

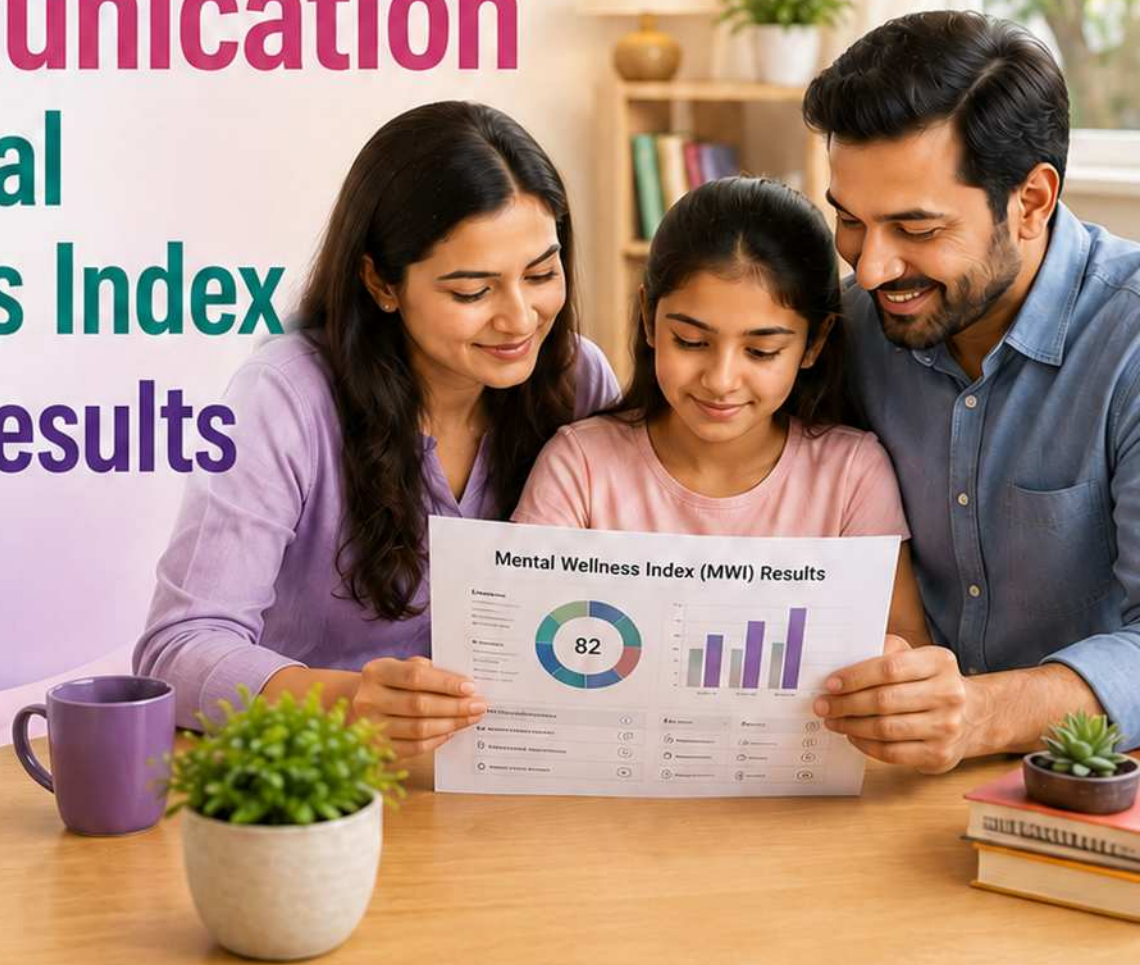


“

Informed Parents.
Stronger Support.
Healthier Minds.
Brighter Futures.

”

Parent Communication of Mental Wellness Index (MWI) Results



VOLUME
14

 AUTHOR BY
Various

 PUBLISHER
Johan Brown

 EDITION
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 SERIES
Mental Wellness
Management
SOPs

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STANDARD OPERATING PROCEDURES

•

Preface

*On the Discipline of Care —
A Foreword to This Compendium of Mental Wellness SOPs*

INSTITUTIONAL MENTAL HEALTH & WELLNESS GOVERNANCE

FOR INTERNAL USE & INSTITUTIONAL REFERENCE

Preface

This book exists because good intentions are not enough. Every institution that takes on the care of another human being's mind — a school, a hospital, a workplace, a home — eventually discovers the same hard truth: compassion, unrecorded and unstructured, evaporates the moment it is needed most. A caring teacher forgets to document a warning sign. A well-meaning counselor makes a judgment call with no record of why. A crisis unfolds, and no one is quite sure who was supposed to act, by when, or how. What follows in these pages is an attempt to close that gap — not by replacing compassion with paperwork, but by giving compassion a spine.

Each Standard Operating Procedure gathered here, regardless of the specific domain it addresses — policy, curriculum, crisis response, staff training, referral, review, or record-keeping — is built on the same conviction: that the wellbeing of the people an institution serves deserves the same rigor we would demand of a safety protocol or a financial audit. Mental wellness has too often been treated as a matter of individual sensitivity rather than institutional design. This collection insists otherwise. It treats care as a system: something that can be planned for, trained on, measured, escalated, reviewed, and improved — without ever losing sight of the person at the center of it.

WHY A BOOK OF PROCEDURES

A single well-written policy can inspire. A single procedure can guide one moment. But only a coherent body of interlocking SOPs can hold an institution accountable across time — through the ordinary Tuesday afternoon and the extraordinary emergency alike. Every document in this compendium follows a common architecture: purpose and scope, roles and authorities, step-by-step procedure, required forms and checklists, escalation pathways, documentation standards, audit protocols, and a record of lessons learned. This structure is deliberate. It ensures that whoever opens any SOP in this book — whether they are reading it for the first time or the hundredth — finds the same discipline of clarity: what to do, who does it, when it must happen, what evidence proves it happened, and what "good" looks like when it is done.

A procedure is not the opposite of empathy. It is empathy that has learned to survive a bad day.

HOW THIS BOOK SHOULD BE USED

- Not as a shelf reference consulted only in crisis, but as a living working document — read, trained on, rehearsed, and revisited long before it is ever needed.
- Not as a substitute for professional judgment, but as the scaffolding within which that judgment can be exercised safely, consistently, and defensibly.
- Not as a static text, but as a document under continuous review — each SOP carries its own audit protocol and improvement log precisely so that it can be questioned, corrected, and strengthened over time.
- Not in isolation, but in relation — many procedures reference and depend on one another, and their real strength lies in how they interlock across an institution's departments and roles.

AN ACKNOWLEDGMENT

No procedure, however carefully drafted, can anticipate every circumstance a young person, an employee, a patient, or a family will bring through the door. These pages will occasionally fall short of the moment they were written for. When they do, the responsibility of everyone who uses this book is the same: act in the best interest of the person in front of you, document what you did and why, and bring that gap back to this book so it can be closed for whoever comes next. That feedback loop — practice informing procedure, procedure guiding practice — is the quiet, ongoing work this compendium asks of its readers.

What follows, then, is not a bureaucratic exercise. It is an attempt to make sure that care, once promised, is also delivered — reliably, accountably, and with dignity — to every person an institution is entrusted to protect.

EDITORIAL & GOVERNANCE BOARD

**Standard Operating Procedures for Mental
Wellness**

Confidential Reporting and Grievance Redressal Awareness

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STANDARD OPERATING PROCEDURE

Parent Communication of Mental Wellness Index (MWI) Results

Mental Wellness Management System - Parental Engagement and
Communication

SOP ID

MWI-011

VERSION

1.0

STATUS

Finalized

CLASSIFICATION

Confidential - Restricted Access

EFFECTIVE DATE

2026-01-01

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1 Executive Overview

SOP ID	MWI-011
DOMAIN	Mental Wellness Management System
SUB DOMAIN	Parental Engagement and Communication
TITLE	Parent Communication of Mental Wellness Index (MWI) Results
VERSION	1.0
CLASSIFICATION	Confidential - Restricted Access
EFFECTIVE DATE	2026-01-01
REVIEW DATE	2027-01-01
DOCUMENT STATUS	Finalized
LANGUAGE	English

2 Purpose & Rationale

OBJECTIVE

To establish a standardized, empathetic, and legally compliant procedure for communicating Mental Wellness Index (MWI) results to parents while ensuring student privacy and providing actionable support.

RISK CONTROLLED

- Data breach of sensitive mental health information
- Parental panic due to clinical jargon or raw scores
- Misinterpretation of wellness status by caregivers
- Failure to identify high-risk students requiring immediate intervention

DESIRED OUTCOMES

- Increased parent engagement in student wellness
- Reduced stigma around mental health discussions
- Timely initiation of support plans for at-risk students
- 100% compliance with data privacy regulations

3 Scope of Application

INSTITUTION TYPES

- High Schools
- Secondary Schools
- Residential Schools

KEY STAKEHOLDERS

- Counselors
- Parents

- School Management
- Wellness Committees

AGE GROUPS

- All Ages (High School focus)

LOCATIONS

- Urban Schools
- Rural Schools
- Remote Learning Centers

4 Definitions & Terminology

Term	Definition
Mental Wellness Index (MWI)	A non-clinical screening tool used to assess student emotional, social, and academic wellbeing.
Wellness Zone	A simplified classification (Green, Yellow, Orange, Red) used to communicate MWI results without using clinical scores.
CWSN	Children with Special Needs, including neurodivergent students.

Term	Definition
Tele-MANAS	Government of India's 24/7 mental health helpline for tele-consultation.

5 Key Stakeholders

Stakeholder Type	Stakeholder Title	Role In SOP
Internal	School Counselor	Primary executor of parent communication and risk assessment.
Internal	Wellness Coordinator	Oversees the MWI process, ensures compliance, and manages escalations.
Internal	School Principal	Final approver for high-risk communications and resource allocation.
External	Parents/Guardians	Recipients of MWI results, partners in student wellness support.
External	Medical Professionals	Referral points for clinical interventions (Orange/Red zones).

6 Governance & Ownership KEY FOCUS

OWNER

School Wellness Coordinator

APPROVER

Principal / School Management Board

REVIEW FREQUENCY

Annual

LINKED POLICIES

- Student Data Privacy Policy
- Child Protection Policy
- Mental Health Support Framework

LINKED SOPs

- MWI-010: Mental Wellness Index Assessment
- MWI-012: Student Intervention Planning
- MWI-015: Crisis Response Protocol

7 Roles & Responsibilities

Item 1 ROLE School Counselor RESPONSIBILITIES • Conduct MWI score mapping to Wellness Zones. • Execute parent communications for all risk levels. • Provide resource handouts and guidance. • Maintain Student Wellness Records.	RESPONSIBILITIES • Oversee MWI process compliance. • Manage high-risk (Red Zone) escalations. • Update management dashboards. • Conduct quarterly audits of communication logs.
Item 2 ROLE Wellness Coordinator	Item 3 ROLE School Principal RESPONSIBILITIES • Final approval for high-risk communications. • Resource allocation for wellness programs. • Oversight of grievance redressal.

8 RACI Accountability Matrix

Item 1 ACTIVITY MWI Score Mapping	RESPONSIBLE • School Counselor ACCOUNTABLE
--	---

- Wellness Coordinator
- CONSULTED**
- Teachers
- INFORMED**
- Principal

Item 2

ACTIVITY

High-Risk Parent Communication

RESPONSIBLE

- School Counselor

ACCOUNTABLE

- Wellness Coordinator

CONSULTED

- Principal
- INFORMED**
- Parents

Item 3

ACTIVITY

Data Privacy Audit

RESPONSIBLE

- Wellness Coordinator

ACCOUNTABLE

- Principal

INFORMED

- School Board

9 Preconditions

- Signed MWI Consent Form from parent/guardian
- Completion of student MWI screening
- Access to secure Student Wellness Record system
- Counselor training in non-numerical communication

10 Activation Triggers

- Completion of MWI screening
- Identification of high-risk (Orange/Red) student
- Parent request for wellness update
- External medical referral
- Significant behavioral change report from teacher

11 SOP Trigger Matrix

MANDATORY TRIGGERS

- Completion of annual MWI screening
- Identification of a student in Yellow, Orange, or Red zone
- Request from parent for wellness status update

OPTIONAL TRIGGERS

- Mid-term progress review
- Student self-referral for wellness check

AUTOMATIC TRIGGERS

- MWI score exceeding Orange threshold automatically alerts Wellness Coordinator

EXTERNAL TRIGGERS

- Referral from external medical professional
- Report of significant behavioral change by community members

RECURRING TRIGGERS

- Monthly wellness summary for Green zone students (optional)
- Quarterly review of high-risk cases

12 Required Inputs

- Student MWI Scores
- Parent Consent Forms
- Teacher Observation Notes
- Language Preferences
- Tele-MANAS Guidelines

13 Process Map

13.1 Phase 1

NAME

Data Preparation & Consent

OBJECTIVE

Ensure legal compliance and data readiness.

ACTIVITIES

- Verify signed MWI consent forms for all students
- Aggregate MWI scores into Wellness Zones
- Anonymize data for management reporting

13.2 Phase 2

NAME

Risk Stratification

OBJECTIVE

Categorize student needs based on severity.

ACTIVITIES

- Counselor review of all scores
- Flagging Orange and Red zone cases for priority communication
- Assigning primary counselor for each case

13.3 Phase 3

NAME

Communication Planning

OBJECTIVE

Select appropriate channels and scripts.

ACTIVITIES

- Identify parent contact preferences (Phone, Email, In-person)

- Determine language requirements (Hindi, English, Regional)
- Prepare specific scripts for high-risk conversations

13.4 Phase 4

NAME

Parent Engagement

OBJECTIVE

Deliver results and provide support.

ACTIVITIES

- Send non-numerical wellness summaries
- Conduct risk-based phone calls or meetings
- Provide resource handouts and referral links

13.5 Phase 5

NAME

Documentation & Follow-up

OBJECTIVE

Ensure continuity of care.

ACTIVITIES

- Log parent acknowledgment in Student Wellness Record
- Schedule follow-up meetings
- Update management dashboard

14 Workflow Overview

START

MWI Screening Completion

STEPS

- Consent Verification
- Wellness Zone Mapping
- Communication Planning
- Initial Notification Delivery
- Risk-Based Conversation
- Resource Provisioning
- Parent Acknowledgment
- Follow-up Scheduling
- Data Logging & Dashboard Update
- Closure or Ongoing Monitoring

CRITICAL DECISION POINTS

- Consent Verification (DP-01)
- Risk Stratification (DP-02)
- Parent Response Type (DP-03)

END

Record Closure or Transition to Ongoing Monitoring

15 Standard Procedure

1. Consent Verification

PURPOSE & RATIONALE

Ensure legal right to share MWI data.

ACTOR

Wellness Coordinator

ACTION

Cross-reference student ID with signed consent forms.

REQUIRED INPUTS

- Student MWI Data
- Parent Consent Forms

EXPECTED OUTPUTS

- Verified list of students for communication

CRITICAL DECISION POINTS

- If no consent: Notify parent to sign; do not share results.

DOCUMENTATION REQUIRED

- Consent Audit Log

EVIDENCE REQUIRED

- Signed Consent Forms

APPROVAL REQUIRED

No

TIME LIMIT

48 hours

EXPECTED OUTCOME

Verified list of students for communication.

ESCALATION IF FAILED

Notify Principal if parent refuses consent.

RELATED FORMS

- Consent Audit Log

2. Wellness Zone Mapping

PURPOSE & RATIONALE

Translate scores into non-numerical zones.

ACTOR

School Counselor

ACTION

Apply scoring logic to assign Green, Yellow, Orange, or Red status.

REQUIRED INPUTS

- Raw MWI Scores

EXPECTED OUTPUTS

- Stratified student list

CRITICAL DECISION POINTS

- Score > 75% distress = Orange
- Score > 90% distress = Red

DOCUMENTATION REQUIRED

- Wellness Zone Summary Report

EVIDENCE REQUIRED

- Raw MWI Scores

APPROVAL REQUIRED

No

TIME LIMIT

24 hours

EXPECTED OUTCOME

Stratified student list.

ESCALATION IF FAILED

Wellness Coordinator (for ambiguous scores).

RELATED FORMS

- Wellness Zone Summary Report

3. Communication Planning

PURPOSE & RATIONALE

Ensure accessibility and parent preference.

ACTOR

School Counselor

ACTION

Identify primary language (e.g., Hindi, English) and select mode.

REQUIRED INPUTS

- Parent Contact Preferences
- Language Requirements

EXPECTED OUTPUTS

- Scheduled communication list

CRITICAL DECISION POINTS

- Orange/Red Zone = Mandatory Phone Call or Meeting
- Green/Yellow Zone = Email or Letter

DOCUMENTATION REQUIRED

- Communication Plan Matrix

EVIDENCE REQUIRED

- Parent Contact Preferences

APPROVAL REQUIRED

No

TIME LIMIT

24 hours

EXPECTED OUTCOME

Scheduled communication list.

ESCALATION IF FAILED

Wellness Coordinator (if contact info is missing).

RELATED FORMS

- Communication Plan Matrix

4. Initial Notification Delivery

PURPOSE & RATIONALE

Inform parents of wellness status.

ACTOR

School Counselor

ACTION

Send non-numerical summary (e.g., 'Your child is in the Green zone').

REQUIRED INPUTS

- Wellness Zone Summary Report
- Communication Plan Matrix

EXPECTED OUTPUTS

- Sent notifications

CRITICAL DECISION POINTS

- If parent asks for scores: Explain policy on non-numerical reporting.

DOCUMENTATION REQUIRED

- Parent Notification Letter/Email
- Communication Log

EVIDENCE REQUIRED

- Sent Email Logs

APPROVAL REQUIRED

No

TIME LIMIT

48 hours

EXPECTED OUTCOME

Sent notifications.

ESCALATION IF FAILED

Wellness Coordinator (if parent is hostile).

RELATED FORMS

- Parent Notification Letter/Email
- Communication Log

5. Risk-Based Conversation

PURPOSE & RATIONALE

Detailed discussion for Orange/Red zones.

ACTOR

School Counselor / Wellness Coordinator

ACTION

Conduct phone call/meeting using specific scripts and provide resources.

REQUIRED INPUTS

- Student Behavior Logs
- MWI Scores

EXPECTED OUTPUTS

- Parental awareness of risk
- Agreed support plan

CRITICAL DECISION POINTS

- If student is in Red Zone: Immediate referral to medical professional.

DOCUMENTATION REQUIRED

- Risk Conversation Log
- Referral Slip

EVIDENCE REQUIRED

- Student Behavior Logs

APPROVAL REQUIRED

No

TIME LIMIT

24 hours for Red; 72 hours for Orange

EXPECTED OUTCOME

Parental awareness of risk and agreed support plan.

ESCALATION IF FAILED

Principal (for high-risk safety concerns).

RELATED FORMS

- Risk Conversation Log
- Referral Slip

6. Resource Provisioning

PURPOSE & RATIONALE

Empower parents with support tools.

ACTOR

School Counselor

ACTION

Send digital/physical handouts and Tele-MANAS info.

REQUIRED INPUTS

- Parent's specific needs identified in conversation

EXPECTED OUTPUTS

- Parental resources provided

DECISION CRITERIA

- If parent is low-literacy: Provide audio recordings or visual guides.

DOCUMENTATION REQUIRED

- Resource Distribution Checklist

EVIDENCE REQUIRED

- Conversation Log

APPROVAL REQUIRED

No

TIME LIMIT

24 hours

EXPECTED OUTCOME

Parental resources provided.

ESCALATION IF FAILED

Wellness Coordinator (if specific resource requested is unavailable).

RELATED FORMS

- Resource Distribution Checklist

7. Parent Acknowledgment

PURPOSE & RATIONALE

Confirm receipt and understanding.

ACTOR

School Counselor

ACTION

Request parent to acknowledge receipt via email/form.

REQUIRED INPUTS

- Communication Log

EXPECTED OUTPUTS

- Confirmed parent engagement

DECISION CRITERIA

- If no response in 48 hours: Follow up with phone call.

DOCUMENTATION REQUIRED

- Acknowledgment Record

EVIDENCE REQUIRED

- Communication Log

APPROVAL REQUIRED

No

TIME LIMIT

48 hours

EXPECTED OUTCOME

Confirmed parent engagement.

ESCALATION IF FAILED

Wellness Coordinator (if no response after 3 attempts).

RELATED FORMS

- Acknowledgment Record

8. Follow-up Scheduling

PURPOSE & RATIONALE

Ensure continuity of support.

ACTOR

School Counselor

ACTION

Schedule next check-in date and send reminder.

REQUIRED INPUTS

- Agreed support plan

EXPECTED OUTPUTS

- Scheduled follow-ups

DECISION CRITERIA

- Frequency based on risk zone.

DOCUMENTATION REQUIRED

- Follow-up Schedule

EVIDENCE REQUIRED

- Agreed support plan

APPROVAL REQUIRED

No

TIME LIMIT

24 hours

EXPECTED OUTCOME

Scheduled follow-ups.

ESCALATION IF FAILED

Wellness Coordinator (if student status deteriorates).

RELATED FORMS

- Follow-up Schedule

9. Data Logging & Dashboard Update

PURPOSE & RATIONALE

Maintain accurate records for audit.

ACTOR

Wellness Coordinator

ACTION

Update Student Wellness Record and institutional dashboard.

REQUIRED INPUTS

- Communication Logs
- Risk Conversation Logs

EXPECTED OUTPUTS

- Updated records
- Real-time dashboard visibility

DECISION CRITERIA

- Ensure all data is anonymized for management view.

DOCUMENTATION REQUIRED

- Management Dashboard Update

EVIDENCE REQUIRED

- Communication Logs
- Risk Conversation Logs

APPROVAL REQUIRED

No

TIME LIMIT

48 hours

EXPECTED OUTCOME

Updated records and dashboard visibility.

ESCALATION IF FAILED

Principal (if data integrity issues found).

RELATED FORMS

- Management Dashboard Update

10. Closure or Ongoing Monitoring

PURPOSE & RATIONALE

Finalize the communication cycle.

ACTOR

School Counselor

ACTION

Mark status as 'Complete' or 'Ongoing' and archive records.

REQUIRED INPUTS

- Student Wellness Record

EXPECTED OUTPUTS

- Finalized record status

DECISION CRITERIA

- Closure only if student returns to Green zone and parent agrees.

DOCUMENTATION REQUIRED

- Closure Report (if applicable)

EVIDENCE REQUIRED

- Student Wellness Record

APPROVAL REQUIRED

No

TIME LIMIT

7 days

EXPECTED OUTCOME

Finalized record status.

ESCALATION IF FAILED

Wellness Coordinator

RELATED FORMS

- Closure Report

16 Detailed Workflow Steps

1. Consent Verification

PURPOSE & RATIONALE

Ensure legal right to share MWI data.

TRIGGER

Completion of MWI screening

ACTOR

Wellness Coordinator

REQUIRED INPUTS

- Student MWI Data
- Parent Consent Forms

ACTIONS

- Cross-reference student ID with signed consent forms
- Identify students without valid consent

DECISION CRITERIA

- If no consent: Notify parent to sign; do not share results.

DOCUMENTS GENERATED

- Consent Audit Log

EXPECTED OUTPUTS

- Verified list of students for communication

SERVICE LEVEL AGREEMENT (SLA)

48 hours from screening completion

ESCALATION PATH

- Principal (if parent refuses consent)

EVIDENCE GENERATED

- Consent Audit Log

School Counselor

REQUIRED INPUTS

- Raw MWI Scores

ACTIONS

- Apply scoring logic to assign Green, Yellow, Orange, or Red status
- Flag any student in Orange/Red for immediate priority

DECISION CRITERIA

- Score > 75% distress = Orange
- Score > 90% distress = Red

DOCUMENTS GENERATED

- Wellness Zone Summary Report

EXPECTED OUTPUTS

- Stratified student list

SERVICE LEVEL AGREEMENT (SLA)

24 hours from data verification

ESCALATION PATH

- Wellness Coordinator (for ambiguous scores)

EVIDENCE GENERATED

- Wellness Zone Summary Report

3. Communication Channel Selection

PURPOSE & RATIONALE

Ensure accessibility and parent preference.

TRIGGER

Stratified student list received

ACTOR

School Counselor

REQUIRED INPUTS

- Parent Contact Preferences
- Language Requirements

ACTIONS

- Identify primary language (e.g., Hindi, English)

2. Wellness Zone Mapping

PURPOSE & RATIONALE

Translate scores into non-numerical zones.

TRIGGER

Verified data list received

ACTOR

- Select mode: Phone call (High Risk), Email (Low Risk), In-person (Complex)

DECISION CRITERIA

- Orange/Red Zone = Mandatory Phone Call or Meeting
- Green/Yellow Zone = Email or Letter

DOCUMENTS GENERATED

- Communication Plan Matrix

EXPECTED OUTPUTS

- Scheduled communication list

SERVICE LEVEL AGREEMENT (SLA)

24 hours from mapping

ESCALATION PATH

- Wellness Coordinator (if contact info is missing)

EVIDENCE GENERATED

- Communication Plan Matrix

4. Initial Notification Delivery

PURPOSE & RATIONALE

Inform parents of wellness status.

TRIGGER

Scheduled communication list ready

ACTOR

School Counselor

REQUIRED INPUTS

- Wellness Zone Summary Report
- Communication Plan Matrix

ACTIONS

- Send non-numerical summary (e.g., 'Your child is in the Green zone')
- Provide brief overview of what MWI means

DECISION CRITERIA

- If parent asks for scores: Explain policy on non-numerical reporting.

DOCUMENTS GENERATED

- Parent Notification Letter/Email
- Communication Log

EXPECTED OUTPUTS

- Sent notifications

SERVICE LEVEL AGREEMENT (SLA)

48 hours from planning

ESCALATION PATH

- Wellness Coordinator (if parent is hostile)

EVIDENCE GENERATED

- Communication Log

5. Risk-Based Conversation

PURPOSE & RATIONALE

Detailed discussion for Orange/Red zones.

TRIGGER

Orange or Red zone identification

ACTOR

School Counselor / Wellness Coordinator

REQUIRED INPUTS

- Student Behavior Logs
- MWI Scores

ACTIONS

- Conduct phone call/meeting using specific scripts
- Discuss observable behaviors and school support plans
- Provide resources (handouts, Tele-MANAS info)

DECISION CRITERIA

- If student is in Red Zone: Immediate referral to medical professional.

DOCUMENTS GENERATED

- Risk Conversation Log
- Referral Slip

EXPECTED OUTPUTS

- Parental awareness of risk
- Agreed support plan

SERVICE LEVEL AGREEMENT (SLA)

24 hours from mapping for Red Zone; 72 hours for Orange

ESCALATION PATH

- Principal (for high-risk safety concerns)

EVIDENCE GENERATED

- Risk Conversation Log
- Referral Slip

6. Resource Provisioning

PURPOSE & RATIONALE

Empower parents with support tools.

TRIGGER

Completion of conversation

ACTOR

School Counselor

REQUIRED INPUTS

- Parent's specific needs identified in conversation

ACTIONS

- Send digital/physical handouts
- Provide links to school wellness portal
- Share Tele-MANAS contact details

DECISION CRITERIA

- If parent is low-literacy: Provide audio recordings or visual guides.

DOCUMENTS GENERATED

- Resource Distribution Checklist

EXPECTED OUTPUTS

- Parental resources provided

SERVICE LEVEL AGREEMENT (SLA)

24 hours from conversation

ESCALATION PATH

- Wellness Coordinator (if specific resource requested is unavailable)

EVIDENCE GENERATED

- Resource Distribution Checklist

7. Parent Acknowledgment

PURPOSE & RATIONALE

Confirm receipt and understanding.

TRIGGER

Resources provided

ACTOR

School Counselor

REQUIRED INPUTS

- Communication Log

ACTIONS

- Request parent to acknowledge receipt via email/form
- Log acknowledgment date

DECISION CRITERIA

- If no response in 48 hours: Follow up with phone call.

DOCUMENTS GENERATED

- Acknowledgment Record

EXPECTED OUTPUTS

- Confirmed parent engagement

SERVICE LEVEL AGREEMENT (SLA)

48 hours from resource provision

ESCALATION PATH

- Wellness Coordinator (if no response after 3 attempts)

EVIDENCE GENERATED

- Acknowledgment Record

8. Follow-up Scheduling

PURPOSE & RATIONALE

Ensure continuity of support.

TRIGGER

Acknowledgment received

ACTOR

School Counselor

REQUIRED INPUTS

- Agreed support plan

ACTIONS

- Schedule next check-in date (weekly for Red, monthly for Yellow)
- Send calendar invite/reminder

DECISION CRITERIA

- Frequency based on risk zone.

DOCUMENTS GENERATED

- Follow-up Schedule

EXPECTED OUTPUTS

- Scheduled follow-ups

SERVICE LEVEL AGREEMENT (SLA)

24 hours from acknowledgment

ESCALATION PATH

- Wellness Coordinator (if student status deteriorates)

EVIDENCE GENERATED

- Follow-up Schedule

9. Data Logging & Dashboard Update

PURPOSE & RATIONALE

Maintain accurate records for audit.

TRIGGER

Completion of follow-up scheduling

ACTOR

Wellness Coordinator

REQUIRED INPUTS

- Communication Logs
- Risk Conversation Logs

ACTIONS

- Update Student Wellness Record
- Update institutional dashboard metrics

DECISION CRITERIA

- Ensure all data is anonymized for management view.

DOCUMENTS GENERATED

- Management Dashboard Update

EXPECTED OUTPUTS

- Updated records
- Real-time dashboard visibility

SERVICE LEVEL AGREEMENT (SLA)

48 hours from follow-up scheduling

ESCALATION PATH

- Principal (if data integrity issues found)

EVIDENCE GENERATED

- Management Dashboard Update

10. Closure or Ongoing Monitoring

PURPOSE & RATIONALE

Finalize the communication cycle.

TRIGGER

Completion of follow-up schedule

ACTOR

School Counselor

REQUIRED INPUTS

- Student Wellness Record

ACTIONS

- Mark communication cycle as 'Complete' or 'Ongoing'
- Archive records according to retention policy

DECISION CRITERIA

- Closure only if student returns to Green zone and parent agrees.

DOCUMENTS GENERATED

- Closure Report (if applicable)

EXPECTED OUTPUTS

- Finalized record status

SERVICE LEVEL AGREEMENT (SLA)

7 days from last interaction

ESCALATION PATH

- Wellness Coordinator

EVIDENCE GENERATED

- Closure Report

17 Critical Decision Points

Item 1

DECISION ID

DP-01

DECISION NAME

Consent Verification

CRITERIA

- Is there a signed MWI consent form for this student?

POSSIBLE OUTCOMES

- Yes: Proceed to mapping
- No: Notify parent and pause communication

NEXT STEPS

- Step 2 (Mapping)
- Parent Outreach for Consent

- Yes: High-priority risk conversation required
- No: Standard wellness summary sufficient

NEXT STEPS

- Step 5 (Risk Conversation)
- Step 4 (Initial Notification)

Item 2

DECISION ID

DP-02

DECISION NAME

Risk Stratification

CRITERIA

- Is the student in Orange or Red zone?

POSSIBLE OUTCOMES

Item 3

DECISION ID

DP-03

DECISION NAME

Parent Response Type

CRITERIA

- Is the parent cooperative, in denial, or hostile?

POSSIBLE OUTCOMES

- Cooperative: Proceed with support plan
- In Denial: Provide evidence-based behavior logs and request meeting
- Hostile: Escalate to Wellness Coordinator/Principal

NEXT STEPS

- Step 6 (Resource Provisioning)
- Escalation Protocol
- Case Conference

18 Decision Tree

GREEN

- Send non-numerical summary via email.
- Schedule optional check-in.

YELLOW

- Conduct phone call to discuss support plan.
- Provide resource handouts.

ORANGE

- Mandatory in-person or detailed phone meeting.
- Develop specific intervention plan.

- Consider external referral if needed.

RED

- Immediate risk assessment.
- Activate crisis protocol.
- Urgent parent notification and medical referral.

BLACK

- Activate Emergency Response SOP.
- Contact emergency services immediately.
- Notify parents/guardians instantly.

19 Decision Rules

Rule ID	Condition	Decision	Next Action
DR-01	If Parent is Unresponsive for 3 attempts.	Escalate to Wellness Coordinator.	Initiate 'Unresponsive Parent' protocol.
DR-02	If Student is in Red Zone.	Immediate Risk Assessment required.	Activate Crisis Protocol.

20 Exception Handling Protocol

Item 1

EXCEPTION

Parent refuses to acknowledge risk or communication.

DETECTION METHOD

No response after 3 attempts (Phone/Email).

RESPONSE

Document all attempts; notify Wellness Coordinator; initiate 'Unresponsive Parent' protocol.

ESCALATION

Wellness Coordinator → Principal.

DOCUMENTATION REQUIRED

- Communication Log with timestamps
- Escalation Approval Record

Activate Emergency Response SOP; contact emergency services/parents immediately.

ESCALATION

Immediate escalation to Principal and Emergency Services.

DOCUMENTATION REQUIRED

- Risk Assessment Record
- Emergency Call Log

Item 2

EXCEPTION

Student in Red Zone shows immediate self-harm risk during call.

DETECTION METHOD

Parent disclosure or counselor observation during conversation.

RESPONSE

Item 3

EXCEPTION

Parent requests specific clinical scores or raw data.

DETECTION METHOD

Direct request during communication.

RESPONSE

Politely decline based on privacy policy; explain the 'Wellness Zone' focus on support over scoring.

ESCALATION

None (Standard procedure).

DOCUMENTATION REQUIRED

- Communication Log

21 Exceptions

Exception Type	Condition	Action
Emergency Disclosure	Imminent threat to life.	Immediate disclosure to authorities without parent consent.
Court Order	Legal mandate for data release.	Release records only after legal review by Principal.

22 Expected Outputs

- Wellness Zone Summary Report
- Risk Conversation Logs
- Resource Handouts

- Follow-up Schedules
- Management Dashboard Updates

23 Document Control & Version Governance

23.1 Version History

Version	Date	Author	Description
1.0	2025-10-27	MWMS Architect	Initial SOP creation for MWI result communication.

REVIEW TRIGGERS

- Change in CBSE guidelines
- Supreme Court rulings on student privacy
- Annual institutional audit findings
- Significant incident of miscommunication

23.3 Approval History

Approver	Date	Status
School Principal	2025-10-30	Approved

24 Inclusive Implementation Mandate

MULTILINGUAL SUPPORT REQUIRED

Yes

ACCESSIBILITY REVIEW REQUIRED

Yes

LOW RESOURCE SCHOOL ALTERNATIVE REQUIRED

Yes

EQUITY IMPACT ASSESSMENT REQUIRED

Yes

25 Regulatory Compliance Mapping

KEY FOCUS

MANDATORY REQUIREMENTS

- Supreme Court of India Judgment 2025 INSC 893 (Student Privacy)
- CBSE Circular TRG-03/2026 (Wellbeing Guidelines)
- NEP 2020 Holistic Development Mandate

RECOMMENDED REQUIREMENTS

- WHO School Mental Health Framework
- UNESCO Whole School Wellbeing Framework

BEST PRACTICES

- Manodarpan Guidelines
- Tele-MANAS Integration
- Trauma-Informed Communication

COMPLIANCE GAPS IF NOT IMPLEMENTED

- Legal liability for data breach
- Non-compliance with CBSE affiliation bye-laws
- Failure to provide equitable support to marginalized groups

26 Compliance Traceability Matrix

KEY FOCUS

1. Supreme Court of India

REQUIREMENT REFERENCE

2025 INSC 893

REQUIREMENT TEXT

Protection of student psychological data and right to privacy.

IMPLEMENTED IN SECTIONS

- Confidentiality Controls
- Data Governance
- Record Retention

2. CBSE Circular TRG-03/2026

REQUIREMENT REFERENCE

TRG-03/2026

REQUIREMENT TEXT

Mandatory communication of student wellbeing status to parents with actionable support.

IMPLEMENTED IN SECTIONS

- Parent Communication Strategy
- SLA by Risk Level

27 Documentation & Evidentiary Requirements

EVENT DOCUMENTATION PRINCIPLES

- Factual, objective language only
- No clinical diagnoses unless provided by a medical professional
- Focus on observable behaviors and parent responses
- Timely entry (within 24 hours of interaction)

MANDATORY CONVERSATION RECORDS

- Parent Conversation Log (Date, Time, Mode, Summary)
- Risk Assessment Record (for Orange/Red zones)
- Consent Form (Signed by parent/guardian)

MANDATORY OBSERVATION RECORDS

- Teacher Observation Notes
- Student Behavior Logs

MANDATORY DECISION RECORDS

- Escalation Approval Record

- Referral Decision Log

MANDATORY ACTION RECORDS

- Intervention Plan Summary
- Resource Distribution Checklist

MANDATORY FOLLOWUP RECORDS

- Follow-up Review Record
- Closure Report

EVIDENCE REQUIREMENTS

- Signed Consent Forms
- Communication Logs
- Referral Slips
- Management Dashboard Screenshots

DOCUMENTATION QUALITY STANDARDS

- Must be legible and organized in a secure folder
- Must use standardized templates
- Must be accessible only to authorized personnel

28 The Student Journey

ENTRY

- Student completes MWI screening assessment.

ASSESSMENT

- Data is anonymized and processed by the Wellness Team.
- Student is assigned to a Wellness Zone (Green, Yellow, Orange, Red).

INTERVENTION

- Parent is notified of student's wellness status.

- Support plan is developed based on parent-counselor consultation.

MONITORING

- Student receives regular check-ins from the counselor.
- Progress is tracked against initial MWI baseline.

CLOSURE

- Student returns to Green zone status.
- Parent acknowledges successful completion of support plan.

29 The Parent Journey

NOTIFICATION

- Receives non-numerical wellness summary via preferred channel (Email/Letter).

CONSULTATION

- Participates in a risk-based conversation for Yellow, Orange, or Red zones.
- Discusses student's behavior and school support.

CONSENT

- Provides initial MWI data sharing consent.

- Signs acknowledgment of received communication.

ENGAGEMENT

- Receives resource handouts (Tele-MANAS, School Portal).
- Collaborates on a home-school support plan.

CLOSURE

- Receives final summary and closure report.
- Confirms satisfaction with the support provided.

30 The Staff Journey

OBSERVATION

- Teachers/Staff provide behavioral observations to counselors.

REPORTING

- Counselors input data into MWI system and map zones.

INTERVENTION SUPPORT

- Counselors conduct parent calls and meetings.

- Wellness Coordinator manages high-risk escalations.

DOCUMENTATION

- Staff log all interactions in the Student Wellness Record.
- Records are audited for compliance quarterly.

31 Resource Evidence

COMMITTEE MINUTES

- Monthly Wellness Committee meeting minutes regarding MWI trends.

ATTENDANCE RECORDS

- Parent consultation attendance logs.

TRAINING RECORDS

- Counselor training certificates for 'Trauma-Informed Communication'.

FORMS

- MWI Consent Form
- Risk Conversation Log
- Referral Slip
- Resource Distribution Checklist
- Acknowledgment Record

REPORTS

- Quarterly MWI Trend Report
- Annual Wellness Impact Audit

DASHBOARDS

- Real-time Student Wellness Zone Dashboard (Anonymized)

32 Grievance Redressal Mechanism

KEY FOCUS

COMPLAINT CHANNELS

- Anonymous feedback box
- Wellness Coordinator email
- School Principal's office

INVESTIGATION PROCESS

- Complaint logged within 24 hours.
- Internal review by Wellness Committee.

- Fact-finding meeting with involved parties.

RESOLUTION TIMELINES

- Initial response: 48 hours
- Full resolution: 10 business days

APPEAL PROCESS

- Appeal to School Management Board within 5 days of resolution.

33 Referral Framework

33.1 Internal Referrals

ELIGIBLE REFERRERS

- Teachers
- Counsellors
- School Management
- Wellness Committee
- Student Support Team

REFERRAL TRIGGERS

- Persistent academic decline
- Social withdrawal
- Aggressive behavior
- Self-harm ideation

DOCUMENTATION REQUIRED

- Internal Referral Form

- Teacher Observation Notes

SERVICE LEVEL AGREEMENT (SLA)

24 hours from referral submission

33.2 Self Referrals

ALLOWED

Yes

SUBMISSION METHODS

- Online portal
- Physical drop-box
- Direct email to counselor

CONFIDENTIALITY PROTECTIONS

- Encrypted digital records
- Limited access for non-wellness staff

33.3 Anonymous Referrals

ALLOWED

Yes

CHANNELS

- Anonymous QR codes in classrooms
- School website feedback form

VALIDATION PROCESS

- Counselor review of anonymous reports for immediate risk.

FOLLOW UP PROCEDURE

- Direct outreach to student if identity can be inferred safely.

33.4 External Referrals

ELIGIBLE AGENCIES

- Tele-MANAS
- Child Helpline (1098)
- District Child Protection Services
- Private Psychiatrists
- Pediatricians

ELIGIBILITY CRITERIA

- Clinical diagnosis required

- Severe risk of self-harm
- Abuse disclosure
- Persistent symptoms not responding to school support

CONSENT REQUIREMENTS

- Mandatory written parent consent for external referral.

HANDOVER PROTOCOL

- Summary of internal observations shared with medical professional.
- Parental contact details provided.

33.5 Referral Tracking

TRACKING REQUIRED

Yes

STATUS CATEGORIES

- Pending
- In-Progress
- Completed
- Closed/Withdrawn

CLOSURE REQUIREMENTS

- Counselor confirmation of referral completion.
- Parent acknowledgment.

34 Confidentiality Exception Matrix

KEY FOCUS

MANDATORY DISCLOSURES

- Imminent risk of self-harm
- Imminent risk to others
- Child abuse
- Child neglect
- Sexual abuse
- Court orders
- Legal reporting obligations

LEGAL REPORTING EVENTS

- Police notification for violent threats
- Mandatory reporting of physical abuse to District Authorities

CONSENT OVERRIDE CONDITIONS

- Life-threatening emergency
- Court order requiring disclosure

AUTHORIZED DISCLOSURE RECIPIENTS

- Emergency Services
- Child Protection Officers
- School Principal (for safety)
- Legal Authorities

DOCUMENTATION REQUIREMENTS

- Confidentiality Breach Log
- Reason for Disclosure Record

APPROVAL REQUIREMENTS

- Principal approval required for all non-emergency disclosures.

35 Record Retention & Disposal

RETENTION PERIOD

7 years post-graduation

RETENTION BASIS

Legal compliance and institutional policy

RECORD CATEGORIES

- MWI Scores
- Parent Communication Logs
- Risk Assessment Records
- Referral Slips

35.4 Storage Requirements

PHYSICAL RECORDS

- Locked filing cabinets in Counselor's office

DIGITAL RECORDS

- Encrypted, password-protected cloud storage with MFA

ACCESS CONTROL REQUIREMENTS

- Role-based access control (RBAC)
- Audit logs for all record views

DISPOSAL METHOD

Secure shredding for physical; permanent deletion for digital

DISPOSAL AUTHORITY

Wellness Coordinator

AUDIT FREQUENCY

Annual

DESTRUCTION LOG REQUIRED

Yes

36 Anonymised Reporting Framework

36.1 Management Reports

ANONYMIZATION REQUIRED

Yes

PERMITTED DATA

- Aggregate Wellness Zone counts
- Trend analysis by grade level
- Resource utilization rates

PROHIBITED DATA

- Student names
- Specific dates of incidents
- Unique identifiers (ID numbers)

36.2

Research Usage

ALLOWED

No

36.3 Statistical Reporting

AGGREGATE ONLY

Yes

MINIMUM GROUP SIZE

10 students

RE IDENTIFICATION CONTROLS

- Data scrubbing of unique descriptors

DATA SHARING CONTROLS

- No sharing with third-party marketing agencies
- Strict internal data silos

37 Guiding Ethical Principles

37.1 Autonomy

DESCRIPTION

Respecting the student's and parent's right to choose support.

IMPLEMENTATION REQUIREMENTS

- Informed consent for all MWI activities.

37.2 Beneficence

DESCRIPTION

Acting in the best interest of the student's mental health.

IMPLEMENTATION REQUIREMENTS

- Prioritizing wellness over academic metrics.

37.3 Non Maleficence

DESCRIPTION

Avoiding harm through accurate and non-stigmatizing communication.

IMPLEMENTATION REQUIREMENTS

- Non-numerical reporting to prevent unnecessary anxiety.

37.4 Justice And Equity

DESCRIPTION

Ensuring equal access to wellness support for all students.

IMPLEMENTATION REQUIREMENTS

- Multilingual and low-resource adaptations.

37.5 Confidentiality

KEY FOCUS

DESCRIPTION

Protecting sensitive mental health information.

IMPLEMENTATION REQUIREMENTS

- Strict RBAC and encryption protocols.

37.6 Privacy

DESCRIPTION

Respecting the student's right to a private educational environment.

IMPLEMENTATION REQUIREMENTS

- Data minimization principles.

37.7 Professional Integrity

DESCRIPTION

Maintaining high standards of counseling and governance.

IMPLEMENTATION REQUIREMENTS

- Regular supervision and peer review for counselors.

38 Communication & Awareness Strategy

38.1 Student Communication

CHANNELS

- Classroom sessions
- School portal
- Counselor meetings

FREQUENCY

Weekly/Monthly

MANDATORY TOPICS

- Stress management
- Help-seeking behaviors
- Wellness Zone definitions

38.4 Awareness Campaigns

MINIMUM FREQUENCY

Quarterly

TARGET GROUPS

- Students
- Parents
- Staff

CAMPAIGN TYPES

- Social media posts
- School newsletters
- Workshops

38.2 Parent Communication

CHANNELS

- Email
- Phone calls
- In-person meetings
- SMS (for alerts)

FREQUENCY

As per risk level

MANDATORY TOPICS

- Wellness status summary
- Support plan actions
- Resource links

38.5 Workshops And Training

STUDENT WORKSHOPS

- Resilience building
- Peer support training

PARENT WORKSHOPS

- Understanding adolescent mental health
- Navigating the MWI process

STAFF WORKSHOPS

- Trauma-informed pedagogy
- Identifying risk indicators

38.3 Staff Communication

CHANNELS

- Staff meetings
- Internal newsletters
- Training workshops

FREQUENCY

Monthly

MANDATORY TOPICS

- Early warning signs
- Referral protocols
- Confidentiality reminders

38.6 Helpline Visibility

REQUIRED

Yes

DISPLAY LOCATIONS

- Counselor office
- School website footer
- Student handbook

DIGITAL VISIBILITY REQUIREMENTS

- Prominent link on school portal

FEEDBACK CHANNELS

- Parent survey
- Student feedback box
- Staff suggestion form

39 Competency & Qualification Requirements

MANDATORY QUALIFICATIONS

- Master's in Psychology/Counseling or equivalent

- Registered with relevant Indian counseling board

PREFERRED QUALIFICATIONS

- Certification in School Counseling
- Experience in adolescent mental health

CERTIFICATIONS

- First Aid for Mental Health
- Child Protection Certification

MINIMUM EXPERIENCE REQUIREMENTS

- 2 years in an educational setting

BACKGROUND VERIFICATION REQUIREMENTS

- Police clearance certificate (mandatory)
- Reference checks from previous schools

39.6 Continuing Professional Development

ANNUAL TRAINING HOURS

40

MANDATORY TOPICS

- Crisis intervention
- Cultural competency
- Data privacy laws

RECERTIFICATION REQUIREMENTS

Every 3 years

39.7 Supervision Requirements

CLINICAL SUPERVISION REQUIRED

Yes

PEER REVIEW REQUIRED

Yes

39.7.3 Wellness Case Review Framework

DESCRIPTION Weekly review of high-risk cases by the Wellness Committee.

FREQUENCY Weekly

39.7.4 Second Level Review Process

DESCRIPTION Principal/Wellness Coordinator review for Red Zone escalations.

39.7. High Risk Decision 5 Validation

KEY
FOCUS

DESCRIPTION Dual-signoff required for all external referrals.

FREQUENCY

Weekly

40 Feedback & Effectiveness Measurement

40.1 Student Feedback

REQUIRED

Yes

COLLECTION METHODS

- Anonymous surveys
- Focus group discussions

FREQUENCY

Bi-annually

40.2 Parent Feedback

REQUIRED

Yes

COLLECTION METHODS

- Post-communication survey
- Annual satisfaction survey

FREQUENCY

After each high-risk communication

40.3 Staff Feedback

REQUIRED

Yes

COLLECTION METHODS

- Staff surveys

- Focus groups

FREQUENCY

Annually

40.4 Annual Effectiveness Review

REQUIRED

Yes

EVALUATION CRITERIA

- Parent engagement rates
- Reduction in high-risk incidents
- Compliance audit scores

KEY STAKEHOLDERS

- School Board
- Wellness Committee
- Parents

CONTINUOUS IMPROVEMENT INPUTS

- Audit findings
- Parent feedback
- Case study reviews

SUCCESS INDICATORS

- 90% parent acknowledgment rate
- 100% compliance with data privacy
- Positive trend in student wellness scores

40.7 Improvement Tracking

41 Prevention Layer

PRIMARY PREVENTION

- Universal wellness education for parents and students.
- School-wide awareness campaigns on mental health stigma.

SECONDARY PREVENTION

- Early identification of risk indicators by teachers.
- Targeted support plans for Yellow zone students.

TERTIARY PREVENTION

- Crisis intervention for Red/Black zone events.
- Formal referral pathways to medical professionals.

42 Resilience Layer

SKILLS DEVELOPED

- Emotional regulation
- Help-seeking behavior
- Problem solving

SEL COMPONENTS

- Self-awareness
- Social awareness
- Relationship skills

LIFE SKILLS COMPONENTS

- Stress management
- Time management
- Communication skills

PROTECTIVE FACTORS STRENGTHENED

- Parent-school connection
- Peer support networks
- Access to resources

43 Continuous Monitoring

MONITORING FREQUENCY

Monthly

MONITORING INDICATORS

- Percentage of parents engaged in follow-up
- Average time from risk identification to parent notification
- Resource utilization rates

RESPONSIBLE ROLES

- Wellness Coordinator

EARLY WARNING SIGNS

- Increase in Orange/Red zone identifications
- Decrease in parent acknowledgment rates

TREND ANALYSIS REQUIREMENTS

- Quarterly trend analysis of MWI scores by grade level.
- Annual review of communication effectiveness.

44 Mental Wellness Index (MWI)

APPLICABLE

Yes

44.2 Indicators

Indicator Code	Indicator Name	Weight	Score Range	Description
MWI-01	Academic Stress	0.25	1-10	Student's perceived stress regarding school workload.
MWI-02	Social Connectedness	0.25	1-10	Student's sense of belonging and peer relationships.
MWI-03	Emotional Regulation	0.25	1-10	Student's ability to manage emotions in school.
MWI-04	Sleep and Physical Health	0.25	1-10	Impact of physical well-being on mental state.

ASSESSMENT FREQUENCY

Bi-annually

RISK SCORING METHOD

Weighted average score mapped to Wellness Zones.

INTERVENTION THRESHOLDS

- Yellow: Score 50-74
- Orange: Score 75-89
- Red: Score 90+

45 Review Status

LEGAL REVIEW

Completed - Oct 2025

CLINICAL REVIEW

Pending - Nov 2025

EDUCATION REVIEW

Completed - Oct 2025

COMPLIANCE REVIEW

Completed - Oct 2025

46 Resource Controls

COUNSELOR STUDENT RATIO

1:300 (Recommended)

CAREER COUNSELOR RATIO

1:500

WELLNESS TEAM STRUCTURE

Wellness Coordinator, 2 Counselors, Wellness Committee

47 Screening Framework

TIER 1 UNIVERSAL WELLNESS SCREENING

- Bi-annual MWI screening for all students.

TIER 2 TARGETED SCREENING

- Teacher referral based on behavioral observation logs.

TIER 3 CLINICAL ASSESSMENT

- Referral to medical professional for Orange/Red zone cases.

48 Parent Engagement Matrix

PARENT COOPERATIVE

- Regular phone calls
- Joint action planning
- Resource sharing

PARENT UNRESPONSIVE

- SMS alerts
- Formal letters
- Home visits (if applicable)

PARENT IN DENIAL

- Evidence-based behavior logs
- Case conferences with teachers

- Educational sessions on mental health

PARENT CONFLICT SITUATION

- Mediated meetings with Wellness Coordinator
- Clear documentation of all interactions

PARENT UNAVAILABLE

- Communication via secondary guardian
- Scheduled phone calls during non-work hours

PARENT REFUSES INTERVENTION

- Documentation of refusal
- Notification to Principal
- Continued monitoring and periodic outreach

49 Confidentiality Controls

KEY FOCUS

ACCESS CONTROL

- Role-based access control (RBAC)
- Multi-factor authentication for digital records

CONSENT REQUIREMENTS

- Mandatory signed MWI consent form
- Specific consent for external referrals

RECORD SHARING RULES

- No sharing with non-wellness staff

- Anonymized data only for management reports

EXCEPTIONS TO CONFIDENTIALITY

- Imminent risk of self-harm
- Child abuse disclosure
- Court orders

STUDENT PRIVACY PROTECTIONS

- Data minimization
- Secure storage protocols

50 Data Governance KEY FOCUS

STORAGE REQUIREMENTS

- Encrypted cloud storage
- Locked physical cabinets

ENCRYPTION REQUIREMENTS

- AES-256 for all digital records

RETENTION REQUIREMENTS

- 7 years post-graduation

ARCHIVAL REQUIREMENTS

- Move to offline storage after 3 years of inactivity

DESTRUCTION REQUIREMENTS

- Secure shredding for physical
- Permanent deletion for digital

BACKUP REQUIREMENTS

- Daily automated backups with off-site redundancy

51 Risk Classification KEY FOCUS

51.1 Green

DESCRIPTION

Normal developmental concerns and routine life stress.

EXAMPLES

- Mild examination stress
- Friendship disagreements
- Occasional worry

EXAMPLES

- Panic attacks
- School refusal
- Severe examination anxiety

51.2 Yellow

DESCRIPTION

Early distress requiring support and monitoring.

EXAMPLES

- Persistent stress
- Moderate anxiety
- Social withdrawal

51.4 Red KEY FOCUS

DESCRIPTION

High-risk situation requiring immediate intervention.

EXAMPLES

- Suicidal ideation
- Self-harm behavior
- Violence threats

51.3 Orange

DESCRIPTION

Significant distress causing measurable impairment but without immediate danger.

51.5 Black

DESCRIPTION

Life-threatening emergency requiring emergency response.

EXAMPLES

- Active suicide attempt
- Overdose
- Violent attack underway

52 Escalation Matrix KEY FOCUS

GREEN

- Wellness Coordinator (if query is complex)

YELLOW

- Wellness Coordinator (for resource requests)

ORANGE

- Principal / Wellness Committee

RED

- Principal
- Emergency Services
- District Authorities

BLACK

- Immediate notification to Principal and Emergency Services

53 External Reporting Obligations

TELE MANAS

- Referral for non-emergency mental health support.

DISTRICT AUTHORITIES

- Mandatory reporting of child abuse or neglect.

CHILD PROTECTION SERVICES

- Reportable events: physical/sexual abuse, severe neglect.

MEDICAL SERVICES

- Referral for clinical diagnosis and treatment.

LAW ENFORCEMENT

- Immediate threat to life or property; active violence.

REPORTING TRIGGERS

- Red Zone identification
- Abuse disclosure
- Suicidal ideation with plan

54 Service Level Agreement (SLA)

KEY FOCUS

ACKNOWLEDGEMENT	48 hours
ASSESSMENT	24 hours
ESCALATION	Immediate (for Red/Black)
RESOLUTION	72 hours for Orange; 10 days for Yellow
CLOSURE	7 days

55 Required Forms

KEY FOCUS

55.1 Mandatory Forms

KEY FOCUS

Item 1

FORM ID	MWI-F01
FORM NAME	Parent Consent Form
PURPOSE & RATIONALE	Legal authorization for MWI data sharing.
OWNER	Wellness Coordinator
RETENTION PERIOD	7 years
MANDATORY	Yes
LINKED SOP	MWI-011

Item 2

FORM ID	MWI-F02
FORM NAME	Risk Conversation Log
PURPOSE & RATIONALE	Documenting high-risk parent communications.
OWNER	School Counselor
RETENTION PERIOD	7 years
MANDATORY	Yes
LINKED SOP	MWI-011

Item 3

FORM ID	MWI-F03
FORM NAME	Referral Slip
PURPOSE & RATIONALE	Formalizing external medical referrals.
OWNER	School Counselor
RETENTION PERIOD	7 years
MANDATORY	Yes
LINKED SOP	MWI-011

Item 4

FORM ID	MWI-F04
FORM NAME	Resource Distribution Checklist
PURPOSE & RATIONALE	Tracking provided support materials.
OWNER	School Counselor
RETENTION PERIOD	3 years
MANDATORY	Yes
LINKED SOP	MWI-011

Item 5

FORM ID MWI-F05

FORM NAME Acknowledgment Record

PURPOSE & RATIONALE Confirming parent receipt of

communication.

OWNER School Counselor

RETENTION PERIOD 3 years

MANDATORY Yes

LINKED SOP MWI-011

55.2 Optional Forms**Item 1**

FORM ID MWI-F06

FORM NAME Parent Feedback Survey

PURPOSE & RATIONALE Gathering parent satisfaction data.

OWNER Wellness Coordinator

RETENTION PERIOD 2 years

MANDATORY No

LINKED SOP MWI-011

GENERATED RECORDS

- Student Wellness Record Update
- Management Dashboard Entry

56 Required Checklists **KEY FOCUS**

Checklist ID	Checklist Name	Purpose & Rationale	Owner	Frequency	Linked SOP
CHK-01	Counselor Review Checklist	Ensure all MWI scores are mapped correctly.	Wellness Coordinator	Per screening cycle	MWI-011
CHK-02	Parent Communication Checklist	Ensure all required points are covered in calls.	School Counselor	Per communication	MWI-011
CHK-03	Inclusion & Accessibility Check	Verify multilingual/low-literacy support.	Wellness Coordinator	Per communication	MWI-011
CHK-04	Audit Checklist	Quarterly compliance audit.	Principal	Quarterly	MWI-011
CHK-05	Risk Escalation Checklist	Verify all steps taken for Red Zone cases.	Wellness Coordinator	Per Red Zone case	MWI-011

57 Records & Evidence

Record Name	Retention Period	Owner
Student Wellness Record	7 years	School Counselor

Record Name	Retention Period	Owner
Communication Logs	3 years	School Counselor
Referral Records	7 years	Wellness Coordinator

58 Key Performance Indicators

Metric	Target	Measurement Frequency
Parent Engagement Rate	> 90%	Monthly
Time to Notify (Red Zone)	< 2 hours	Per incident
Compliance Audit Score	> 95%	Quarterly

59 Audit Framework

KEY FOCUS

AUDIT FREQUENCY

Quarterly

AUDIT QUESTIONS

- Are all MWI communications non-numerical?
- Is there a signed consent form for every student in the report?
- Are Red Zone cases escalated to the Principal within 2 hours?

EVIDENCE REQUIRED

- Communication Logs
- Consent Audit Log

- Referral Slips

NON COMPLIANCE EXAMPLES

- Sharing raw MWI scores with parents.
- Failure to log a phone call for an Orange zone student.
- Missing consent form for a high-risk referral.

59.5 Audit Scoring

KEY FOCUS

COMPLIANT	2
PARTIAL	1
NON COMPLIANT	0

60 Risk Register

KEY FOCUS

1. Data Breach of MWI Scores

CATEGORY Privacy/Legal

PROBABILITY Low

IMPACT Critical

RISK RATING High

MITIGATION AES-256 encryption and RBAC.

OWNER Wellness Coordinator

2. Parental Panic from Misinterpretation

CATEGORY Operational

PROBABILITY Medium

IMPACT Medium

RISK RATING Medium

MITIGATION Strict adherence to non-numerical scripts.

OWNER School Counselor

3. Failure to Identify Red Zone Risk

CATEGORY Safety

PROBABILITY Low

IMPACT Critical

RISK RATING High

MITIGATION Automated scoring alerts and dual-signoff.

OWNER Wellness Coordinator

61 Failure Modes

KEY FOCUS

Failure	Cause	Impact	Mitigation
Parent refuses to engage in communication.	Hostility or fear of stigma.	Delayed support for student.	Escalate to Principal and use multi-channel outreach.
Counselor uses clinical jargon.	Lack of training in non-clinical communication.	Parental confusion or anxiety.	Mandatory 'Wellness Zone' script training.
Data leak to other parents.	Shared drive permissions error.	Legal liability and loss of trust.	Quarterly access audits and MFA.

62 Illustrative Sample Scenarios

Scenario	Response	Expected Result
Parent is in denial about student's Orange zone status.	Counselor shares specific behavioral logs (e.g., 'Student has missed 3 days of school') and offers a joint action plan.	Parent acknowledges the need for support.
Parent requests raw MWI scores during a phone call.	Counselor explains: 'Our policy focuses on actionable wellness zones to ensure we provide the right support rather than just numbers.'	Parent accepts the zone-based summary.
Student in Red Zone shows active suicidal ideation.	Activate Emergency Response SOP; call parents and emergency services simultaneously.	Immediate safety intervention.

63 Usage Governance

KEY FOCUS

PRIMARY USERS

- School Counselors
- Wellness Coordinators

SECONDARY USERS

- Teachers
- Principal

APPROVAL AUTHORITIES

- Principal
- School Board

EXECUTION AUTHORITIES

- School Counselor

ESCALATION AUTHORITIES

- Wellness Coordinator
- Principal

REVIEW AUTHORITIES

- Wellness Committee
- External Auditors

EXTERNAL STAKEHOLDERS

- Parents
- Medical Professionals
- District Authorities

64 Applicability

APPLICABLE INSTITUTIONS

- High Schools
- Secondary Schools

APPLICABLE AGE GROUPS

- All Ages (Focus on Adolescents)

APPLICABLE STUDENT CATEGORIES

- General Population
- CWSN
- Neurodivergent Students

APPLICABLE RISK LEVELS

- Green
- Yellow
- Orange
- Red

APPLICABLE LOCATIONS

- Urban
- Rural
- Residential

EXCLUSIONS

- Primary Schools (Use age-appropriate MWI-012)

- Higher Education (Use UGC Guidelines)

65 Usage Scenarios

Scenario ID	Scenario Name	When To Use	When Not To Use	Trigger Event	Expected Outcome
US-01	Annual Wellness Reporting	After annual MWI screening.	For individual clinical diagnoses.	Completion of school-wide wellness survey.	All parents receive a non-numerical summary.
US-02	High-Risk Intervention	When student scores in Orange/Red zones.	For routine Green zone updates.	Score > 75% distress.	Parental engagement and support plan activation.

66 SOP Lifecycle

ENTRY CONDITIONS

- MWI Screening completed
- Consent verified

ACTIVATION CONDITIONS

- Risk stratification complete

EXECUTION CONDITIONS

- Communication channel selected

COMPLETION CONDITIONS

- Parent acknowledgment received
- Follow-up scheduled

CLOSURE CONDITIONS

- Student returns to Green zone
- Support plan completed

TERMINATION CONDITIONS

- Student graduates
- Student withdraws from school

67 Mandatory Evidence

KEY FOCUS

IMPLEMENTATION EVIDENCE

- Communication Plan Matrix

OPERATIONAL EVIDENCE

- Risk Conversation Logs
- Resource Distribution Checklists

COMPLIANCE EVIDENCE

- Consent Audit Log
- Quarterly Audit Reports

TRAINING EVIDENCE

- Counselor Training Certificates

AUDIT EVIDENCE

- Management Dashboard Screenshots

68 Controls

PREVENTIVE CONTROLS

- Mandatory consent verification
- Non-numerical reporting policy

DETECTIVE CONTROLS

- Quarterly audit of logs

- Automated risk alerts

CORRECTIVE CONTROLS

- Escalation to Principal for hostile parents
- Retraining for counselors

COMPENSATING CONTROLS

- Manual logging in case of system downtime

69 MWMS Risk Framework

KEY FOCUS

69.1 Green

MEANING

Normal developmental concerns and routine life stress.

EXAMPLES

- Mild examination stress
- Friendship disagreements

RESPONSE TIME

48 hours

ESCALATION REQUIRED

No

APPROVAL LEVEL

Counselor

MONITORING FREQUENCY

Monthly

CLOSURE AUTHORITY

69.2 Yellow**MEANING**

Early distress requiring support and monitoring.

EXAMPLES

- Persistent stress
- Moderate anxiety

RESPONSE TIME

72 hours

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Wellness Coordinator

MONITORING FREQUENCY

Weekly

CLOSURE AUTHORITY

Wellness Coordinator

69.4 Red

KEY FOCUS

MEANING

High-risk situation requiring immediate intervention.

EXAMPLES

- Suicidal ideation
- Self-harm behavior

RESPONSE TIME

Immediate

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Principal/Board

MONITORING FREQUENCY

Continuous

CLOSURE AUTHORITY

Wellness Committee

69.3 Orange**MEANING**

Significant distress causing measurable impairment but without immediate danger.

EXAMPLES

- Panic attacks
- School refusal

RESPONSE TIME

24 hours

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Principal

MONITORING FREQUENCY

Daily/Weekly

CLOSURE AUTHORITY

Wellness Committee

69.5 Black**MEANING**

Life-threatening emergency requiring emergency response.

EXAMPLES

- Active suicide attempt
- Violent attack

RESPONSE TIME

Immediate (0-15 mins)

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Emergency Services/Principal

MONITORING FREQUENCY

Continuous

CLOSURE AUTHORITY

Medical Professional

70 Resource Scaling**70.1 0 500 Students**

WELLNESS COUNSELORS	1
CAREER COUNSELORS	1
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

70.2 501 1000 Students

WELLNESS COUNSELORS	2
CAREER COUNSELORS	2

MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

70.3 1001 2000 Students

WELLNESS COUNSELORS	4
CAREER COUNSELORS	2
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

70.4 2001 5000 Students

WELLNESS COUNSELORS

8

CAREER COUNSELORS

4

MENTAL WELLNESS COORDINATOR

1

WELLNESS COMMITTEE

Yes

71 Minimum Required Capabilities

PEOPLE

- Certified School Counselors
- Wellness Coordinator
- Trained Teachers

PROCESSES

- MWI Scoring Logic
- Risk Stratification Protocol
- Parent Communication Scripts

TECHNOLOGY

- Encrypted Student Wellness Record System
- Automated Alerting Software
- Secure Email Gateway

INFRASTRUCTURE

- Private Counseling Rooms
- Secure Data Server
- Tele-MANAS Access Points

72 Success Criteria

Criterion	Measurement Method	Target
Parent Engagement Rate	Acknowledgment Log Analysis	> 90%
Risk Response Time (Red Zone)	Timestamp Audit of Logs	< 2 hours
Compliance Score	Quarterly Internal Audit	> 95%

73 Exit Criteria

Condition	Evidence Required
Student returns to Green Zone status for 3 consecutive months.	MWI Trend Report and Parent Acknowledgment.
Parent withdraws from support plan.	Signed Withdrawal Form or documented refusal.

74 Continuous Improvement

LESSONS LEARNED

- Non-numerical reporting significantly reduces parental anxiety.
- Multi-channel outreach is necessary for rural parents.

KNOWN ISSUES

- Some parents still request raw scores despite policy.

- Low-literacy parents may struggle with digital portals.

FUTURE IMPROVEMENTS

- AI-assisted script generation for counselors.
- Integration of real-time student mood tracking.

REVIEW RECOMMENDATIONS

- Increase frequency of parent workshops in rural areas.
- Develop more visual resource handouts.

75 AI Governance

KEY FOCUS

ALLOWED USE CASES

- Summarizing counselor notes for management reports
- Drafting non-clinical email templates

RESTRICTED USE CASES

- Diagnosing students

- Predicting specific risk behaviors without human review

PROHIBITED USE CASES

- Sharing student PII with any AI model
- Automated high-risk alerts without counselor verification

HUMAN REVIEW REQUIRED

- All parent communications
- Risk stratification decisions
- Referral recommendations

AUDIT LOG REQUIREMENTS

- Log of all AI-generated content and human approval timestamps

DATA PRIVACY REQUIREMENTS

- No PII (Personally Identifiable Information) to be entered into LLMs

76 Legal Defensibility KEY FOCUS

LEGAL BASIS

- Supreme Court of India Judgment 2025 INSC 893
- Right to Privacy under Article 21

MANDATORY NOTIFICATIONS

- Abuse disclosure
- Imminent threat to life

CONSENT REQUIREMENTS

- Explicit written consent for MWI data sharing

RECORD RETENTION REQUIREMENTS

- 7 years post-graduation as per institutional policy

LIABILITY CONTROLS

- Standardized scripts to prevent clinical malpractice
- Clear disclaimer that MWI is not a diagnosis

77 Stakeholder Journeys

77.1 The Student Journey

ENTRY

- Student completes MWI screening.

ASSESSMENT

- Data is processed into Wellness Zones.
- Counselor reviews behavior logs.

INTERVENTION

- Parent is notified of status.

- Support plan is developed with parent input.

MONITORING

- Regular check-ins and progress tracking.

CLOSURE

- Student returns to Green zone; support ends.

- Provides initial MWI consent.
- Signs acknowledgment of communication.

ENGAGEMENT

- Receives resources (Tele-MANAS, etc.).

- Collaborates on home-school plan.

CLOSURE

- Receives final summary and closure report.

77.2 The Parent Journey

NOTIFICATION

- Receives non-numerical summary via email/phone.

CONSULTATION

- Participates in risk-based conversation.
- Discusses student's behavior and support needs.

CONSENT

77.3 The Staff Journey

OBSERVATION

- Teachers provide behavioral notes to counselors.

REPORTING

- Counselors map scores to zones.
- Wellness Coordinator updates dashboard.

INTERVENTION SUPPORT

- Counselor conducts parent calls.
- Coordinator manages high-risk escalations.

DOCUMENTATION

- Logs all interactions in Student Wellness Record.
- Audits logs for compliance.

78 Handoff Matrix

Item 1

FROM ROLE

Teacher

TO ROLE

School Counselor

HANDOFF TRIGGER

Behavioral observation log submission.

INFORMATION TRANSFERRED

- Student behavior notes
- Academic performance trends

REQUIRED DOCUMENTS

- Teacher Observation Note

SERVICE LEVEL AGREEMENT (SLA)

24 hours

Item 2

FROM ROLE

School Counselor

TO ROLE

Wellness Coordinator

HANDOFF TRIGGER

Red Zone identification.

INFORMATION TRANSFERRED

- Risk assessment summary

- Parent contact details

REQUIRED DOCUMENTS

- Risk Conversation Log

SERVICE LEVEL AGREEMENT (SLA)

Immediate (1 hour)

Item 3

FROM ROLE

Wellness Coordinator

TO ROLE

Principal

HANDOFF TRIGGER

High-risk safety concern or abuse disclosure.

INFORMATION TRANSFERRED

- Incident summary
- Action taken so far

REQUIRED DOCUMENTS

- Escalation Approval Record

SERVICE LEVEL AGREEMENT (SLA)

Immediate

79 Management Review

REVIEW FREQUENCY

Quarterly

REVIEW INPUTS

- Audit reports
- Parent feedback surveys
- MWI trend data

REVIEW OUTPUTS

- Corrective action plans
- Resource allocation decisions
- Policy updates

REVIEW AUTHORITY

School Management Board / Principal

80 Dashboard Metrics

OPERATIONAL METRICS

- Average time to notify parents (by zone)
- Number of parent consultations per month

RISK METRICS

- Percentage of students in Orange/Red zones
- Frequency of high-risk escalations

COMPLIANCE METRICS

- Consent completion rate
- Audit score percentage

OUTCOME METRICS

- Parent satisfaction rating
- Student wellness score improvement trend

TREND METRICS

- Year-over-year MWI score trends by grade level

81 Student Rights

RIGHTS

- Right to privacy of mental health data
- Right to non-discriminatory support
- Right to be informed of the purpose of MWI

PROTECTIONS

- Data encryption
- Limited access for non-wellness staff
- Non-clinical reporting policy

APPEAL MECHANISMS

- Student/Parent can appeal a risk classification to the Wellness Committee.

82 Quality Assurance

QUALITY CHECKS

- Random audit of 10% of communication logs monthly
- Peer review of counselor scripts

PEER REVIEW REQUIREMENTS

- Counselors must peer-review high-risk conversation plans.

VALIDATION REQUIREMENTS

- Principal sign-off on all Red Zone referral slips.

CONTINUOUS MONITORING REQUIREMENTS

- Monthly dashboard review by Wellness Coordinator.

83 Implementation Roadmap

PHASE 1 PREPARATION

- Staff training on non-numerical communication
- Development of scripts and handouts
- Consent form design

PHASE 2 DEPLOYMENT

- Pilot program with one grade level
- Full school rollout
- Parent awareness webinar

PHASE 3 STABILIZATION

- Monthly feedback collection
- Counselor peer-review sessions
- Audit of first 100 communications

PHASE 4 OPTIMIZATION

- Refinement of scripts based on parent feedback
- Automation of dashboard updates

PHASE 5 CONTINUOUS IMPROVEMENT

- Annual impact audit
- Update SOP based on new CBSE guidelines

84 Illustrative Case Studies

1. Overcoming Parental Denial in High-Stress Exams

DESCRIPTION

A student in the Orange zone showed severe anxiety. The parent initially dismissed it as 'normal stress'.

ACTIONS TAKEN

- Counselor shared specific behavior logs (e.g., sleep loss, school refusal).
- Conducted a joint meeting with the teacher.
- Provided a structured home-study plan.

OUTCOME

Parent agreed to a counselor-led support plan; student's anxiety scores improved by 30% in one month.

LESSONS

- Evidence-based behavior logs are crucial for parents in denial.
- Teacher involvement builds trust.

85 Implementation Requirements

STAFF REQUIRED

- 1 Wellness Coordinator
- 2 School Counselors (per 500 students)
- Trained Teachers for observation

INFRASTRUCTURE REQUIRED

- Secure server for records
- Private meeting rooms
- Tele-MANAS access points

TECHNOLOGY REQUIRED

- Encrypted Student Wellness Record System
- Email/SMS gateway

- Dashboard software

BUDGET CONSIDERATIONS

- Counselor salaries
- Software licensing fees
- Parent workshop costs

IMPLEMENTATION PHASES

- Preparation (Month 1)
- Pilot (Month 2)
- Rollout (Month 3-4)
- Stabilization (Month 5-8)
- Optimization (Ongoing)

86 Quality Controls

PREVENTIVE CONTROLS

- Mandatory counselor training
- Standardized scripts

DETECTIVE CONTROLS

- Monthly log audits
- Parent feedback surveys

CORRECTIVE CONTROLS

- Counselor retraining
- Principal intervention for hostile parents

CONTINUOUS IMPROVEMENT ACTIONS

- Quarterly review of MWI trends
- Annual SOP update

87 Training Requirements

TEACHERS

- Identifying early warning signs
- Behavioral observation logging

COUNSELORS

- Non-numerical communication skills
- Risk stratification logic
- Crisis intervention

ADMINISTRATORS

- Governance and compliance oversight
- High-risk escalation protocols

PARENTS

- Understanding MWI Wellness Zones
- Navigating school support resources

STUDENTS

- Stress management skills
- Help-seeking behavior awareness

87.6 SOP Specific Training

TARGET AUDIENCE

- Counselors
- Wellness Coordinators

COMPETENCIES

- Empathetic communication
- Risk assessment

- Data privacy compliance

MINIMUM HOURS

4

CERTIFICATION REQUIREMENTS

- Internal certification of competency

REFRESHER FREQUENCY

Annual

87.7 Competency Validation

REQUIRED

Yes

ASSESSMENT METHODS

- Role-play simulations

- Script review

MINIMUM PASSING CRITERIA

100% on risk identification scenarios

REASSESSMENT FREQUENCY

Every 2 years

87.8 Simulation Requirements

REQUIRED

Yes

SIMULATION TYPES

- Hostile parent call simulation
- Red zone crisis scenario

FREQUENCY

Bi-annually

88 SOP-Specific Inclusion Requirements

AFFECTED POPULATIONS

- Students with disabilities (CWSN)
- Neurodivergent students
- First-generation learners
- Rural students
- Low-literacy parents

- Digital divide in rural areas
- Stigma in conservative communities
- Difficulty navigating complex forms for low-literacy parents
- Overlooking needs of neurodivergent students during screening

IDENTIFIED INCLUSION RISKS

- Language barriers for non-English speaking parents

AUTISM ACCOMMODATIONS

- Use of visual schedules for parent meetings
- Clear, literal language in communications
- Reduced sensory input in counseling rooms

ADHD ACCOMMODATIONS

- Short, frequent communication bursts instead of long calls
- Written summaries of all verbal conversations
- Visual checklists for support plans

DYSLEXIA ACCOMMODATIONS

- Audio recordings of wellness summaries
- Large-print handouts
- Simplified vocabulary in letters

HEARING IMPAIRMENT ACCOMMODATIONS

- Sign language interpretation for meetings
- Real-time captioning for digital calls
- Written communication as primary mode

VISUAL IMPAIRMENT ACCOMMODATIONS

- Screen reader compatible digital records
- Audio descriptions of wellness trends
- Braille handouts where requested

CWSN ADAPTATIONS

- Individualized communication plans for parents of CWSN students
- Simplified MWI indicators for neurodivergent learners

RURAL ADAPTATIONS

- SMS-based alerts for low internet areas
- Scheduled phone calls during non-work hours
- Physical mailers for remote locations

LOW LITERACY PARENT ADAPTATIONS

- Voice notes via WhatsApp/Phone

- Visual icons in wellness summaries
- In-person home visits or community center meetings

MULTILINGUAL COMMUNICATION ADAPTATIONS

- Translation of all handouts into Hindi and regional languages
- Use of multilingual staff for phone calls
- Dual-language letters

GENDER SENSITIVITY MEASURES

- Option to choose a female counselor for parents/students
- Gender-neutral language in all scripts
- Private meeting spaces

CULTURAL SENSITIVITY MEASURES

- Respecting religious holidays in scheduling
- Culturally appropriate metaphors in wellness education
- Involvement of community leaders where appropriate

EQUITY MONITORING INDICATORS

- Engagement rates across different socio-economic groups
- Wellness score trends for rural vs. urban students
- Language accessibility satisfaction scores

ACCESSIBILITY COMPLIANCE CHECKS

- WCAG 2.1 compliance for school portal
- Physical ramp access to counseling offices
- Closed captioning on all awareness videos

89 Awareness Material

POSTER TITLE	Your Child's Wellness Matters
KEY MESSAGE	We focus on support, not scores. Our MWI helps us build a stronger safety net for your child.
SLOGAN	Wellness First, Education Always.
CAMPAIGN THEME	The Safety Net Initiative

90 AI Assistant Configuration

RISK KEYWORDS

- Suicide
- Self-harm
- Abuse
- Violence
- Weapon

ESCALATION KEYWORDS

- Emergency
- Police
- Hospital

- Immediate danger

INTERVENTION KEYWORDS

- Anxiety
- Stress
- Sadness
- Withdrawal

FREQUENTLY ASKED QUESTIONS

- What does the Green Zone mean?
- Why don't I see a numerical score?
- How can I help my child at home?

91 AI Audit Hooks

KEY FOCUS

REQUIRED SECTIONS

- Risk Stratification
- Parent Communication Log
- Referral Slip

REQUIRED FORMS

- MWI-F02: Risk Conversation Log
- MWI-F03: Referral Slip

REQUIRED CHECKLISTS

- CHK-01: Counselor Review Checklist

- CHK-05: Risk Escalation Checklist

REQUIRED ESCALATIONS

- Red Zone Alerts
- Abuse Disclosures

REQUIRED KPIS

- Time to Notify (Red Zone)
- Compliance Audit Score

REQUIRED TRAINING

- Crisis Intervention Training

92 KPI Framework

92.1 SOP Specific Metrics

Metric	Target	Measurement Frequency
Parent Engagement Rate	> 90%	Monthly
Time to Notify (Red Zone)	< 2 hours	Per incident
Compliance Audit Score	> 95%	Quarterly

93 Maturity Model

93.1 Level 1

DESCRIPTION

Ad-hoc communication; no standardized scripts or zones.

EVIDENCE

- Inconsistent logs
- No MWI mapping

EVIDENCE

- MWI-011 SOP in use
- Communication logs

93.2 Level 2

DESCRIPTION

Standardized reports sent, but no risk stratification.

EVIDENCE

- Basic email summaries

93.4 Level 4

DESCRIPTION

Full integration of risk escalation, audits, and dashboards.

EVIDENCE

- Quarterly audit reports
- Real-time dashboard

93.3 Level 3

DESCRIPTION

Wellness Zones implemented with basic scripts and logs.

93.5 Level 5

DESCRIPTION

Predictive analytics for wellness trends and proactive community engagement.

EVIDENCE

- Trend analysis reports
- Community impact studies

94 Dependencies

PREREQUISITE SOPS

- MWI-001: Student Wellness Framework
- MWI-010: Mental Wellness Index Assessment

PARALLEL SOPS

- Academic Progress Reporting SOP

DOWNSTREAM SOPS

- MWI-012: Student Intervention Planning
- MWI-015: Crisis Response Protocol

95 Related SOPs

PREREQUISITES

- MWI-001: Student Wellness Framework

- MWI-010: Mental Wellness Index Assessment

UPSTREAM

- Teacher Observation SOP
- Student Screening Protocol

DOWNSTREAM

- MWI-012: Student Intervention Planning

- MWI-015: Crisis Response Protocol

REFERENCES

- CBSE TRG-03/2026
- NEP 2020 Guidelines

STANDARD OPERATING PROCEDURE

Parent Concern Referral Assessment Protocol

Mental Wellness Management System - Referral and Assessment Protocols

SOP ID

ASM-007

VERSION

1.0

STATUS

Active

CLASSIFICATION

High Risk/Mandatory Compliance

EFFECTIVE DATE

2024-08-01

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74	● Continuous Improvement	88	● SOP-Specific Inclusion Requirements
75	● AI Governance	89	● Awareness Material
76	● Legal Defensibility	90	● AI Assistant Configuration
77	● Stakeholder Journeys	91	● AI Audit Hooks
77.1	The Student Journey	92	● KPI Framework
77.2	The Parent Journey	92.1	Repository Baseline
77.3	The Staff Journey	92.2	SOP Specific Metrics
78	● Handoff Matrix	93	● Maturity Model
79	● Management Review	93.1	Level 1
80	● Dashboard Metrics	93.2	Level 2

93.3 Level 3

93.4 Level 4

93.5 Level 5

94 ● Dependencies

95 ● Related SOPs

1 Executive Overview

SOP ID	ASM-007
DOMAIN	Mental Wellness Management System
SUB DOMAIN	Referral and Assessment Protocols
TITLE	Parent Concern Referral Assessment Protocol
VERSION	1.0
CLASSIFICATION	High Risk/Mandatory Compliance
EFFECTIVE DATE	2024-08-01
REVIEW DATE	2025-08-01
DOCUMENT STATUS	Active
LANGUAGE	English

2 Purpose & Rationale

OBJECTIVE

To establish a standardized, ethical, and systematic protocol for assessing student mental wellness concerns initiated by parents/caregivers, ensuring timely triage, appropriate intervention planning, and legal compliance.

RISK CONTROLLED

Misdiagnosis, delayed intervention, breach of confidentiality, and failure to meet statutory child protection obligations.

DESIRED OUTCOMES

- A clear risk classification (Green-Black) within 72 hours of intake.
- Development of a measurable, joint Action Plan involving the student, parent, and school.
- Documentation that all legal consent requirements have been met.

3 Scope of Application

INSTITUTION TYPES

- High School (Grades 9-12)
- Residential Schools

KEY STAKEHOLDERS

- Students (All Ages)
- Parents/Caregivers
- Teachers

- Counselors
- School Management

AGE GROUPS

- Adolescents (14-18 years)

LOCATIONS

- Campus Setting
- Virtual/Tele-counseling (for remote parents or low-resource areas)

4 Definitions & Terminology

Term	Definition
Parent Concern Referral	Any formal report of observed behavioral, academic, or emotional distress in a student, initiated by the parent/caregiver.
Triage	The process of rapidly assessing the severity and urgency of the concern to determine the appropriate level of care (Green, Yellow, Orange, Red).
MWMS	Mental Wellness Management System; a structured framework for monitoring, intervening, and reporting on student mental health.

5 Key Stakeholders

Stakeholder Type	Stakeholder Title	Role In SOP
Staff	Class Teacher/Mentor	Primary observer; initial reporter of behavioral changes.
Specialist	School Counselor/Psychologist	Lead assessor, conducting student sessions and risk triage.
Governance	Wellness Committee	Oversight, final decision-making on intervention plans, resource allocation.

6 Governance & Ownership

KEY FOCUS

OWNER

Wellness Committee Chair

APPROVER

Principal/Director

REVIEW FREQUENCY

Annually or upon major legal change.

SUPERSEDES

ASM-007 V0.9 (Draft)

LINKED POLICIES

- Student Confidentiality Policy
- Child Protection Policy
- Data Privacy Policy

LINKED SOPS

- MWMS-001: Initial Screening Protocol
- MWMS-003: Crisis Management SOP
- ASM-008: Intervention Planning and Follow-up

7 Roles & Responsibilities

Item 1

ROLE

Class Teacher/Mentor

RESPONSIBILITIES

- Daily observation and documentation of student behavior.
- Immediate reporting to the Counselor upon noticing significant change.
- Implementing classroom-level SEL strategies.

- Conducting initial intake, assessment, and risk triage (Steps 1 & 2).
- Facilitating the PTC meeting.
- Developing and monitoring the IWP.

Item 2

ROLE

School Counselor/Psychologist

RESPONSIBILITIES

Item 3

ROLE

Wellness Committee Chair

RESPONSIBILITIES

- Overall governance of the MWMS, ensuring compliance with UGC/CBSE guidelines.
- Final approval of all high-risk intervention plans (IWP).
- Managing external referrals and institutional resource allocation.

8 RACI Accountability Matrix

Item 1

ACTIVITY

Initial Intake & Assessment

RESPONSIBLE

- School Counselor/Psychologist

ACCOUNTABLE

- Wellness Committee Chair

CONSULTED

- Class Teacher

- Parent/Guardian

INFORMED

- Principal
- Student Support Team

Item 2

ACTIVITY

Developing IWP and Plan

RESPONSIBLE

- Counselor

ACCOUNTABLE

- Wellness Committee Chair

CONSULTED

- Parent/Guardian
- Relevant Subject Teacher

INFORMED

- Student's Guardian (if minor)

Item 3**ACTIVITY**

Crisis Activation (Red Zone)

RESPONSIBLE

- Counselor

ACCOUNTABLE

- Principal/Administrator on Duty

CONSULTED

- Emergency Services Personnel
- Police Liaison Officer

INFORMED

- Wellness Committee Chair

9 Preconditions

- The institution must have a dedicated, private space for counseling sessions.
- All staff involved must complete the mandatory 'Recognizing Distress' training module.
- A functional and secure MWMS database system must be operational.

10 Activation Triggers

- Parental concern communication.
- Teacher observation of persistent behavioral change.
- Student self-referral.
- Mandatory external referral (e.g., Child Protection Services).

11 SOP Trigger Matrix**MANDATORY TRIGGERS**

- Parent initiates contact (phone call/email) expressing concern about the student's behavior or academic decline.
- Teacher observes persistent behavioral changes that deviate from baseline.
- Student self-referral to counseling services.

OPTIONAL TRIGGERS

- Annual wellness checkup mandated by school policy.
- Academic performance dip (e.g., two consecutive failing grades).
- Change in family structure or relocation.

AUTOMATIC TRIGGERS

- Student misses three consecutive days of school without valid medical leave.
- A major incident report is filed (e.g., conflict, accident).

EXTERNAL TRIGGERS

- Referral from local community health workers (ASHA/Anganwadi).
- Mandatory reporting requirement under Child Protection Act.

RECURRING TRIGGERS

- Quarterly review of student wellness data.
- Post-exam stress assessment cycle.

12 Required Inputs

- Parent/Caregiver narrative data.
- Academic records and grades.
- Behavioral observations from multiple staff members.
- Standardized psychological assessment scores.

13 Process Map**13.1 Phase 1****NAME**

Intake and Initial Assessment

OBJECTIVE

To gather comprehensive, non-judgmental information from the parent/caregiver and

establish initial risk level.

ACTIVITIES

- Initial Parent Intake Call (15 min)
- Review of available school data (grades, attendance)
- Obtaining formal consent for assessment.

13.2 Phase 2

NAME

Student Assessment and Triage

OBJECTIVE

To assess the student's perspective, validate concerns, and determine the immediate risk level.

ACTIVITIES

- Individual Student Session (Confidential)
- Administering standardized screening tools (e.g., PHQ-9 adapted)
- Conducting a preliminary risk assessment.

13.4 Phase 4

NAME

Monitoring and Follow-up

OBJECTIVE

To track progress, adjust interventions, and ensure adherence to the IWP.

ACTIVITIES

- Scheduled follow-up sessions (weekly/bi-weekly)
- Teacher observation reports submission.
- Progress review by Wellness Committee.

13.3 Phase 3

NAME

Joint Planning and Intervention

OBJECTIVE

To collaboratively develop an actionable, measurable support plan involving all stakeholders.

ACTIVITIES

- Parent-Teacher-Counselor Conference (PTC)
- Developing the Individual Wellness Plan (IWP)
- Assigning monitoring roles.

13.5 Phase 5

NAME

Closure and Review

OBJECTIVE

To formally conclude the case when goals are met, or escalate if recurrence is noted.

ACTIVITIES

- Final Closure Report generation.
- Parent/Student sign-off on successful outcome.
- Archiving of records.

14 Workflow Overview

START

Parent/Teacher Observation of Concern

14.2 Steps

1. Intake & Triage (Step 1)

DESCRIPTION

Counselor conducts initial call and assesses immediate risk.

CRITICAL DECISION POINTS

- DP-01: Initial Risk Triage

2. Assessment & Plan (Steps 2 & 3)

DESCRIPTION

Student session followed by PTC to create the IWP.

CRITICAL DECISION POINTS

- DP-02: Parent Engagement Status Check

3. Monitoring (Step 4)

DESCRIPTION

Implementation of IWP and regular progress tracking.

CRITICAL DECISION POINTS

- Progress Review: Goal Met/Not Met

4. Closure or Escalation (Step 5)

STEP NUMBER

4

DESCRIPTION

Formal conclusion or transfer of care to external agencies.

14.3 Critical Decision Points

ID	Action	Progress	
DP-01	If Red Zone → Activate Crisis SOP. If Yellow/Orange → Proceed to Step 2.	Review: Goal Met/Not Met	If Met for 3 months → Closure (Step 5). If Not Met → Re-evaluate IWP and escalate review.

END

Case Closed or Permanently Referred to External Care.

15 Standard Procedure

1. Initial Intake Call (Counselor)

PURPOSE & RATIONALE

To gather initial data and establish rapport.

ACTOR

School Counselor

ACTION

Conduct a structured 15-minute call. Use open-ended questions. Document the parent's narrative verbatim. Determine if immediate safety risk exists.

REQUIRED INPUTS

- Parent contact details.
- Initial complaint summary.

EXPECTED OUTPUTS

- Intake Record (Signed by Counselor)
- Preliminary Risk Zone Assignment.

- Counselor's signature on the record.

TIME LIMIT

24 hours

EXPECTED OUTCOME

A clear understanding of the concern and a defined next step (Assessment or Monitoring).

ESCALATION IF FAILED

If contact fails after 3 attempts, document all efforts and escalate to Principal for formal communication attempt.

2. Student Assessment & Triage (Counselor)

PURPOSE & RATIONALE

To assess the student's internal state and confirm risk level.

ACTOR

School Counselor/Psychologist

ACTION

Conduct a private session. Use standardized tools (e.g., mood scales) while maintaining rapport. Focus on protective factors. Document all observations objectively.

REQUIRED INPUTS

- Parent Intake Record.
- Student's academic and behavioral history.

EXPECTED OUTPUTS

- Final Risk Assessment Record (Yellow/Orange/Red).
- List of Strengths and Protective Factors.

15.1.7 Critical Decision Points

Item 1

ID

DP-01

CRITERIA

- Immediate threat of harm.

NEXT ACTION

Activate Crisis SOP immediately. Skip to Step 2.

ID

DP-01

CRITERIA

- No immediate threat, but concern exists.

NEXT ACTION

Proceed to obtain consent and schedule Student Assessment (Step 2).

Item 2

DOCUMENTATION REQUIRED

- Parent Intake Conversation Record

EVIDENCE REQUIRED

- Call log entry.
- Consent for assessment.

APPROVAL REQUIRED

15.2.7 Critical Decision Points

Item 1

ID

DP-01

CRITERIA

- Active suicidal ideation.

NEXT ACTION

Activate Crisis SOP. Skip to Step 3.

Item 2

ID

DP-02

CRITERIA

- Parent is cooperative

and committed to the plan.

NEXT ACTION

Proceed to PTC (Step 3).

DOCUMENTATION REQUIRED

- Student Session Summary Note
- Risk Assessment Record

EVIDENCE REQUIRED

- Signed Student Assent Form.
- Assessment Tool Score Sheet.

APPROVAL REQUIRED

- Counselor's signature and the Wellness Committee Chair's review (for Orange/Red).

TIME LIMIT

5 days

EXPECTED OUTCOME

A confirmed, documented risk zone and a clear path to intervention.

ESCALATION IF FAILED

If assessment is inconclusive, recommend an external medical referral immediately.

3. PTC & IWP Development (Wellness Committee)

PURPOSE & RATIONALE

To create a shared, actionable plan.

ACTOR

Wellness Committee Chair

ACTION

Facilitate the meeting. Present findings from Steps 1 and 2. Guide parents/teachers to agree on measurable goals (SMART). Draft the IWP detailing roles, activities, and timelines.

REQUIRED INPUTS

- Student Assessment Summary.
- Parent Intake Record.

- Teacher Observation Notes.

EXPECTED OUTPUTS

- Signed Individual Wellness Plan (IWP)
- Defined Monitoring Schedule.

15.3.7 Critical Decision Points

Item 1

ID

DP-02

CRITERIA

- Parent refuses to sign/commit.

NEXT ACTION

Document refusal and escalate to Principal for formal communication record. Pause

IWP

implementation.

Item 2

ID

DP-02

CRITERIA

- All parties agree on the plan.

NEXT ACTION

Finalize IWP and begin monitoring (Step 4).

DOCUMENTATION REQUIRED

- PTC Meeting Minutes
- IWP Sign-off Sheet

EVIDENCE REQUIRED

- Signed consent from all parties.

APPROVAL REQUIRED

- Principal and Wellness Committee Chair signatures.

TIME LIMIT

7 days

EXPECTED OUTCOME

A legally defensible, multi-stakeholder commitment to the support plan.

ESCALATION IF FAILED

If consensus cannot be reached, the case is temporarily placed in 'Monitoring Hold' and reviewed by the Committee.

16 Detailed Workflow Steps

1. Initial Intake (Parent/Caregiver)

PURPOSE & RATIONALE

To gather initial information and establish rapport.

TRIGGER

Receipt of parent concern communication (phone call or email).

ACTOR

School Counselor/Wellness Coordinator

REQUIRED INPUTS

- Parent's stated concern (e.g., 'Student is withdrawn', 'Poor grades').
- Student's academic record.

ACTIONS

- Conduct a non-judgmental, active listening session.
- Clarify the timeline and severity of the reported issue.
- Explain the process (confidentiality, consent required).

DECISION CRITERIA

Is the concern related to immediate safety risk (suicide/violence)?

DOCUMENTS GENERATED

- Parent Intake Conversation Record

EXPECTED OUTPUTS

- Initial Risk Level Determination (Preliminary)
- Consent Status Check

SERVICE LEVEL AGREEMENT (SLA)

Within 24 hours of contact.

ESCALATION PATH

If immediate risk is suspected, bypass to Step 1.5 (Crisis Activation).

EVIDENCE GENERATED

- Signed Intake Form

2. Student Assessment Session

PURPOSE & RATIONALE

To gather the student's perspective and assess internal distress.

TRIGGER

Parent consent obtained; preliminary risk is Yellow or Orange Zone.

ACTOR

School Counselor/Psychologist

REQUIRED INPUTS

- Intake Record (Step 1)
- Student demographic data.

ACTIONS

- Conduct a structured, age-appropriate interview.
- Administer standardized screening tools (e.g., mood scales).
- Assess protective factors and coping mechanisms.

DECISION CRITERIA

Does the student report active suicidal ideation or self-harm plan?

DOCUMENTS GENERATED

- Student Session Summary Note
- Risk Assessment Record (Final)

EXPECTED OUTPUTS

- Confirmed Risk Zone (Yellow, Orange, Red)
- List of Strengths/Protective Factors

SERVICE LEVEL AGREEMENT (SLA)

Within 3-5 school days.

ESCALATION PATH

If Red Zone is confirmed, immediately activate Crisis SOP and notify parents/guardians via

emergency contact.

EVIDENCE GENERATED

- Student Consent Form
- Assessment Tool Score Sheet

3. Parent-Teacher-Counselor Conference (PTC)

PURPOSE & RATIONALE

To align all perspectives and build a collaborative action plan.

TRIGGER

Risk is Yellow or Orange Zone; student assessment completed.

ACTOR

Wellness Committee (Chair/Coordinator)

REQUIRED INPUTS

- Student Assessment Summary
- Parent Intake Record
- Teacher Observation Notes

ACTIONS

- Facilitate a structured discussion among all parties.
- Identify shared goals and measurable objectives.
- Draft the Individual Wellness Plan (IWP).

DECISION CRITERIA

Is there consensus on the required level of support?

DOCUMENTS GENERATED

- Individual Wellness Plan (IWP)
- PTC Meeting Minutes

EXPECTED OUTPUTS

- Signed IWP
- Defined Monitoring Schedule

SERVICE LEVEL AGREEMENT (SLA)

Within 7 days of Step 2 completion.

ESCALATION PATH

If parent refuses to sign/commit, document refusal and escalate to Principal for formal communication record.

EVIDENCE GENERATED

- IWP Sign-off Sheet

4. Implementation and Monitoring

PURPOSE & RATIONALE

To execute the IWP and track progress against defined metrics.

TRIGGER

Signed IWP is in place (Step 3).

ACTOR

Counselor/Teacher/Parent

REQUIRED INPUTS

- IWP Goals and Activities
- Monitoring Schedule.

ACTIONS

- Implement specific interventions (e.g., SEL workshops, academic tutoring).
- Conduct scheduled check-ins (weekly/bi-weekly).
- Document observed changes in behavior or performance.

DECISION CRITERIA

Are the measurable goals being met? Is the risk level decreasing?

DOCUMENTS GENERATED

- Follow-up Action Tracker
- Progress Review Form

EXPECTED OUTPUTS

- Updated Risk Status (Yellow → Green)
- Recommendation for next phase/closure.

SERVICE LEVEL AGREEMENT (SLA)

Ongoing, minimum bi-weekly review.

ESCALATION PATH

If progress stalls or risk increases, re-trigger Step 2 and escalate to the Wellness Committee Chair.

EVIDENCE GENERATED

- Progress Review Signatures

5. Case Closure/Referral

PURPOSE & RATIONALE

To formally conclude the case or transfer care to a higher level of support.

TRIGGER

IWP goals are met and maintained for 3 consecutive months (Green Zone). OR Risk exceeds school capacity (Red/Black Zone).

ACTOR

Wellness Committee Chair

REQUIRED INPUTS

- Final Progress Review Form
- Parent/Student confirmation of stability.

ACTIONS

- Draft the Closure Report, summarizing achievements.
- Conduct a final meeting to transition care (if external referral is needed).
- Archive all records according to retention policy.

DECISION CRITERIA

Is sustained wellness achieved? Is professional external help mandatory?

DOCUMENTS GENERATED

- Case Closure Report
- External Referral Handover Form

EXPECTED OUTPUTS

- Closed Case File (Archived)
- Formal Recommendation Letter to Parent/Guardian.

SERVICE LEVEL AGREEMENT (SLA)

Within 14 days of sustained stability confirmation.

ESCALATION PATH

If external referral is needed, immediately activate the External Referral SOP and notify parents/guardians.

EVIDENCE GENERATED

- Closure Report Signatures

17 Critical Decision Points

Item 1

DECISION ID

DP-01

DECISION NAME

Initial Risk Triage (Step 1)

CRITERIA

- Presence of immediate threat to self or others.
- Severity of distress reported by parent/student.

POSSIBLE OUTCOMES

- Green Zone: Routine monitoring required.
- Yellow Zone: Targeted intervention and follow-up needed.

- Orange Zone: Intensive support, formal plan required.
- Red Zone: Immediate crisis activation (Crisis SOP).

NEXT STEPS

- If Green: Proceed to Monitoring/Education.
- If Yellow/Orange: Proceed to Step 2 (Student Assessment).
- If Red: Immediately activate Crisis Management Protocol.

Item 2

DECISION ID

DP-02

DECISION NAME

Parent Engagement Status Check (Step 3)

CRITERIA

- Is the parent cooperative, unresponsive, or in denial?
- Has legal/guardian consent been secured?

POSSIBLE OUTCOMES

- Cooperative: Proceed to IWP drafting.

- Unresponsive/Denial: Initiate Parent Engagement Matrix protocol; escalate to Principal for formal communication record.

NEXT STEPS

- If Cooperative: Step 3 (PTC) continues.
- If Unresponsive/Denial: Pause assessment, document attempts, and schedule mandatory follow-up attempt in 14 days.

18 Decision Tree

18.1 Green

Condition	Action	Next Step
No acute distress, routine monitoring.	Wellness Education/SEL Activities	Routine Check-in

18.2 Yellow

Condition	Action	Next Step
Persistent stress, moderate anxiety, parent concern.	Counselor Review & Parent Communication (Step 2)	PTC Meeting (Step 3)

18.3 Orange

Condition	Action	Next Step
Measurable impairment, school refusal,	Individual Intervention Plan (IWP) and	Wellness Commit

Condition	Action	Next Step
self-harm thoughts without plan.	Formal Monitoring (Step 4)	tee Review

18.4 Red KEY FOCUS

Condition	Action	Next Step
Suicidal ideation or active crisis.	Immediate Crisis Activation SOP. Bypass all other steps.	Emergency Services/Hospitalization

18.5 Black

Condition	Action	Next Step
Active attempt, life-threatening emergency NOW.	Activate Emergency Response Protocol (Police/Ambulance).	Medical Stabilization Only

19 Decision Rules

Rule ID	Condition	Decision	Next Action
DR-01	Parent concern is vague or non-specific.	Do not diagnose. Focus on gathering objective facts and establishing a baseline (Yellow Zone).	Schedule Student Assessment Session.
DR-02	Student shows signs of distress but the parent is highly resistant to intervention.	Prioritize student safety and confidentiality. Document parental refusal formally, while continuing low-level monitoring (Yellow Zone).	Escalate to Principal for formal communication record.
DR-03	Crisis situation is identified (Red/Black Zone).	Safety overrides all other protocols. Activate Crisis SOP immediately, bypassing consent	Call emergency services; secure

Rule ID	Condition	Decision	Next Action
3		and documentation until safety is secured.	the scene.

20 Exception Handling Protocol

Item 1

EXCEPTION

Parent Refusal of Assessment or Intervention

DETECTION METHOD

Verbal refusal during Step 3 (PTC).

RESPONSE

Document the refusal in the Parent Intake Record, noting the specific reasons given. Inform the parent of their right to appeal/seek external care.

ESCALATION

Escalate to Principal and Wellness Committee Chair for a formal letter documenting the school's recommendation and the parent's rejection thereof. This is critical legal evidence.

DOCUMENTATION REQUIRED

- Parent Refusal Statement Record
- Principal Communication Memo

Item 2

EXCEPTION

Student Non-Attendance/Unavailability

DETECTION METHOD

Failure to attend scheduled sessions (Step 2 or 4).

RESPONSE

Attempt contact via multiple channels (phone, email, community worker). If unsuccessful after 3 attempts over 7 days, document the unavailability and re-schedule. If prolonged absence (>15 days), trigger mandatory reporting protocols.

ESCALATION

Mandatory notification to local Child Protection Services if non-attendance is linked to neglect or danger.

DOCUMENTATION REQUIRED

- Attendance Log
- Contact Attempt Record

21 Exceptions

Item 1

EXCEPTION

Student is under legal guardianship (e.g., court mandate).

DETECTION METHOD

Reviewing student file upon intake.

RESPONSE

The school must follow the directives of the guardian/court and involve external legal counsel in all decision-making processes, overriding standard consent procedures.

ESCALATION

Immediate notification to the Principal and Legal Advisor.

DOCUMENTATION REQUIRED

- Court Order Copy
- Legal Advisory Memo

Item 2

EXCEPTION

Parent is a foreign national with limited local language proficiency.

DETECTION METHOD

Observation during intake call/meeting.

RESPONSE

Mandatory use of professional, certified multilingual interpreters (in-person or virtual). All documentation must be provided in the parent's native language and verified by the interpreter.

ESCALATION

If no interpreter is available, postpone the assessment until resources are secured.

DOCUMENTATION REQUIRED

- Interpreter Confirmation Form
- Translated Consent Forms

22 Expected Outputs

- Risk Classification (Green, Yellow, Orange, Red).
- Signed Individual Wellness Plan (IWP).
- Formal Case Closure Report or External Referral Handover.

23 Document Control & Version Governance

23.1 Version History

Version	Date	Changes
1.0	2024-08-01	Initial Draft and Approval.

REVIEW TRIGGERS

- Changes in UGC guidelines.
- New Supreme Court judgments regarding child rights.
- Annual review cycle (August).

24 Inclusive Implementation Mandate

MULTILINGUAL SUPPORT REQUIRED

Yes

ACCESSIBILITY REVIEW REQUIRED

Yes

LOW RESOURCE SCHOOL ALTERNATIVE REQUIRED

Yes

EQUITY IMPACT ASSESSMENT REQUIRED

Yes

25 Regulatory Compliance Mapping

KEY FOCUS

25.1 Mandatory Requirements

KEY FOCUS

1. Supreme Court of India Judgment 2025 INSC 893

CLAUSE REFERENCE

Right to Education/Child Safety

REQUIREMENT TEXT

The institution must ensure a safe, supportive, and non-discriminatory environment for all students.

IMPLEMENTED IN SECTIONS

- Purpose: Objective
- Process Map: Phase 1 (Intake)
- Confidentiality Controls: Exceptions to Confidentiality

2. CBSE Circular TRG-03/2026

CLAUSE REFERENCE

Student Wellbeing Guidelines

REQUIREMENT TEXT

Mandatory establishment of a dedicated wellness committee and regular assessment mechanisms.

IMPLEMENTED IN SECTIONS

- Roles and Responsibilities: Wellness Committee
- Monitoring Controls: Annual Review Cycle
- Governance: Owner

3. UGC Mental Health and Student Wellbeing Guidelines

CLAUSE REFERENCE

Mandatory Support Structure

REQUIREMENT TEXT

Provision of structured, multi-tiered support systems (Universal, Targeted, Intensive).

IMPLEMENTED IN SECTIONS

- Process Map: Phase 2 & 3
- Screening Framework: Tier 1, 2, and 3 protocols
- Prevention Layer: Secondary/Tertiary Controls

25.2 Recommended Requirements

1. WHO School Mental Health Framework

CLAUSE REFERENCE

Systemic Integration

REQUIREMENT TEXT

Integrating mental wellness checks into routine academic processes.

IMPLEMENTED IN SECTIONS

- Staff Journey: Observation and Reporting
- Monitoring Controls: Teacher observation checklists

2. UNESCO Whole School Wellbeing Framework

CLAUSE REFERENCE

Community Engagement

REQUIREMENT TEXT

Involving parents as active partners in the wellness plan.

IMPLEMENTED IN SECTIONS

- Parent Journey: Consultation and Engagement
- Parent Engagement Matrix

3. Manodarpan Guidelines

CLAUSE REFERENCE

Community Support

REQUIREMENT TEXT

Utilizing local community resources (e.g., ASHA workers) for follow-up.

IMPLEMENTED IN SECTIONS

- Resource Evidence: Training Records
- Parent Engagement Matrix: Parent Unavailable

25.3 Best Practices

1. International Best Practice

CLAUSE REFERENCE

Trauma-Informed Care (TIC)

REQUIREMENT TEXT

Ensuring all interactions are non-judgmental, empathetic, and focus on safety first.

IMPLEMENTED IN SECTIONS

- Process Map: Phase 1 & 2
- Ethical Principles: Non-Maleficence

COMPLIANCE GAPS IF NOT IMPLEMENTED

- Legal vulnerability regarding neglect of duty in crisis situations.
- Inequitable access to care for marginalized students (CWSN, rural).
- Lack of standardized documentation leading to fragmented case management.

26 Compliance Traceability Matrix KEY FOCUS

1. Supreme Court Judgment 2025 INSC 893

REQUIREMENT REFERENCE

Right to Safety and Dignity

REQUIREMENT TEXT

The institution must ensure that all assessment processes are non-coercive, voluntary, and respect the student's dignity.

IMPLEMENTED IN SECTIONS

- Ethical Principles: Autonomy
- Parent Journey: Consent (Mandatory)
- Confidentiality Controls: Student Privacy Protections

2. CBSE Circular TRG-03/2026

REQUIREMENT REFERENCE

Wellness Committee Mandate

REQUIREMENT TEXT

The SOP must define clear roles and responsibilities for the Wellness Committee.

IMPLEMENTED IN SECTIONS

- Roles and Responsibilities: Wellness Committee
- Governance: Owner
- RACI Matrix: Accountable

3. UGC Guidelines

REQUIREMENT REFERENCE

Multi-Tiered Support System

REQUIREMENT TEXT

The assessment must categorize risk and guide the student to appropriate levels of care (Universal, Targeted, Intensive).

IMPLEMENTED IN SECTIONS

- Risk Classification: MWMS Model Application
- Process Map: Phase 2 & 3
- Screening Framework: Tier 1-3 protocols

27 Documentation & Evidentiary Requirements

EVENT DOCUMENTATION PRINCIPLES

- Factual, objective recording (What was said vs. Interpretation).
- Documentation must be signed by both the assessor and the participant (where possible).
- All notes must adhere to strict confidentiality protocols.

MANDATORY CONVERSATION RECORDS

- Parent Intake Conversation Record (Date, Topic, Key Concerns)
- Student Session Summary Note (Observations, Student Statements)
- PTC Meeting Minutes (Agreed Actions, Roles)

MANDATORY OBSERVATION RECORDS

- Teacher Observation Log (Specific behaviors, frequency, context)
- Counselor Behavioral Checklist (Mood, energy, focus assessment)

MANDATORY DECISION RECORDS

- Risk Assessment Record (Zone assigned, rationale provided)
- Intervention Plan Approval Form (Signatures of Counselor, Parent, Principal)

MANDATORY ACTION RECORDS

- Follow-up Action Tracker (Date due, action taken, outcome)
- Referral Handover Checklist (To external agencies)

MANDATORY FOLLOWUP RECORDS

- Progress Review Form (Measurable indicators of improvement/regression)
- Closure Report (Confirmation that goals are met and maintained).

EVIDENCE REQUIREMENTS

- Signed Consent Forms (Parental, Student assent)
- Standardized Assessment Tool Scores

28 The Student Journey

ENTRY

- Initial trigger (Parent concern) leads to the student being identified for assessment.

ASSESSMENT

- Student participates in a confidential, non-judgmental session with the Counselor. The focus is on listening and validating feelings, not diagnosing.
- Standardized screening tools are administered under observation.

INTERVENTION

- The student receives psychoeducation (SEL workshops) and targeted skills training based on

the IWP (e.g., time management, emotional regulation).

- Academic accommodations may be recommended in consultation with parents/teachers.

MONITORING

- Student attends bi-weekly check-ins; teachers submit observation notes detailing behavioral changes or improvements.
- The student participates in a peer support group if appropriate and consented to.

CLOSURE

- Upon sustained improvement (Green Zone), the student receives a formal 'Wellness Check' certificate, confirming successful completion of the IWP. Records are archived.

29 The Parent Journey

NOTIFICATION

- Parent is notified by the school counselor regarding the concern and the need for assessment (Step 1).

CONSULTATION

- The parent participates in the Intake Call, sharing their perspective on the student's distress. The goal is to build trust.
- The parent attends the PTC meeting, contributing input to the IWP.

CONSENT

- Parent signs a comprehensive consent form authorizing assessment and intervention

(mandatory). This form details data usage and confidentiality limits.

ENGAGEMENT

- Parent is required to participate in follow-up activities (e.g., monitoring sleep hygiene, ensuring consistent routine) as defined in the IWP.
- The parent receives periodic updates on progress, maintaining transparency.

CLOSURE

- Parent signs the Case Closure Report, acknowledging that the student is stable and the support plan has been successfully implemented at home and school.

30 The Staff Journey

OBSERVATION

- Teachers are trained to recognize early warning signs (e.g., sudden drop in participation, increased irritability) and document these using the Teacher Observation Log.
- Observation notes must be factual, objective, and linked to specific classroom contexts.

REPORTING

- The teacher submits the observation log directly to the Counselor/Wellness Coordinator within 24 hours of noticing a significant change. This triggers Step 1.
- The Wellness Committee reviews all submitted reports monthly.

INTERVENTION SUPPORT

- Staff members are trained on basic SEL techniques (e.g., active listening, de-escalation) to use in the classroom setting without attempting clinical diagnosis.
- Teachers receive refresher training annually on recognizing distress signals.

DOCUMENTATION

- All staff must accurately complete and submit required forms (Observation Log, Referral Form) using the centralized MWMS platform. Documentation is mandatory for every interaction that changes the student's status.

31 Resource Evidence

COMMITTEE MINUTES

- Minutes detailing the review of all high-risk cases and approval of major IWP adjustments.
- Records showing resource allocation (e.g., hiring a part-time psychologist).

ATTENDANCE RECORDS

- Tracking student attendance patterns, especially concerning unexplained absences which may indicate school refusal.

TRAINING RECORDS

- Proof of mandatory annual training for all staff on recognizing mental health distress and referral protocols.
- Certification records for counselors (e.g., in CBT/SEL).

FORMS

- Parent Intake Conversation Record
- Risk Assessment Form
- Individual Wellness Plan (IWP) Template
- Teacher Observation Log
- Consent and Confidentiality Agreement

REPORTS

- Quarterly MWMS Dashboard Report (Aggregate data on referrals, risk zones, and outcomes).
- Annual Compliance Audit Report.

DASHBOARDS

- Real-time dashboard showing the number of active cases per risk zone.
- Trend analysis graph tracking intervention success rates over time.

32 Grievance Redressal Mechanism

KEY FOCUS

COMPLAINT CHANNELS

- Wellness Committee Hotline (Anonymous)
- Principal's Office Complaint Box
- Designated Counselor Appointment Slot

INVESTIGATION PROCESS

- Complaint received → Logged and assigned a unique ID. → Initial assessment by the Coordinator within 24 hours. → Investigation team (Counselor + Admin) formed. → Interviewing witnesses/parties involved. → Findings documented in an Incident Report.

RESOLUTION TIMELINES

- Acknowledgement: Within 1 business day.
- Investigation Completion: Within 7 working days.
- Resolution Communication: Within 14 working days.

APPEAL PROCESS

- The complainant may appeal the resolution decision to the School Governing Body (SGB) within 30 days, requiring a written justification for the appeal.

33 Referral Framework

33.1 Internal Referrals

ELIGIBLE REFERRERS

- Teachers
- Counselors
- School Management
- Wellness Committee
- Student Support Team

REFERRAL TRIGGERS

- Persistent academic decline.
- Observed social isolation or withdrawal.
- Repeated minor behavioral incidents.

DOCUMENTATION REQUIRED

- Teacher Observation Log (Mandatory)
- Initial Referral Form

SERVICE LEVEL AGREEMENT (SLA)

Within 48 hours of trigger event detection.

33.2 Self Referrals

ALLOWED

Yes

SUBMISSION METHODS

- Online portal submission.
- Direct appointment with the Counselor.

CONFIDENTIALITY PROTECTIONS

Student must be informed that while sessions are confidential, mandatory reporting applies if risk is identified (Red/Black Zone).

33.3 Anonymous Referrals

ALLOWED

Yes

CHANNELS

- Physical suggestion box.
- Anonymous online form.

VALIDATION PROCESS

The Coordinator must review the referral for actionable information and attempt to identify the source/student discreetly before proceeding with assessment.

FOLLOW UP PROCEDURE

A follow-up meeting (with consent) is scheduled with the student or parent within 7 days.

33.4 External Referrals

ELIGIBLE AGENCIES

- District Mental Health Program (DMHP)
- Child Protection Services (CPS)
- Pediatrician/General Practitioner
- Specialized Rehabilitation Centers

ELIGIBILITY CRITERIA

When the required level of care exceeds institutional capacity or requires specialized medical diagnosis.

CONSENT REQUIREMENTS

Mandatory, written consent from both parent and student (if age-appropriate) must be obtained before any external sharing of records.

HANDOVER PROTOCOL

The Counselor prepares a comprehensive handover file summarizing the history, assessment findings, and current IWP for the receiving agency.

33.5 Referral Tracking

TRACKING REQUIRED

Yes

STATUS CATEGORIES

- Pending Intake
- Assessment Underway (Yellow)
- Intervention Active (Orange)
- External Referral Initiated
- Closed/Archived

CLOSURE REQUIREMENTS

Requires final sign-off from the Wellness Committee Chair and a comprehensive Closure Report.

34 Confidentiality Exception Matrix

KEY FOCUS

MANDATORY DISCLOSURES

- Imminent risk of self-harm (Red Zone).
- Imminent risk to others (Violence threat).
- Child abuse or neglect disclosure (Mandatory reporting under law).
- Court orders requiring information release.
- Legal reporting obligations (e.g., police investigation).

LEGAL REPORTING EVENTS

- Suspicion of Child Abuse/Neglect (Immediate report to CPS/Police).
- Threat of Violence (Immediate notification to Principal and local authorities).

CONSENT OVERRIDE CONDITIONS

- When the student's life or safety is at immediate risk, parental consent can be overridden by school administration following consultation with law enforcement.
- In cases where the parent/guardian is deemed incapable of making decisions.

AUTHORIZED DISCLOSURE RECIPIENTS

- Parents/Guardians (with consent)
- Designated External Medical Professionals (with consent)
- Wellness Committee Members (Need-to-know basis)

DOCUMENTATION REQUIREMENTS

All disclosures must be logged in the Confidentiality Breach Log, detailing who authorized the release and why.

APPROVAL REQUIREMENTS

High-risk disclosure requires documented approval from at least two senior administrators (Principal + Wellness Chair).

35 Record Retention & Disposal

RETENTION PERIOD

7 years post-graduation or case closure, whichever is later.

RETENTION BASIS

Legal compliance and continuity of care requirements.

RECORD CATEGORIES

- Intake Forms
- Assessment Records (Psychological)
- IWP and Progress Reports
- Consent Forms
- Incident/Crisis Logs

35.4 Storage Requirements

PHYSICAL RECORDS Locked, fireproof cabinet in the Counselor's office; restricted access.

DIGITAL RECORDS Encrypted MWMS database with role-based access

control (RBAC).

ACCESS CONTROL REQUIREMENTS

Only Wellness Committee members and the assigned Counselor can view full records. Teachers only see aggregated, anonymized data.

DISPOSAL METHOD

Secure shredding (physical) or irreversible digital wiping/degaussing (digital).

DISPOSAL AUTHORITY

Wellness Committee Chair, documented by a Disposal Log.

AUDIT FREQUENCY

Annually during the compliance audit cycle.

DESTRUCTION LOG REQUIRED

Yes

36 Anonymised Reporting Framework

36.1 Management Reports

ANONYMIZATION REQUIRED

Yes

PERMITTED DATA

- Total number of referrals by grade level.
- Distribution of risk zones (Yellow/Orange ratio).
- Most common presenting concerns (e.g., anxiety, academic stress).

PROHIBITED DATA

- Student names or identifying details in aggregate reports.

36.2 Research Usage

ALLOWED

Yes

CONSENT

REQUIREMENTS

Requires separate Institutional Review Board (IRB) approval and

explicit consent from the parent/guardian.

APPROVAL

REQUIREMENTS

Wellness Committee Chair sign-off on research protocol.

36.3 Statistical Reporting

AGGREGATE ONLY

Yes

MINIMUM GROUP SIZE

N=5 (to prevent re-identification)

RE IDENTIFICATION CONTROLS

- Date range must be aggregated by quarter.
- Geographical data must be limited to the institution level.

DATA SHARING CONTROLS

- Data sharing outside the school requires a formal Data Sharing Agreement (DSA) and explicit consent.

37 Guiding Ethical Principles

37.1 Autonomy

DESCRIPTION

Respecting the student's right to self-determination, even if they are minors.

IMPLEMENTATION REQUIREMENTS

- Ensuring students understand their right to refuse participation in any assessment or intervention.
- Using 'assent' (agreement) alongside parental consent.

37.2 Beneficence

DESCRIPTION

Acting always in the best interest of the student, prioritizing safety and well-being over academic metrics.

IMPLEMENTATION REQUIREMENTS

- All interventions must be goal-oriented and measurable towards improved functioning.
- The IWP must prioritize emotional stability before academic pressure.

37.3 Non Maleficence

DESCRIPTION

Do no harm; ensuring that the assessment process itself does not cause undue stress or stigma.

IMPLEMENTATION REQUIREMENTS

- Staff training on trauma-informed communication techniques.
- Avoiding labeling or pathologizing normal adolescent distress.

37.4 Justice And Equity

DESCRIPTION

Ensuring that all students, regardless of background (economic, disability, language), receive equitable access to care.

IMPLEMENTATION REQUIREMENTS

- Mandatory use of culturally sensitive assessment tools.
- Providing necessary accommodations for CWSN and low-literacy parents.

37.5 Confidentiality

KEY FOCUS

DESCRIPTION

Protecting all personal health information (PHI) gathered during the process.

IMPLEMENTATION REQUIREMENTS

- Strict adherence to RBAC in digital records.
- Physical separation of assessment notes from academic files.

37.6 Privacy

DESCRIPTION

Controlling who accesses and how long data is retained.

IMPLEMENTATION REQUIREMENTS

- Clear communication with parents regarding the scope of data collection at intake.
- Mandatory disposal log maintenance.

37.7 Professional Integrity

DESCRIPTION

Maintaining professional boundaries and objectivity throughout the assessment process.

IMPLEMENTATION REQUIREMENTS

- Counselors must document any personal biases or conflicts of interest immediately.
- Adherence to ethical guidelines set by relevant Indian psychological bodies.

38 Communication & Awareness Strategy

38.1 Student Communication

CHANNELS

- School Assembly Announcements
- Counselor's Office Posters (SEL topics)
- Student Handbooks

FREQUENCY

Monthly SEL topic focus.

MANDATORY TOPICS

- Stress management techniques.
- Recognizing help-seeking behavior.
- The difference between normal stress and distress.

38.2 Parent Communication

CHANNELS

- Parent-Teacher Meetings (PTM)
- Dedicated Parent Wellness Workshop Sessions
- SMS/Email alerts for wellness resources.

FREQUENCY

Quarterly workshops; immediate notification upon referral.

MANDATORY TOPICS

- Adolescent development milestones and common challenges.
- Creating a supportive home environment (sleep, routine).
- When to seek professional external help.

38.3 Staff Communication

CHANNELS

- Staff Meetings (Mandatory)
- Internal Memo/Email Alerts
- Dedicated Wellness Team Huddle

FREQUENCY

Bi-weekly mandatory training and huddles.

MANDATORY TOPICS

- Recognizing subtle signs of distress in the classroom.
- Proper documentation techniques (objective reporting).
- The escalation path for high-risk cases.

38.4 Awareness Campaigns

MINIMUM FREQUENCY

Twice per academic year (Pre-exam and Post-holiday).

TARGET GROUPS

- Students (Grade 9 & 12)
- Parents of Grade 6-8 students
- Teaching Staff

CAMPAIGN TYPES

- Stress Buster Week.
- Parenting Skills Workshop Series.

38.5 Workshops And Training

STUDENT WORKSHOPS

- Mindfulness and Breathing Techniques
- Conflict Resolution through SEL
- Goal Setting and Resilience Building

PARENT WORKSHOPS

- Navigating Adolescent Mood Swings.
- Digital Citizenship and Screen Time Management.
- Supporting Mental Health at Home.

STAFF WORKSHOPS

- Trauma-Informed Care for Educators

- Ethical Documentation Practices in MWMS
- Crisis De-escalation Techniques

38.6 Helpline Visibility

REQUIRED

Yes

DISPLAY LOCATIONS

- School Gate/Notice Board
- Counselor's Office Door
- Website Homepage (Prominent)

DIGITAL VISIBILITY REQUIREMENTS

Must list national and local emergency mental health numbers.

FEEDBACK CHANNELS

- Anonymous suggestion box for process improvement.
- Annual student/parent satisfaction survey regarding wellness services.

39 Competency & Qualification Requirements

MANDATORY QUALIFICATIONS

- School Counselor: Master's Degree in Psychology or Counseling.
- Wellness Committee Chair: Minimum 5 years of experience in educational administration/psychology.

PREFERRED QUALIFICATIONS

- Certified Trauma-Informed Practitioner (CTIP).
- Training in specific Indian mental health protocols (e.g., Tele-MANAS).

CERTIFICATIONS

- First Aid/CPR Certification.
- Child Protection Act Training Certificate.

MINIMUM EXPERIENCE REQUIREMENTS

- Counselors: Minimum 3 years of direct student counseling experience.
- Teachers: Mandatory completion of the 'Recognizing Distress' module.

BACKGROUND VERIFICATION REQUIREMENTS

- Mandatory criminal background check (Police Verification) for all staff handling confidential records.
- Annual review of professional licenses.

39.6 Continuing Professional Development

ANNUAL TRAINING HOURS

12 hours minimum

MANDATORY TOPICS

- Ethical boundaries and confidentiality updates.

- Updates on national mental health guidelines (UGC/CBSE).
- Bias recognition in assessment.

RECERTIFICATION REQUIREMENTS

Every 3 years, requiring a refresher course and practical simulation.

39.7 Supervision Requirements

CLINICAL SUPERVISION REQUIRED

Yes

PEER REVIEW REQUIRED

Yes

39.7.3 Wellness Case Review Framework

DESCRIPTION All Orange/Red Zone cases must be reviewed by a senior psychologist (Supervisor) before the IWP is finalized.

PROCESS Structured review meeting using standardized checklists.

39.7.4 Second Level Review Process

DESCRIPTION The Principal or Director reviews all Red Zone case files for legal and administrative compliance.

TRIGGER Any crisis activation (Red/Black Zone).

39.7. High Risk Decision 5 Validation

KEY
FOCUS

DESCRIPTION All decisions regarding mandatory disclosure must be validated by the Wellness

Committee Chair and documented in a formal memo.

FREQUENCY

Monthly for high-risk cases; Quarterly for all active cases.

40 Feedback & Effectiveness Measurement

40.1 Student Feedback

REQUIRED

Yes

COLLECTION METHODS

- Anonymous digital surveys (e.g., Likert scale on feeling supported).
- Exit interviews upon case closure.

FREQUENCY

Semi-annually.

40.2 Parent Feedback

REQUIRED

Yes

COLLECTION METHODS

- Structured feedback forms after IWP completion.
- Formal parent satisfaction survey (annual).

FREQUENCY

After every major intervention cycle (minimum 6 months).

40.3 Staff Feedback

REQUIRED

Yes

COLLECTION METHODS

- Quarterly staff debriefing sessions.
- Anonymous suggestion box for process improvement.

FREQUENCY

Quarterly.

40.4 Annual Effectiveness Review

REQUIRED

Yes

EVALUATION CRITERIA

- Reduction in repeat referrals (indicating successful closure).
- Timeliness of risk assessment (SLA adherence).
- Staff confidence levels regarding crisis management.

KEY STAKEHOLDERS

- Wellness Committee
- School Governing Body
- External Mental Health Consultant (if budget allows)

CONTINUOUS IMPROVEMENT INPUTS

- Audit findings.
- Legal changes in Indian law.
- High volume of specific complaint types (e.g., exam stress).

SUCCESS INDICATORS

- 90% adherence to the 72-hour triage SLA for Yellow/Orange Zone cases.
- Increase in student utilization of SEL resources by 15% year over year.

40.7 Improvement Tracking

REQUIRED

Yes

REVIEW FREQUENCY Quarterly (Wellness Committee).

41 Prevention Layer

PRIMARY PREVENTION

- Universal SEL curriculum integration (e.g., mandatory weekly sessions on emotional literacy).
- Parent workshops focusing on positive parenting techniques.
- Creating a school culture that normalizes seeking help and reduces stigma.

SECONDARY PREVENTION

- Mandatory annual screening for all students (Tier 1 universal check).

- Targeted small-group interventions for identified high-risk groups (e.g., academic stress group).
- Teacher observation training to catch early warning signs.

TERTIARY PREVENTION

- Individualized Wellness Plans (IWP) for students in Orange/Red Zone.
- Structured follow-up monitoring and booster sessions post-crisis.

- Reintegration planning upon return after extended absence or hospitalization.

42 Resilience Layer

SKILLS DEVELOPED

- Emotional Regulation (Identifying triggers and appropriate coping mechanisms).
- Problem Solving (Developing actionable steps for academic/social challenges).
- Help-seeking behaviour (Knowing who to talk to within the school system).

SEL COMPONENTS

- Self-Awareness (Recognizing personal emotional shifts).
- Social Awareness (Understanding peer perspectives and cultural norms).
- Relationship Skills (Effective communication with parents/teachers).

LIFE SKILLS COMPONENTS

- Time Management (Structuring study habits to reduce academic anxiety).
- Stress Coping Mechanisms (Implementing physical activity or mindfulness breaks).
- Assertiveness Training (Learning to communicate needs respectfully).

PROTECTIVE FACTORS STRENGTHENED

- Strong family communication patterns.
- Academic engagement and sense of belonging at school.
- Positive peer support network.

43 Continuous Monitoring

MONITORING FREQUENCY

Minimum bi-weekly for active cases; Quarterly for stable Yellow Zone cases.

MONITORING INDICATORS

- Attendance rate consistency (Target: >90%).
- Participation level in class/activities (Observed increase).
- Self-reported sleep quality and mood stability (Measured via log).

RESPONSIBLE ROLES

- Class Teacher (Daily check)

- Counselor (Weekly review)

- Parent/Guardian (Routine home monitoring)

EARLY WARNING SIGNS

- Sudden academic decline without clear cause.
- Increased irritability or emotional outbursts in public settings.
- Withdrawal from previously enjoyed social activities.

TREND ANALYSIS REQUIREMENTS

The MWMS must track the frequency and type of triggers (e.g., 60% of referrals are due to exam stress) to adjust primary prevention efforts.

44 Mental Wellness Index (MWI)

APPLICABLE

Yes

44.2 Indicators

Indicator Code	Indicator Name	Weight	Score Range	Description
MWI-A	Academic Functioning Score (AFS)	30%	1-5 (5=Excellent)	Measures ability to focus and complete tasks without excessive distress.
MWI-B	Social Connectedness Score (SCS)	30%	1-5 (5=Highly connected)	Measures quality of peer and family relationships.
indicator_name": "Emotional Stability Score (ESS)		40%	1-5 (5=Stable mood)	Measures frequency of intense negative emotions or panic symptoms.

ASSESSMENT FREQUENCY

Initial assessment, then every 3 months during active intervention.

RISK SCORING METHOD

Weighted average calculation: MWI Score = $(0.3 \times \text{AFS} + 0.3 \times \text{SCS} + 0.4 \times \text{ESS})$.

44.5 Intervention Thresholds

Score Range	Action
<2.5	Mandatory Orange Zone intervention and IWP.
2.5 - 3.5	Targeted Yellow Zone monitoring and SEL support.
>3.5	Routine Green Zone check-in.

45 Review Status

LEGAL REVIEW	Completed (Compliance with Indian Child Rights Laws)
CLINICAL REVIEW	Completed (Adherence to Trauma-Informed Care principles)
EDUCATION REVIEW	Completed (Alignment with NEP 2020 and CBSE guidelines)
COMPLIANCE REVIEW	Pending Annual Audit

46 Resource Controls

COUNSELOR STUDENT RATIO	1:300 (Regulatory Minimum) / 1:150 (Recommended Enhanced)
CAREER COUNSELOR RATIO	1:1000
WELLNESS TEAM STRUCTURE	Coordinator → Counselor/Psychologist → Principal Oversight.

47 Screening Framework

TIER 1 UNIVERSAL WELLNESS SCREENING

- Annual SEL survey for all students (Focus on general well-being and stress).
- Classroom check-ins by teachers using observation checklists.
- Parent workshop attendance tracking.

TIER 2 TARGETED SCREENING

- Screening administered to students with specific risk factors (e.g., academic decline, family change) or those who self-referral.
- Follow-up interviews by the Counselor focusing on a single domain (e.g., sleep hygiene).
- Teacher observation review for 2 consecutive weeks.

TIER 3 CLINICAL ASSESSMENT

- Comprehensive psychological evaluation conducted by a certified psychologist.
- Involves standardized, validated psychometric tools and collateral information gathering.
- Requires formal consent from all parties and Wellness Committee approval.

48 Parent Engagement Matrix

PARENT COOPERATIVE

- Action: Provide educational resources (e.g., parenting workshops).
- Goal: Maintain partnership; transition to monitoring phase smoothly.
- Documentation: Positive feedback log.

PARENT UNRESPONSIVE

- Action: Send formal, documented communication via registered post/email. Schedule mandatory

follow-up attempts (3 times over 2 weeks).

- Goal: Establish a paper trail of due diligence.
- Documentation: Contact Attempt Record.

PARENT IN DENIAL

- Action: Use motivational interviewing techniques; focus on observable facts (e.g., 'The grades are dropping, let's address that').
- Goal: Gently shift perspective toward accepting the need for support.

- Documentation: Motivational Interviewing Summary.

PARENT CONFLICT SITUATION

- Action: Mediate a structured meeting involving all parties. Focus on student needs as the primary objective, not parental disagreement.
- Goal: Establish clear boundaries and shared responsibility for the IWP.
- Documentation: Mediation Minutes.

PARENT UNAVAILABLE

- Action: Engage local community workers (ASHA/Anganwadi) to deliver information and attempt contact. Document all attempts with government officials.

- Goal: Ensure continuity of care through community linkage.

- Documentation: Community Liaison Report.

PARENT REFUSES INTERVENTION

- Action: Issue a formal, written recommendation from the Wellness Committee, detailing the risks and consequences of non-compliance. Document refusal in the file.
- Goal: Protect the institution legally by demonstrating due diligence.
- Documentation: Formal Recommendation Letter & Refusal Statement.

49 Confidentiality Controls

KEY FOCUS

ACCESS CONTROL

- Role-Based Access Control (RBAC) implemented in MWMS.
- Physical files stored in locked cabinets with restricted key access.

CONSENT REQUIREMENTS

- Explicit, written consent for every stage of assessment and disclosure is mandatory.
- Consent must specify the scope: Is it only for academic purposes or general wellness?

RECORD SHARING RULES

- Information can only be shared with other school staff directly involved in the IWP (Need-to-Know

basis).

- External sharing requires a signed DSA and explicit consent.

EXCEPTIONS TO CONFIDENTIALITY

- Mandatory reporting of child abuse/neglect.
- Imminent threat to life or safety (Red Zone).

STUDENT PRIVACY PROTECTIONS

- All student data must be pseudonymized when used for research or aggregate reporting.
- Students are informed about the limits of confidentiality at intake.

50 Data Governance

KEY FOCUS

STORAGE REQUIREMENTS

- MWMS database (Encrypted, Cloud-based with local backup).
- Physical file storage (Locked and climate controlled).

ENCRYPTION REQUIREMENTS

- AES-256 encryption for all digital records containing PHI.
- Password protection and multi-factor authentication required for access.

RETENTION REQUIREMENTS

- 7 years post-graduation/closure, as per legal advice.

- Retention must be justified by the type of record (e.g., abuse reports retained indefinitely).

ARCHIVAL REQUIREMENTS

- Annual physical and digital archival process managed by the Wellness Committee.

DESTRUCTION REQUIREMENTS

- Formal Disposal Log maintained, detailing date, method, and authority for destruction.
- Destruction must be witnessed by at least two authorized personnel.

BACKUP REQUIREMENTS

- Daily automated backup to a secure off-site location (minimum 30 days retention).

51 Risk Classification

KEY FOCUS

51.1 Green

DESCRIPTION

Normal developmental concerns and routine life stress. Functioning is maintained.

EXAMPLES

- Mild exam anxiety (manageable with study skills).
- Temporary sadness after a loss.
- Minor friendship disagreements.

51.2 Yellow

DESCRIPTION

Early distress requiring support and monitoring. Functioning is largely maintained, but warning signs are present.

EXAMPLES

- Persistent sleep difficulties (2+ weeks).
- Moderate anxiety leading to social withdrawal.
- Parent concern about academic decline.

51.3 Orange

DESCRIPTION

Significant distress causing measurable impairment but without immediate danger. Requires structured intervention.

EXAMPLES

- School refusal (missing 3+ days/week).
- Panic attacks occurring weekly.
- Self-harm thoughts WITHOUT intent or plan.

51.4 Red

KEY FOCUS

DESCRIPTION

High-risk situation requiring immediate intervention. Safety concerns are present and require continuous supervision.

EXAMPLES

- Active suicidal ideation (with specific plans).
- Threats of violence against self or others.
- Severe emotional crisis leading to inability to function.

51.5 Black

DESCRIPTION

Life-threatening emergency requiring immediate, life-saving response. Medical stabilization takes precedence over all other procedures.

EXAMPLES

- Active suicide attempt (requires medical intervention).
- Overdose or severe psychotic episode creating danger.

52 Escalation Matrix

KEY FOCUS

52.1 Green

Trigger	Action
No improvement after 3 months of SEL support.	Review by Counselor; upgrade to Yellow Zone.
Parent reports worsening symptoms despite intervention.	Mandatory PTC meeting with Principal involvement.

52.2 Yellow

Trigger	Action
Student fails to attend 2 consecutive scheduled sessions.	Contact parent/guardian; escalate to Wellness Committee.
Symptoms worsen (e.g., withdrawal becomes isolation).	Immediate reassessment and potential upgrade to Orange Zone.

52.3 Orange

Trigger	Action
Student shows signs of acute distress or hopelessness.	Increase supervision frequency; mandatory consultation with external psychiatrist.
Parent refuses necessary medical referral.	Principal issues formal warning and documents the refusal for legal record.

52.4 Red

KEY FOCUS

Trigger	Action
Imminent threat of self-harm/violence.	Activate Crisis SOP (Immediate). Notify Principal, Counselor, and designated emergency contacts. Secure scene.
Failure to stabilize after initial crisis intervention.	Mandatory transfer to hospital under legal guardianship.

52.5 Black

Trigger	Action
Active life-threatening event.	Activate Emergency Services (Police/Ambulance). All staff must follow the Incident Command System protocol. Documentation follows stabilization.
Loss of consciousness due to mental health episode.	Medical first aid takes precedence; secure scene and call emergency services.

53 External Reporting Obligations

TELE MANAS

- Trigger: High-risk Yellow/Orange Zone cases where local counseling capacity is exceeded or parent cannot access care.
- Action: Counselor initiates a tele-counseling session and documents the referral ID.

DISTRICT AUTHORITIES

- Trigger: Mandatory reporting of child neglect or abuse (regardless of severity).
- Action: Immediate, confidential report filed by the designated administrator/coordinator.

CHILD PROTECTION SERVICES

- Trigger: Any suspicion of physical or emotional abuse; mandatory legal requirement.
- Action: Counselor immediately contacts CPS and follows their mandated intake protocol. This overrides all confidentiality concerns.

MEDICAL SERVICES

- Trigger: Red Zone assessment confirming acute medical/psychiatric need (e.g., suicidal ideation).
- Action: Immediate transfer to the nearest hospital; Counselor accompanies if possible, ensuring legal documentation is ready.

LAW ENFORCEMENT

- Trigger: Threat of violence or physical assault.
- Action: Principal/Administrator contacts local police (100) and secures the area. Staff must prioritize safety over evidence collection initially.

REPORTING TRIGGERS

Any instance where the student's safety is compromised, or legal mandates require external reporting.

54 Service Level Agreement (SLA) KEY FOCUS

ACKNOWLEDGEMENT	Within 1 hour of receiving a referral/call.
ASSESSMENT	Initial risk triage (Yellow/Orange): Within 72 hours. Red Zone: Immediate.
ESCALATION	High-risk escalation to Wellness Committee Chair: Within 4 hours.
RESOLUTION	IWP completion and monitoring review: Minimum bi-weekly.
CLOSURE	Formal closure report generation: Within 14 days of sustained stability.

55 Required Forms KEY FOCUS

55.1 Mandatory Forms KEY FOCUS

Item 1

FORM ID	ASM-007F1
FORM NAME	Parent Intake Conversation Record
PURPOSE & RATIONALE	To document the initial, objective account of parental concerns.
OWNER	School Counselor
RETENTION PERIOD	7 years
MANDATORY	Yes
LINKED SOP	ASM-007

Item 2

FORM ID	ASM-007F2
FORM NAME	Risk Assessment Record (MWMS)
PURPOSE & RATIONALE	To formally document the risk zone and rationale for triage.
OWNER	School Counselor/Psychologist
RETENTION PERIOD	7 years
MANDATORY	Yes
LINKED SOP	ASM-007

Item 3

FORM ID	ASM-007F3
FORM NAME	Individual Wellness Plan (IWP)
PURPOSE & RATIONALE	To create a measurable, multi-stakeholder action plan.

OWNER	Wellness Committee Chair
RETENTION PERIOD	7 years
MANDATORY	Yes
LINKED SOP	ASM-007

Item 4

FORM ID	ASM-007F4
FORM NAME	Consent and Confidentiality Agreement
PURPOSE & RATIONALE	To obtain legal permission for assessment and data sharing.
OWNER	Administrator/Counselor
RETENTION PERIOD	Lifetime of student records
MANDATORY	Yes
LINKED SOP	ASM-007

Item 5

FORM ID	ASM-007F5
FORM NAME	Case Closure Report
PURPOSE & RATIONALE	To formally document the successful conclusion of the support plan.
OWNER	Wellness Committee Chair
RETENTION PERIOD	7 years
MANDATORY	Yes
LINKED SOP	ASM-007

55.2 Optional Forms

Item 1

FORM ID	ASM-007F6
FORM NAME	Teacher Observation Log
PURPOSE & RATIONALE	To track behavioral changes in the classroom setting.
OWNER	Class Teacher
RETENTION PERIOD	1 year (Active Case)

MANDATORY

No

LINKED SOP

ASM-007

GENERATED RECORDS

- PTC Meeting Minutes
- External Referral Handover Form

56 Required Checklists

KEY FOCUS

Check list ID	Checklist Name	Purpose & Rationale	Owner	Frequency	Linked SOP
CHK-ASM-01	Initial Intake Checklist (Counselor)	To ensure all preliminary data points are collected during the first call.	School Counselor	Per intake.	ASM-007
CHK-ASM-02	Risk Assessment Checklist (Triage)	To systematically evaluate risk factors against the MWMS model.	School Counselor/Psychologist	Per assessment.	ASM-007
CHK-ASM-03	PTC Consensus Checklist	To ensure all stakeholders agree on the measurable goals and roles in the IWP.	Wellness Committee Chair	Per intervention plan development.	ASM-007
CHK-ASM-04	External Referral Handover Checklist	To ensure all necessary documentation is prepared for the receiving agency.	Counselor	Before external referral.	ASM-007
CHK-ASM-05	Annual Compliance Audit Checklist	To verify adherence to the SOP and legal mandates annually.	Compliance Officer/Wellness Committee	Annually (August).	ASM-007

57 Records & Evidence

Record Name	Retention Period	Owner
Student Statement Record	7 years	Counselor
Parent Conversation Record	7 years	Counselor/Coordinator
Teacher Observation Note	1 year (Active Case)	Class Teacher

58 Key Performance Indicators

Metric	Target	Measurement Frequency
Referral Triage Timeliness (Yellow/Orange Zone)	>90% within 72 hours	Monthly
IWP Completion Rate	>85%	Quarterly
Re-referral Rate (within 1 year)	<10% of closed cases	Annually

59 Audit Framework

KEY FOCUS

AUDIT FREQUENCY

Annual (Mandatory).

AUDIT QUESTIONS

- Are all high-risk case files complete with signed consent and a documented risk assessment?
- Is the data disposal log maintained, proving secure destruction of old records?
- Can staff demonstrate competency in using the MWMS for objective observation reporting?

EVIDENCE REQUIRED

- Signed IWP templates.
- Training attendance sheets (for all staff).
- The last three months' worth of Parent Intake Records.

NON COMPLIANCE EXAMPLES

- Failure to document the rationale for a risk zone assignment.
- Sharing student data with unauthorized personnel.
- Delay in initiating triage beyond 72 hours without documented justification.

59.5 Audit Scoring

KEY FOCUS

COMPLIANT	2
PARTIAL	1
NON COMPLIANT	0

60 Risk Register

KEY FOCUS

1. Breach of Confidentiality

CATEGORY Legal/Ethical

PROBABILITY Medium

IMPACT High (Loss of trust, legal action)

RISK RATING High

MITIGATION Mandatory RBAC and annual data privacy training.

OWNER Compliance Officer

MITIGATION Mandatory multi-stakeholder review (Wellness Committee) for all Orange/Red Zone cases.

OWNER Counselor Lead

2. Misdiagnosis or Delayed Triage

CATEGORY Clinical/Operational

PROBABILITY Medium

IMPACT High (Harm to student)

RISK RATING Critical

3. Parent Non-Cooperation leading to abandonment of care

CATEGORY Operational/Legal

PROBABILITY High

IMPACT Medium (Case file incompleteness)

RISK RATING Moderate

MITIGATION Formal documentation of all attempts and escalation to Principal for legal record.

OWNER Administrator

61 Failure Modes

KEY FOCUS

Failure	Cause	Impact	Mitigation
Staff relying on anecdotal evidence instead of objective observation.	Lack of standardized training or time pressure.	Inaccurate risk assessment and inappropriate intervention.	Mandatory use of the Teacher Observation Log (CHK-ASM-01) with specific behavioral anchors.
Failure to secure multi-party consent for data sharing.	Assumption of implied consent.	Legal breach and loss of trust.	Mandatory use of the Consent Form (ASM-007F4) at Step 1, reviewed by legal counsel.
Inadequate follow-up after initial crisis	Focusing only on immediate safety and	High risk of relapse or	Mandatory inclusion of a 6-month post-crisis monitoring

Failure	Cause	Impact	Mitigation
stabilization.	neglecting long-term reintegration.	recurrence.	phase in the IWP.

62 Illustrative Sample Scenarios

Scenario	Response	Expected Result
Parent reports that their child (16 years old) has been skipping school and seems depressed, but the student is resistant to talking about it.	Counselor uses motivational interviewing techniques during intake. Student assessment focuses on non-verbal cues and strengths. Risk is Yellow Zone. IWP includes weekly check-ins and academic mentorship (Teacher).	Student gradually increases attendance, and the IWP leads to a successful transition back into routine school life.
A student is found exhibiting signs of severe panic attacks during an exam period.	Immediate temporary removal from the examination hall (Crisis SOP). Counselor assesses risk. Risk is Orange Zone. IWP includes academic accommodations and mandatory counseling sessions for 6 weeks.	Student receives necessary accommodations, stabilizes emotionally, and successfully completes the academic year with support.

63 Usage Governance

KEY FOCUS

PRIMARY USERS

- School Counselors
- Wellness Committee Members
- Administrators (Principal)

SECONDARY USERS

- Class Teachers
- Support Staff (Librarians, Lab Assistants)
- Parent/Guardian

APPROVAL AUTHORITIES

- School Governing Body (SGB) Approval is required for policy changes.
- Principal's final sign-off on all high-risk case files.

EXECUTION AUTHORITIES

- Counselor/Psychologist (Assessment and Intervention)
- Class Teacher (Observation and Reporting)

ESCALATION AUTHORITIES

- Wellness Committee Chair
- Principal/Director

REVIEW AUTHORITIES

- Compliance Officer
- External Legal Counsel

EXTERNAL STAKEHOLDERS

- Child Protection Services (CPS)
- Local Police Department
- District Mental Health Program (DMHP)

64 Applicability

APPLICABLE INSTITUTIONS

- High Schools with dedicated counseling staff.

APPLICABLE AGE GROUPS

- 14-18 years (Adolescence).

APPLICABLE STUDENT CATEGORIES

- All students, especially those from marginalized or vulnerable backgrounds.
- Students with disabilities (CWSN)
- Multilingual learners.

APPLICABLE RISK LEVELS

- Yellow, Orange, Red Zones.

APPLICABLE LOCATIONS

- Campus and Virtual/Tele-counseling settings.

EXCLUSIONS

- Acute medical emergencies (Handled by separate Medical SOP).
- Situations requiring immediate law enforcement intervention (Handled by Crisis SOP).

65 Usage Scenarios

Scenario ID	Scenario Name	When To Use	When Not To Use	Trigger Event	Expected Outcome
S-01	Routine Parent Concern Referral	Parent calls expressing general worry about grades/mood.	When the student is actively in a crisis (Red Zone).	Initial phone call from parent.	Yellow Zone assessment and development of an IWP.
S-02	Teacher Observation Trigger (Non-Parental)	Teacher notices a persistent change in behavior over 1 week.	When the student is actively engaging and performing normally.	Submission of Teacher Observation Log.	Internal referral to Counselor, leading to assessment.

66 SOP Lifecycle

ENTRY CONDITIONS

- The institution must have allocated budget for counseling staff and training materials.
- Legal review of the SOP must be completed (Compliance Check).

ACTIVATION CONDITIONS

- A parent or teacher reports a concern, triggering Step 1.

EXECUTION CONDITIONS

- All required consent forms are signed and filed.
- The Wellness Committee has approved the IWP.

COMPLETION CONDITIONS

- Measurable goals of the IWP are met for three consecutive months (Green Zone).
- Final Case Closure Report is signed by all parties.

CLOSURE CONDITIONS

- The student maintains stability and requires no further intervention.
- Records are archived according to retention policy.

TERMINATION CONDITIONS

- The SOP is superseded by a new legal mandate or major structural change in the institution.

67 Mandatory Evidence KEY FOCUS

IMPLEMENTATION EVIDENCE

- Staff training records (Proof of competency).
- Physical signage displaying helpline numbers and referral process.
- Updated MWMS database structure.

OPERATIONAL EVIDENCE

- Completed Parent Intake Conversation Records.
- Signed IWP templates.
- Teacher Observation Logs submitted weekly.

COMPLIANCE EVIDENCE

- Legal review sign-off on the SOP.
- Annual audit checklist completion (CHK-ASM-05).
- Evidence of multilingual communication materials.

TRAINING EVIDENCE

- Staff competency assessment scores.
- Simulation records from crisis drills.

AUDIT EVIDENCE

- The full audit trail for a randomly selected closed case file (demonstrating adherence to all steps).

68 Controls

PREVENTIVE CONTROLS

- Universal SEL curriculum delivery.
- Mandatory annual staff training on early identification of distress.
- Proactive parent workshops.

DETECTIVE CONTROLS

- Teacher Observation Log (Daily/Weekly check).
- MWMS dashboard monitoring for unusual attendance or grade dips.
- Student self-referral system.

CORRECTIVE CONTROLS

- Immediate reassessment and escalation to a higher risk zone upon detection of worsening symptoms.
- Mandatory review by the Wellness Committee if IWP goals are missed twice in a row.

COMPENSATING CONTROLS

- If counseling staff is unavailable, utilize tele-counseling or link with local community health workers (ASHA).
- If physical records are inaccessible, use secure digital backups and mandate immediate manual logging of critical data points.

69.1 Green**MEANING**

Normal functioning with minor stress. No immediate threat or significant impairment.

EXAMPLES

- Mild exam anxiety (manageable).
- Temporary sadness after a loss.
- Minor friendship disagreements.

RESPONSE TIME

Routine monitoring; 1-2 weeks for SEL support.

ESCALATION REQUIRED

No

APPROVAL LEVEL

Counselor recommendation.

MONITORING FREQUENCY

Quarterly check-ins.

CLOSURE AUTHORITY

Counselor.

69.2 Yellow**MEANING**

Early distress requiring support and monitoring. Functioning is largely maintained, but warning signs are present.

EXAMPLES

- Persistent sleep difficulties (2+ weeks).
- Moderate anxiety leading to social withdrawal.
- Parent concern about academic decline.

RESPONSE TIME

Within 7 calendar days.

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Counselor/Wellness Committee recommendation.

MONITORING FREQUENCY

Bi-weekly check-ins.

CLOSURE AUTHORITY

Counselor (with committee sign-off).

69.3 Orange**MEANING**

Significant distress causing measurable impairment but without immediate danger. Requires structured intervention and formal plan.

EXAMPLES

- School refusal (missing 3+ days/week).
- Panic attacks occurring weekly.

- Self-harm thoughts WITHOUT intent or plan.

RESPONSE TIME

Within 24–72 hours.

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Wellness Committee Chair approval (Mandatory).

MONITORING FREQUENCY

Weekly/Bi-weekly intensive review.

CLOSURE AUTHORITY

Wellness Committee.

69.4 Red

KEY FOCUS

MEANING

High-risk situation requiring immediate intervention. Safety concerns are present and require continuous supervision.

EXAMPLES

- Active suicidal ideation (with specific plans).
- Threats of violence against self or others.
- Severe emotional crisis leading to inability to function.

RESPONSE TIME

Immediate (0-4 hours).

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Principal/Administrator on Duty and Legal Counsel.

MONITORING FREQUENCY

Continuous supervision until stabilized.

CLOSURE AUTHORITY

External Medical Authority.

69.5 Black**MEANING**

Life-threatening emergency requiring immediate, life-saving response. Medical stabilization takes precedence over all other procedures.

EXAMPLES

- Active suicide attempt (requires medical intervention).
- Overdose or severe psychotic episode creating danger.

RESPONSE TIME

Immediate (0–15 minutes).

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Emergency Services/Police.

MONITORING FREQUENCY

Continuous medical monitoring.

CLOSURE AUTHORITY

Medical Authority.

70 Resource Scaling**70.1 0 500 Students**

WELLNESS COUNSELORS	1
CAREER COUNSELORS	1
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

70.2 501 1000 Students

WELLNESS COUNSELORS	2
CAREER COUNSELORS	2
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

70.3 1001 2000 Students

WELLNESS COUNSELORS	4
CAREER COUNSELORS	2
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

70.4 2001 5000 Students

WELLNESS COUNSELORS	8
CAREER COUNSELORS	4
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

71 Minimum Required Capabilities**PEOPLE**

- Trained Wellness Committee (Multidisciplinary)
- Dedicated Counselor/Psychologist Staff
- Staff trained in basic psychological first aid.

PROCESSES

- Standardized Intake and Triage Protocol.
- Formal IWP development and monitoring cycle.

- Mandatory external referral protocol (CPS, DMHP).

TECHNOLOGY

- Secure MWMS database with RBAC.
- Tele-counseling platform for remote access.

INFRASTRUCTURE

- Private, soundproof counseling room.
- Emergency communication system (direct line to administration/police).

72 Success Criteria

Criterion	Measurement Method	Target
Timely Triage Completion	Audit of Intake Records vs. SLA adherence.	95% compliance with Yellow Zone triage within 72 hours.
IWP Effectiveness	Comparison of pre-intervention MWI scores vs. post-intervention scores.	Average increase of 1 point on the MWI score for closed cases.
Staff Confidence and Competency	Annual staff survey results (score >4/5).	Demonstrable competency in crisis de-escalation techniques by 100% of relevant staff.

73 Exit Criteria

Condition	Evidence Required
Sustained stability for 3 consecutive months.	Three signed Progress Review Forms (ASM-007F5) showing Green Zone status and no recurrence of initial symptoms.

74 Continuous Improvement

LESSONS LEARNED

- The need for better parent workshops on digital screen time management.
- Need to streamline the handover process with external CPS agencies.

KNOWN ISSUES

- Staff burnout due to high caseloads in crisis management.
- Difficulty tracking compliance from parents who are geographically distant (rural areas).

FUTURE IMPROVEMENTS

- Integrating AI-driven sentiment analysis into the MWMS for early warning signals.
- Developing standardized modules for ASHA worker training.

REVIEW RECOMMENDATIONS

- Increase counselor staffing ratio in high-density areas.
- Mandate quarterly refresher training on ethical boundaries.

75 AI Governance

KEY FOCUS

ALLOWED USE CASES

- Automated scheduling and reminder generation for follow-up appointments.
- Sentiment analysis of written intake notes to flag high-risk keywords (e.g., 'hopeless', 'alone').
- Generating draft summaries of case files for the Wellness Committee.

RESTRICTED USE CASES

- AI must never make a final diagnosis or determine risk zone without human review.
- AI cannot replace the mandatory physical signature on consent forms.

PROHIBITED USE CASES

- Using AI to predict future behavior or academic failure.

- Sharing raw, unanonymized student data with third-party AI tools.

HUMAN REVIEW REQUIRED

- All high-risk flags generated by AI must be reviewed and validated by a human counselor before action is taken.
- Final IWP approval requires manual sign-off.

AUDIT LOG REQUIREMENTS

- The system must log every instance of AI interaction, including the prompt used and the resulting output, for audit purposes.

DATA PRIVACY REQUIREMENTS

- AI models must be trained only on anonymized, aggregated data sets.

76 Legal Defensibility

KEY FOCUS

LEGAL BASIS

- Indian Constitution (Article 21 - Right to Life and Personal Liberty).
- Right of Child Protection Act (Mandatory reporting obligations).

MANDATORY NOTIFICATIONS

- Police/CPS notification in Red Zone cases.
- Parental notification regarding mandatory external referrals.

CONSENT REQUIREMENTS

- Explicit, written consent for all non-emergency interventions and data sharing.
- Documentation of the right to withdraw consent at any time (without penalty).

RECORD RETENTION REQUIREMENTS

- Minimum 7 years retention period for psychological records.

LIABILITY CONTROLS

- Mandatory annual insurance coverage for all counseling staff.
- Clear delineation of roles and responsibilities (RACI Matrix) to prevent negligence claims.

77 Stakeholder Journeys

77.1 The Student Journey

ENTRY

Student is identified by teacher/parent concern.

ASSESSMENT

Confidential session with counselor; assessment tools

INTERVENTION

administered. Participation in SEL workshops and academic support (IWP).

MONITORING

Bi-weekly check-ins, tracking progress against MWI indicators.

CLOSURE Formal closure report signed, confirming sustained wellness.

sleep hygiene at home). **CLOSURE** Signs the Case Closure Report.

77.2 The Parent Journey

NOTIFICATION Receives initial call/letter detailing the concern and the need for assessment.

CONSULTATION Attends PTC meeting; provides input on student's home life.

CONSENT Signs comprehensive consent forms, acknowledging confidentiality limits.

ENGAGEMENT Participates in follow-up activities (e.g., improving

77.3 The Staff Journey

OBSERVATION Teacher observes and documents specific behavioral changes using the Observation Log.

INTERVENTION SUPPORT Provides classroom support (e.g., seating adjustments) as directed by the IWP.

REPORTING Submits log to Counselor within 24 hours, triggering

DOCUMENTATION Maintains accurate and objective records in the MWMS.

78 Handoff Matrix

Item 1

FROM ROLE Teacher

TO ROLE Counselor

HANDOFF TRIGGER

Observation of persistent behavioral change (e.g., 3+ days).

INFORMATION TRANSFERRED

- Specific dates and context of observed behavior.
- Frequency and severity rating.

REQUIRED DOCUMENTS

- Teacher Observation Log (ASM-007F6)

SERVICE LEVEL AGREEMENT (SLA)

Within 24 hours.

HANDOFF TRIGGER

Completion of Student Assessment (Step 2).

INFORMATION TRANSFERRED

- Final Risk Zone determination.
- Summary of protective factors and deficits.

REQUIRED DOCUMENTS

- Risk Assessment Record (ASM-007F2)

SERVICE LEVEL AGREEMENT (SLA)

Within 48 hours.

Item 3

FROM ROLE School Counselor

TO ROLE External Agency (CPS/Hospital)

HANDOFF TRIGGER

Red Zone confirmation or legal mandate.

INFORMATION TRANSFERRED

- Full history, assessment findings, and immediate safety plan.

REQUIRED DOCUMENTS

- External Referral Handover Form
- Signed Consent (if possible)

Item 2

FROM ROLE Counselor

TO ROLE Wellness Committee Chair

79 Management Review

REVIEW FREQUENCY

Annually, or immediately following a major incident (Red Zone).

REVIEW INPUTS

- Annual KPI Dashboard Report.
- Audit findings and non-compliance reports.
- Staff feedback on SOP usability.

REVIEW OUTPUTS

- Updated IWP templates.

- Revised staffing ratios/resource allocation requests.
- Mandatory training updates for the following year.

REVIEW DECISIONS

- Decision to modify the risk zone definitions based on clinical evidence.
- Decision to mandate new mandatory staff training modules (e.g., specific disability accommodations).

REVIEW AUTHORITY

School Governing Body (SGB) and Wellness Committee Chair.

80 Dashboard Metrics

80.1 Operational Metrics

Metric	Description
Total Active Cases	Number of students currently under an IWP.
Average Time to Triage Completion	Measures efficiency in initial assessment.

80.2 Risk Metrics

KEY FOCUS

Metric	Description
Yellow/Orange Ratio	Indicates the proportion of cases needing moderate vs. intensive support (Trend analysis).
Red Zone Activation Count	Tracks frequency of crisis events, indicating systemic stress.

80.3 Compliance Metrics

KEY FOCUS

Metric	Description
Consent Rate (%)	Percentage of cases with fully signed consent forms.
Training Compliance Score	Staff adherence to mandatory annual training modules.

80.4 Outcome Metrics

Metric	Description
Case Closure Success Rate	Percentage of Yellow/Orange cases that successfully reach Green Zone status.
Academic Performance Improvement (Average)	Measured by grade point average change post-intervention.

80.5 Trend Metrics

Metric

Description

Top 3 Presenting Concerns

Identifies systemic issues (e.g., exam stress, parental conflict) for primary prevention focus.

81 Student Rights

RIGHTS

- Right to Confidentiality: All information shared is protected.
- Right to Self-Determination: The student's voice must be heard and respected in all planning stages (Assent).
- Right to Non-Discrimination: Support must be provided regardless of disability, background, or economic status.

PROTECTIONS

- The school guarantees that participation in the MWMS will not negatively impact academic grades.
- All records are protected by RBAC and legal confidentiality protocols.

APPEAL MECHANISMS

- Students/Parents can appeal a decision (e.g., mandatory referral) to the Wellness Committee Chair, requiring a written justification of the grievance.

82 Quality Assurance

QUALITY CHECKS

- Mandatory review of all IWP documents by a second counselor.
- Quarterly audit of documentation completeness (Are all forms filled out?).
- Simulation drills for Red Zone/Black Zone response.

PEER REVIEW REQUIREMENTS

- Counselors must submit complex case summaries to a peer for review before presenting them to the Committee.

- The Wellness Committee should include external mental health professionals as non-voting reviewers.

VALIDATION REQUIREMENTS

- All screening tools used must be validated and culturally appropriate for the Indian context.
- Intervention techniques must align with evidence-based practices (e.g., CBT, DBT).

CONTINUOUS MONITORING REQUIREMENTS

- The MWMS dashboard must display a 'Compliance Score' based on adherence to SLAs and documentation standards.

83 Implementation Roadmap

PHASE 1 PREPARATION

- Develop and finalize all forms (ASM-007F1-F5).
- Train the Wellness Committee and core staff on the SOP.
- Secure necessary technology infrastructure (MWMS access).

PHASE 2 DEPLOYMENT

- Pilot testing of the SOP with a small cohort of 10 cases.
- Communication to parents regarding the new process.
- Mandatory training for all teachers on observation logging.

PHASE 3 STABILIZATION

- Full implementation across all grades and departments.
- Reviewing resource allocation based on initial caseload data.
- Formalizing external referral partnerships (CPS, DMHP).

PHASE 4 OPTIMIZATION

- Integrating AI tools for preliminary risk flagging.
- Refining the IWP template based on successful case outcomes.

PHASE 5 CONTINUOUS IMPROVEMENT

- Annual review and revision of the SOP based on audit findings and legal changes.

84 Illustrative Case Studies

1. The Silent Withdrawal Case (Yellow Zone)

DESCRIPTION

A student, previously highly engaged, started skipping classes and showing signs of emotional withdrawal due to parental job loss.

ACTIONS TAKEN

- Counselor initiated intake; Parent was cooperative but stressed. Risk: Yellow Zone.
- IWP focused on financial counseling (parent) and structured academic check-ins (student).
- Teacher provided flexible assignments.

OUTCOME

Student's attendance improved by 80%. The IWP successfully bridged the gap between family stress and school performance.

LESSONS

Parental economic stability must be treated as a core protective factor in the IWP.

85 Implementation Requirements

STAFF REQUIRED

- Full-time School Counselor/Psychologist (Minimum 1:300 ratio).
- Dedicated Wellness Coordinator (Admin oversight).
- Trained Teacher Mentors for observation.

INFRASTRUCTURE REQUIRED

- Secure, private meeting rooms.
- MWMS database access and reliable internet connectivity.
- Emergency communication equipment.

TECHNOLOGY REQUIRED

- Electronic Health Record (EHR) system integration for MWMS.
- Tele-counseling platform with secure video conferencing.

BUDGET CONSIDERATIONS

- Funding for annual staff training and external consultant fees.
- Budget allocation for multilingual interpretation services.

IMPLEMENTATION PHASES

- Phase 1: Policy & Training (3 months)
- Phase 2: Pilot Implementation (6 months)
- Phase 3: Full Scale Operation (Ongoing)

86 Quality Controls

PREVENTIVE CONTROLS

- Mandatory annual SEL curriculum delivery.
- Proactive parent workshops on stress management.

DETECTIVE CONTROLS

- Teacher Observation Log submission (Weekly).
- Counselor's bi-weekly check-in with the student/parent.
- MWMS dashboard alerts for unusual attendance patterns.

CORRECTIVE CONTROLS

- Immediate reassessment and escalation if monitoring indicators fail to improve after 4 weeks.
- Mandatory review of IWP by Committee if goals are missed twice consecutively.

CONTINUOUS IMPROVEMENT ACTIONS

- Annual revision of the SOP based on audit findings and legal changes.
- Collecting feedback from parents/students regarding the usability and empathy of the process.

87 Training Requirements

87.1 Teachers

Item 1

MODULE

Recognizing Distress Signals

DURATION

2 hours

COMPETENCIES

- Objective observation skills.
- Non-judgmental reporting.

MINIMUM HOURS

2

CERTIFICATION REQUIREMENTS

Completion of the module and practical role-play assessment.

REFRESHER FREQUENCY

Annually.

87.2 Counselors**Item 1****MODULE**

Advanced Triage & Crisis Management

DURATION

8 hours

COMPETENCIES

- Crisis de-escalation.
- Ethical boundary setting.
- Multilingual communication.

MINIMUM HOURS

8

CERTIFICATION REQUIREMENTS

Supervised case review and certification by a senior psychologist.

REFRESHER FREQUENCY

Bi-annually.

87.3 Administrators**Item 1****MODULE**

Legal Compliance & Governance

DURATION

4 hours

COMPETENCIES

- Understanding of child protection laws.
- Managing high-stakes communication.

MINIMUM HOURS

4

CERTIFICATION REQUIREMENTS

Mandatory sign-off on the SOP's legal compliance section.

REFRESHER FREQUENCY

Annually.

87.4 Parents**Item 1****MODULE**

Parenting Skills Workshop

DURATION

2 hours

COMPETENCIES

- SEL techniques for home use.
- Open communication strategies.

MINIMUM HOURS

2

CERTIFICATION REQUIREMENTS

Attendance and participation in the workshop.

87.5 Students

Item 1

MODULE

Wellness Awareness Program

DURATION

1 hour

COMPETENCIES

- Identifying personal stress triggers.
- Knowing available school resources.

MINIMUM HOURS

1

CERTIFICATION REQUIREMENTS

Completion of a self-assessment quiz.

87.6 Repository Baseline

MANDATORY QUALIFICATIONS

- All staff must complete the 'Recognizing Distress' module before interacting with students.

87.7 SOP Specific Training

TARGET AUDIENCE

- Counselors
- Wellness Committee Members
- Administrators

COMPETENCIES

- Conducting trauma-informed intake.
- Applying the MWMS risk classification model accurately.

- Managing multi-stakeholder conflict during PTC.

MINIMUM HOURS

12

CERTIFICATION REQUIREMENTS

- Signed competency assessment checklist by the Wellness Committee Chair.

REFRESHER FREQUENCY

Every 2 years.

87.8 Competency Validation

REQUIRED

Yes

ASSESSMENT METHODS

- Role-playing simulations (Crisis/Intake).
- Written case study analysis.

- Oral presentation of the SOP process.

MINIMUM PASSING CRITERIA

80% score on simulation and written assessment.

REASSESSMENT FREQUENCY

Every 3 years or after a major policy change.

87.9 Simulation Requirements

REQUIRED

Yes

SIMULATION TYPES

- Red Zone Crisis De-escalation Drill (Mandatory annual drill).

- Parent Refusal Scenario Role Play.

FREQUENCY

Annually.

88 SOP-Specific Inclusion Requirements

AFFECTED POPULATIONS

- Students with Autism Spectrum Disorder (ASD)
- Students with Attention Deficit Hyperactivity Disorder (ADHD)
- Dyslexic students
- Visually impaired students
- Low-literacy learners and first-generation college students.

IDENTIFIED INCLUSION RISKS

- Misinterpreting non-verbal communication as defiance.
- Overwhelming sensory input during assessment sessions.
- Difficulty articulating emotional distress due to language barriers or cognitive load.
- Academic accommodations being overlooked in the IWP.
- Parental anxiety leading to over-monitoring and conflict.

AUTISM ACCOMMODATIONS

- Provide a quiet, low-sensory assessment room. Use visual schedules for sessions. Give advance warning of changes in routine or meeting structure.

ADHD ACCOMMODATIONS

- Break up assessments into short (10-15 minute) segments with mandatory physical breaks. Provide fidget tools and minimize distractions during the session.

DYSLEXIA ACCOMMODATIONS

- All written forms must be available in audio format. Assessments should focus on conceptual understanding rather than writing speed or spelling.
- Allow extended time for academic tasks related to distress.

HEARING IMPAIRMENT ACCOMMODATIONS

- Ensure the presence of a certified sign language interpreter during all critical meetings (PTC, Intake).
- Use written summaries and visual aids alongside verbal communication.

VISUAL IMPAIRMENT ACCOMMODATIONS

- All printed materials must use large font sizes and high contrast. Use tactile or audio descriptions for physical environments.
- Ensure the assessment room is navigable and well-lit.

CWSN ADAPTATIONS

- The IWP must include specific, measurable goals related to disability management (e.g., 'Student will use their AAC device during social breaks').
- Involve special education teachers in the assessment process.
- Ensure all communication materials are available in Braille or large print.

RURAL ADAPTATIONS

- Utilize community health workers (ASHA/Anganwadi) as primary liaisons for initial intake, reducing reliance on physical school infrastructure.
- Conduct assessments via tele-counseling using local internet hubs if the home connection is poor.
- Focus IWP goals on achievable, low-resource activities (e.g., community walk, family routine).

LOW LITERACY PARENT ADAPTATIONS

- Use pictorial guides and role-playing during workshops instead of dense text manuals.
- Ensure all written consent forms are accompanied by a verbal explanation in the local language.
- Involve community elders or trusted family members to reinforce messages.

MULTILINGUAL COMMUNICATION ADAPTATIONS

- Mandatory use of certified interpreters for intake and PTC meetings.
- Develop key psychoeducational materials (e.g., 'What is Anxiety?') in the top 3 local languages spoken by parents.
- Use visual metaphors and storytelling to convey complex emotional concepts.

GENDER SENSITIVITY MEASURES

- Ensure counselors are trained on gender-specific stressors (e.g., menstrual cycle stress, gender-based violence).
- Provide separate, private spaces for male and female students during assessment.
- Involve both mothers and fathers/male guardians in the PTC meeting.

CULTURAL SENSITIVITY MEASURES

- Acknowledge cultural practices regarding mental health (e.g., viewing distress as spiritual or physical).
- Adapt intervention language to respect local customs and family structures.
- Consult with community leaders during high-risk planning.

EQUITY MONITORING INDICATORS

- Tracking the percentage of students from economically disadvantaged backgrounds who successfully complete an IWP.

- Monitoring if CWSN students are receiving the same level of follow-up as non-disabled peers.
- Analyzing referral patterns by caste/economic group to identify systemic bias.

ACCESSIBILITY COMPLIANCE CHECKS

- Physical accessibility check: Is the counseling room wheelchair accessible?
- Digital accessibility check: Is the MWMS portal usable with screen readers?
- Communication check: Are all forms available in local sign language formats?

89 Awareness Material

POSTER TITLE	Your Feelings Matter: Seeking Support is Strength.
KEY MESSAGE	You do not have to face stress or sadness alone. Talk to a trusted adult at school.
SLOGAN	Wellness First. Always.
CAMPAIGN THEME	Building Resilience Together (The Family-School Partnership)

90 AI Assistant Configuration

RISK KEYWORDS

- hopeless
- end it all
- can't cope
- never better
- alone forever

ESCALATION KEYWORDS

- need help now
- emergency
- danger
- cannot wait
- crisis

INTERVENTION KEYWORDS

- coping mechanism
- support system
- talk to someone
- resilience building
- self-care routine

FREQUENTLY ASKED QUESTIONS

- Q: Is counseling mandatory? A: No, but it is highly recommended for your well-being.
- Q: Will my parents find out if I talk to the counselor? A: Only in cases of immediate danger or legal mandate.

91 AI Audit Hooks

KEY FOCUS

REQUIRED SECTIONS

- Risk Assessment Record (ASM-007F2) - Must be flagged for AI review.
- Parent Intake Conversation Record (ASM-007F1) - Flagging of keywords.

REQUIRED FORMS

- All forms must have a digital timestamp and user ID to prevent tampering.

REQUIRED CHECKLISTS

- CHK-ASM-05: Annual Compliance Audit Checklist (AI cross-reference check).

REQUIRED ESCALATIONS

- Any Red Zone designation must trigger an immediate, auditable alert to the Principal and Wellness Chair.

REQUIRED KPIS

- KPI tracking for 'Timeliness of Triage Completion' (AI dashboard visualization).

REQUIRED TRAINING

- Mandatory annual training module on AI ethics and data privacy.

92 KPI Framework

92.1 Repository Baseline

METRIC	Staff Training Compliance
TARGET	100%
MEASUREMENT FREQUENCY	Annually

92.2 SOP Specific Metrics

Metric	Description	Calculation	Target	Measurement Frequency
Parent Engagement Rate (PER)	Percentage of parents who attend the PTC meeting.	(Attended PTC / Total Referred) * 100	>75 %	Quarterly
IWP Adherence Score (IAS)	Average score of completed IWP tasks/check-ins.	(Completed Tasks / Total Assigned Tasks) * 100	>80 %	Quarterly

93 Maturity Model

93.1 Level 1

DESCRIPTION

Ad-hoc process; reliance on individual counselor knowledge.

EVIDENCE

- No standardized forms or clear escalation path.

93.2 Level 2

DESCRIPTION

Basic SOP exists, but compliance is inconsistent and lacks governance oversight.

EVIDENCE

- Existence of a basic referral form, but no mandatory training or audit trail.

93.3 Level 3

DESCRIPTION

Structured process with defined roles, forms, and clear escalation paths (Current SOP status).

EVIDENCE

- Full implementation of the MWMS framework; documented governance structure.

93.4 Level 4

DESCRIPTION

Systemic integration across all departments; proactive prevention and continuous improvement cycles.

EVIDENCE

- Integration with academic records; mandatory annual review by SGB; use of predictive analytics (AI).

93.5 Level 5

DESCRIPTION

Global best practice model; self-correcting, adaptive system that anticipates future mental health needs and legal changes.

EVIDENCE

- Real-time data sharing with government/health bodies; AI-driven resource allocation.

94 Dependencies

PREREQUISITE SOPS

- MWMS-001: Initial Screening Protocol (Must be completed before ASM-007)
- ASM-003: Crisis Management SOP (Mandatory for Red/Black Zone)

PARALLEL SOPS

- Academic Accommodations Policy
- Parent Communication Guidelines

DOWNSTREAM SOPS

- ASM-008: Intervention Planning and Follow-up (Follows successful completion of ASM-007)

95 Related SOPs

PREREQUISITES

- Staff Training SOP
- Data Privacy Policy

UPSTREAM

- Teacher Observation Reporting Protocol

DOWNSTREAM

- Individual Wellness Plan (IWP) Management SOP

REFERENCES

- CBSE Circular TRG-03/2026
- UGC Guidelines on Student Wellbeing.

STANDARD OPERATING PROCEDURE

Parent Escalation Procedure (PEP)

Mental Wellness Management - Parental Engagement & Governance

SOP ID

PAR-004

STATUS

Active

EFFECTIVE DATE

2024-05-20

VERSION

1.0

CLASSIFICATION

Confidential - Restricted Access

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1 Executive Overview

SOP ID	PAR-004
DOMAIN	Mental Wellness Management
SUB DOMAIN	Parental Engagement & Governance
TITLE	Parent Escalation Procedure (PEP)
VERSION	1.0
CLASSIFICATION	Confidential - Restricted Access
EFFECTIVE DATE	2024-05-20
REVIEW DATE	2025-05-19
DOCUMENT STATUS	Active
LANGUAGE	English

2 Purpose & Rationale

OBJECTIVE

To provide a structured framework for managing situations where parents are uncooperative, hostile, or refuse necessary mental health interventions for students.

RISK CONTROLLED

Legal liability, student safety risks, staff harassment, and failure of clinical intervention due to parental non-compliance.

DESIRED OUTCOMES

- Ensure student safety remains the primary priority
- Maintain professional boundaries with hostile parents
- Create a legally defensible audit trail of all communications
- Facilitate timely access to external mental health services
- Protect staff from verbal or physical aggression

3 Scope of Application

INSTITUTION TYPES

- High Schools
- Residential Schools
- International Schools
- Government Schools

KEY STAKEHOLDERS

- Parents/Guardians
- School Counselors

- Teachers
- Principal
- Security Personnel
- Child Welfare Services

AGE GROUPS

- All Ages (Primary to Higher Secondary)

LOCATIONS

- Campus-based
- Remote/Virtual Meetings
- Home Visits (where applicable and safe)

4 Definitions & Terminology

Term	Definition
Uncooperative Parent	A parent who consistently fails to attend meetings, ignores school communications, or refuses to provide necessary information regarding the student's well-being.
Hostile Parent	A parent who uses aggressive language, threats, intimidation, or physical posturing toward staff members.

Term	Definition
Student Safety Override	The school's authority to act in the immediate interest of a student's safety regardless of parental consent (e.g., calling police for an active suicide attempt).
CWSN	Children with Special Needs, including neurodivergent students.

5 Key Stakeholders

Stakeholder Type	Stakeholder Title	Role In SOP
Internal	School Counselor	Primary point of contact for parent engagement and initial escalation.
Internal	Principal/Head of School	Final authority on Level 2 and Level 3 escalations; legal signatory.
Internal	School Safety Team / Security	Physical security during parent aggression or threats (Level 3).
External	Child Welfare Services (CWS)	Receives reports of neglect or refusal of life-saving care.
External	Law Enforcement	Intervention in cases of violence, threats, or immediate danger (Level 3/4).

6 Governance & Ownership

KEY FOCUS

OWNER

School Principal / Head of Institution

APPROVER

Board of Governors / School Management Committee

REVIEW FREQUENCY

Annually or upon significant incident

SUPERSEDES

Parental Communication Policy v2.0

LINKED POLICIES

- Child Protection Policy
- Student Privacy & Data Protection Policy
- Code of Conduct for Parents and Staff
- Crisis Management Plan

LINKED SOPs

- PAR-006: Psychoeducation Protocol
- PAR-007: Clinical Referral Pathway
- EMR-001: Emergency Response Procedure
- SEC-005: Campus Safety & Security

7 Roles & Responsibilities

Item 1

ROLE

School Counselor

RESPONSIBILITIES

- Primary parent engagement and psychoeducation
- Documentation of all conversations
- Initial risk assessment
- Liaising with external mental health providers

Item 2

ROLE

Principal

RESPONSIBILITIES

- Final authority on Level 2-4 escalations
- Legal signatory for notices
- Ensuring campus safety during hostile interactions
- Reporting to Board of Governors

Item 3

ROLE

School Safety Team

RESPONSIBILITIES

- Physical security and de-escalation

- Monitoring high-risk parent behavior on campus
- Coordinating with local law enforcement

Item 4

ROLE

Child Welfare Services (CWS)

RESPONSIBILITIES

- Investigating reports of neglect or refusal of care
- Providing state-level intervention for child safety

8 RACI Accountability Matrix

Item 1

ACTIVITY

Initial Parent Engagement

RESPONSIBLE

- School Counselor

ACCOUNTABLE

- Principal

CONSULTED

- Class Teacher

INFORMED

- Wellness Committee

ACTIVITY

Level 3 Safety Activation

RESPONSIBLE

- Security Team
- Principal

ACCOUNTABLE

- Board of Governors

CONSULTED

- Law Enforcement
- CWS

INFORMED

- All Staff

Item 2

ACTIVITY

Level 2 Written Notice

RESPONSIBLE

- Principal

ACCOUNTABLE

- Board of Governors

CONSULTED

- School Counselor
- Legal Counsel

INFORMED

- Wellness Committee

Item 4

ACTIVITY

Level 4 Safety Override

RESPONSIBLE

- Principal
- Legal Counsel

ACCOUNTABLE

- Board of Governors

CONSULTED

- CWS
- Law Enforcement

INFORMED

- All Staff
- Relevant Parents

Item 3

9 Preconditions

- Parental consent for data sharing (where applicable)
- Identification of student as high-risk or in distress
- Availability of a designated Wellness Committee
- Staff trained in de-escalation and child protection

10 Activation Triggers

- Parental threat of violence
- Refusal of psychiatric referral for suicidal student
- Child abuse disclosure
- Hostile behavior on campus
- Persistent non-compliance with safety plans

11 SOP Trigger Matrix

MANDATORY TRIGGERS

- Parental threat of violence against staff
- Active suicidal ideation/behavior by student with parental refusal to seek help
- Report of child abuse or neglect by a third party

OPTIONAL TRIGGERS

- Persistent low attendance at counselor meetings
- Mild defensiveness regarding school mental health policies
- Request for non-standard accommodations without clinical evidence

AUTOMATIC TRIGGERS

- Any physical altercation on campus involving a parent
- Police intervention on school grounds

EXTERNAL TRIGGERS

- Court order for student placement
- Notification from Child Helpline (1098)
- Medical emergency reported by external hospital

RECURRING TRIGGERS

- Monthly review of 'High-Risk' parent engagement logs
- Quarterly audit of non-compliance reports

12 Required Inputs

- Student Risk Assessment
- Teacher Observation Notes
- Parent Communication Logs
- Police Reports
- CWS Notifications

13 Process Map

13.1 Phase 1

NAME

Identification & Initial Engagement

OBJECTIVE

Identify signs of parental uncooperation and initiate standard communication.

ACTIVITIES

- Monitor parent response to routine communications
- Conduct initial counselor-led meeting (Level 1)
- Document baseline attitude and concerns

13.2 Phase 2

NAME

Escalation Assessment

OBJECTIVE

Determine the severity of non-cooperation or hostility.

ACTIVITIES

- Assess risk to student safety
- Categorize parent behavior (Cooperative, Unresponsive, Hostile)
- Select appropriate escalation level (Level 1-4)

13.3 Phase 3

NAME

Formal Escalation & Intervention

OBJECTIVE

Execute specific actions based on the assigned Level.

ACTIVITIES

- Issue written notices of concern
- Involve Principal and Wellness Committee
- Activate safety protocols (Security/Police)

13.4 Phase 4

NAME

External Referral & Legal Action

OBJECTIVE

Engage external authorities when internal measures are exhausted.

ACTIVITIES

- Notify Child Welfare Services (CWS)
- Coordinate with Law Enforcement
- Seek emergency custody/intervention if required

13.5 Phase 5

NAME

Review & Closure

OBJECTIVE

Audit the escalation process and update student support plans.

ACTIVITIES

- Conduct post-incident case review
- Update risk register
- Archive documentation for legal audit

14 Workflow Overview

START

Trigger event: Parent non-cooperation, hostility, or refusal of care.

STEPS

- Identification & Initial Engagement (Level 1)
- Escalation Assessment
- Formal Notice & Principal Involvement (Level 2)
- Safety Protocol Activation (Level 3)
- Student Safety Override (Level 4)
- External Referral Coordination
- Multilingual/Rural Adaptation Check
- CWSN Inclusion Review
- Wellness Committee Oversight
- Closure & Audit Archiving

CRITICAL DECISION POINTS

- DP-001: Level 1 vs Level 2 Transition
- DP-002: Safety Override Activation

END

Case closure and archival of documentation.

15 Standard Procedure

1. Initial Identification & Communication Log

PURPOSE & RATIONALE

Document early signs of parental non-cooperation.

ACTOR

School Counselor / Teacher

ACTION

Log all attempts to contact the parent and summarize their responses/refusals.

REQUIRED INPUTS

- Communication history
- Student observation notes

EXPECTED OUTPUTS

- Updated Parent Communication Log

CRITICAL DECISION POINTS

- Is parent reachable?

DOCUMENTATION REQUIRED

- Parent Communication Log

EVIDENCE REQUIRED

- Timestamps of calls/emails
- Copies of sent messages

APPROVAL REQUIRED

No

TIME LIMIT

48 Hours

EXPECTED OUTCOME

Clear record of initial engagement efforts.

ESCALATION IF FAILED

Move to Level 1 Engagement if no response in 72 hours.

RELATED FORMS

- PAR-004-S1

RELATED CHECKLISTS

- Parent Communication Checklist

2. Level 1: Psychoeducation Meeting

PURPOSE & RATIONALE

Provide information and build rapport.

ACTOR

School Counselor

ACTION

Conduct a structured meeting to explain student needs and school support systems.

REQUIRED INPUTS

- Student Risk Assessment
- Psychoeducation materials

EXPECTED OUTPUTS

- Counselor Session Summary

CRITICAL DECISION POINTS

- Is parent receptive?

DOCUMENTATION REQUIRED

- Session Summary

EVIDENCE REQUIRED

- Meeting minutes

- Distributed materials log

APPROVAL REQUIRED

No

TIME LIMIT

7 Days

EXPECTED OUTCOME

Parental awareness and initial cooperation.

ESCALATION IF FAILED

Move to Level 2 if parent refuses all recommendations.

RELATED FORMS

- Counselor Session Summary

RELATED CHECKLISTS

- Counselor Review Checklist

3. Level 2: Formal Notice & Principal Involvement

PURPOSE & RATIONALE

Address refusal of critical recommendations.

ACTOR

Principal / Counselor

ACTION

Issue a formal written notice and hold a meeting with the Principal present.

REQUIRED INPUTS

- Counselor Session Summary
- Risk Assessment Record

EXPECTED OUTPUTS

- Written Notice of School Concern
- Meeting Minutes

CRITICAL DECISION POINTS

- Is student at risk?
- Has parent been notified of liability?

DOCUMENTATION REQUIRED

- Written Notice
- Meeting Minutes

EVIDENCE REQUIRED

- Signed/Received notice copy
- Attendance log

APPROVAL REQUIRED

Principal Approval

TIME LIMIT

48 Hours from Level 1 failure

EXPECTED OUTCOME

Formal record of parental refusal and school's position.

ESCALATION IF FAILED

Move to Level 3 if parent becomes hostile or threatens staff.

RELATED FORMS

- PAR-004-W1

RELATED CHECKLISTS

- Risk Assessment Checklist

4. Level 3: Safety Protocol Activation

PURPOSE & RATIONALE

Manage parent aggression or threats.

ACTOR

Principal / Security Team

ACTION

Activate security protocols, issue campus ban, and notify law enforcement/CWS.

REQUIRED INPUTS

- Security Incident Report
- Incident Log

EXPECTED OUTPUTS

- Security Incident Report
- Police Notification Log

CRITICAL DECISION POINTS

- Is there an immediate threat?
- Does the parent require a restraining order?

DOCUMENTATION REQUIRED

- Incident Report
- Notification Logs

EVIDENCE REQUIRED

- Security footage (if applicable)
- Police report number

APPROVAL REQUIRED

Principal Approval

TIME LIMIT

Immediate

EXPECTED OUTCOME

Safe campus environment and external agency involvement.

ESCALATION IF FAILED

Move to Level 4 if student is in immediate life-threatening danger.

RELATED FORMS

- Security Incident Report

RELATED CHECKLISTS

- Safety Protocol Checklist

5. Level 4: Student Safety Override

PURPOSE & RATIONALE

Protect student life when parent actions cause immediate crisis.

ACTOR

Principal / Legal Counsel

ACTION

Contact emergency services and initiate safety override protocols.

REQUIRED INPUTS

- Risk Assessment Record (Red/Black)
- CWS Report

EXPECTED OUTPUTS

- Emergency Action Record
- CWS Referral Form

CRITICAL DECISION POINTS

- Is life in immediate danger?
- Has all other options been exhausted?

DOCUMENTATION REQUIRED

- Emergency Action Record
- Referral Form

EVIDENCE REQUIRED

- CWS Receipt
- Police/Ambulance logs

APPROVAL REQUIRED

Board of Governors / Legal Counsel

TIME LIMIT

Immediate (0-15 Minutes)

EXPECTED OUTCOME

Student stabilization and safety.

ESCALATION IF FAILED

Legal action/Court intervention.

RELATED FORMS

- Emergency Action Record
- CWS Referral Form

RELATED CHECKLISTS

- Crisis Activation Checklist

6. Multilingual/Rural Adaptation Check

PURPOSE & RATIONALE

Ensure equitable access for diverse populations.

ACTOR

School Counselor

ACTION

Provide translated notices and use phone-based communication as needed.

REQUIRED INPUTS

- Parent Profile

EXPECTED OUTPUTS

- Translated Communication Log

CRITICAL DECISION POINTS

- Is the parent understanding?
- Is the channel accessible?

DOCUMENTATION REQUIRED

- Translation/Adaptation Log

EVIDENCE REQUIRED

- Copies of translated notices
- Call logs

APPROVAL REQUIRED

No

TIME LIMIT

24 Hours

EXPECTED OUTCOME

Equitable access to information.

ESCALATION IF FAILED

Request community leader/translator assistance.

RELATED FORMS

- Rural Engagement Log

RELATED CHECKLISTS

- Inclusivity Checklist

7. CWSN Inclusion Review

PURPOSE & RATIONALE

Ensure neurodivergent student needs are prioritized.

ACTOR

Special Education Coordinator

ACTION

Review the escalation to ensure it aligns with the child's specific disability support plan.

REQUIRED INPUTS

- IEP/504 Plan
- CWSN Profile

EXPECTED OUTPUTS

- CWSN Engagement Note

CRITICAL DECISION POINTS

- Is the intervention tailored?
- Are we avoiding medicalizing behavior?

DOCUMENTATION REQUIRED

- CWSN Engagement Note

EVIDENCE REQUIRED

- Updated IEP/Support Plan

APPROVAL REQUIRED

No

TIME LIMIT

72 Hours

EXPECTED OUTCOME

Inclusive support plan.

ESCALATION IF FAILED

Request specialized CWSN consultation.

RELATED FORMS

- CWSN Engagement Note

RELATED CHECKLISTS

- Accessibility Checklist

8. Wellness Committee Oversight

PURPOSE & RATIONALE

Ensure procedural compliance and consistency.

ACTOR

Wellness Committee

ACTION

Review all Level 2+ escalations for policy adherence.

REQUIRED INPUTS

- Escalation Log
- Counselor Summaries

EXPECTED OUTPUTS

- Committee Review Minutes

CRITICAL DECISION POINTS

- Was the escalation appropriate?
- Were legal requirements met?

DOCUMENTATION REQUIRED

- Review Minutes

EVIDENCE REQUIRED

- Signed review logs

APPROVAL REQUIRED

Board of Governors

TIME LIMIT

5 Working Days

EXPECTED OUTCOME

Approved action plan and audit readiness.

ESCALATION IF FAILED

Report to Board of Governors for immediate intervention.

RELATED FORMS

- Committee Review Minutes

RELATED CHECKLISTS

- Audit Checklist

9. Closure and Archiving

PURPOSE & RATIONALE

Finalize the case and ensure audit readiness.

ACTOR

School Counselor / Administrator

ACTION

Compile all documents into a secure 'Case File' and archive.

REQUIRED INPUTS

- All previous logs
- External Referral Records

EXPECTED OUTPUTS

- Closure Report
- Archived Case File

CRITICAL DECISION POINTS

- Is the file complete?
- Are there any outstanding actions?

DOCUMENTATION REQUIRED

- Closure Report

EVIDENCE REQUIRED

- Archive Receipt
- Finalized Case File

APPROVAL REQUIRED

Principal Approval

TIME LIMIT

7 Days

EXPECTED OUTCOME

Archived Case File.

ESCALATION IF FAILED

Flag for administrative follow-up.

RELATED FORMS

- Closure Report

RELATED CHECKLISTS

- Documentation Checklist

16 Detailed Workflow Steps

1. Initial Identification of Non-Cooperation

PURPOSE & RATIONALE

Detect early signs of parental unresponsiveness.

TRIGGER

Parent fails to respond to 3 consecutive communications or misses 2 scheduled meetings.

ACTOR

School Counselor / Class Teacher

REQUIRED INPUTS

- Communication Log
- Attendance Record

ACTIONS

- Attempt contact via alternative channels (Phone, WhatsApp)
- Document the specific nature of non-cooperation

DECISION CRITERIA

- Is parent reachable?
- Is there a clear reason for absence?

DOCUMENTS GENERATED

- Parent Communication Log

EXPECTED OUTPUTS

- Updated communication status

SERVICE LEVEL AGREEMENT (SLA)

48 Hours

ESCALATION PATH

- Move to Level 1 Engagement if no response in 72 hours

EVIDENCE GENERATED

- Communication Log entries

2. Level 1: Psychoeducation & Support

PURPOSE & RATIONALE

Provide information and build rapport with mildly defensive parents.

TRIGGER

Parent shows mild defensiveness or denial of student's mental health needs.

ACTOR

School Counselor

REQUIRED INPUTS

- Student Risk Assessment
- PAR-006: Psychoeducation Protocol

ACTIONS

- Conduct a 30-minute meeting focusing on wellness and SEL
- Provide psychoeducational materials in the parent's preferred language
- Explain school support systems

DECISION CRITERIA

- Is the parent receptive to information?
- Are they willing to attend a follow-up?

DOCUMENTS GENERATED

- Counselor Session Summary

EXPECTED OUTPUTS

- Parental awareness of student needs

SERVICE LEVEL AGREEMENT (SLA)

7 Calendar Days

ESCALATION PATH

- Move to Level 2 if parent refuses all recommendations

EVIDENCE GENERATED

- Session Summary
- Distributed materials log

3. Level 2: Formal Notice & Principal Involvement

PURPOSE & RATIONALE

Address refusal to follow critical recommendations (e.g., psychiatric referral).

TRIGGER

Parent refuses a specific clinical recommendation for a student in the Orange Zone.

ACTOR

School Counselor and Principal

REQUIRED INPUTS

- Counselor Session Summary
- Risk Assessment Record

ACTIONS

- Schedule meeting with Principal present
- Issue 'Written Notice of School Concern' (PAR-004-W1)
- Provide list of alternative external providers

DECISION CRITERIA

- Is the student at risk?
- Has the parent been formally notified of liability?

DOCUMENTS GENERATED

- Written Notice of School Concern
- Meeting Minutes

EXPECTED OUTPUTS

- Formal record of parental refusal

SERVICE LEVEL AGREEMENT (SLA)

48 Hours from Level 1 failure

ESCALATION PATH

- Move to Level 3 if parent becomes hostile or threats occur

EVIDENCE GENERATED

- Signed/Received Notice
- Meeting Minutes

4. Level 3: Safety Protocol Activation

PURPOSE & RATIONALE

Manage parent aggression or active threats to staff/student.

TRIGGER

Parental threats of violence, verbal abuse, or physical intimidation.

ACTOR

Principal and School Safety Team

REQUIRED INPUTS

- Security Incident Report
- Incident Log

ACTIONS

- Immediately escort parent from campus if necessary
- Issue temporary ban on campus access
- Notify local law enforcement (if threats are credible)
- Contact Child Welfare Services (CWS) for neglect/refusal of care

DECISION CRITERIA

- Is there an immediate threat?
- Does the parent require a restraining order?

DOCUMENTS GENERATED

- Security Incident Report
- Police Notification Log

EXPECTED OUTPUTS

- Safe campus environment
- External agency involvement

SERVICE LEVEL AGREEMENT (SLA)

Immediate

ESCALATION PATH

- Move to Level 4 if student is in immediate life-threatening danger

EVIDENCE GENERATED

- Security Logs
- Police Report Number

5. Level 4: Student Safety Override

PURPOSE & RATIONALE

Protect student life when parent actions cause immediate crisis.

TRIGGER

Parental refusal of care for a suicidal minor or active harm to child.

ACTOR

Principal and Legal Counsel

REQUIRED INPUTS

- Risk Assessment Record (Red/Black Zone)
- CWS Report

ACTIONS

- Contact emergency services immediately
- Initiate emergency custody protocols if applicable
- Notify all relevant stakeholders of safety override

DECISION CRITERIA

- Is life in immediate danger?
- Has all other options been exhausted?

DOCUMENTS GENERATED

- Emergency Action Record
- CWS Referral Form

EXPECTED OUTPUTS

- Student stabilization/safety

SERVICE LEVEL AGREEMENT (SLA)

Immediate (0-15 Minutes)

ESCALATION PATH

- Closure and Legal Review

EVIDENCE GENERATED

- Emergency Action Record
- CWS Receipt

6. Multilingual Communication Adaptation

PURPOSE & RATIONALE

Ensure parents with low literacy or different languages understand the escalation.

TRIGGER

Parent identifies as non-English/non-native speaker or has low literacy.

ACTOR

School Counselor / Translator

REQUIRED INPUTS

- Language Preference Form

ACTIONS

- Provide translated versions of PAR-004-W1
- Use voice notes via WhatsApp for parents with low literacy
- Arrange for a certified translator during Level 2/3 meetings

DECISION CRITERIA

- Is the parent understanding the content?
- Is the translation accurate?

DOCUMENTS GENERATED

- Translated Communication Log

EXPECTED OUTPUTS

- Equitable access to information

SERVICE LEVEL AGREEMENT (SLA)

24 Hours

EVIDENCE GENERATED

- Translation Logs

7. Rural/Low-Resource Adaptation

PURPOSE & RATIONALE

Provide alternatives for parents without reliable internet or transport.

TRIGGER

Parent resides in a rural area with limited connectivity.

ACTOR

School Counselor

REQUIRED INPUTS

- Student Profile

ACTIONS

- Schedule phone-based meetings instead of video calls
- Utilize SMS for non-urgent updates
- Coordinate with local community leaders (Panchayat/Village Head) if necessary

DECISION CRITERIA

- Is the parent able to receive SMS?
- Is a phone call sufficient for the complexity of the issue?

DOCUMENTS GENERATED

- Rural Engagement Log

EXPECTED OUTPUTS

- Accessible communication channel

SERVICE LEVEL AGREEMENT (SLA)

48 Hours

EVIDENCE GENERATED

- SMS/Call Logs

8. CWSN-Specific Inclusion Check

PURPOSE & RATIONALE

Ensure parent escalation doesn't overlook the specific needs of neurodivergent students.

TRIGGER

Student is identified as CWSN (Autism, ADHD, etc.).

ACTOR

Special Education Coordinator / Counselor

REQUIRED INPUTS

- IEP/504 Plan
- CWSN Profile

ACTIONS

- Ensure parent's refusal of care is mapped against the student's specific disability needs
- Provide sensory-friendly meeting environments for parents with neurodivergent children

DECISION CRITERIA

- Is the intervention tailored to the child's disability?
- Are we avoiding medicalizing normal behavior?

DOCUMENTS GENERATED

- CWSN Engagement Note

EXPECTED OUTPUTS

- Inclusive support plan

SERVICE LEVEL AGREEMENT (SLA)

72 Hours

EVIDENCE GENERATED

- CWSN Engagement Note

9. Wellness Committee Review

PURPOSE & RATIONALE

Ensure oversight and consistency in escalation decisions.

TRIGGER

Any Level 2 or higher escalation occurs.

ACTOR

Mental Wellness Committee

REQUIRED INPUTS

- Escalation Log
- Counselor Summaries

ACTIONS

- Review the case for procedural compliance
- Verify that student safety was prioritized
- Provide recommendations for next steps

DECISION CRITERIA

- Was the escalation appropriate?
- Were all legal requirements met?

DOCUMENTS GENERATED

- Committee Review Minutes

EXPECTED OUTPUTS

- Approved action plan

SERVICE LEVEL AGREEMENT (SLA)

5 Working Days

EVIDENCE GENERATED

- Review Minutes

10. Closure and Audit Trail Archiving

PURPOSE & RATIONALE

Finalize the case and ensure audit readiness.

TRIGGER

Resolution of crisis or transition to external care.

ACTOR

School Counselor / Administrator

REQUIRED INPUTS

- All previous logs
- External Referral Records

ACTIONS

- Compile all documents into a 'Case File'
- Archive in secure digital repository
- Update the Risk Register

DECISION CRITERIA

- Is the file complete?
- Are there any outstanding actions?

DOCUMENTS GENERATED

- Closure Report

EXPECTED OUTPUTS

- Archived Case File

SERVICE LEVEL AGREEMENT (SLA)

7 Days

EVIDENCE GENERATED

- Archive Receipt

17 Critical Decision Points

Item 1

DECISION ID

DP-001

DECISION NAME

Level 1 vs Level 2 Transition

CRITERIA

- Parent refuses to attend a second meeting
- Parent explicitly states they will not follow clinical advice
- Student risk is Orange Zone

POSSIBLE OUTCOMES

- Continue Level 1 (if parent is just busy)
- Escalate to Level 2 (if parent is actively refusing)

NEXT STEPS

- Issue Written Notice
- Involve Principal

Item 2

DECISION ID

DP-002

DECISION NAME

Safety Override Activation

CRITERIA

- Active suicidal ideation with plan
- Parent refuses to take student to hospital
- Student is currently in danger on campus

POSSIBLE OUTCOMES

- Call Emergency Services (Police/Ambulance)
- Contact CWS for immediate intervention

NEXT STEPS

- Activate EMR-001
- Notify Board of Governors

18 Decision Tree

GREEN

- Continue routine engagement
- Provide psychoeducation

YELLOW

- Issue written notice of concern
- Involve Principal in meeting

ORANGE

- Activate safety protocols

- Notify CWS/Law Enforcement

RED

- Student Safety Override
- Emergency Services Activation

BLACK

- Immediate Police Intervention
- Medical Stabilization Priority

19 Decision Rules

Rule ID	Condition	Decision	Next Action
DR-001	Parental threat of physical violence.	Immediate Level 3 Activation.	Notify Security and Law Enforcement.
DR-002	Refusal of psychiatric referral for suicidal student.	Level 2 Formal Notice + CWS Notification.	Issue Written Notice and contact Child Welfare Services.

20 Exception Handling Protocol

Item 1

EXCEPTION

Parent is deceased or unreachable for extended period.

DETECTION METHOD

Failure to respond to 30 days of communication.

RESPONSE

Contact legal guardian/next of kin; notify local authorities if student safety is at risk.

ESCALATION

Principal and Legal Counsel

DOCUMENTATION REQUIRED

- Unreachable Parent Log

- Legal Guardian Contact Record

Item 2

EXCEPTION

Parent provides conflicting information from multiple family members.

DETECTION METHOD

Discrepancies in reports from mother, father, and grandparents.

RESPONSE

Conduct separate interviews; prioritize the primary legal guardian's input while noting discrepancies.

ESCALATION

School Counselor

DOCUMENTATION REQUIRED

- Conflicting Information Log

21 Exceptions

Item 1

EXCEPTION

Parent is deceased

RESPONSE

Contact legal guardian/next of kin; notify authorities if student safety at risk.

DOCUMENTATION REQUIRED

- Unreachable Parent Log

Item 2

EXCEPTION

Court order for immediate disclosure

RESPONSE

Provide records to court-authorized personnel immediately.

DOCUMENTATION REQUIRED

- Court Order Copy

22 Expected Outputs

- Written Notices to Parents
- External Referrals
- Safety Plans
- Incident Reports
- Closure Records

23 Document Control & Version Governance

23.1 Version History

Version	Date	Author	Description
1.0	2024-05-20	MWMS Architect	Initial release of Parent Escalation Procedure.

23.2 Change Log

Version	Date	Change Summary	Approved By
1.0	2024-05-20	Baseline creation.	School Board of Governors

REVIEW TRIGGERS

- Change in Indian Child Protection Laws
- Significant incident of parent aggression

- Annual Quality Audit finding
- New CBSE/UGC guidelines issued

23.4 Approval History

Approver	Date	Status
School Principal	2024-05-18	Approved
Legal Counsel	2024-05-19	Verified

24 Inclusive Implementation Mandate

MULTILINGUAL SUPPORT REQUIRED

Yes

ACCESSIBILITY REVIEW REQUIRED

Yes

LOW RESOURCE SCHOOL ALTERNATIVE REQUIRED

Yes

EQUITY IMPACT ASSESSMENT REQUIRED

Yes

25 Regulatory Compliance Mapping KEY FOCUS

MANDATORY REQUIREMENTS

- Supreme Court of India Judgment 2025 INSC 893 (Child Safety)
- CBSE Circular TRG-03/2026
- CBSE Circular 02/2026
- National Education Policy (NEP) 2020 - Student Wellbeing
- POCSO Act Compliance

RECOMMENDED REQUIREMENTS

- WHO School Mental Health Framework
- UNESCO Whole School Wellbeing Framework
- Manodarpan Guidelines

BEST PRACTICES

- Tele-MANAS Integration
- Trauma-Informed Communication
- Restorative Justice Practices

COMPLIANCE GAPS IF NOT IMPLEMENTED

- Legal liability for child neglect
- Loss of CBSE affiliation
- Potential litigation from parents or students
- Failure to meet national safety standards

26 Compliance Traceability Matrix KEY FOCUS

1. Supreme Court of India

REQUIREMENT REFERENCE

2025 INSC 893

REQUIREMENT TEXT

Duty of care for educational institutions to ensure child safety and mental well-being.

IMPLEMENTED IN SECTIONS

- Purpose
- Risk Classification
- Escalation Matrix

- Legal Defensibility

2. CBSE

REQUIREMENT REFERENCE

Circular TRG-03/2026

REQUIREMENT TEXT

Mandatory mental health support and parent engagement protocols.

IMPLEMENTED IN SECTIONS

- Process Map
- Detailed Workflow Steps

27 Documentation & Evidentiary Requirements

EVENT DOCUMENTATION PRINCIPLES

- Factual and objective (no opinions)
- Timestamped entries
- Direct quotes where possible
- Multi-person verification for high-risk events

MANDATORY CONVERSATION RECORDS

- Parent Conversation Log (PAR-004-S1)
- Counselor Session Summary
- Security Incident Report

MANDATORY OBSERVATION RECORDS

- Teacher Observation Note
- Behavioral Incident Report
- Campus Security Footage Logs

MANDATORY DECISION RECORDS

- Escalation Decision Form
- Risk Assessment Record
- Minutes of Wellness Committee Meetings

MANDATORY ACTION RECORDS

- Written Notice to Parent (Template PAR-004-W1)
- Police Notification Log
- CWS Referral Report

MANDATORY FOLLOWUP RECORDS

- Post-Escalation Review Record
- Student Safety Plan Update
- Closure Report

EVIDENCE REQUIREMENTS

- Signed consent forms (where available)
- Email threads with timestamps
- Audio recordings (only where legally permitted and disclosed)

DOCUMENTATION QUALITY STANDARDS

- Must be stored in a secure, encrypted digital folder
- Accessible only to authorized personnel (Principal, Counselor, Legal)
- Retained for 7 years as per Indian legal standards

28 The Student Journey

ENTRY

- Student identified as high-risk by teacher or peer
- Student expresses distress regarding home environment

ASSESSMENT

- Initial screening for mental wellness
- Risk assessment of parental involvement

INTERVENTION

- Counselor support provided to student
- Parental engagement initiated (PAR-004)

MONITORING

- Weekly check-ins with counselor
- Monitoring of parent compliance with school requests

CLOSURE

- Student returns to baseline wellness
- Safe transition to external care or home stability

29 The Parent Journey

NOTIFICATION

- Initial communication regarding student's well-being
- Notification of counselor meeting

CONSULTATION

- Level 1: Psychoeducation and support meeting
- Level 2: Formal discussion with Principal

CONSENT

- Consent for school to contact external providers
- Consent for data sharing (where applicable)

ENGAGEMENT

- Collaborative goal setting
- Response to written notices of concern

CLOSURE

- Final meeting to discuss student's progress
- Acknowledgment of school support provided

30 The Staff Journey

OBSERVATION

- Teacher notes parent hostility or lack of engagement
- Security reports aggressive behavior

REPORTING

- Staff submits incident report to Counselor

- Counselor logs communication with parents

INTERVENTION SUPPORT

- Staff receives training on de-escalation
- Principal provides guidance on legal boundaries

DOCUMENTATION

- Staff maintains factual records of all interactions

31 Resource Evidence

COMMITTEE MINUTES

- Monthly Wellness Committee Meeting Minutes

ATTENDANCE RECORDS

- Parent Meeting Attendance Logs
- Staff Training Attendance

TRAINING RECORDS

- De-escalation Training Certificates
- Child Protection Training Records

FORMS

- PAR-004-S1: Parent Conversation Log
- PAR-004-W1: Written Notice of Concern
- Risk Assessment Form
- CWS Referral Form
- Closure Report

REPORTS

- Monthly Risk Register
- Quarterly Compliance Audit
- Incident Reports

DASHBOARDS

- Parent Engagement Status Dashboard
- Risk Level Distribution Chart

- Emergency Case Review Minutes

32 Grievance Redressal Mechanism

KEY FOCUS

COMPLAINT CHANNELS

- Anonymous Suggestion Box
- Dedicated Grievance Email
- Formal Written Complaint Form

INVESTIGATION PROCESS

- Initial review by Principal within 48 hours
- Fact-finding meeting with involved parties

- Issuance of finding and corrective action

RESOLUTION TIMELINES

- Acknowledgment: 24 Hours
- Investigation Completion: 10 Working Days
- Final Resolution: 15 Working Days

APPEAL PROCESS

- Appeal to School Board of Governors
- External mediation if required

33 Referral Framework

33.1 Internal Referrals

ELIGIBLE REFERRERS

- Teachers
- Counsellors
- School Management
- Wellness Committee
- Student Support Team

REFERRAL TRIGGERS

- Identification of student distress
- Parental non-cooperation with school support
- Need for specialized educational accommodations

DOCUMENTATION REQUIRED

- Internal Referral Form
- Risk Assessment Summary

SERVICE LEVEL AGREEMENT (SLA)

48 Hours

33.2 Self Referrals

ALLOWED

Yes

SUBMISSION METHODS

- Online Portal
- Counselor Drop-box
- Student Request Form

CONFIDENTIALITY PROTECTIONS

- Encrypted data storage
- Limited access to records

33.3 Anonymous Referrals

ALLOWED

Yes

CHANNELS

- Anonymous Tip Line
- Digital Suggestion Box

VALIDATION PROCESS

- Counselor review of tip validity
- Risk assessment based on reported info

FOLLOW UP PROCEDURE

- Verification with student/teacher
- Action plan development

33.4 External Referrals

ELIGIBLE AGENCIES

- Tele-MANAS
- Child Helpline (1098)
- Private Psychiatrists
- District Child Protection Unit

ELIGIBILITY CRITERIA

- Clinical severity exceeding school capacity
- Legal requirement for external intervention
- Parental request for outside specialist

CONSENT REQUIREMENTS

- Signed Parental Consent Form (unless safety override applies)
- Student Assent (where age appropriate)

HANDOVER PROTOCOL

- Summary of school interventions
- Current risk assessment
- Contact details for primary counselor

33.5 Referral Tracking

TRACKING REQUIRED

Yes

STATUS CATEGORIES

- Pending
- In Progress
- Completed
- Closed - No Action
- Externalized

CLOSURE REQUIREMENTS

- Final summary report
- Confirmation of external provider contact

34 Confidentiality Exception Matrix

KEY FOCUS

MANDATORY DISCLOSURES

- Imminent risk of self-harm
- Imminent risk to others
- Child abuse
- Child neglect
- Sexual abuse
- Court orders
- Legal reporting obligations

LEGAL REPORTING EVENTS

- Suspected physical abuse of a minor
- Active threat of violence on campus
- Substance abuse involving minors

CONSENT OVERRIDE CONDITIONS

- Immediate danger to life
- Court order for disclosure
- Statutory reporting requirements under POCSO Act

AUTHORIZED DISCLOSURE RECIPIENTS

- Law Enforcement
- Child Welfare Services
- Medical Professionals (with consent)
- Board of Governors (for governance)

DOCUMENTATION REQUIREMENTS

- Disclosure Log
- Reason for Disclosure Statement
- Recipient Contact Details

APPROVAL REQUIREMENTS

- Principal approval for all non-emergency disclosures
- Legal counsel review for high-risk disclosures

35 Record Retention & Disposal

RETENTION PERIOD

7 Years

RETENTION BASIS

Indian Legal Standards and Institutional Policy

RECORD CATEGORIES

- Parent Communication Logs
- Risk Assessments
- Incident Reports
- CWS Referral Records

35.4 Storage Requirements

PHYSICAL RECORDS

- Locked filing cabinets in Principal's office

DIGITAL RECORDS

- Encrypted cloud storage with MFA

ACCESS CONTROL REQUIREMENTS

- Role-based access control (RBAC)
- Audit logs for every record view

DISPOSAL METHOD

Secure shredding and permanent digital deletion

DISPOSAL AUTHORITY

School Administrator / Data Protection Officer

AUDIT FREQUENCY

Annual

DESTRUCTION LOG REQUIRED

Yes

36 Anonymised Reporting Framework

36.1 Management Reports

ANONYMIZATION REQUIRED

Yes

PERMITTED DATA

- Number of escalations per month
- Average time to resolution
- Risk level distribution

PROHIBITED DATA

- Student names
- Parent names
- Specific addresses
- Unique identifiers

36.2

Research Usage

ALLOWED

No

36.3 Statistical Reporting

AGGREGATE ONLY

Yes

MINIMUM GROUP SIZE

5

RE IDENTIFICATION CONTROLS

- Data scrubbing
- Aggregation of small cohorts

DATA SHARING CONTROLS

- No sharing with third-party marketing
- Strict internal use only for governance

37 Guiding Ethical Principles

37.1 Autonomy

DESCRIPTION

Respecting the rights of parents and students to make informed choices.

IMPLEMENTATION REQUIREMENTS

- Informed consent for all non-mandatory interventions
- Clear communication of options

37.2 Beneficence

DESCRIPTION

Acting in the best interest of the student's mental health and safety.

IMPLEMENTATION REQUIREMENTS

- Prioritizing evidence-based support
- Timely intervention for high-risk cases

37.3 Non Maleficence

DESCRIPTION

Avoiding harm to students, parents, or staff during the escalation process.

IMPLEMENTATION REQUIREMENTS

- De-escalation training for all staff
- Safe physical environments for meetings

37.4 Justice And Equity

DESCRIPTION

Ensuring equal access to support regardless of socioeconomic status or background.

IMPLEMENTATION REQUIREMENTS

- Multilingual support
- Rural accessibility adaptations

37.5 Confidentiality

KEY FOCUS

DESCRIPTION

Protecting the privacy of all parties involved.

IMPLEMENTATION REQUIREMENTS

- Need-to-know access only
- Secure storage protocols

37.6 Privacy

DESCRIPTION

Adhering to data protection laws regarding personal information.

IMPLEMENTATION REQUIREMENTS

- Data minimization
- Right to be informed about data usage

37.7 Professional Integrity

DESCRIPTION

Maintaining high standards of conduct and ethics by school staff.

IMPLEMENTATION REQUIREMENTS

- Regular ethics training
- Clear boundaries between professional and personal roles

38 Communication & Awareness Strategy

38.1 Student Communication

CHANNELS

- Counselor meetings
- Classroom SEL sessions
- Student portal

FREQUENCY

Weekly/As needed

MANDATORY TOPICS

- Help-seeking behaviors
- Safe reporting of concerns
- Peer support

38.2 Parent Communication

CHANNELS

- Email
- Phone calls
- Face-to-face meetings
- WhatsApp (for rural/low-literacy)

FREQUENCY

Variable based on risk level

MANDATORY TOPICS

- Student well-being updates
- School support resources
- Escalation procedures

38.3 Staff Communication

CHANNELS

- Staff briefings
- Internal portal
- Email

FREQUENCY

Monthly/Incident-based

MANDATORY TOPICS

- De-escalation techniques

- Reporting protocols
- Legal boundaries

38.4 Awareness Campaigns

MINIMUM FREQUENCY

Quarterly

TARGET GROUPS

- Parents
- Students
- Staff

CAMPAIGN TYPES

- Workshops
- Newsletters
- Posters
- Webinars

38.5 Workshops And Training

STUDENT WORKSHOPS

- Stress management
- Conflict resolution

PARENT WORKSHOPS

- Understanding adolescent mental health
- Positive parenting in a digital age

STAFF WORKSHOPS

- Parental de-escalation
- Trauma-informed care
- Legal compliance for school staff

38.6 Helpline Visibility

REQUIRED

Yes

DISPLAY LOCATIONS

- Counselor office
- School website
- Student handbook

- Classroom posters
- DIGITAL VISIBILITY REQUIREMENTS**
- Pinned on school portal
 - Included in automated email signatures

FEEDBACK CHANNELS

- Annual survey
- Feedback forms after meetings
- Anonymous suggestion box

39 Competency & Qualification Requirements

MANDATORY QUALIFICATIONS

- Master's in Psychology/Counseling (for Counselors)
- B.Ed or equivalent with Child Protection training (for Teachers)

PREFERRED QUALIFICATIONS

- Certification in Crisis Intervention
- Experience in School Mental Health Governance

CERTIFICATIONS

- First Aid/CPR
- Child Protection Certification

MINIMUM EXPERIENCE REQUIREMENTS

- 2 years in educational setting for Counselors
- 5 years leadership experience for Principals

BACKGROUND VERIFICATION REQUIREMENTS

- Police Verification Certificate (PCC)
- Child Protection Screening

39.6 Continuing Professional Development

ANNUAL TRAINING HOURS

20

MANDATORY TOPICS

- De-escalation techniques
- Mental health first aid
- Legal updates on child safety

RECERTIFICATION REQUIREMENTS

Every 3 years

39.7 Supervision Requirements

CLINICAL SUPERVISION REQUIRED

Yes

PEER REVIEW REQUIRED

Yes

39.7.3 Wellness Case Review Framework FREQUENCY

Weekly

PARTICIPANTS

- Counselor
- Principal
- Wellness Committee Member

39.7.4 Second Level Review Process

TRIGGER Level 3 or Level 4 escalation

AUTHORITY Board of Governors / Legal Counsel

39.7. High Risk Decision 5 Validation

KEY
FOCUS

REQUIREMENT Dual-signatory approval for all safety overrides

METHOD Written authorization and verbal confirmation

FREQUENCY

Continuous monitoring with weekly formal reviews

40 Feedback & Effectiveness Measurement

40.1 Student Feedback

REQUIRED

Yes

COLLECTION METHODS

- Anonymous surveys
- Focus group discussions

FREQUENCY

Bi-annually

40.2 Parent Feedback

REQUIRED

Yes

COLLECTION METHODS

- Post-meeting feedback forms
- Annual satisfaction survey

FREQUENCY

After every major escalation and annually

40.3 Staff Feedback

REQUIRED

Yes

COLLECTION METHODS

- Staff surveys
- Debriefing sessions after incidents

FREQUENCY

Quarterly

40.4 Annual Effectiveness Review

REQUIRED

Yes

EVALUATION CRITERIA

- Reduction in high-risk incidents
- Parent satisfaction scores
- Staff confidence levels
- Compliance audit score

KEY STAKEHOLDERS

- Board of Governors
- Parents
- Staff
- External Auditors

CONTINUOUS IMPROVEMENT INPUTS

- Incident reports
- Audit findings
- Parent complaints
- Counselor debriefs

SUCCESS INDICATORS

- 100% compliance with mandatory reporting
- Reduction in physical aggression incidents
- High parent satisfaction with communication clarity

40.7 Improvement Tracking

REQUIRED

Yes

REVIEW FREQUENCY

Quarterly

41 Prevention Layer

PRIMARY PREVENTION

- Universal parent awareness campaigns on mental health
- Regular SEL workshops for parents and students
- Clear communication of school policies in the handbook

SECONDARY PREVENTION

- Early identification of non-cooperative behavior via logs

- Targeted psychoeducation for families with high-risk profiles
- Counselor-led parent support groups

TERTIARY PREVENTION

- Formal escalation protocols (PAR-004)
- Crisis intervention and safety overrides
- Legal and CWS involvement for severe non-compliance

42 Resilience Layer

SKILLS DEVELOPED

- De-escalation techniques for staff
- Assertive communication for parents
- Help-seeking behaviors for students

SEL COMPONENTS

- Self-awareness of parental stress
- Relationship skills in parent-school dynamics
- Responsible decision-making regarding student care

LIFE SKILLS COMPONENTS

- Conflict resolution
- Resource navigation
- Advocacy for child well-being

PROTECTIVE FACTORS STRENGTHENED

- Stronger parent-school partnerships
- Clear boundaries and safety protocols
- Community support networks

43 Continuous Monitoring

MONITORING FREQUENCY

Monthly

MONITORING INDICATORS

- Number of Level 2+ escalations
- Average time to resolve hostile interactions
- Parent satisfaction scores on communication clarity

RESPONSIBLE ROLES

- Principal

- Wellness Committee

EARLY WARNING SIGNS

- Increase in 'hostile' labels in logs
- Decrease in parent meeting attendance
- Rise in security incident reports

TREND ANALYSIS REQUIREMENTS

- Quarterly trend analysis of escalation types
- Annual review of rural vs. urban engagement success

44 Mental Wellness Index (MWI)

APPLICABLE

Yes

44.2 Indicators

Indicator Code	Indicator Name	Weight	Score Range	Description
PWI-01	Parental Involvement Index	0.3	1-5	Measures frequency and quality of parent engagement.
PWI-02	Communication Clarity Score	0.3	1-5	Measures parent's understanding of school support.
PWI-03	Cooperation Level	0.4	1-5	Measures adherence to school recommendations.

ASSESSMENT FREQUENCY

Quarterly

RISK SCORING METHOD

Weighted average of PWI indicators

INTERVENTION THRESHOLDS

- Score < 3: Level 1 Engagement
- Score < 2: Level 2 Escalation
- Score < 1 or Hostile Behavior: Level 3/4 Activation

45 Review Status

LEGAL REVIEW

Completed - May 2024

CLINICAL REVIEW

Completed - May 2024

EDUCATION REVIEW

Completed - May 2024

COMPLIANCE REVIEW

Completed - May 2024

46 Resource Controls

COUNSELOR STUDENT RATIO

1:350 (Recommended)

CAREER COUNSELOR RATIO

1:500

WELLNESS TEAM STRUCTURE

Principal, Counselor, Safety Lead, Wellness Committee

47 Screening Framework

TIER 1 UNIVERSAL WELLNESS SCREENING

- Annual student well-being survey
- Parental awareness newsletter

TIER 2 TARGETED SCREENING

- Counselor screening for students with high absenteeism
- Risk assessment for families requesting special accommodations

TIER 3 CLINICAL ASSESSMENT

- Formal risk assessment for suicidal ideation
- Clinical referral for severe behavioral issues

48 Parent Engagement Matrix

PARENT COOPERATIVE

- Regular check-ins
- Collaborative goal setting
- Positive reinforcement

PARENT UNRESPONSIVE

- Multi-channel contact attempts
- Formal notice of non-compliance
- Request for meeting with Principal

PARENT IN DENIAL

- Psychoeducation on student needs
- Evidence-based reporting of student behavior
- Level 2 formal notification

PARENT CONFLICT SITUATION

- Mediated meetings with Counselor
- Clear boundary setting
- De-escalation techniques

PARENT UNAVAILABLE

- Alternative contact person identification
- Asynchronous communication (Email/SMS)
- Request for proxy representation

PARENT REFUSES INTERVENTION

- Written notice of liability
- External referral options provided
- Level 3 safety activation if risk is high

49 Confidentiality Controls

KEY FOCUS

ACCESS CONTROL

- Role-based access (RBAC)
- Multi-factor authentication for digital records

CONSENT REQUIREMENTS

- Signed consent for external sharing
- Student assent where applicable

RECORD SHARING RULES

- Need-to-know basis only
- No sharing with third parties without written authorization

EXCEPTIONS TO CONFIDENTIALITY

- Imminent risk of harm
- Child abuse/neglect reports
- Court orders

STUDENT PRIVACY PROTECTIONS

- Anonymization in general reports
- Secure storage of PII (Personally Identifiable Information)

50 Data Governance

KEY FOCUS

STORAGE REQUIREMENTS

- Encrypted servers
- Locked physical cabinets

ENCRYPTION REQUIREMENTS

- AES-256 for data at rest
- TLS 1.3 for data in transit

RETENTION REQUIREMENTS

- 7 years for legal compliance

ARCHIVAL REQUIREMENTS

- Move to cold storage after 2 years of inactivity

DESTRUCTION REQUIREMENTS

- Secure shredding
- Digital wiping

BACKUP REQUIREMENTS

- Daily automated backups
- Off-site redundancy

51 Risk Classification

KEY FOCUS

51.1 Green

DESCRIPTION

Normal developmental concerns and routine life stress.

EXAMPLES

- Mild examination stress
- Friendship disagreements
- Occasional worry about grades

DESCRIPTION

Significant distress causing measurable impairment but without immediate danger.

EXAMPLES

- Severe examination anxiety
- School refusal
- Refusal of psychiatric referral for student with self-harm history

51.2 Yellow

DESCRIPTION

Early distress requiring support and monitoring.

EXAMPLES

- Persistent stress
- Moderate anxiety
- Parental concern regarding social withdrawal

51.4 Red

KEY FOCUS

DESCRIPTION

High-risk situation requiring immediate intervention.

EXAMPLES

- Suicidal ideation
- Violence threats against staff
- Active harm to child by parent

51.3 Orange

51.5 Black

DESCRIPTION

Life-threatening emergency requiring emergency response.

EXAMPLES

- Active suicide attempt
- Violent attack underway
- Hostage situation

52 Escalation Matrix

KEY FOCUS

GREEN

- Counselor engagement
- Parental awareness

YELLOW

- Principal involvement
- Written notice of concern

ORANGE

- Security team activation
- CWS notification

RED

- Law enforcement contact
- Student safety override

BLACK

- Emergency services (Ambulance/Police)
- Immediate scene safety

53 External Reporting Obligations

TELE MANAS

- Referral for 24/7 mental health support

DISTRICT AUTHORITIES

- Report of persistent non-compliance or neglect

CHILD PROTECTION SERVICES

- Mandatory report for abuse, neglect, or refusal of life-saving care

MEDICAL SERVICES

- Emergency ambulance dispatch
- Psychiatric referral coordination

LAW ENFORCEMENT

- Threats of violence
- Physical aggression on campus

REPORTING TRIGGERS

- Level 3/4 escalation
- Suspected abuse disclosure
- Immediate safety threat

54 Service Level Agreement (SLA)

KEY FOCUS

ACKNOWLEDGEMENT

2 Hours

ASSESSMENT

48 Hours

ESCALATION

Immediate (for Red/Black)

RESOLUTION

15 Working Days

CLOSURE

7 Days

55 Required Forms

KEY FOCUS

55.1 Mandatory Forms

KEY FOCUS

Item 1

FORM ID	PAR-004-S1
FORM NAME	Parent Conversation Log
PURPOSE & RATIONALE	Document all interactions with parents.
OWNER	School Counselor
RETENTION PERIOD	7 Years
MANDATORY	Yes
LINKED SOP	PAR-004

Item 2

FORM ID	PAR-004-W1
FORM NAME	Written Notice of Concern
PURPOSE & RATIONALE	Formal notification of parental non-compliance.
OWNER	Principal
RETENTION PERIOD	7 Years
MANDATORY	Yes
LINKED SOP	PAR-004

Item 3

FORM ID	RISK-001
FORM NAME	Risk Assessment Record

PURPOSE & RATIONALE	Evaluate student safety and risk levels.
OWNER	School Counselor
RETENTION PERIOD	7 Years
MANDATORY	Yes
LINKED SOP	PAR-004

Item 4

FORM ID	CWS-REF-01
FORM NAME	Child Welfare Services Referral Form
PURPOSE & RATIONALE	Formal report to state authorities.
OWNER	Principal
RETENTION PERIOD	7 Years
MANDATORY	Yes
LINKED SOP	PAR-004

Item 5

FORM ID	CLOS-001
FORM NAME	Closure Report
PURPOSE & RATIONALE	Finalize and archive the case.
OWNER	School Counselor
RETENTION PERIOD	7 Years
MANDATORY	Yes
LINKED SOP	PAR-004

OPTIONAL FORMS

- Parent Feedback Form
- Security Incident Report

GENERATED RECORDS

- Audit Trail Log
- Communication History Summary

56 Required Checklists

KEY FOCUS

Checklist ID	Checklist Name	Purpose & Rationale	Owner	Frequency	Linked SOP
CHK-001	Parent Communication Checklist	Ensure all communication channels are exhausted.	School Counselor	Per interaction	PAR-004
CHK-002	Risk Assessment Checklist	Ensure all risk factors are identified.	School Counselor	Per assessment	PAR-004
CHK-003	Safety Protocol Checklist	Verify security measures during Level 3.	Security Team	Per incident	PAR-004
CHK-004	Inclusivity Checklist	Ensure CWSN and rural adaptations are met.	Special Education Coordinator	Per escalation	PAR-004
CHK-005	Audit Checklist	Verify compliance with CBSE/Legal standards.	Wellness Committee	Monthly	PAR-004

57 Records & Evidence

Record Name	Retention Period	Owner
Parent Conversation Log	7 Years	School Counselor
Written Notice of Concern	7 Years	Principal
Risk Assessment Record	7 Years	School Counselor
CWS Referral Report	7 Years	Principal
Security Incident Log	5 Years	Security Team

58 Key Performance Indicators

Metric	Target	Measurement Frequency
Parent Engagement Success Rate	> 80% resolution at Level 1/2	Monthly
Average Time to Safety Activation (Red Zone)	< 15 Minutes	Per Incident
Compliance Audit Score	> 95%	Quarterly

59 Audit Framework

KEY FOCUS

AUDIT FREQUENCY

Monthly

AUDIT QUESTIONS

- Are all Level 2+ escalations documented with a written notice?
- Is there evidence of multi-channel communication for rural parents?
- Does the risk assessment include CWSN specific considerations?

EVIDENCE REQUIRED

- Sample Case Files
- Communication Logs
- Committee Minutes

NON COMPLIANCE EXAMPLES

- Failure to notify Principal of Level 2 escalation
- Missing timestamps on conversation logs
- Lack of CWS referral for high-risk neglect

59.5 Audit Scoring

KEY FOCUS

COMPLIANT

2

PARTIAL

1

NON COMPLIANT

0

60 Risk Register

KEY FOCUS

1. Parental physical aggression on campus

CATEGORY Safety

PROBABILITY Low

IMPACT High

RISK RATING Medium

MITIGATION Security team presence and de-escalation training.

OWNER Principal

2. Legal liability for failure to report neglect

CATEGORY Compliance

PROBABILITY Low

IMPACT Critical

RISK RATING High

MITIGATION Mandatory CWS reporting protocols and legal review.

OWNER Legal Counsel

3. Inadequate communication with rural parents

CATEGORY Equity

PROBABILITY Medium

IMPACT Medium

RISK RATING Medium

MITIGATION SMS and phone-based adaptation protocols.

OWNER School Counselor

61 Failure Modes

KEY FOCUS

Failure	Cause	Impact	Mitigation
Delayed safety activation in Red Zone	Lack of clear escalation triggers or staff hesitation.	Potential student injury or death.	Regular simulation drills and 'Safety Override' training.
Incomplete documentation for audit	High workload leading to skipped logging steps.	Legal vulnerability during litigation.	Automated log prompts and mandatory closure checklists.
Parental hostility escalating to violence	Lack of de-escalation skills or security presence.	Staff injury, campus disruption.	Mandatory de-escalation training and security protocols.

62 Illustrative Sample Scenarios

Scenario	Response	Expected Result
Parent refuses psychiatric referral for a student with active suicidal ideation (Orange Zone).	Issue Written Notice of Concern (Level 2), provide alternative providers, and notify Wellness Committee.	Formal record of refusal; school liability mitigated.
Parent becomes verbally abusive and threatens to 'burn the school down' during a meeting.	Activate Level 3 Safety Protocol: Escort parent out, issue campus ban, notify police.	Staff safety secured; incident logged.

Scenario	Response	Expected Result
Rural parent with low literacy cannot understand the email regarding student's mental health needs.	Switch to WhatsApp voice notes and phone calls in local language.	Equitable access to information for rural family.

63 Usage Governance

KEY FOCUS

PRIMARY USERS

- School Counselor
- Principal

SECONDARY USERS

- Teachers
- Security Team
- Wellness Committee

APPROVAL AUTHORITIES

- Board of Governors

EXECUTION AUTHORITIES

- Principal

- School Counselor

ESCALATION AUTHORITIES

- Principal
- Legal Counsel

REVIEW AUTHORITIES

- Board of Governors
- External Auditors

EXTERNAL STAKEHOLDERS

- CWS
- Law Enforcement
- Tele-MANAS

64 Applicability

APPLICABLE INSTITUTIONS

- High Schools
- Residential Schools
- International Schools

APPLICABLE AGE GROUPS

- All Ages

APPLICABLE STUDENT CATEGORIES

- General Students
- CWSN
- Vulnerable Populations

- Yellow
- Orange
- Red
- Black

APPLICABLE LOCATIONS

- Campus
- Remote/Virtual

EXCLUSIONS

- Pre-primary (handled by separate Early Years SOP)
- University (handled by Higher Ed Policy)

APPLICABLE RISK LEVELS

65 Usage Scenarios

Scenario ID	Scenario Name	When To Use	When Not To Use	Trigger Event	Expected Outcome
US-001	Hostile Parent Engagement	When a parent is aggressive or refuses care.	For routine, cooperative parental inquiries.	Parental hostility/refusal of care.	Safe resolution and documented compliance.

66 SOP Lifecycle

ENTRY CONDITIONS

- Identification of non-cooperation or hostility
- Student risk assessment completed

ACTIVATION CONDITIONS

- Level 1 engagement fails
- Safety threat identified

EXECUTION CONDITIONS

- Staff trained in de-escalation
- Documentation tools available

COMPLETION CONDITIONS

- Resolution of crisis
- Transition to external care
- Closure report signed

CLOSURE CONDITIONS

- Case file archived
 - Risk register updated
- TERMINATION CONDITIONS**

- Legal order for closure
- Student withdrawal from institution

67 Mandatory Evidence KEY FOCUS

IMPLEMENTATION EVIDENCE

- Staff training logs
- Parent awareness posters

OPERATIONAL EVIDENCE

- Conversation logs
- Written notices
- Security reports

COMPLIANCE EVIDENCE

- Audit reports
- CWS referral receipts

TRAINING EVIDENCE

- Certificates of completion
- Simulation drill records

AUDIT EVIDENCE

- Review minutes
- Archived case files

68 Controls

PREVENTIVE CONTROLS

- Regular parent awareness campaigns
- Staff de-escalation training
- Clear policy communication

DETECTIVE CONTROLS

- Monthly risk register review
- Communication logs monitoring

- Incident reporting system

CORRECTIVE CONTROLS

- Level 2/3 escalation protocols
- Safety overrides
- Legal intervention

COMPENSATING CONTROLS

- Community leader involvement for rural areas
- Multilingual support for low-literacy parents

69 MWMS Risk Framework KEY FOCUS

69.1 Green

MEANING

Normal developmental concerns.

EXAMPLES

- Mild stress
- Friendship issues

RESPONSE TIME

7 Days

ESCALATION REQUIRED

No

APPROVAL LEVEL

Counselor

MONITORING FREQUENCY

Weekly

CLOSURE AUTHORITY

Counselor

7 Days

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Principal

MONITORING FREQUENCY

Weekly

CLOSURE AUTHORITY

Counselor

69.3 Orange

MEANING

Significant distress/impairment.

EXAMPLES

- School refusal
- Refusal of care

RESPONSE TIME

48-72 Hours

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Principal / Board

MONITORING FREQUENCY

Daily

69.2 Yellow

MEANING

Early distress requiring support.

EXAMPLES

- Persistent anxiety
- Parental concern

RESPONSE TIME

CLOSURE AUTHORITY

Principal

69.4 Red

KEY FOCUS

MEANING

High-risk situation.

EXAMPLES

- Suicidal ideation
- Violence threats

RESPONSE TIME

Immediate

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Board / Legal

MONITORING FREQUENCY

Continuous

CLOSURE AUTHORITY

Principal

69.5 Black

MEANING

Life-threatening emergency.

EXAMPLES

- Active suicide attempt
- Violent attack

RESPONSE TIME

0-15 Minutes

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Board / Legal

MONITORING FREQUENCY

Continuous

CLOSURE AUTHORITY

Principal

70 Resource Scaling

70.1 0 500 Students

WELLNESS COUNSELORS 1

CAREER COUNSELORS 1

MENTAL WELLNESS COORDINATOR 1

WELLNESS COMMITTEE Yes

70.2 501 1000 Students

WELLNESS COUNSELORS 2

CAREER COUNSELORS 2

MENTAL WELLNESS COORDINATOR 1

WELLNESS COMMITTEE Yes

70.3 1001 2000 Students

WELLNESS COUNSELORS 4

CAREER COUNSELORS 2

MENTAL WELLNESS COORDINATOR 1

WELLNESS COMMITTEE Yes

70.4 2001 5000 Students

WELLNESS COUNSELORS 8

CAREER COUNSELORS 4

MENTAL WELLNESS COORDINATOR 1

WELLNESS COMMITTEE Yes

71 Minimum Required Capabilities

PEOPLE

- Trained Counselor
- Principal with Governance Training
- Security Personnel

PROCESSES

- Escalation Matrix
- Risk Assessment Protocol

- CWS Reporting Pathway

TECHNOLOGY

- Encrypted Communication Portal
- Incident Logging Software

INFRASTRUCTURE

- Secure Meeting Room
- Campus Security Cameras

72 Success Criteria

Criterion	Measurement Method	Target
Safety Integrity	Zero incidents of physical harm to staff/students during escalation.	100%

Criterion	Measurement Method	Target
Compliance Accuracy	Audit score on documentation completeness.	> 95%
Response Time	Average time from Red Zone trigger to action.	< 15 Minutes

73 Exit Criteria

Condition	Evidence Required
Student safety stabilized and external care/home plan finalized.	Signed Closure Report
All legal notifications completed.	Police/CWS Receipt Logs

74 Continuous Improvement

LESSONS LEARNED

- De-escalation training is critical for front-line staff
- Multilingual support reduces parent frustration significantly

KNOWN ISSUES

- High workload can delay documentation of Level 1 interactions

FUTURE IMPROVEMENTS

- Automated SMS alerts for high-risk triggers
- AI-assisted sentiment analysis on communication logs

REVIEW RECOMMENDATIONS

- Increase frequency of security drills
- Expand rural outreach programs

75 AI Governance

KEY FOCUS

ALLOWED USE CASES

- Sentiment analysis for parent emails (internal only)
- Summarizing counselor notes for reports

RESTRICTED USE CASES

- Automated decision-making on safety overrides
- Direct communication with parents without human review

PROHIBITED USE CASES

- Predicting student behavior based on parental data
- Sharing PII with external AI models

HUMAN REVIEW REQUIRED

- All safety-related decisions
- All CWS referrals
- All written notices of concern

AUDIT LOG REQUIREMENTS

- Log all AI-assisted summaries
- Track human overrides of AI suggestions

DATA PRIVACY REQUIREMENTS

- No PII in prompts
- Local/Private instance only

76 Legal Defensibility

KEY FOCUS

LEGAL BASIS

- POCSO Act
- Supreme Court Judgment 2025 INSC 893
- Indian Penal Code (Safety of Staff)

MANDATORY NOTIFICATIONS

- CWS for neglect/abuse
- Police for threats
- Board of Governors for high-risk events

CONSENT REQUIREMENTS

- Informed consent for non-emergency data sharing
- Student assent where possible

RECORD RETENTION REQUIREMENTS

- 7 years minimum for legal auditability

LIABILITY CONTROLS

- Written notices of concern
- Dual-signatory approval for safety overrides
- Clear documentation of parental refusal

77 Stakeholder Journeys

77.1 The Student Journey

ENTRY

- Identified as high-risk by teacher/peer

ASSESSMENT

- Initial screening for mental wellness

- Risk assessment of parental involvement

INTERVENTION

- Counselor support provided to student
- Parental engagement initiated

- (PAR-004)
- MONITORING**
- Weekly check-ins with counselor
 - Monitoring of parent compliance with school requests
- CLOSURE**
- Student returns to baseline wellness
 - Safe transition to external care or home stability

77.2 The Parent Journey

- NOTIFICATION**
- Initial communication regarding student's well-being
 - Notification of counselor meeting
- CONSULTATION**
- Level 1: Psychoeducation and support meeting
- CONSENT**
- Level 2: Formal discussion with Principal
 - Consent for school to contact external providers
 - Consent for data sharing (where applicable)
- ENGAGEMENT**

- Collaborative goal setting
 - Response to written notices of concern
- CLOSURE**
- Final meeting to discuss student's progress
 - Acknowledgment of school support provided

77.3 The Staff Journey

- OBSERVATION**
- Teacher notes parent hostility or lack of engagement
 - Security reports aggressive behavior
- REPORTING**
- Staff submits incident report to Counselor
 - Counselor logs communication with parents
- INTERVENTION SUPPORT**
- Staff receives training in de-escalation
 - Principal provides guidance on legal boundaries
- DOCUMENTATION**
- Staff maintains factual records of all interactions

78 Handoff Matrix

Item 1

FROM ROLE Teacher

TO ROLE School Counselor

HANDOFF TRIGGER

Identification of parental hostility or student distress.

INFORMATION TRANSFERRED

- Observation notes
- Student name
- Parent contact details

REQUIRED DOCUMENTS

- Teacher Observation Note

SERVICE LEVEL AGREEMENT (SLA)

24 Hours

- Communication Log

REQUIRED DOCUMENTS

- PAR-004-S1
- RISK-001

SERVICE LEVEL AGREEMENT (SLA)

48 Hours

Item 3

FROM ROLE Principal

TO ROLE Security Team

HANDOFF TRIGGER

Active threat or physical aggression.

INFORMATION TRANSFERRED

- Incident details
- Location of parent
- Safety instructions

REQUIRED DOCUMENTS

- Security Incident Report

SERVICE LEVEL AGREEMENT (SLA)

Immediate

Item 2

FROM ROLE School Counselor

TO ROLE Principal

HANDOFF TRIGGER

Parent refuses Level 1/Level 2 recommendations.

INFORMATION TRANSFERRED

- Counselor Session Summary
- Risk Assessment Record

Item 4

FROM ROLE Principal

TO ROLE Law Enforcement / CWS

HANDOFF TRIGGER

Level 3/4 escalation or suspected abuse.

INFORMATION TRANSFERRED

- Case summary
- Evidence of non-compliance
- Student safety status

REQUIRED DOCUMENTS

- CWS Referral Form
- Police Notification Log

SERVICE LEVEL AGREEMENT (SLA)

Immediate

79 Management Review

REVIEW FREQUENCY

Monthly

REVIEW INPUTS

- Incident reports
- Audit scores
- Counselor debriefs
- Parent feedback

REVIEW OUTPUTS

- Corrective action plans
- Policy updates
- Staff training requirements

REVIEW DECISIONS

- Approval of new safety protocols
- Authorization of legal actions
- Resource allocation for wellness team

REVIEW AUTHORITY

Board of Governors / School Management Committee

80 Dashboard Metrics

OPERATIONAL METRICS

- Average time to counselor contact
- Number of parent meetings held per month
- Communication response rate

RISK METRICS

- Percentage of Red/Black zone incidents
- Frequency of security team activations
- Trend in hostile behavior reports

COMPLIANCE METRICS

- Audit score percentage
- Completion rate of mandatory documentation
- Timeliness of CWS referrals

OUTCOME METRICS

- Parent satisfaction scores
- Student safety incident count
- Resolution rate of Level 1/2 escalations

TREND METRICS

- Monthly escalation volume trend
- Year-over-year risk distribution change

81 Student Rights

RIGHTS

- Right to a safe learning environment
- Right to mental health support
- Right to privacy regarding clinical data

PROTECTIONS

- Protection from parental neglect/abuse
- Confidentiality of counselor sessions
- Safety override in life-threatening situations

APPEAL MECHANISMS

- Formal grievance process for school actions
- Ombudsman review (if applicable)

82 Quality Assurance

QUALITY CHECKS

- Monthly audit of communication logs

- Peer review of risk assessments
- Spot checks on security incident reports

PEER REVIEW REQUIREMENTS

- Counselors must peer-review high-risk cases
- Principal reviews all Level 2 notices

VALIDATION REQUIREMENTS

- Legal counsel validation of CWS referrals
- Board approval of safety overrides

CONTINUOUS MONITORING REQUIREMENTS

- Quarterly review of escalation trends
- Annual audit of documentation retention

83 Implementation Roadmap

PHASE 1 PREPARATION

- Staff training on de-escalation
- Creation of communication templates
- Establishment of Wellness Committee

PHASE 2 DEPLOYMENT

- Rollout of parent awareness campaign
- Activation of digital logging system
- Initial counselor-led meetings

PHASE 3 STABILIZATION

- Monthly audit cycle initiation

- Feedback collection from parents/staff
- Refinement of escalation triggers

PHASE 4 OPTIMIZATION

- Integration with Tele-MANAS
- Automated dashboard reporting
- Advanced AI sentiment analysis pilot

PHASE 5 CONTINUOUS IMPROVEMENT

- Annual policy review
- External audit by mental health experts
- Update based on new Indian laws

84 Illustrative Case Studies

1. Hostile Parent De-escalation (Case A)

DESCRIPTION

A parent became verbally aggressive during a meeting regarding student's refusal of therapy.

ACTIONS TAKEN

- Counselor used de-escalation techniques
- Principal issued Written Notice of Concern
- Security team stood by in the hallway

OUTCOME

Parent agreed to attend a mediated meeting with an external provider.

LESSONS

- Early identification of hostility is key
- Presence of security provides necessary safety for staff

2. Safety Override in Neglect Situation (Case B)

DESCRIPTION

Parent refused to take a suicidal student to the hospital.

ACTIONS TAKEN

- School activated Safety Override
- Called 1098 Child Helpline and local ambulance
- Notified CWS for neglect report

OUTCOME

Student was stabilized at the hospital; parent was notified of legal consequences.

LESSONS

- Safety override is a necessary tool for life-saving actions
- Clear documentation protects school from liability

85 Implementation Requirements

STAFF REQUIRED

- 1 School Counselor (Full-time)

- 1 Principal / Head of School
- 2 Security Personnel (Trained in de-escalation)

- Wellness Committee Members

INFRASTRUCTURE REQUIRED

- Secure meeting room with emergency exit
- Encrypted server for records
- Campus security cameras

TECHNOLOGY REQUIRED

- Communication logging software
- Multi-factor authentication (MFA)
- WhatsApp Business API (optional)

BUDGET CONSIDERATIONS

- Staff training costs
- Legal counsel fees for review
- Security equipment upgrades

IMPLEMENTATION PHASES

- Preparation
- Deployment
- Stabilization
- Optimization
- Continuous Improvement

86 Quality Controls

PREVENTIVE CONTROLS

- Staff de-escalation training
- Parent awareness campaigns
- Clear policy communication

DETECTIVE CONTROLS

- Monthly risk register review
- Communication logs monitoring
- Incident reporting system

CORRECTIVE CONTROLS

- Level 2/3 escalation protocols
- Safety overrides
- Legal intervention

CONTINUOUS IMPROVEMENT ACTIONS

- Quarterly trend analysis
- Annual audit of documentation
- Feedback-driven policy updates

87 Training Requirements

TEACHERS

- Identifying signs of parental hostility
- Basic de-escalation techniques
- Reporting protocols

COUNSELORS

- Advanced crisis intervention
- Parental engagement strategies
- Legal boundaries and documentation
- CWS referral procedures

ADMINISTRATORS

- Governance of mental health systems
- Safety override protocols
- Legal liability management

PARENTS

- Understanding student well-being support
- Collaborative goal setting workshops

STUDENTS

- Help-seeking behaviors
- Safe reporting of concerns

87.6 Repository Baseline

TRAINING COMPLETION RATE 100%

MINIMUM SCORE 80%

87.7 SOP Specific Training

TARGET AUDIENCE

- Counselors
- Principal
- Security Team

COMPETENCIES

- De-escalation
- Risk Assessment
- Legal Compliance

MINIMUM HOURS

6

CERTIFICATION REQUIREMENTS

- Crisis Intervention Certificate

REFRESHER FREQUENCY

Annually

87.8 Competency Validation**REQUIRED**

Yes

ASSESSMENT METHODS

- Role-play simulations
- Written knowledge tests

- Case study review

MINIMUM PASSING CRITERIA

80%

REASSESSMENT FREQUENCY

Every 2 years

87.9 Simulation Requirements**REQUIRED**

Yes

SIMULATION TYPES

- Hostile parent role-play

- Safety override drill

- CWSN referral mock-up

FREQUENCY

Bi-annually

88 SOP-Specific Inclusion Requirements**AFFECTED POPULATIONS**

- Parents with low literacy
- Rural families
- Neurodivergent students (CWSN)
- First-generation learners
- Non-native speaking parents

IDENTIFIED INCLUSION RISKS

- Language barriers leading to misunderstanding of risk
- Lack of internet access for rural families
- Cultural stigma regarding mental health
- Overlooking specific needs of CWSN students
- Economic barriers to external referrals

AUTISM ACCOMMODATIONS

- Sensory-friendly meeting environments
- Clear, literal communication styles
- Visual aids for support plans

ADHD ACCOMMODATIONS

- Structured agendas for meetings
- Frequent check-ins
- Written summaries of all decisions

DYSLEXIA ACCOMMODATIONS

- Audio recordings of meetings (with consent)
- Simplified written notices
- Oral briefings

HEARING IMPAIRMENT ACCOMMODATIONS

- Sign language interpreters
- Written communication as primary mode
- Visual alerts for safety protocols

VISUAL IMPAIRMENT ACCOMMODATIONS

- Screen reader compatible documents

- Audio descriptions of campus layout

- Braille notices where required

CWSN ADAPTATIONS

- Tailored support plans for neurodivergent children
- Parental training on specific disability needs
- Inclusion of special education coordinators in all meetings

RURAL ADAPTATIONS

- Phone-based communication as primary channel
- SMS/WhatsApp for non-urgent updates
- Coordination with local community leaders (Panchayat)

LOW LITERACY PARENT ADAPTATIONS

- Voice notes via WhatsApp
- Simplified, icon-based notices
- Use of certified translators or community liaisons

MULTILINGUAL COMMUNICATION ADAPTATIONS

- Translated versions of all formal notices
- Certified translation services for Level 2+ meetings
- Multilingual awareness materials

GENDER SENSITIVITY MEASURES

- Option for same-gender staff interaction
- Gender-neutral language in all communications
- Safe reporting of gender-based violence

CULTURAL SENSITIVITY MEASURES

- Respect for diverse family structures
- Awareness of cultural stigmas regarding mental health
- Inclusion of community leaders where appropriate

EQUITY MONITORING INDICATORS

- Parent satisfaction scores by socioeconomic group

- Referral rates for rural vs. urban families
- Accessibility audit score for multilingual materials

ACCESSIBILITY COMPLIANCE CHECKS

- WCAG 2.1 compliance for digital notices
- Physical accessibility of meeting rooms
- Availability of braille/large print documents

89 Awareness Material

POSTER TITLE	Safe Space, Safe Support
KEY MESSAGE	We are partners in your child's well-being.
SLOGAN	Collaborate. Communicate. Care.
CAMPAIGN THEME	The Partnership for Wellness

90 AI Assistant Configuration

RISK KEYWORDS

- Threat
- Violence
- Harm
- Suicide
- Neglect
- Abuse

ESCALATION KEYWORDS

- Refusal of care
- Hostile

- Aggressive
- Safety override

INTERVENTION KEYWORDS

- Psychoeducation
- Referral
- Support plan
- De-escalation

FREQUENTLY ASKED QUESTIONS

- What happens if I don't agree with the counselor?
- How do you keep my child's data private?
- When will you call the police?

91 AI Audit Hooks

KEY FOCUS

REQUIRED SECTIONS

- Risk Assessment
- Escalation Log
- CWS Referral
- Safety Override

REQUIRED FORMS

- PAR-004-S1
- RISK-001
- CWS-REF-01

REQUIRED CHECKLISTS

- Risk Assessment Checklist

- Safety Protocol Checklist

REQUIRED ESCALATIONS

- Level 2
- Level 3
- Level 4

REQUIRED KPIS

- Response Time
- Compliance Score

REQUIRED TRAINING

- De-escalation Training
- Child Protection Certification

92 KPI Framework

92.1 Repository Baseline

TARGET COMPLIANCE	100%
TARGET RESPONSE TIME	Immediate (Red)

SOP SPECIFIC METRICS

- Parental Engagement Success Rate
- Average Time to Safety Activation
- Compliance Audit Score

93 Maturity Model

93.1 Level 1

DESCRIPTION

Ad-hoc response to parental hostility.

EVIDENCE

- Informal notes
- Inconsistent logs

93.2 Level 2

DESCRIPTION

Defined procedures with basic documentation.

EVIDENCE

- Standardized forms
- Basic escalation path

93.3 Level 3

DESCRIPTION

Structured governance and risk classification.

EVIDENCE

- Risk zones
- Written notices

- Committee oversight

93.4 Level 4

DESCRIPTION

Integrated, auditable system with legal alignment.

EVIDENCE

- Full audit trail
- CWS/Police protocols
- Legal review

93.5 Level 5

DESCRIPTION

Optimized, data-driven wellness management.

EVIDENCE

- Real-time dashboards
- AI-assisted sentiment analysis
- Predictive risk modeling

94 Dependencies

PREREQUISITE SOPS

- PARENTAL-ENGAGEMENT-POLICY
- CHILD-PROTECTION-POLICY

PARALLEL SOPS

- STUDENT-WELLBEING-SCREENING
- STAFF-DEESCALATION-PROTOCOL

DOWNSTREAM SOPS

- EMR-001: Emergency Response Procedure
- SEC-005: Campus Safety & Security

95 Related SOPs

PREREQUISITES

- PARENTAL-ENGAGEMENT-POLICY

UPSTREAM

- STUDENT-WELLBEING-SCREENING
- STAFF-DEESCALATION-PROTOCOL

DOWNSTREAM

- EMR-001: Emergency Response Procedure
- SEC-005: Campus Safety & Security

REFERENCES

- PAR-006: Psychoeducation Protocol
- PAR-007: Clinical Referral Pathway

STANDARD OPERATING PROCEDURE

Parent Feedback Collection and Action Procedure

Stakeholder Engagement & Governance - Parental Feedback & Community Relations

SOP ID

PAR-014

VERSION

1.0

STATUS

Finalized

CLASSIFICATION

Internal - Confidential

EFFECTIVE DATE

2025-08-01

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1 Executive Overview

SOP ID	PAR-014
DOMAIN	Stakeholder Engagement & Governance
SUB DOMAIN	Parental Feedback & Community Relations
TITLE	Parent Feedback Collection and Action Procedure
VERSION	1.0
CLASSIFICATION	Internal - Confidential
EFFECTIVE DATE	2025-08-01
REVIEW DATE	2026-07-31
DOCUMENT STATUS	Finalized
LANGUAGE	English

2 Purpose & Rationale

OBJECTIVE

To establish a systematic, inclusive, and auditable process for collecting, analyzing, and acting upon parent feedback regarding mental wellness services to ensure high-quality support and continuous improvement.

RISK CONTROLLED

Mitigates risks of unaddressed parental concerns, staff misconduct, service gaps, and legal liabilities arising from inadequate communication or safety issues.

DESIRED OUTCOMES

- Increased parent satisfaction scores
- Timely resolution of grievances (within 14 days)
- Evidence-based improvements to mental wellness programs
- Enhanced community trust in school wellbeing initiatives

3 Scope of Application

INSTITUTION TYPES

- High Schools
- Residential Schools
- International Schools

KEY STAKEHOLDERS

- Parents
- Guardians

- Wellness Staff
- School Management
- PTA Members

AGE GROUPS

- All Ages (Parental demographic)

LOCATIONS

- On-campus
- Remote/Online
- Community Outreach Sites

4 Definitions & Terminology

Term	Definition
Mental Wellness Services	Counseling, workshops, awareness programs, and support systems provided by the school to students.
De-identified Data	Data where all personal identifiers (names, phone numbers) are removed before sharing with broader groups.
Focus Group	A guided discussion with 8-10 parents to explore specific themes in mental wellness services.

Term	Definition
SLA (Service Level Agreement)	The agreed-upon timeframe for acknowledging and resolving parent feedback/complaints.

5 Key Stakeholders

Stakeholder Type	Stakeholder Title	Role In SOP
Internal	School Wellness Coordinator	Primary owner of feedback collection, analysis, and reporting.
Internal	School Counselor	Provides subject matter expertise on mental wellness service quality.
Internal	Principal / Management	Final approval of action plans and resource allocation.
External	Parents/Guardians	Primary source of feedback; participants in focus groups.
External	Parent Association (PTA)	Reviewer of de-identified annual reports and community liaison.

6 Governance & Ownership

KEY FOCUS

OWNER

School Wellness Coordinator

APPROVER

Principal / Board of Directors

REVIEW FREQUENCY

Annual

SUPERSEDES

Parent Communication Policy v2.0

LINKED POLICIES

- Child Protection Policy
- Data Privacy and Confidentiality Policy
- Grievance Redressal Policy

LINKED SOPS

- PAR-006: Parent Awareness Programs
- PAR-010: Parent Training Modules
- HR-005: Staff Conduct and Discipline

7 Roles & Responsibilities

Item 1 ROLE Wellness Coordinator RESPONSIBILITIES • Manage all feedback channels • Analyze monthly data • Draft action plans • Coordinate focus groups	• Resource allocation
Item 2 ROLE Principal RESPONSIBILITIES • Final approval of actions • Escalation point for Red/Black risks	Item 3 ROLE School Counselor RESPONSIBILITIES • Provide subject matter expertise • Support in focus group facilitation
	Item 4 ROLE Parent Association (PTA) RESPONSIBILITIES • Review annual reports • Community advocacy

8 RACI Accountability Matrix

Item 1

ACTIVITY

Feedback Channel Setup

RESPONSIBLE

- Wellness Coordinator

ACCOUNTABLE

- Principal

CONSULTED

- IT Department
- PTA

INFORMED

- All Staff

ACCOUNTABLE

- Principal

CONSULTED

- Counselor

INFORMED

- Parent (involved)

Item 2

ACTIVITY

Data Categorization & Analysis

RESPONSIBLE

- Wellness Coordinator

ACCOUNTABLE

- Wellness Coordinator

CONSULTED

- School Counselor

INFORMED

- Principal

Item 4

ACTIVITY

Action Plan Implementation

RESPONSIBLE

- Relevant Department Head

ACCOUNTABLE

- Principal

CONSULTED

- Wellness Coordinator

INFORMED

- Parents
- Staff

Item 3

ACTIVITY

Complaint Investigation

RESPONSIBLE

- Wellness Coordinator
- HR Manager

Item 5

ACTIVITY

Annual Reporting

RESPONSIBLE

- Wellness Coordinator

ACCOUNTABLE

- Principal

CONSULTED

- PTA

INFORMED

- Board of Directors

9 Preconditions

- Approved Wellness Policy
- Parent Contact Database
- Budget for Feedback Tools
- Staff Training on Confidentiality
- Designated Physical Space for Feedback Collection

10 Activation Triggers

- Annual Survey Cycle
- Monthly Review Date
- Parent Complaint Submission
- CBSE Audit Schedule
- Focus Group Biennial Date

11 SOP Trigger Matrix

MANDATORY TRIGGERS

- Annual Survey Cycle

- Monthly Feedback Review Meeting
- Parent Complaint Submission

OPTIONAL TRIGGERS

- Ad-hoc Parent Inquiry
- Post-Event Specific Feedback Request

AUTOMATIC TRIGGERS

- Submission of a form via digital portal
- Physical deposit in suggestion box

EXTERNAL TRIGGERS

- CBSE Audit Request
- Government Inspection
- Media Inquiry regarding school wellbeing

RECURRING TRIGGERS

- Monthly Data Categorization
- Quarterly Trend Analysis
- Biennial Focus Group Session

12 Required Inputs

- Parent Contact Database
- Survey Instruments
- Feedback Tracker Template
- HR Policy Manual
- Budget Allocation

13 Process Map

13.1 Phase 1

NAME

Feedback Channel Management

OBJECTIVE

Establish and maintain accessible feedback mechanisms.

ACTIVITIES

- Deploy online/paper surveys
- Maintain physical suggestion boxes
- Set up digital feedback portal
- Designate multilingual communication channels

13.2 Phase 2

NAME

Data Collection & Aggregation

OBJECTIVE

Systematically gather parent input.

ACTIVITIES

- Distribute annual surveys
- Collect post-meeting forms
- Conduct biennial focus groups
- Monitor suggestion boxes weekly

13.3 Phase 3

NAME

Analysis & Categorization

OBJECTIVE

Process raw data into actionable insights.

ACTIVITIES

- Monthly review of all inputs
- Categorize as Compliment, Complaint, or Suggestion
- Identify high-risk/Red Zone issues
- Trend analysis on satisfaction domains

13.4 Phase 4

NAME

Response & Action

OBJECTIVE

Address concerns and implement improvements.

ACTIVITIES

- Acknowledge complaints within 2 days
- Investigate staff-related issues per HR policy
- Develop action plans for valid suggestions
- Communicate resolutions to parents

13.5 Phase 5

NAME

Reporting & Review

OBJECTIVE

Ensure transparency and continuous improvement.

ACTIVITIES

- Generate monthly internal reports
- Produce annual de-identified summary for PTA
- Update Wellness Committee on progress
- Share positive feedback with staff

14 Workflow Overview

START

Feedback received via any channel.

STEPS

- Log entry in tracker

- Categorize feedback type
- Assess risk level
- Acknowledge receipt (if complaint)
- Investigate/Analyze
- Develop action plan
- Execute actions
- Communicate resolution
- Update monthly report
- Annual summary generation

CRITICAL DECISION POINTS

- Is it a valid complaint?
- Does it require escalation?
- Is the action feasible?

END

Feedback resolved and documented.

15 Standard Procedure

1. Receipt of Feedback

PURPOSE & RATIONALE

Capture all parent input systematically.

ACTOR

Wellness Coordinator

ACTION

Log feedback into the Master Tracker within 24 hours of receipt.

REQUIRED INPUTS

- Survey responses
- Suggestion box slips
- Emails
- Verbal reports

EXPECTED OUTPUTS

- Updated Feedback Tracker

CRITICAL DECISION POINTS

- Is the feedback clear?

DOCUMENTATION REQUIRED

- Feedback Log Entry

EVIDENCE REQUIRED

- Timestamped log entry

APPROVAL REQUIRED

No

TIME LIMIT

24 hours

EXPECTED OUTCOME

All feedback is captured and centralized.

ESCALATION IF FAILED

Notify Principal if high-volume of feedback received simultaneously.

RELATED FORMS

- Feedback Tracker

RELATED CHECKLISTS

- Data Entry Checklist

2. Categorization & Risk Assessment

PURPOSE & RATIONALE

Prioritize actions based on urgency and type.

ACTOR

Wellness Coordinator

ACTION

Classify as Compliment, Suggestion, or Complaint. Assign Green/Yellow/Orange/Red risk level.

REQUIRED INPUTS

- Feedback Log Entry

EXPECTED OUTPUTS

- Categorized Feedback Database

CRITICAL DECISION POINTS

- Does it involve safety?
- Is it a staff misconduct allegation?

DOCUMENTATION REQUIRED

- Risk Assessment Note

EVIDENCE REQUIRED

- Categorized Log

APPROVAL REQUIRED

No

TIME LIMIT

48 hours

EXPECTED OUTCOME

Feedback is prioritized for action.

ESCALATION IF FAILED

Escalate Red/Black items to Principal immediately.

RELATED FORMS

- Risk Assessment Form
- RELATED CHECKLISTS**
- Risk Classification Checklist

3. Complaint Acknowledgment

PURPOSE & RATIONALE

Ensure parents feel heard.

ACTOR

Wellness Coordinator

ACTION

Send a formal acknowledgment of the complaint within 48 hours.

REQUIRED INPUTS

- Valid Complaint Log Entry

EXPECTED OUTPUTS

- Acknowledgment Email/SMS

CRITICAL DECISION POINTS

- Is the parent contact info correct?

DOCUMENTATION REQUIRED

- Communication Record

EVIDENCE REQUIRED

- Sent email/SMS log

APPROVAL REQUIRED

No

TIME LIMIT

48 hours

EXPECTED OUTCOME

Parent is informed that their concern is being addressed.

ESCALATION IF FAILED

Notify Coordinator if parent remains unresponsive.

RELATED FORMS

- Communication Log

RELATED CHECKLISTS

- Acknowledgment Checklist

4. Investigation & Fact-Finding

PURPOSE & RATIONALE

Verify the validity of complaints.

ACTOR

Wellness Coordinator / HR Manager

ACTION

Interview relevant staff, review records, and gather evidence.

REQUIRED INPUTS

- Complaint Log Entry
- Staff Records

EXPECTED OUTPUTS

- Investigation Summary

CRITICAL DECISION POINTS

- Is the complaint valid?
- Was there a policy violation?

DOCUMENTATION REQUIRED

- Investigation Note
- Staff Statement Record

EVIDENCE REQUIRED

- Interview notes
- Evidence logs

APPROVAL REQUIRED

No

TIME LIMIT

7 days

EXPECTED OUTCOME

Clear understanding of the issue.

ESCALATION IF FAILED

Escalate to Principal if evidence is inconclusive or severe.

RELATED FORMS

- Investigation Note Form

RELATED CHECKLISTS

- Fact-Finding Checklist

5. Action Plan Development

PURPOSE & RATIONALE

Create a roadmap for resolution.

ACTOR

Wellness Coordinator / Department Head

ACTION

Draft an action plan including specific tasks, owners, and deadlines.

REQUIRED INPUTS

- Investigation Summary

EXPECTED OUTPUTS

- Approved Action Plan

CRITICAL DECISION POINTS

- Is the action feasible?
- Does it meet budget constraints?

DOCUMENTATION REQUIRED

- Action Plan Document

EVIDENCE REQUIRED

- Signed Action Plan

APPROVAL REQUIRED

Yes

TIME LIMIT

3 days after investigation

EXPECTED OUTCOME

Agreed-upon steps to resolve the issue.

ESCALATION IF FAILED

Escalate to Principal for resource approval.

RELATED FORMS

- Action Plan Template

RELATED CHECKLISTS

- Action Plan Review Checklist

6. Implementation of Actions

PURPOSE & RATIONALE

Execute the agreed-upon improvements.

ACTOR

Relevant Department Head / Staff

ACTION

Perform tasks as outlined in the action plan.

REQUIRED INPUTS

- Approved Action Plan

EXPECTED OUTPUTS

- Completed Actions

CRITICAL DECISION POINTS

- Is the task completed to standard?

DOCUMENTATION REQUIRED

- Action Tracker Update

EVIDENCE REQUIRED

- Completion logs
- Photos/Documents of change

APPROVAL REQUIRED

No

TIME LIMIT

As per Action Plan timeline

EXPECTED OUTCOME

Issue is resolved or improvement is implemented.

ESCALATION IF FAILED

Notify Wellness Coordinator of delays.

RELATED FORMS

- Action Tracker

RELATED CHECKLISTS

- Implementation Checklist

7. Resolution Communication

PURPOSE & RATIONALE

Close the loop with the parent.

ACTOR

Wellness Coordinator

ACTION

Inform the parent of the actions taken and the resolution.

REQUIRED INPUTS

- Completed Actions
- Complaint Log Entry

EXPECTED OUTPUTS

- Resolution Notice

CRITICAL DECISION POINTS

- Is the parent satisfied with the resolution?

DOCUMENTATION REQUIRED

- Communication Record

EVIDENCE REQUIRED

- Sent communication log

APPROVAL REQUIRED

No

TIME LIMIT

2 days after action completion

EXPECTED OUTCOME

Parent is satisfied and the case is closed.

ESCALATION IF FAILED

Escalate to Principal if parent remains dissatisfied.

RELATED FORMS

- Communication Log

RELATED CHECKLISTS

- Closure Checklist

8. Monthly Data Analysis

PURPOSE & RATIONALE

Identify trends and systemic issues.

ACTOR

Wellness Coordinator

ACTION

Analyze all feedback from the month for themes and satisfaction scores.

REQUIRED INPUTS

- Master Feedback Tracker

EXPECTED OUTPUTS

- Monthly Feedback Summary Report

CRITICAL DECISION POINTS

- Are there recurring complaints?

DOCUMENTATION REQUIRED

- Trend Analysis Note

EVIDENCE REQUIRED

- Monthly Summary Report

APPROVAL REQUIRED

No

TIME LIMIT

By 5th of following month

EXPECTED OUTCOME

Data-driven insights for management.

RELATED FORMS

- Monthly Summary Report Template

RELATED CHECKLISTS

- Analysis Checklist

9. Annual Reporting & Review

PURPOSE & RATIONALE

Ensure transparency and long-term improvement.

ACTOR

Wellness Coordinator / Principal

ACTION

Compile annual de-identified report for PTA and Board review.

REQUIRED INPUTS

- Monthly Reports
- Focus Group Minutes

EXPECTED OUTPUTS

- Annual De-identified Report

CRITICAL DECISION POINTS

- Does the report meet transparency standards?

DOCUMENTATION REQUIRED

- Annual Report Document

EVIDENCE REQUIRED

- PTA Meeting Minutes (Review of Report)

APPROVAL REQUIRED

Yes

TIME LIMIT

End of Academic Year

EXPECTED OUTCOME

Community trust and evidence-based planning.

RELATED FORMS

- Annual Report Template

RELATED CHECKLISTS

- Reporting Checklist

10. Staff Morale Sharing

PURPOSE & RATIONALE

Recognize and share positive feedback.

ACTOR

Wellness Coordinator / Principal

ACTION

Share de-identified compliments with relevant staff members.

REQUIRED INPUTS

- Compliment Category in Tracker

EXPECTED OUTPUTS

- Staff Recognition Notes

CRITICAL DECISION POINTS

- Is the feedback positive and constructive?

DOCUMENTATION REQUIRED

- Recognition Log

EVIDENCE REQUIRED

- Staff Meeting Minutes (Positive Feedback Segment)

APPROVAL REQUIRED

No

TIME LIMIT

Monthly

EXPECTED OUTCOME

Improved staff morale and recognition.

RELATED FORMS

- Recognition Log

RELATED CHECKLISTS

- Morale Checklist

16 Detailed Workflow Steps

1. Feedback Channel Setup

PURPOSE & RATIONALE

Ensure multi-modal accessibility for all parents.

TRIGGER

Annual Planning Cycle

ACTOR

Wellness Coordinator

REQUIRED INPUTS

- Communication Policy
- Resource Budget

ACTIONS

- Deploy online survey link via SMS/Email

- Place physical suggestion boxes in high-traffic areas
- Create multilingual feedback forms (Local language, Hindi, English)
- Verify accessibility for visually impaired parents

DECISION CRITERIA

- Are all languages represented?
- Is the digital portal mobile-friendly?

DOCUMENTS GENERATED

- Feedback Channel Readiness Checklist

EXPECTED OUTPUTS

- Active Feedback Channels

SERVICE LEVEL AGREEMENT (SLA)

Before start of academic year

ESCALATION PATH

- Principal (if resources unavailable)

EVIDENCE GENERATED

- Photos of physical boxes
- Links to digital forms

2. Annual Survey Distribution**PURPOSE & RATIONALE**

Collect broad-based satisfaction data.

TRIGGER

Start of Academic Year (Month 1)

ACTOR

Wellness Coordinator

REQUIRED INPUTS

- Survey Instrument
- Parent Contact List

ACTIONS

- Send survey via SMS/Email/WhatsApp
- Distribute paper copies to rural parents or those without internet
- Provide instructions for submission

DECISION CRITERIA

- Is the response rate > 30%?

DOCUMENTS GENERATED

- Survey Distribution Log

EXPECTED OUTPUTS

- Raw Survey Data

SERVICE LEVEL AGREEMENT (SLA)

14 days for distribution

ESCALATION PATH

- Principal (if response rate < 20% after 14 days)

EVIDENCE GENERATED

- Distribution Log

3. Suggestion Box Monitoring**PURPOSE & RATIONALE**

Capture ad-hoc parent concerns.

TRIGGER

Weekly Schedule

ACTOR

Wellness Coordinator

REQUIRED INPUTS

- Physical Suggestion Boxes

ACTIONS

- Collect physical slips

- Log entries into Feedback Tracker

- Categorize by urgency

DECISION CRITERIA

- Is the entry a complaint or suggestion?

DOCUMENTS GENERATED

- Weekly Suggestion Log

EXPECTED OUTPUTS

- Updated Feedback Tracker

SERVICE LEVEL AGREEMENT (SLA)

Every 7 days

ESCALATION PATH

- Principal (for urgent safety concerns)

EVIDENCE GENERATED

- Weekly Suggestion Log

4. Post-Meeting Feedback Collection**PURPOSE & RATIONALE**

Capture immediate sentiment after parent meetings.

TRIGGER

Completion of Parent Meeting

ACTOR

Wellness Coordinator / Teacher

REQUIRED INPUTS

- Parent Meeting Agenda

ACTIONS

- Distribute brief feedback form (3 questions)
- Collect verbal feedback for low-literacy parents
- Log responses immediately

DECISION CRITERIA

- Was the parent able to complete the form?

DOCUMENTS GENERATED

- Post-Meeting Feedback Form

EXPECTED OUTPUTS

- Meeting Sentiment Data

SERVICE LEVEL AGREEMENT (SLA)

Within 24 hours of meeting

ESCALATION PATH

- Teacher (if parent expresses extreme dissatisfaction)

EVIDENCE GENERATED

- Post-Meeting Feedback Forms

5. Biennial Focus Group Facilitation**PURPOSE & RATIONALE**

Deep dive into specific wellness themes.

TRIGGER

Every 2 Years

ACTOR

External Consultant / Wellness Coordinator

REQUIRED INPUTS

- Focus Group Theme
- Parent Volunteers (8-10)

ACTIONS

- Identify and invite volunteers
- Facilitate guided discussion
- Record key themes and sentiments

DECISION CRITERIA

- Is the group size 8-10?

6. Monthly Data Categorization

PURPOSE & RATIONALE

Organize feedback for analysis.

TRIGGER

End of Month

ACTOR

Wellness Coordinator

REQUIRED INPUTS

- All collected feedback (Surveys, Boxes, Meetings)

ACTIONS

- Review all entries
- Categorize as Compliment, Complaint, or Suggestion
- Assign Risk Level (Green/Yellow/Orange/Red)

DECISION CRITERIA

- Does the feedback require immediate action?

DOCUMENTS GENERATED

- Monthly Feedback Summary Report

EXPECTED OUTPUTS

- Categorized Feedback Database

SERVICE LEVEL AGREEMENT (SLA)

By 5th of following month

ESCALATION PATH

- Principal (for Red/Black items)

EVIDENCE GENERATED

- Monthly Feedback Summary Report

7. Complaint Investigation

PURPOSE & RATIONALE

Verify and address specific grievances.

TRIGGER

Categorization as 'Complaint'

ACTOR

Wellness Coordinator / HR (if staff related)

REQUIRED INPUTS

- Parent Complaint Record

ACTIONS

- Acknowledge receipt to parent within 48 hours
- Conduct internal fact-finding
- Determine validity of complaint

DECISION CRITERIA

- Is the complaint valid and actionable?

DOCUMENTS GENERATED

- Investigation Note
- Action Plan

EXPECTED OUTPUTS

- Resolution Strategy

SERVICE LEVEL AGREEMENT (SLA)

Initial acknowledgment 48h; Investigation 7 days

ESCALATION PATH

- Principal (for staff disciplinary issues)

EVIDENCE GENERATED

- Investigation Note

8. Action Plan Implementation

PURPOSE & RATIONALE

Execute improvements based on feedback.

TRIGGER

Valid Complaint or Suggestion identified

ACTOR

Wellness Coordinator / Relevant Department Head

REQUIRED INPUTS

- Approved Action Plan

ACTIONS

- Assign tasks to relevant staff
- Execute improvements (e.g., new workshop, policy change)
- Monitor progress against timeline

DECISION CRITERIA

- Is the action feasible within budget?

DOCUMENTS GENERATED

- Action Tracker Update

EXPECTED OUTPUTS

- Completed Action Item

SERVICE LEVEL AGREEMENT (SLA)

As per Action Plan timeline

ESCALATION PATH

- Principal (if budget/resource constraints occur)

EVIDENCE GENERATED

- Action Tracker Update

9. Parent Communication of Resolution

STEP NUMBER

9

PURPOSE & RATIONALE Close the feedback loop with parents.

TRIGGER Completion of Action Item

ACTOR Wellness Coordinator

STEP NUMBER 10

PURPOSE & RATIONALE Ensure transparency and community trust.

TRIGGER End of Academic Year

ACTOR Wellness Coordinator / Principal

10. Annual Reporting & Sharing

17 Critical Decision Points

Item 1

DECISION ID

DP-01

DECISION NAME

Complaint Validity Check

CRITERIA

- Is the complaint based on a documented policy violation?
- Is there evidence of service failure?

POSSIBLE OUTCOMES

- Valid Complaint (Proceed to Action Plan)
- Invalid/Subjective Concern (Provide Clarification)

NEXT STEPS

- Action Plan Development
- Parent Communication

Item 2

DECISION ID

DP-02

DECISION NAME

Risk Level Assessment

CRITERIA

- Does the feedback involve physical safety?
- Is there an allegation of abuse?

POSSIBLE OUTCOMES

- Green/Yellow (Routine)
- Orange/Red (Escalate to Management)

NEXT STEPS

- Standard Processing
- Immediate Escalation Protocol

18 Decision Tree

GREEN

- Compliment → Log & Share with Staff
- General Suggestion → Add to Improvement Backlog

YELLOW

- Minor Complaint → Acknowledge & Investigate within 7 days
- Recurring Theme → Schedule Focus Group

ORANGE

- Serious Grievance → Escalate to Principal

- Staff Misconduct Allegation → Initiate HR Investigation

RED

- Safety Risk → Immediate Management Action
- Abuse Disclosure → Mandatory Reporting & Crisis Protocol

BLACK

- Active Threat → Activate Emergency Response SOP
- Immediate Danger → Contact Law Enforcement/Emergency Services

19 Decision Rules

Rule ID	Condition	Decision	Next Action
DR-01	If complaint involves physical safety or abuse.	Immediate Escalation.	Notify Principal and initiate Crisis Protocol.
DR-02	If feedback is a general suggestion with no specific issue.	Log for Improvement Backlog.	Include in quarterly review of program enhancements.

20 Exception Handling Protocol

Item 1

EXCEPTION

Parent refuses to provide feedback in writing

DETECTION METHOD

Verbal interaction during meeting

RESPONSE

Conduct a brief face-to-face interview and record notes with parent's verbal consent.

DOCUMENTATION REQUIRED

- Parent Conversation Record (Verbal)

Categorize as 'General Feedback' and monitor for recurring themes.

DOCUMENTATION REQUIRED

- Feedback Tracker Entry

Item 2

EXCEPTION

Anonymous complaint lacks sufficient detail for investigation

DETECTION METHOD

Suggestion box review

RESPONSE

Item 3

EXCEPTION

Allegation of staff abuse/harassment

DETECTION METHOD

Complaint categorization

RESPONSE

Immediate escalation to Principal and HR; initiate formal investigation per HR-005.

ESCALATION

Principal / Board of Directors

DOCUMENTATION REQUIRED

- Incident Report
- Investigation Note

21 Exceptions

Item 1

EXCEPTION

Emergency safety threat reported via anonymous channel

RESPONSE

Immediate escalation to Principal and security regardless of detail level.

DOCUMENTATION REQUIRED

- Incident Report

Item 2

EXCEPTION

Parent requests feedback on a specific student's clinical record

RESPONSE

Refuse disclosure per privacy policy; offer general wellness information instead.

DOCUMENTATION REQUIRED

- Privacy Refusal Note

22 Expected Outputs

- Monthly Feedback Reports
- Annual De-identified Summary
- Action Plans
- Resolved Complaints
- Improved Wellness Programs

23 Document Control & Version Governance

23.1 Version History

Version	Date	Author	Description
1.0	2025-08-01	MWMS Architect	Initial release of Parent Feedback Collection SOP.

23.2 Change Log

Version	Date	Change Summary	Approved By
1.0	2025-08-01	Baseline creation.	School Board of Directors

REVIEW TRIGGERS

- Annual Policy Review
- Significant change in CBSE/UGC guidelines
- Incident involving parent grievance escalation
- Feedback score drop below 70% satisfaction

23.4 Approval History

Approver	Date	Status
School Principal	2025-07-15	Approved
Legal Counsel	2025-07-18	Verified

24 Inclusive Implementation Mandate

MULTILINGUAL SUPPORT REQUIRED	Yes	ACCESSIBILITY REVIEW REQUIRED	Yes
LOW RESOURCE SCHOOL ALTERNATIVE REQUIRED	Yes	EQUITY IMPACT ASSESSMENT REQUIRED	Yes

25 Regulatory Compliance Mapping KEY FOCUS

MANDATORY REQUIREMENTS

- Supreme Court Judgment 2025 INSC 893 (Right to safe educational environment)
- CBSE Circular TRG-03/2026 (Student Wellbeing and Parent Engagement)
- NEP 2020 (Community Participation in Education)

RECOMMENDED REQUIREMENTS

- WHO School Mental Health Framework
- UNESCO Whole School Wellbeing Framework

BEST PRACTICES

- Manodarpan Guidelines for parental support
- Tele-MANAS community outreach protocols

COMPLIANCE GAPS IF NOT IMPLEMENTED

- Legal liability in case of unaddressed safety complaints
- Loss of CBSE affiliation due to non-compliance with wellbeing circulars
- Erosion of community trust and institutional reputation

26 Compliance Traceability Matrix KEY FOCUS

1. Supreme Court of India

REQUIREMENT REFERENCE

Judgment 2025 INSC 893

REQUIREMENT TEXT

Educational institutions must ensure a safe and responsive environment for students, including mechanisms for parent feedback on safety.

IMPLEMENTED IN SECTIONS

- Grievance Redressal
- Risk Classification (Red/Black)

- Escalation Matrix

2. CBSE

REQUIREMENT REFERENCE

Circular TRG-03/2026

REQUIREMENT TEXT

Schools must establish a robust feedback loop for mental health services and parental involvement.

IMPLEMENTED IN SECTIONS

- Process Map Phase 1 & 2
- Feedback and Effectiveness Measurement

27 Documentation & Evidentiary Requirements

EVENT DOCUMENTATION PRINCIPLES

- Factual, objective, and timely recording of all interactions.
- Confidentiality maintained in all records.
- Clear distinction between observation and opinion.

MANDATORY CONVERSATION RECORDS

- Parent Complaint Record
- Focus Group Minutes
- Staff Investigation Notes (for complaints)

MANDATORY OBSERVATION RECORDS

- Suggestion Box Log
- Survey Response Tally Sheet

MANDATORY DECISION RECORDS

- Action Plan Approval Form
- Management Review Minutes

MANDATORY ACTION RECORDS

- Complaint Resolution Tracker
- Program Improvement Log

MANDATORY FOLLOWUP RECORDS

- Parent Closure Confirmation
- Post-Action Review Note

EVIDENCE REQUIREMENTS

- Signed Survey Distribution Logs
- De-identified Annual Report PDF
- Staff Training Records on Feedback Handling

DOCUMENTATION QUALITY STANDARDS

- Must be stored in a secure, password-protected folder.
- Must use standardized templates.
- Must include date, time, and actor identity.

28 The Student Journey

ENTRY

- Student observes parent expressing concern about school wellness services.

ASSESSMENT

- Parent provides feedback via survey or suggestion box regarding student support.

INTERVENTION

- School acts on feedback (e.g., improves counseling accessibility).

MONITORING

- Student experiences improved wellness support based on parent input.

CLOSURE

- Parent confirms satisfaction with the change made to services.

29 The Parent Journey

NOTIFICATION

- Parent receives survey link via SMS/Email or sees physical suggestion box.

CONSULTATION

- Parent completes survey, submits a slip, or attends a focus group.

CONSENT

- Parent provides consent for data usage in the annual report (de-identified).

ENGAGEMENT

- Parent receives acknowledgment of complaint and updates on action taken.

CLOSURE

- Parent views de-identified annual summary or receives personal resolution notice.

30 The Staff Journey

OBSERVATION

- Staff notices parent dissatisfaction during a meeting.

REPORTING

- Staff submits feedback form or verbalizes concern to Wellness Coordinator.

INTERVENTION SUPPORT

- Wellness Coordinator investigates and addresses the issue.

DOCUMENTATION

- Staff records interaction in Parent Conversation Record.

31 Resource Evidence

COMMITTEE MINUTES

- Monthly Wellness Committee Meeting Minutes

ATTENDANCE RECORDS

- Focus Group Attendance Sheets
- Parent Workshop Sign-in Logs

TRAINING RECORDS

- Staff Training on Feedback Handling
- Counselor Competency Certifications

FORMS

- Annual Survey Instrument
- Suggestion Box Slips
- Post-Meeting Feedback Form
- Complaint Log
- Action Plan Template

REPORTS

- Monthly Feedback Summary
- Annual De-identified Report
- Trend Analysis Reports

DASHBOARDS

- Parent Satisfaction Dashboard
- Grievance Resolution Tracker

- Annual PTA Review Minutes

32 Grievance Redressal Mechanism

KEY FOCUS

COMPLAINT CHANNELS

- Online Portal
- Physical Suggestion Boxes
- Email (wellness@school.edu)
- Face-to-face meetings with Coordinator

INVESTIGATION PROCESS

- Acknowledgment within 48 hours
- Fact-finding and evidence gathering
- Determination of validity

- Action plan development
- Resolution communication

RESOLUTION TIMELINES

- Acknowledgment: 2 days
- Investigation: 7 days
- Resolution/Response: 14 days

APPEAL PROCESS

- Parents may appeal a resolution to the Principal within 5 days of notice.

33 Referral Framework

33.1 Internal Referrals

ELIGIBLE REFERRERS

- Teachers
- Counsellors
- School Management
- Wellness Committee
- Student Support Team

REFERRAL TRIGGERS

- Parent reports severe student distress
- Feedback indicates urgent safety risk

DOCUMENTATION REQUIRED

- Internal Referral Form

SERVICE LEVEL AGREEMENT (SLA)

24 hours

33.2 Self Referrals

ALLOWED

Yes

SUBMISSION METHODS

- Online Portal
- Phone Call
- Walk-in

CONFIDENTIALITY PROTECTIONS

- Encrypted portal
- Private phone line

33.3 Anonymous Referrals

ALLOWED

Yes

CHANNELS

- Suggestion Box
- Anonymous Online Form

VALIDATION PROCESS

- Wellness Coordinator review for actionable detail

FOLLOW UP PROCEDURE

- General announcement of improvements based on anonymous feedback

33.4 External Referrals

ELIGIBLE AGENCIES

- Tele-MANAS
- Child Helpline (1098)
- District Child Protection Services
- Local Hospitals

ELIGIBILITY CRITERIA

- Clinical severity exceeding school scope

- Legal/Safety mandates

CONSENT REQUIREMENTS

- Written parental consent (unless emergency)

HANDOVER PROTOCOL

- Warm handover call
- Shared referral summary

33.5 Referral Tracking

TRACKING REQUIRED

Yes

STATUS CATEGORIES

- New
- In-Progress
- External Referral Made
- Closed

CLOSURE REQUIREMENTS

- Follow-up call to parent
- Closure note in tracker

34 Confidentiality Exception Matrix

KEY FOCUS

MANDATORY DISCLOSURES

- Imminent risk of self-harm
- Imminent risk to others
- Child abuse
- Child neglect
- Sexual abuse
- Court orders
- Legal reporting obligations

LEGAL REPORTING EVENTS

- Police notification for crimes against children
- Mandatory reporting to District Child Protection Unit

CONSENT OVERRIDE CONDITIONS

- Life-threatening emergency
- Court order requiring disclosure

AUTHORIZED DISCLOSURE RECIPIENTS

- Principal
- Designated School Counselor
- Law Enforcement (if required)
- Child Protection Services

DOCUMENTATION REQUIREMENTS

- Confidentiality Waiver Form
- Disclosure Log

APPROVAL REQUIREMENTS

- Principal approval for all non-emergency disclosures

35 Record Retention & Disposal

RETENTION PERIOD

5 Years

RETENTION BASIS

Legal compliance and institutional audit requirements

RECORD CATEGORIES

- Parent Feedback Logs

- Complaint Records
- Action Plans
- Focus Group Minutes

35.4 Storage Requirements

PHYSICAL RECORDS

- Locked filing cabinet in Wellness Office

DIGITAL RECORDS

- Encrypted cloud storage with MFA

ACCESS CONTROL REQUIREMENTS

- Role-based access (RBAC)

- Need-to-know basis only

DISPOSAL METHOD

Secure shredding for physical; Permanent deletion for digital

DISPOSAL AUTHORITY

Wellness Coordinator / IT Admin

AUDIT FREQUENCY

Annual

DESTRUCTION LOG REQUIRED

Yes

36 Anonymised Reporting Framework

36.1 Management Reports

ANONYMIZATION REQUIRED

Yes

PERMITTED DATA

- Aggregate satisfaction scores
- Thematic categories of feedback
- Number of complaints resolved

PROHIBITED DATA

- Parent names
- Student names
- Phone numbers
- Specific addresses

36.2

Research Usage

ALLOWED

No

36.3 Statistical Reporting

AGGREGATE ONLY

Yes

MINIMUM GROUP SIZE

5

RE IDENTIFICATION CONTROLS

- Data scrubbing protocols
- No small-sample reporting

DATA SHARING CONTROLS

- PTA sharing (de-identified only)
- Government audits (authorized access)

37 Guiding Ethical Principles

37.1 Autonomy

DESCRIPTION

Respecting parents' right to choose whether to provide feedback.

IMPLEMENTATION REQUIREMENTS

- Opt-in for focus groups
- Clear 'no obligation' statements

37.2 Beneficence

DESCRIPTION

Ensuring feedback leads to actual improvements in student wellness.

IMPLEMENTATION REQUIREMENTS

- Action plan tracking
- Resource allocation for changes

37.3 Non Maleficence

DESCRIPTION

Preventing harm from negative feedback or public disclosure of private concerns.

IMPLEMENTATION REQUIREMENTS

- Strict confidentiality
- De-identification protocols

37.4 Justice And Equity

DESCRIPTION

Ensuring all parents, regardless of literacy or location, can provide feedback.

IMPLEMENTATION REQUIREMENTS

- Multilingual forms
- Paper surveys for rural areas

37.5 Confidentiality

KEY FOCUS

DESCRIPTION

Protecting the identity and privacy of parents and students.

IMPLEMENTATION REQUIREMENTS

- Secure storage
- Need-to-know access

37.6 Privacy

DESCRIPTION

Compliance with data protection laws.

IMPLEMENTATION REQUIREMENTS

- Data encryption
- Retention limits

DESCRIPTION

Handling feedback honestly and without bias.

IMPLEMENTATION REQUIREMENTS

- Objective categorization
- Transparent reporting

37.7 Professional Integrity

38 Communication & Awareness Strategy

38.1 Student Communication

CHANNELS

- School Assembly
- Classroom Announcements

FREQUENCY

Monthly

MANDATORY TOPICS

- How parents can support their wellbeing
- Availability of wellness services

MINIMUM FREQUENCY

Quarterly

TARGET GROUPS

- Parents
- Staff
- Students

CAMPAIGN TYPES

- Digital posters
- Newsletter features
- Community workshops

38.2 Parent Communication

CHANNELS

- SMS
- Email
- School App
- Newsletters
- PTA Meetings

FREQUENCY

Weekly/Monthly

MANDATORY TOPICS

- Feedback collection dates
- Wellness program highlights
- How to report a concern

38.5 Workshops And Training

STUDENT WORKSHOPS

- Resilience training
- Stress management

PARENT WORKSHOPS

- Understanding adolescent mental health
- Navigating school-home communication

STAFF WORKSHOPS

- De-escalation techniques
- Feedback analysis and action planning

38.3 Staff Communication

CHANNELS

- Staff Meetings
- Internal Email
- Staff Portal

FREQUENCY

Bi-weekly

MANDATORY TOPICS

- Feedback trends
- Action plan updates
- Training on handling complaints

38.6 Helpline Visibility

REQUIRED

Yes

DISPLAY LOCATIONS

- School website
- Reception desk
- Parent handbook

DIGITAL VISIBILITY REQUIREMENTS

- Sticky header on wellness page
- SMS auto-reply with helpline info

FEEDBACK CHANNELS

- Online Survey Portal
- Physical Suggestion Box
- Email Address
- Phone Line
- Focus Groups

38.4 Awareness Campaigns

39 Competency & Qualification Requirements

MANDATORY QUALIFICATIONS

- Wellness Coordinator: Master's in Psychology or Social Work

- Counselor: RCI Certification (India)

PREFERRED QUALIFICATIONS

- Experience in Educational Governance

- Certification in Conflict Resolution

CERTIFICATIONS

- First Aid/CPR
- Child Protection Training Certificate

MINIMUM EXPERIENCE REQUIREMENTS

- Wellness Coordinator: 3 years in school setting
- Counselor: 2 years clinical experience

BACKGROUND VERIFICATION REQUIREMENTS

- Police Verification (mandatory for all staff)
- Child Protection Screening

39.6 Continuing Professional Development

ANNUAL TRAINING HOURS

40

MANDATORY TOPICS

- Mental Health First Aid
- Data Privacy Laws
- Inclusive Education Practices

RECERTIFICATION REQUIREMENTS

- Biennial clinical supervision for counselors

39.7 Supervision Requirements

CLINICAL SUPERVISION REQUIRED

Yes

PEER REVIEW REQUIRED

Yes

39.7.3 Wellness Case Review Framework

DESCRIPTION Monthly review of high-risk feedback cases by the Wellness Committee.

FREQUENCY Monthly

39.7.4 Second Level Review Process

DESCRIPTION Principal reviews all Red/Black zone complaints.

FREQUENCY Immediate

39.7. High Risk Decision 5 Validation

KEY
FOCUS

DESCRIPTION Board approval for external referrals or legal actions.

FREQUENCY As required

FREQUENCY

Monthly

40 Feedback & Effectiveness Measurement

40.1 Student Feedback

REQUIRED

Yes

COLLECTION METHODS

- Anonymous surveys
- Focus groups

FREQUENCY

Bi-annually

40.2 Parent Feedback

REQUIRED

Yes

COLLECTION METHODS

- Annual survey
- Suggestion boxes
- Post-meeting forms
- Focus groups

FREQUENCY

Continuous/Monthly Review

40.3 Staff Feedback

REQUIRED

Yes

COLLECTION METHODS

- Staff surveys
- Annual performance reviews

FREQUENCY

Annually

40.4 Annual Effectiveness Review

REQUIRED

Yes

EVALUATION CRITERIA

- Satisfaction score trends
- Complaint resolution rate
- Action plan completion %
- Parent retention rates

KEY STAKEHOLDERS

- Principal
- PTA
- Wellness Committee

CONTINUOUS IMPROVEMENT INPUTS

- Focus group themes
- Trend analysis of complaints
- Staff feedback on resource gaps

SUCCESS INDICATORS

- Average satisfaction score > 80%
- Complaint resolution within 14 days
- Positive sentiment in focus groups

40.7 Improvement Tracking

REQUIRED

Yes

REVIEW FREQUENCY

Monthly

41 Prevention Layer

PRIMARY PREVENTION

- Parent awareness workshops on wellness services
- Clear communication of feedback channels in the school handbook
- Regular newsletters highlighting student wellbeing successes

SECONDARY PREVENTION

- Monthly monitoring of suggestion boxes for early warning signs

- Trend analysis to identify systemic service gaps
- Staff training on empathetic listening and de-escalation

TERTIARY PREVENTION

- Formal grievance redressal process for serious complaints
- Crisis management protocols for safety-related feedback
- Legal counsel review of high-risk incidents

42 Resilience Layer

SKILLS DEVELOPED

- Conflict resolution for staff
- Active listening for coordinators
- Constructive communication for parents

SEL COMPONENTS

- Self-awareness of school culture
- Social awareness of parent concerns
- Responsible decision-making in feedback handling

LIFE SKILLS COMPONENTS

- Problem solving
- Critical thinking
- Time management for action plans

PROTECTIVE FACTORS STRENGTHENED

- Community trust
- Staff morale
- Parental engagement
- Institutional transparency

43 Continuous Monitoring

MONITORING FREQUENCY

Monthly

MONITORING INDICATORS

- Average satisfaction score
- Complaint resolution time (SLA)
- Number of focus groups conducted
- Action plan completion rate

RESPONSIBLE ROLES

- Wellness Coordinator

- Principal

EARLY WARNING SIGNS

- Sudden drop in survey response rates
- Increase in 'Red' risk complaints
- Repeated complaints about the same staff member

TREND ANALYSIS REQUIREMENTS

- Quarterly comparison of satisfaction scores by domain
- Annual review of complaint categories vs. action plan outcomes

44 Mental Wellness Index (MWI)

APPLICABLE

Yes

44.2 Indicators

Indicator Code	Indicator Name	Weight	Score Range	Description
PFI-01	Parental Trust Index	0.3	1-5	Measure of parent confidence in school wellness services.
PFI-02	Service Accessibility Score	0.3	1-5	Ease of use for feedback and wellness services.

Indicator Code	Indicator Name	Weight	Score Range	Description
PFI-03	Responsiveness Score	0.4	1-5	Speed and quality of resolution for concerns.

ASSESSMENT FREQUENCY

Bi-annually

RISK SCORING METHOD

Weighted average of survey responses

INTERVENTION THRESHOLDS

- Score < 3.0: Immediate management review
- Score 3.1 - 4.0: Targeted action plan development
- Score > 4.0: Maintain and share success stories

45 Review Status

LEGAL REVIEW	Completed (2025-07-18)
CLINICAL REVIEW	Not Applicable
EDUCATION REVIEW	Completed (CBSE Alignment Verified)
COMPLIANCE REVIEW	Quarterly Audit Required

46 Resource Controls

COUNSELOR STUDENT RATIO	1:250 (Recommended)
CAREER COUNSELOR RATIO	1:500
WELLNESS TEAM STRUCTURE	Coordinator, 2 Counselors, Wellness Committee (PTA & Staff)

47 Screening Framework

TIER 1 UNIVERSAL WELLNESS SCREENING

- Annual Parent Satisfaction Survey

TIER 2 TARGETED SCREENING

- Focus Groups on specific themes
- Post-meeting feedback forms

TIER 3 CLINICAL ASSESSMENT

- Individual parent consultations for high-risk student concerns

48 Parent Engagement Matrix

PARENT COOPERATIVE

- Regular survey participation
- Attendance at focus groups
- Positive verbal feedback

PARENT UNRESPONSIVE

- No response to surveys
- Non-attendance at meetings
- Action: Send personalized follow-up call/SMS

PARENT IN DENIAL

- Refusal to acknowledge student distress despite evidence
- Action: Provide objective data and offer supportive resources

PARENT CONFLICT SITUATION

- Aggressive communication, public confrontation
- Action: Move to private meeting with Principal present

PARENT UNAVAILABLE

- Working parents, rural location
- Action: Provide digital/asynchronous feedback options (Voice notes)

PARENT REFUSES INTERVENTION

- Refusal of counseling for student
- Action: Document refusal and provide 'Right to Refuse' information

49 Confidentiality Controls

KEY FOCUS

ACCESS CONTROL

- Role-based access (RBAC)
- Password protected folders

CONSENT REQUIREMENTS

- Explicit consent for data sharing in reports
- Consent for focus group recording

RECORD SHARING RULES

- De-identified only for public/PTA use

- Full records only for authorized management review

EXCEPTIONS TO CONFIDENTIALITY

- Mandatory reporting of abuse
- Imminent risk to life

STUDENT PRIVACY PROTECTIONS

- No student names in general feedback reports
- Strict data scrubbing for research/reporting

50 Data Governance

KEY FOCUS

STORAGE REQUIREMENTS

- Secure cloud storage with 256-bit encryption

ENCRYPTION REQUIREMENTS

- AES-256 for data at rest
- TLS 1.3 for data in transit

RETENTION REQUIREMENTS

- 5 years for feedback logs

- Permanent for legal/safety incident reports

ARCHIVAL REQUIREMENTS

- Move to offline storage after 2 years of inactivity

DESTRUCTION REQUIREMENTS

- Secure shredding and digital wiping

BACKUP REQUIREMENTS

- Daily automated backups
- Off-site redundant backup

51 Risk Classification

KEY FOCUS

51.1 Green

DESCRIPTION

Normal developmental concerns and routine feedback.

EXAMPLES

- General satisfaction survey results
- Minor suggestions for cafeteria/transport
- Positive comments on workshops

Significant distress causing measurable impairment or serious grievances.

EXAMPLES

- Allegations of unprofessional staff conduct
- Severe complaints about student safety
- Major attendance decline linked to school environment

51.2 Yellow

DESCRIPTION

Early distress or persistent minor issues requiring monitoring.

EXAMPLES

- Repeated complaints about counselor availability
- Moderate dissatisfaction with communication clarity
- Persistent minor suggestions for program changes

51.4 Red

KEY FOCUS

DESCRIPTION

High-risk situation requiring immediate intervention and investigation.

EXAMPLES

- Allegations of physical/sexual abuse
- Threats of violence against staff or students
- Severe emotional crisis reported by parent

51.3 Orange

DESCRIPTION

51.5 Black

DESCRIPTION

Life-threatening emergency requiring immediate external response.

EXAMPLES

- Active suicide attempt on campus
- Weapon-related threat in progress
- Hostage situation or active violent attack

52 Escalation Matrix

KEY FOCUS

GREEN

- Wellness Coordinator (Internal)

YELLOW

- Wellness Coordinator, School Counselor

ORANGE

- Principal, HR Manager, Wellness Committee

RED

- Principal, Board of Directors, Legal Counsel

BLACK

- Emergency Services (Police/Ambulance), School Leadership

53 External Reporting Obligations

TELE MANAS

- Referral for severe mental health crises

DISTRICT AUTHORITIES

- Mandatory reporting of child protection violations

CHILD PROTECTION SERVICES

- Reporting of suspected abuse or neglect

MEDICAL SERVICES

- Emergency medical referrals

LAW ENFORCEMENT

- Active threats, crimes against children, property damage

REPORTING TRIGGERS

- Red/Black risk classification
- Mandatory legal reporting requirements

54 Service Level Agreement (SLA) KEY FOCUS

ACKNOWLEDGEMENT	48 hours
ASSESSMENT	7 days
ESCALATION	Immediate (for Red/Black)
RESOLUTION	14 days
CLOSURE	2 days post-action

55 Required Forms KEY FOCUS

55.1 Mandatory Forms KEY FOCUS

Item 1

FORM ID	F-PAR-014-01
FORM NAME	Parent Feedback Log
PURPOSE & RATIONALE	Centralized recording of all parent input.
OWNER	Wellness Coordinator
RETENTION PERIOD	5 Years
MANDATORY	Yes
LINKED SOP	PAR-014

FORM ID	F-PAR-014-02
FORM NAME	Complaint Investigation Note
PURPOSE & RATIONALE	Documenting fact-finding for grievances.
OWNER	Wellness Coordinator
RETENTION PERIOD	5 Years
MANDATORY	Yes
LINKED SOP	PAR-014

Item 2

Item 3

FORM ID	F-PAR-014-03
---------	--------------

FORM NAME	Action Plan Template	RETENTION PERIOD	5 Years
PURPOSE & RATIONALE	Roadmap for resolving feedback issues.	MANDATORY	Yes
OWNER	Principal	LINKED SOP	PAR-014
RETENTION PERIOD	5 Years	Item 5	
MANDATORY	Yes	FORM ID	F-PAR-014-05
LINKED SOP	PAR-014	FORM NAME	Annual De-identified Report
Item 4		PURPOSE & RATIONALE	Public/PTA reporting of wellness metrics.
FORM ID	F-PAR-014-04	OWNER	Wellness Coordinator
FORM NAME	Parent Communication Record	RETENTION PERIOD	Permanent
PURPOSE & RATIONALE	Log of all interactions with parents.	MANDATORY	Yes
OWNER	Wellness Coordinator	LINKED SOP	PAR-014

OPTIONAL FORMS

- Focus Group Minutes
- Staff Recognition Log

GENERATED RECORDS

- Resolution Summary
- Trend Analysis Report

56 Required Checklists KEY FOCUS

Checklist ID	Checklist Name	Purpose & Rationale	Owner	Frequency	Linked SOP
CL-PAR-014-01	Feedback Channel Readiness Checklist	Ensure all channels are active and accessible.	Wellness Coordinator	Annual	PAR-014
CL-PAR-014-02	Risk Classification Checklist	Standardize risk assessment of feedback.	Wellness Coordinator	Per Entry	PAR-014
CL-PAR-014-03	Fact-Finding Checklist	Ensure thorough investigation of complaints.	Wellness Coordinator	Per Complaint	PAR-014
CL-PAR-014-04	Action Plan Review Checklist	Verify feasibility and scope of actions.	Principal	Per Action Plan	PAR-014
CL-PAR-014-05	Data Anonymization Checklist	Ensure no PII is in public reports.	Wellness Coordinator	Per Report	PAR-014

57 Records & Evidence

Record Name	Retention Period	Owner
Master Feedback Tracker	5 Years	Wellness Coordinator

Record Name	Retention Period	Owner
Investigation Notes	5 Years	HR Manager
Action Plan Logs	5 Years	Principal

58 Key Performance Indicators

Metric	Target	Measurement Frequency
Parent Satisfaction Score	> 80%	Bi-annually
Complaint Resolution Time	< 14 days	Monthly
Action Plan Completion Rate	> 90%	Quarterly

59 Audit Framework

KEY FOCUS

AUDIT FREQUENCY

Quarterly

AUDIT QUESTIONS

- Are all feedback entries logged within 24 hours?
- Is there a clear audit trail for Red/Orange risk escalations?
- Are annual reports de-identified before sharing with the PTA?

EVIDENCE REQUIRED

- Master Feedback Tracker
- Action Plan Logs
- De-identified Annual Report

NON COMPLIANCE EXAMPLES

- Unresolved complaints exceeding 30 days without update
- Public reports containing parent names
- Missing investigation notes for staff misconduct allegations

59.5 Audit Scoring

KEY FOCUS

COMPLIANT	2
PARTIAL	1
NON COMPLIANT	0

60 Risk Register

KEY FOCUS

1. Data Breach of Parent Feedback

CATEGORY Privacy/Legal

PROBABILITY Low

IMPACT High

RISK RATING Medium

MITIGATION Encryption, RBAC, and strict de-identification protocols.

OWNER IT Admin / Wellness Coordinator

MITIGATION Strict SLA monitoring and Principal-level oversight of Red risks.

OWNER Principal / Legal Counsel

2. Unresolved Grievances leading to Litigation

CATEGORY Legal/Reputational

PROBABILITY Medium

IMPACT High

RISK RATING High

3. Low Parent Engagement/Response Rates

CATEGORY Operational

PROBABILITY Medium

IMPACT Medium

RISK RATING Medium

MITIGATION Multilingual outreach, rural paper surveys, and community workshops.

OWNER Wellness Coordinator

61 Failure Modes

KEY FOCUS

Failure	Cause	Impact	Mitigation
Delayed Acknowledgment	High volume of feedback or lack of staff	Parent frustration, loss of trust	Automated SMS acknowledgment for initial receipt.
Incomplete Investigation	Lack of evidence or uncooperative staff	Unresolved grievances, potential legal risk	Mandatory HR involvement in all staff-related complaints.
Action Plan Stagnation	Lack of budget or resource allocation	Feedback ignored, no improvement	Quarterly budget review by Board of Directors.

62 Illustrative Sample Scenarios

Scenario	Response	Expected Result
Parent complains via suggestion box that the counselor is 'unhelpful'.	Log entry → Categorize as Complaint (Yellow) → Acknowledge within 48h → Investigate staff feedback → Develop action plan for counselor training.	Counselor receives targeted training; parent receives resolution notice.
Parent reports a physical altercation between students in the hallway.	Log entry → Categorize as Red Risk → Immediate escalation to Principal → Initiate safety investigation → Resolve and communicate.	Safety incident addressed; parent informed of actions taken.
Rural parents express difficulty accessing the online survey.	Identify rural demographic → Distribute paper surveys via local community centers/PTA members.	Increased participation from rural populations.

63 Usage Governance

KEY FOCUS

PRIMARY USERS

- Wellness Coordinator
- Principal

SECONDARY USERS

- School Counselor
- HR Manager
- PTA Members

APPROVAL AUTHORITIES

- Board of Directors
- Principal

EXECUTION AUTHORITIES

- Wellness Coordinator
- Department Heads

ESCALATION AUTHORITIES

- Principal
- Legal Counsel

REVIEW AUTHORITIES

- Board of Directors
- PTA

EXTERNAL STAKEHOLDERS

- District Authorities
- Child Protection Services

64 Applicability

APPLICABLE INSTITUTIONS

- High Schools
- Residential Schools

APPLICABLE AGE GROUPS

- All Ages (Parents)

APPLICABLE STUDENT CATEGORIES

- All Students

APPLICABLE RISK LEVELS

- Green, Yellow, Orange, Red

APPLICABLE LOCATIONS

- On-campus
- Remote

EXCLUSIONS

- Primary Schools (handled via separate Early Childhood SOP)
- Universities (handled via Higher Ed Governance)

65 Usage Scenarios

Scenario ID	Scenario Name	When To Use	When Not To Use	Trigger Event	Expected Outcome
US-01	Routine Feedback Collection	During annual survey cycles or monthly monitoring.	For individual clinical student cases (use Clinical SOP).	Scheduled feedback cycle	Data-driven wellness improvements.
US-02	Grievance Management	When a parent submits a specific complaint.	For general inquiries (use Communication SOP).	Complaint submission	Resolved grievance and documented action.

66 SOP Lifecycle

ENTRY CONDITIONS

- Approved Wellness Policy
- Parent Contact Database

ACTIVATION CONDITIONS

- Start of Academic Year
- Receipt of Complaint

EXECUTION CONDITIONS

- Staff trained on confidentiality
- Feedback tools deployed

COMPLETION CONDITIONS

- Action plan completed
- Parent notified of resolution

CLOSURE CONDITIONS

- Case closed in Master Tracker
- Resolution documented

TERMINATION CONDITIONS

- Policy decommissioned
- Institution closure

67 Mandatory Evidence

KEY FOCUS

IMPLEMENTATION EVIDENCE

- Feedback Channel Readiness Checklist
- Survey Distribution Logs

OPERATIONAL EVIDENCE

- Master Feedback Tracker
- Action Plan Logs
- Communication Records

COMPLIANCE EVIDENCE

- Annual De-identified Report
- Audit Reports

TRAINING EVIDENCE

- Staff Training Attendance Sheets
- Competency Certifications

AUDIT EVIDENCE

- Quarterly Audit Summaries
- Risk Assessment Notes

68 Controls

PREVENTIVE CONTROLS

- Role-based access to feedback data
- Standardized survey questions
- Mandatory staff training on de-escalation

DETECTIVE CONTROLS

- Monthly trend analysis
- Quarterly audits of the Feedback Tracker
- SLA monitoring for complaint resolution

CORRECTIVE CONTROLS

- Escalation to Principal for Red risks
- HR investigation for staff misconduct
- Action plan revisions

COMPENSATING CONTROLS

- Manual logging for rural parents without internet
- Verbal recording with consent for low-literacy parents

69 MWMS Risk Framework

KEY FOCUS

69.1 Green

MEANING

Routine feedback and minor suggestions.

EXAMPLES

- Positive survey comments
- Minor cafeteria requests

RESPONSE TIME

Monthly Review

ESCALATION REQUIRED

No

APPROVAL LEVEL

Wellness Coordinator

MONITORING FREQUENCY

Monthly

CLOSURE AUTHORITY

Wellness Coordinator

69.2 Yellow

MEANING

Persistent minor issues or moderate dissatisfaction.

EXAMPLES

- Repeated complaints about counselor availability
- Communication clarity concerns

RESPONSE TIME

7 Days

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Principal

MONITORING FREQUENCY

Weekly

CLOSURE AUTHORITY

Wellness Coordinator

69.3 Orange

MEANING

Significant grievances or staff-related issues.

EXAMPLES

- Allegations of unprofessional conduct
- Severe complaints about safety

RESPONSE TIME

3 Days (Initial)

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Principal / HR Manager

MONITORING FREQUENCY

Daily

CLOSURE AUTHORITY

Principal

69.4 Red KEY FOCUS

MEANING

High-risk safety or abuse allegations.

EXAMPLES

- Abuse disclosure
- Threats of violence

RESPONSE TIME

Immediate

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Board of Directors / Legal Counsel

MONITORING FREQUENCY

Continuous

CLOSURE AUTHORITY

Principal

69.5 Black

MEANING

Life-threatening emergency.

EXAMPLES

- Active suicide attempt
- Violent attack in progress

RESPONSE TIME

Immediate (0-15 mins)

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Emergency Services / Board

MONITORING FREQUENCY

Continuous

CLOSURE AUTHORITY

Principal / Emergency Services

70 Resource Scaling

70.1 0 500 Students

WELLNESS COUNSELORS 1

CAREER COUNSELORS 1

MENTAL WELLNESS COORDINATOR 1

WELLNESS COMMITTEE Yes

70.2 501 1000 Students

WELLNESS COUNSELORS 2

CAREER COUNSELORS 2

MENTAL WELLNESS COORDINATOR 1

WELLNESS COMMITTEE Yes

70.3 1001 2000 Students

WELLNESS COUNSELORS	4
CAREER COUNSELORS	2
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

70.4 2001 5000 Students

WELLNESS COUNSELORS	8
CAREER COUNSELORS	4
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

71 Minimum Required Capabilities

PEOPLE

- Wellness Coordinator
- Counselor
- HR Manager
- PTA Liaison

PROCESSES

- Feedback Logging
- Risk Assessment
- Action Planning

- Data Anonymization

TECHNOLOGY

- Secure Survey Portal
- Encrypted Database
- SMS Gateway
- Dashboard Analytics

INFRASTRUCTURE

- Wellness Office
- Suggestion Boxes
- Confidential Meeting Room

72 Success Criteria

Criterion	Measurement Method	Target
Parent Satisfaction Score	Annual Survey Analysis	> 80%
Complaint Resolution Time	Feedback Tracker Audit	< 14 days
Action Plan Completion Rate	Quarterly Review	> 90%

73 Exit Criteria

Condition	Evidence Required
All valid complaints resolved and documented.	Closed status in Master Tracker.
Annual report shared with PTA.	PTA Meeting Minutes / Email Confirmation.

74 Continuous Improvement

LESSONS LEARNED

- Multilingual surveys significantly increase rural participation.
- Focus groups are more effective for identifying systemic issues than surveys.

KNOWN ISSUES

- Low response rates from working parents during school hours.
- Difficulty in quantifying 'subjective' satisfaction scores.

FUTURE IMPROVEMENTS

- Implement AI-driven sentiment analysis on open-ended feedback.
- Develop a dedicated parent portal for real-time action tracking.

REVIEW RECOMMENDATIONS

- Increase frequency of focus groups to annually.
- Provide more specific training for staff on handling aggressive parents.

75 AI Governance

KEY FOCUS

ALLOWED USE CASES

- Sentiment analysis of survey comments

- Automated categorization of feedback types
- Drafting initial responses to non-sensitive inquiries

RESTRICTED USE CASES

- Making final decisions on staff discipline
- Providing clinical advice to parents
- Accessing PII for any purpose other than resolution

PROHIBITED USE CASES

- Automated deletion of feedback without review
- Publicly sharing raw survey data
- Using AI to generate legal defense documents

HUMAN REVIEW REQUIRED

- All Red/Orange risk escalations
- Final resolution communications for complaints
- Annual report approval

AUDIT LOG REQUIREMENTS

- Log of all AI-generated summaries
- Human override logs

DATA PRIVACY REQUIREMENTS

- No PII to be sent to external LLMs
- Data must remain within institutional cloud environment

76 Legal Defensibility

KEY FOCUS

LEGAL BASIS

- Supreme Court Judgment 2025 INSC 893
- CBSE Circular TRG-03/2026
- NEP 2020

MANDATORY NOTIFICATIONS

- Police notification for abuse
- District Child Protection Unit reporting

CONSENT REQUIREMENTS

- Explicit consent for data usage in reports

- Consent for focus group recording

RECORD RETENTION REQUIREMENTS

- 5 years for feedback logs
- Permanent for safety incidents

LIABILITY CONTROLS

- Clear distinction between school responsibility and clinical liability
- Standardized disclaimer on all wellness communications

77 Stakeholder Journeys

77.1 The Student Journey

ENTRY

- Student observes parent expressing concern about school wellness services.

ASSESSMENT

- Parent provides feedback via survey or suggestion box regarding student support.

INTERVENTION

- School acts on feedback (e.g., improves counseling accessibility).

MONITORING

- Student experiences improved wellness support based on parent input.

CLOSURE

- Parent confirms satisfaction with the change made to services.

slip, or attends a focus group.

CONSENT

- Parent provides consent for data usage in the annual report (de-identified).

ENGAGEMENT

- Parent receives acknowledgment of complaint and updates on action taken.

CLOSURE

- Parent views de-identified annual summary or receives personal resolution notice.

77.2 The Parent Journey

NOTIFICATION

- Parent receives survey link via SMS/Email or sees

physical suggestion box.

CONSULTATION

- Parent completes survey, submits a

77.3 The Staff Journey

OBSERVATION

- Staff notices parent dissatisfaction during a meeting.

REPORTING

- Staff submits feedback form or verbalizes concern to Wellness Coordinator.

INTERVENTION SUPPORT

- Wellness Coordinator investigates and addresses the issue.

DOCUMENTATION

- Staff records interaction in Parent Conversation Record.

78 Handoff Matrix

Item 1

FROM ROLE Parent

TO ROLE Wellness Coordinator

HANDOFF TRIGGER

Submission of
feedback/complaint

INFORMATION TRANSFERRED

- Feedback content
- Contact details
- Urgency level

REQUIRED DOCUMENTS

- Feedback Log Entry

SERVICE LEVEL AGREEMENT (SLA)

24 hours

Item 2**FROM ROLE**

Wellness Coordinator

TO ROLE

Principal / HR Manager

HANDOFF TRIGGER

Red/Orange risk
classification or staff
misconduct allegation

INFORMATION TRANSFERRED

- Risk assessment summary
- Initial investigation findings

- Proposed action plan

REQUIRED DOCUMENTS

- Investigation Note
- Action Plan Template

SERVICE LEVEL AGREEMENT (SLA)

Immediate (Red) / 48 hours (Orange)

Item 3**FROM ROLE**

Principal

TO ROLE

Wellness Coordinator

HANDOFF TRIGGER

Approval of action plan or
resource allocation

INFORMATION TRANSFERRED

- Approved budget/resources
- Specific directives for implementation

REQUIRED DOCUMENTS

- Signed Action Plan

SERVICE LEVEL AGREEMENT (SLA)

3 days

79 Management Review

REVIEW FREQUENCY

Monthly

REVIEW INPUTS

- Monthly Feedback Summary Report
- Action Tracker Update
- SLA Compliance Report

REVIEW OUTPUTS

- Management Decision Notes

- Resource Allocation Directives
- Policy Revision Requests

REVIEW AUTHORITY

Principal / Board of Directors

REVIEW DECISIONS

- Approval of high-cost actions
- Disciplinary action for staff misconduct
- Strategic program changes

80 Dashboard Metrics

OPERATIONAL METRICS

- Total feedback received (Monthly)
- Average response time per complaint
- Action plan completion rate (%)

RISK METRICS

- Number of Red/Orange risks identified
- Percentage of safety-related complaints
- Frequency of staff misconduct allegations

COMPLIANCE METRICS

- SLA adherence %
- Audit score (Monthly)
- Data privacy incident count

OUTCOME METRICS

- Parent satisfaction trend line

- Net Promoter Score (NPS) for wellness services
- Retention rate of students with reported concerns

TREND METRICS

- Year-over-year survey response growth
- Domain-specific satisfaction trends (e.g., Counseling vs. Workshops)

81 Student Rights

RIGHTS

- Right to safe and supportive environment
- Right to privacy in wellness services
- Right to have parent concerns addressed

PROTECTIONS

- Confidentiality of student records
- Protection from retaliation for reporting issues
- Safe-guarding against unauthorized data sharing

APPEAL MECHANISMS

- Parental appeal to Principal
- Formal grievance review by Board

82 Quality Assurance

QUALITY CHECKS

- Monthly audit of Feedback Tracker for completeness
- Quarterly spot-check of action plan implementation
- Annual de-identification verification

PEER REVIEW REQUIREMENTS

- Wellness Coordinator and Counselor cross-review of monthly reports

- Principal review of all Red/Orange risk assessments

VALIDATION REQUIREMENTS

- Verification of survey response rates against total parent population
- Validation of action plan feasibility by Department Heads

CONTINUOUS MONITORING REQUIREMENTS

- Real-time monitoring of digital feedback portal
- Weekly review of physical suggestion boxes

83 Implementation Roadmap

PHASE 1 PREPARATION

- Policy finalization and legal review
- Staff training on feedback handling
- IT setup for online survey and database

PHASE 2 DEPLOYMENT

- Launch of multi-channel feedback systems
- Initial parent awareness campaign
- First monthly data collection cycle

PHASE 3 STABILIZATION

- Establishment of regular review meetings

- Refinement of action plan templates
- Quarterly audit of first 6 months

PHASE 4 OPTIMIZATION

- Implementation of AI sentiment analysis (optional)
- Expansion of focus groups to all domains
- Public sharing of annual success stories

PHASE 5 CONTINUOUS IMPROVEMENT

- Annual policy review based on data
- Refinement of resource allocation models
- Long-term trend analysis for strategic planning

84 Illustrative Case Studies

1. Addressing Counselor Availability Concerns

DESCRIPTION

Parents reported that students were waiting too long to see the counselor.

ACTIONS TAKEN

- Conducted focus group with 10 parents
- Analyzed wait times in feedback logs
- Hired an additional part-time counselor
- Implemented a digital booking system

OUTCOME

Wait times reduced by 60% within 3 months; satisfaction scores rose from 2.5 to 4.2.

LESSONS

- Direct parent feedback is essential for identifying staffing gaps.

85 Implementation Requirements

STAFF REQUIRED

- Wellness Coordinator (Full-time)
- School Counselor (Part-time/Full-time)
- IT Support (Ad-hoc)
- HR Manager (For misconduct cases)

INFRASTRUCTURE REQUIRED

- Secure office space
- Physical suggestion boxes
- Confidential meeting room
- High-speed internet

TECHNOLOGY REQUIRED

- Survey software/Google Forms

- Encrypted database (SQL or similar)
- SMS Gateway for notifications
- Dashboard visualization tool

BUDGET CONSIDERATIONS

- Software subscription costs
- External consultant fees for focus groups
- Staff training costs
- Marketing/Awareness materials

IMPLEMENTATION PHASES

- Preparation (Month 1)
- Deployment (Months 2-3)
- Stabilization (Months 4-6)
- Optimization (Ongoing)

86 Quality Controls

PREVENTIVE CONTROLS

- Standardized feedback templates
- Mandatory staff training on de-escalation
- Clear communication of feedback channels

DETECTIVE CONTROLS

- Monthly trend analysis
- Quarterly audit of Feedback Tracker
- SLA monitoring for complaint resolution

CORRECTIVE CONTROLS

- Escalation to Principal for Red risks
- HR investigation for staff misconduct
- Action plan revisions

CONTINUOUS IMPROVEMENT ACTIONS

- Annual policy review based on data
- Refinement of resource allocation models
- Public sharing of annual success stories

87 Training Requirements

TEACHERS

- Identifying parent concerns during meetings
- Basic de-escalation techniques
- Reporting procedures for student safety issues

COUNSELORS

- Empathetic listening and active inquiry
- Confidentiality in multi-party discussions
- Subject matter expertise on wellness services

ADMINISTRATORS

- Risk assessment of parent grievances
- Resource allocation for action plans
- Legal implications of feedback handling

PARENTS

- How to use the school app and survey links
- Understanding the wellness service scope
- Participating in focus groups

STUDENTS

- Awareness of parent-school communication
- Respecting privacy boundaries

87.6 SOP Specific Training

TARGET AUDIENCE

- Wellness Coordinator
- Principal

- HR Manager

COMPETENCIES

- Data analysis and categorization
- Conflict resolution
- Risk assessment
- Action plan development

MINIMUM HOURS

8

CERTIFICATION REQUIREMENTS

- Internal Certification in Feedback Management

REFRESHER FREQUENCY

Annual

87.7 Competency Validation

REQUIRED

Yes

ASSESSMENT METHODS

- Role-play scenarios
- Written assessment on risk classification

- Audit of first 5 feedback entries

MINIMUM PASSING CRITERIA

90%

REASSESSMENT FREQUENCY

Every 2 years

87.8 Simulation Requirements

REQUIRED

No

SIMULATION TYPES

- Aggressive parent confrontation
- Abuse disclosure scenario

FREQUENCY

Annual

88 SOP-Specific Inclusion Requirements

AFFECTED POPULATIONS

- Parents of students with disabilities (CWSN)
- Neurodivergent student parents
- Rural/Low-resource families
- Non-native English speaking parents
- First-generation learners

IDENTIFIED INCLUSION RISKS

- Language barriers in surveys
- Lack of internet access for digital feedback
- Cultural stigma regarding mental health feedback
- Low literacy hindering form completion
- Time constraints for working parents

AUTISM ACCOMMODATIONS

- Provide clear, literal instructions for focus groups
- Allow for asynchronous (written) feedback options
- Minimize sensory overload in meeting environments

ADHD ACCOMMODATIONS

- Break down long surveys into shorter sections
- Provide verbal summaries of action plans
- Offer multiple submission deadlines

DYSLEXIA ACCOMMODATIONS

- Use large, clear fonts in paper forms
- Allow for voice-recorded feedback
- Simplify complex terminology

HEARING IMPAIRMENT ACCOMMODATIONS

- Provide real-time captioning/transcription for focus groups
- Ensure all written communications are accessible

- Use visual aids in meetings

VISUAL IMPAIRMENT ACCOMMODATIONS

- Screen reader compatible online surveys
- Braille or large print paper forms
- Verbal feedback options with transcription

CWSN ADAPTATIONS

- Dedicated liaison for parents of students with disabilities
- Simplified language in all wellness communications
- Accessible meeting locations

RURAL ADAPTATIONS

- Paper surveys distributed via community leaders
- Phone-based feedback hotlines
- Community outreach focus groups

LOW LITERACY PARENT ADAPTATIONS

- Voice note submission options
- Face-to-face interviews with Wellness Coordinator
- Pictogram-based survey elements

MULTILINGUAL COMMUNICATION ADAPTATIONS

- Surveys in regional languages (Hindi, Marathi, etc.)
- Translation services for focus groups
- Multilingual SMS alerts

GENDER SENSITIVITY MEASURES

- Provide options for female-only parent focus groups
- Gender-neutral language in all feedback forms
- Confidentiality guarantees for sensitive disclosures

CULTURAL SENSITIVITY MEASURES

- Acknowledge cultural variations in mental health perceptions
- Involve community leaders in outreach
- Provide culturally appropriate wellness resources

EQUITY MONITORING INDICATORS

- Participation rate by neighborhood/zip code

- Satisfaction scores across different language groups
- Resolution rates for rural vs. urban complaints

ACCESSIBILITY COMPLIANCE CHECKS

- WCAG 2.1 compliance for online portal
- ADA-compliant physical meeting spaces
- Multilingual audit of all public-facing forms

89 Awareness Material

POSTER TITLE	Your Voice Matters: Parent Feedback Portal
KEY MESSAGE	Help us improve our mental wellness services for your child.
SLOGAN	Partnering for Wellbeing.
CAMPAIGN THEME	Community-Led Wellness

90 AI Assistant Configuration

RISK KEYWORDS

- Abuse
- Violence
- Suicide
- Harassment
- Danger

ESCALATION KEYWORDS

- Police
- Hospital
- Emergency
- Immediate

- Threat

INTERVENTION KEYWORDS

- Counseling
- Support
- Workshop
- Resource
- Help

FREQUENTLY ASKED QUESTIONS

- How do I submit a complaint?
- Is my feedback anonymous?
- How quickly will my concern be addressed?
- Who sees the survey results?

91 AI Audit Hooks

KEY FOCUS

REQUIRED SECTIONS

- Risk Classification
- Action Plan
- Communication Log

REQUIRED FORMS

- Feedback Log
- Investigation Note

REQUIRED CHECKLISTS

- Risk Assessment Checklist
- Data Anonymization Checklist

REQUIRED ESCALATIONS

- Red Risk Escalation
- Abuse Disclosure Alert

REQUIRED KPIS

- SLA Adherence
- Satisfaction Score

REQUIRED TRAINING

- Confidentiality Training
- Risk Assessment Training

92 KPI Framework

92.1 SOP Specific Metrics

Metric	Target	Measurement Frequency
Parent Satisfaction Score	> 80%	Bi-annually
Complaint Resolution Time	< 14 days	Monthly

Metric	Target	Measurement Frequency
Action Plan Completion Rate	> 90%	Quarterly

93 Maturity Model

93.1 Level 1

DESCRIPTION

Ad-hoc feedback collection (Suggestion box only).

EVIDENCE

- Physical suggestion box logs

93.2 Level 2

DESCRIPTION

Systematic survey and monthly logging.

EVIDENCE

- Master Feedback Tracker
- Monthly Summary Reports

93.3 Level 3

DESCRIPTION

Full SLA monitoring, risk classification, and action plans.

EVIDENCE

- Action Plan Logs

- Risk Assessment Notes
- SLA Dashboard

93.4 Level 4

DESCRIPTION

Data-driven improvements with focus groups and trend analysis.

EVIDENCE

- Focus Group Minutes
- Trend Analysis Reports
- Annual De-identified Report

93.5 Level 5

DESCRIPTION

Community-integrated wellness governance with real-time parent portal.

EVIDENCE

- Parent Portal Analytics
- PTA Governance Minutes
- External Audit Reports

94 Dependencies

PREREQUISITE SOPS

- PARENTAL-ENGAGEMENT-FRAMEWORK
- DATA-PRIVACY-POLICY

PARALLEL SOPS

- PAR-015: Parent Awareness Programs
- HR-005: Staff Conduct and Discipline

DOWNSTREAM SOPS

- WELLNESS-PROGRAM-UPDATE
- STAFF-RECOGNITION-PROCEDURE

95 Related SOPs

PREREQUISITES

- DATA-PRIVACY-POLICY
- CHILD-PROTECTION-POLICY

UPSTREAM

- COMMUNICATION-PLAN-01
- PARENT-ORIENTATION-SOP

DOWNSTREAM

- WELLNESS-PROGRAM-UPDATE
- STAFF-RECOGNITION-PROCEDURE

REFERENCES

- CBSE Circular TRG-03/2026
- NEP 2020 Guidelines

STANDARD OPERATING PROCEDURE

Parent Follow-Up Management SOP

Mental Wellness Management System - Parental Engagement and
Communication

SOP ID

CAS-013

VERSION

1.0

STATUS

Finalized

CLASSIFICATION

Confidential - Internal Use Only

EFFECTIVE DATE

2026-01-01

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1 Executive Overview

SOP ID	CAS-013
DOMAIN	Mental Wellness Management System
SUB DOMAIN	Parental Engagement and Communication
TITLE	Parent Follow-Up Management SOP
VERSION	1.0
CLASSIFICATION	Confidential - Internal Use Only
EFFECTIVE DATE	2026-01-01
REVIEW DATE	2027-01-01
DOCUMENT STATUS	Finalized
LANGUAGE	English

2 Purpose & Rationale

OBJECTIVE

To establish a structured, timely, and culturally sensitive framework for communicating with parents regarding student mental wellness.

RISK CONTROLLED

Mitigates risks of child neglect, communication breakdown, legal liability in non-responsive cases, and inconsistent support delivery.

DESIRED OUTCOMES

- Consistent parent engagement across all risk levels
- Timely intervention for students in distress
- Documented audit trail of all communications
- Reduced parental anxiety through clear information sharing
- Legal compliance with Indian child protection laws

3 Scope of Application

INSTITUTION TYPES

- High Schools
- Residential Schools
- Private and Government Schools

KEY STAKEHOLDERS

- Counselors
- Teachers

- Parents
- Guardians
- School Management

AGE GROUPS

- All ages (Minors focus)

LOCATIONS

- Urban Schools
- Rural Schools
- Remote Residential Campuses

4 Definitions & Terminology

Term	Definition
Parental Assent	The voluntary agreement of a parent to participate in specific wellness interventions.
Non-Responsive Parent	A guardian who fails to respond to three consecutive communication attempts within the stipulated SLA.
Strategy Coaching	Providing parents with specific tools and techniques to support their child's mental wellness at home.

Term	Definition
Home Visit	A physical visit by a school social worker or counselor to the student's residence when communication fails.

5 Key Stakeholders

Stakeholder Type	Stakeholder Title	Role In SOP
Internal	School Counselor	Primary executor of follow-up calls and meetings.
Internal	Class Teacher	Initial observer and trigger for parent communication.
Internal	School Principal	Final approver for high-risk escalations and legal notifications.
External	Parent/Guardian	Primary recipient of updates and partner in student support.
External	District Child Protection Officer	Recipient of reports in cases of severe neglect or safety risk.

6 Governance & Ownership

KEY FOCUS

OWNER
School Counselor / Wellness Coordinator

APPROVER
Principal / School Management Committee

REVIEW FREQUENCY
Annual

SUPERSEDES
Parent Communication Policy 2024

LINKED POLICIES

- Child Protection Policy
- Data Privacy and Confidentiality Policy
- Student Code of Conduct
- Anti-Bullying Policy

LINKED SOPS

- CAS-010: Student Risk Assessment
- CAS-012: Crisis Intervention Protocol
- CAS-015: External Referral Pathway

7 Roles & Responsibilities

Item 1 ROLE School Counselor RESPONSIBILITIES - Conduct risk assessments - Execute parent follow-ups - Provide strategy coaching - Maintain documentation logs	Item 3 ROLE School Principal RESPONSIBILITIES - Approve high-risk escalations - Oversee policy compliance - Manage external reporting to authorities
Item 2 ROLE Class Teacher RESPONSIBILITIES - Observe and report student behavior - Initial contact with parents for routine matters - Support counselor in data collection	Item 4 ROLE School Social Worker RESPONSIBILITIES - Conduct home visits - Assess community resources - Support rural family engagement

8 RACI Accountability Matrix

Item 1

ACTIVITY

Risk Identification

RESPONSIBLE

- School Counselor

ACCOUNTABLE

- Wellness Coordinator

CONSULTED

- Class Teacher

INFORMED

- Principal

ACTIVITY

High-Risk Escalation

RESPONSIBLE

- School Counselor

ACCOUNTABLE

- Principal

CONSULTED

- District Authorities

INFORMED

- Parents

Item 2

ACTIVITY

Parent Communication

RESPONSIBLE

- School Counselor

ACCOUNTABLE

- Wellness Coordinator

INFORMED

- Class Teacher

Item 4

ACTIVITY

Home Visits

RESPONSIBLE

- School Social Worker

ACCOUNTABLE

- Wellness Coordinator

CONSULTED

- School Counselor

INFORMED

- Principal

Item 3

9 Preconditions

- Student risk assessment completed
- Parental consent obtained
- Counselor assigned to case
- Communication channel established

10 Activation Triggers

- Student risk assessment score > threshold
- Teacher referral for behavioral change
- Parental request for wellness update
- Absence exceeding 3 days without notice
- Police/Child Helpline notification

11 SOP Trigger Matrix

MANDATORY TRIGGERS

- Student identified in Red Zone (Immediate Risk)
- Formal referral from Teacher/Counselor
- Legal mandate for child safety reporting

OPTIONAL TRIGGERS

- Routine wellness check-in
- Parental request for update
- General academic decline observation

AUTOMATIC TRIGGERS

- Absence exceeding 3 consecutive days without notice
- Significant drop in internal assessment scores (>20%)

EXTERNAL TRIGGERS

- Police notification regarding student safety
- Medical referral from external hospital
- Child Helpline report

RECURRING TRIGGERS

- Bi-weekly check-ins for Yellow Zone students

- Monthly progress updates for Orange Zone students

12 Required Inputs

- Student Risk Assessment Form
- Teacher Observation Notes
- Parent Contact Information
- Consent Forms
- District Child Protection Guidelines

13 Process Map

13.1 Phase 1

NAME

Identification and Initial Contact

OBJECTIVE

Identify the need for parent communication based on student status.

ACTIVITIES

- Review student risk assessment
- Determine risk zone (Green-Black)
- Select appropriate communication channel
- Obtain initial parental consent

ACTIVITIES

- Execute phone calls/emails/meetings
- Document conversation details
- Provide strategy coaching to parents
- Update student wellness plan

13.2 Phase 2

NAME

Communication Planning

OBJECTIVE

Develop a tailored engagement plan.

ACTIVITIES

- Define frequency of contact
- Identify key information to share
- Assign primary and secondary actors
- Prepare cultural/linguistic adaptations

13.4 Phase 4

NAME

Escalation and Non-Response Handling

OBJECTIVE

Manage barriers to communication.

ACTIVITIES

- Track non-response attempts
- Initiate home visits for rural/unresponsive cases
- Notify school management of persistent non-compliance
- Contact legal guardians or authorities

13.3 Phase 3

NAME

Execution of Follow-ups

OBJECTIVE

Conduct structured communication.

13.5 Phase 5

NAME

Closure and Review

OBJECTIVE

Finalize the follow-up cycle.

ACTIVITIES

- Conduct final progress review
- Generate closure report
- Update dashboard metrics
- Archive records securely

14 Workflow Overview

START

Identification of student distress or referral.

STEPS

- Risk Triage
- Consent Acquisition
- Channel Selection
- Routine Follow-up
- High-Risk Intervention
- Non-Response Protocol

- Home Visit Execution
- External Referral
- Case Closure
- Audit & Dashboard Update

CRITICAL DECISION POINTS

- DP-01: Risk Level Classification
- DP-02: Parental Cooperation Status

END

Closure of case and archiving of records.

15 Standard Procedure

1. Risk Identification and Triage

PURPOSE & RATIONALE

Determine urgency.

ACTOR

School Counselor

ACTION

Review student data and classify risk zone (Green-Black).

REQUIRED INPUTS

- Student Risk Assessment Form

EXPECTED OUTPUTS

- Risk Classification Summary

CRITICAL DECISION POINTS

- DP-01: Risk Level Classification

DOCUMENTATION REQUIRED

- Risk Classification Summary

EVIDENCE REQUIRED

- Student Risk Assessment Form

APPROVAL REQUIRED

Yes (for Red/Black)

TIME LIMIT

2 hours for high risk

EXPECTED OUTCOME

Accurate triage and priority assignment.

ESCALATION IF FAILED

Notify Principal immediately.

RELATED FORMS

- Risk Assessment Form

RELATED CHECKLISTS

- Counselor Review Checklist

2. Parental Consent and Information Sharing

PURPOSE & RATIONALE

Legal compliance.

ACTOR

School Counselor

ACTION

Contact parent to explain purpose and obtain signed consent.

REQUIRED INPUTS

- Student Identity Details

EXPECTED OUTPUTS

- Signed Consent Form

DOCUMENTATION REQUIRED

- Parental Consent Form

EVIDENCE REQUIRED

- Signed Consent Form

APPROVAL REQUIRED

No

TIME LIMIT

48 hours

EXPECTED OUTCOME

Legal authorization for follow-up.

ESCALATION IF FAILED

Seek alternative guardian consent.

RELATED FORMS

- Parental Consent Form

3. Communication Channel Selection

PURPOSE & RATIONALE

Effective engagement.

ACTOR

School Counselor

ACTION

Select channel based on parent profile (Phone/Email/In-person).

REQUIRED INPUTS

- Parent Profile

EXPECTED OUTPUTS

- Communication Plan

DOCUMENTATION REQUIRED

- Communication Plan

EVIDENCE REQUIRED

- Communication Plan

APPROVAL REQUIRED

No

TIME LIMIT

24 hours

EXPECTED OUTCOME

Optimized communication path.

ESCALATION IF FAILED

Default to phone call.

RELATED FORMS

- Communication Plan

4. Execution of Routine Follow-up**PURPOSE & RATIONALE**

Wellness support.

ACTOR

School Counselor

ACTION

Conduct scheduled check-in and provide strategy coaching.

REQUIRED INPUTS

- Communication Plan

EXPECTED OUTPUTS

- Updated Progress Notes

DOCUMENTATION REQUIRED

- Parent Conversation Log

EVIDENCE REQUIRED

- Parent Conversation Log

APPROVAL REQUIRED

No

TIME LIMIT

As scheduled

EXPECTED OUTCOME

Consistent parent engagement.

ESCALATION IF FAILED

Escalate to supervisor if risk increases.

RELATED FORMS

- Parent Conversation Log

5. Execution of High-Risk Follow-up**PURPOSE & RATIONALE**

Intensive intervention.

ACTOR

School Counselor / Principal

ACTION

Conduct in-person/intensive meeting and present intervention plan.

REQUIRED INPUTS

- Communication Plan

EXPECTED OUTPUTS

- Intervention Plan

DOCUMENTATION REQUIRED

- Meeting Minutes
- Intervention Plan

EVIDENCE REQUIRED

- Meeting Minutes
- Intervention Plan

APPROVAL REQUIRED

Yes (Principal)

TIME LIMIT

24-72 hours

EXPECTED OUTCOME

Agreed safety and support plan.

ESCALATION IF FAILED

Immediate referral to clinical services.

RELATED FORMS

- Intervention Plan

6. Non-Responsive Parent Protocol**PURPOSE & RATIONALE**

Manage barriers.

ACTOR

School Counselor

ACTION

Execute three attempts (Phone/SMS/Email).

REQUIRED INPUTS

- Parent Contact Details

EXPECTED OUTPUTS

- Attempt Status

DOCUMENTATION REQUIRED

- Communication Log

EVIDENCE REQUIRED

- Communication Log

APPROVAL REQUIRED

No

TIME LIMIT

7 days for non-critical

EXPECTED OUTCOME

Establish communication or trigger escalation.

ESCALATION IF FAILED

Initiate Home Visit protocol.

RELATED FORMS

- Communication Log

7. Home Visit Execution

PURPOSE & RATIONALE

Direct engagement.

ACTOR

School Social Worker

ACTION

Conduct physical visit to residence and assess safety.

REQUIRED INPUTS

- Parent Address
- Risk Assessment Summary

EXPECTED OUTPUTS

- Home Visit Report

DOCUMENTATION REQUIRED

- Home Visit Report

EVIDENCE REQUIRED

- Home Visit Report

APPROVAL REQUIRED

Yes (Principal)

TIME LIMIT

48 hours

EXPECTED OUTCOME

Direct parent engagement and safety confirmation.

ESCALATION IF FAILED

Notify District Child Protection Officer.

RELATED FORMS

- Home Visit Report

8. External Referral Coordination

PURPOSE & RATIONALE

Specialized support.

ACTOR

School Counselor

ACTION

Identify agency and facilitate warm handoff.

REQUIRED INPUTS

- Student Risk Assessment

EXPECTED OUTPUTS

- Referral Record

DOCUMENTATION REQUIRED

- Referral Record

EVIDENCE REQUIRED

- Referral Record

APPROVAL REQUIRED

No

TIME LIMIT

48 hours

EXPECTED OUTCOME

Connection to clinical services.

ESCALATION IF FAILED

Notify Principal of refusal for high-risk cases.

RELATED FORMS

- Referral Record

9. Case Closure and Review

PURPOSE & RATIONALE

Formalize end.

ACTOR

School Counselor

ACTION

Conduct final review meeting and archive records.

REQUIRED INPUTS

- Progress Notes

EXPECTED OUTPUTS

- Closed Status

DOCUMENTATION REQUIRED

- Case Closure Report

EVIDENCE REQUIRED

- Case Closure Report

APPROVAL REQUIRED

No

TIME LIMIT

5 days

EXPECTED OUTCOME

Formal closure of follow-up cycle.

ESCALATION IF FAILED

Re-open case if goals not met.

RELATED FORMS

- Case Closure Report

10. Audit and Dashboard Update

PURPOSE & RATIONALE

Data integrity.

ACTOR

Wellness Coordinator

ACTION

Update dashboard metrics and generate monthly report.

REQUIRED INPUTS

- Counselor Logs

EXPECTED OUTPUTS

- Updated Dashboard

DOCUMENTATION REQUIRED

- Monthly Wellness Report

EVIDENCE REQUIRED

- Monthly Wellness Report

APPROVAL REQUIRED

No

TIME LIMIT

Weekly

EXPECTED OUTCOME

Accurate reporting of system performance.

ESCALATION IF FAILED

Notify Principal of data discrepancies.

RELATED FORMS

- Monthly Wellness Report

16 Detailed Workflow Steps

1. Risk Identification and Triage

PURPOSE & RATIONALE

Determine the urgency of parent communication.

TRIGGER

Student identified in distress or referral received.

ACTOR

School Counselor

REQUIRED INPUTS

- Student Risk Assessment Form
- Teacher Observation Notes

ACTIONS

- Review student data for safety risks
- Classify risk into Green, Yellow, Orange, Red, or Black zones
- Determine immediate action required

DECISION CRITERIA

- Is there an immediate threat to life? (Yes → Black Zone)
- Is there significant impairment but no immediate danger? (Yes → Orange/Red)

DOCUMENTS GENERATED

- Risk Classification Summary

EXPECTED OUTPUTS

- Triage Status
- Priority Level

SERVICE LEVEL AGREEMENT (SLA)

Immediate for Black; 2 hours for Red; 24 hours for Orange/Yellow.

ESCALATION PATH

- Principal (for Red/Black)
- District Authorities (for Child Safety)

EVIDENCE GENERATED

- Risk Classification Summary

ACTOR

School Counselor

REQUIRED INPUTS

- Student Identity Details
- Consent Form Template

ACTIONS

- Contact parent to explain the purpose of follow-up
- Obtain signed consent for communication and intervention
- Explain confidentiality limits

DECISION CRITERIA

- Does parent provide verbal/written consent? (No → Seek alternative guardian)

DOCUMENTS GENERATED

- Parental Consent Form

EXPECTED OUTPUTS

- Signed Consent
- Communication Agreement

SERVICE LEVEL AGREEMENT (SLA)

48 hours from initial contact.

ESCALATION PATH

- School Management (if consent refused for high-risk)

EVIDENCE GENERATED

- Signed Consent Form

3. Communication Channel Selection

PURPOSE & RATIONALE

Select the most effective medium based on parent profile.

TRIGGER

Consent obtained.

ACTOR

School Counselor

REQUIRED INPUTS

- Parent Profile (Language, Literacy, Location)
- Risk Level

ACTIONS

- Identify primary language of the household

2. Parental Consent and Information Sharing

PURPOSE & RATIONALE

Ensure legal compliance and parental buy-in.

TRIGGER

Completion of Risk Identification.

- Assess parent's digital literacy and internet access
- Select channel: Phone (Sensitive), Email (Routine), In-person (High Risk/Rural)

DECISION CRITERIA

- Is the parent in a rural area with low connectivity? (Yes → Phone/SMS)
- Is the topic highly sensitive? (Yes → Phone/In-person)

DOCUMENTS GENERATED

- Communication Plan

EXPECTED OUTPUTS

- Selected Channel
- Scheduled Time

SERVICE LEVEL AGREEMENT (SLA)

24 hours.

EVIDENCE GENERATED

- Communication Plan

4. Execution of Routine Follow-up (Green/Yellow)

PURPOSE & RATIONALE

Provide wellness support and monitoring.

TRIGGER

Scheduled time for routine check-in.

ACTOR

School Counselor

REQUIRED INPUTS

- Communication Plan
- Student Progress Notes

ACTIONS

- Conduct phone call or email
- Share student progress and wellness updates
- Provide strategy coaching for home support
- Document conversation details

DECISION CRITERIA

- Does parent express new concerns? (Yes → Re-assess Risk Zone)

DOCUMENTS GENERATED

- Parent Conversation Log

EXPECTED OUTPUTS

- Updated Progress Notes
- Action Items for Parent

SERVICE LEVEL AGREEMENT (SLA)

As per scheduled frequency.

ESCALATION PATH

- Counselor Supervisor (if risk increases)

EVIDENCE GENERATED

- Parent Conversation Log

5. Execution of High-Risk Follow-up (Orange/Red)

PURPOSE & RATIONALE

Deliver intensive support and intervention planning.

TRIGGER

Scheduled time for high-risk meeting.

ACTOR

School Counselor / Principal

REQUIRED INPUTS

- Communication Plan
- Risk Assessment Summary

ACTIONS

- Conduct in-person or intensive phone meeting
- Present specific intervention plan
- Discuss external referral options if necessary
- Secure commitment to safety actions

DECISION CRITERIA

- Is the parent in denial? (Yes → Escalate to Management)
- Does student require immediate hospitalization? (Yes → Emergency Referral)

DOCUMENTS GENERATED

- Intervention Plan
- Meeting Minutes

EXPECTED OUTPUTS

- Agreed Intervention Plan
- Referral Authorization

SERVICE LEVEL AGREEMENT (SLA)

Within 24-72 hours of risk identification.

ESCALATION PATH

- Principal
- District Authorities

EVIDENCE GENERATED

- Meeting Minutes
- Intervention Plan

6. Non-Responsive Parent Protocol (Attempt 1-3)

PURPOSE & RATIONALE

Manage communication barriers.

TRIGGER

No response to first contact attempt.

ACTOR

School Counselor

REQUIRED INPUTS

- Parent Contact Details
- Communication Plan

ACTIONS

- Attempt 2: Phone call with SMS follow-up
- Attempt 3: Email with 'Urgent' subject line
- Log all attempts and timestamps

DECISION CRITERIA

- Still no response after 3 attempts? (Yes → Initiate Home Visit/Escalation)

DOCUMENTS GENERATED

- Communication Log

EXPECTED OUTPUTS

- Attempt Status
- Next Action Trigger

SERVICE LEVEL AGREEMENT (SLA)

7 days for non-critical; 24 hours for critical.

ESCALATION PATH

- School Social Worker (for Home Visit)

EVIDENCE GENERATED

- Communication Log

7. Home Visit Execution

PURPOSE & RATIONALE

Direct engagement for rural or non-responsive parents.

TRIGGER

Failure of phone/email communication and high risk level.

ACTOR

School Social Worker / Counselor

REQUIRED INPUTS

- Parent Address
- Risk Assessment Summary
- Home Visit Approval Form

ACTIONS

- Conduct physical visit to residence
- Meet with guardian/parent in person
- Assess home environment for safety
- Establish communication agreement

DECISION CRITERIA

- Is the parent hostile? (Yes → Withdraw and notify Management)
- Is the student safe at home? (No → Immediate District Notification)

DOCUMENTS GENERATED

- Home Visit Report

EXPECTED OUTPUTS

- Home Visit Summary
- Safety Confirmation

SERVICE LEVEL AGREEMENT (SLA)

Within 48 hours of non-response trigger.

ESCALATION PATH

- District Child Protection Officer
- Police (if safety risk is immediate)

EVIDENCE GENERATED

- Home Visit Report

8. External Referral Coordination

PURPOSE & RATIONALE

Connect family with specialized clinical services.

TRIGGER

Counselor determines need for external clinical support.

ACTOR

School Counselor

REQUIRED INPUTS

- Student Risk Assessment
- Parental Consent Form

ACTIONS

- Identify appropriate agency (e.g., Tele-MANAS, Hospital)
- Provide parent with referral details and contact info
- Facilitate warm handoff if possible
- Document referral details

DECISION CRITERIA

- Does the parent refuse external referral? (Yes → Document refusal and monitor)

DOCUMENTS GENERATED

- Referral Record

EXPECTED OUTPUTS

- External Referral
- Parental Acknowledgment

SERVICE LEVEL AGREEMENT (SLA)

Within 48 hours of referral decision.

ESCALATION PATH

- Principal (if refusal for high-risk)

EVIDENCE GENERATED

- Referral Record

9. Case Closure and Review

PURPOSE & RATIONALE

Formalize the end of a specific follow-up cycle.

TRIGGER

Student goals met or risk level reduced to Green.

ACTOR

School Counselor

REQUIRED INPUTS

- Progress Notes
- Intervention Plan

ACTIONS

- Conduct final review meeting with parent
- Summarize outcomes and lessons learned
- Generate closure report
- Archive all records securely

DECISION CRITERIA

- Are all safety goals met? (Yes → Close Case)

DOCUMENTS GENERATED

- Case Closure Report

EXPECTED OUTPUTS

- Closed Status
- Archived Records

SERVICE LEVEL AGREEMENT (SLA)

Within 5 days of goal completion.

EVIDENCE GENERATED

- Case Closure Report

Wellness Coordinator

REQUIRED INPUTS

- Counselor Logs
- Closure Reports

ACTIONS

- Update school wellness dashboard
- Verify documentation completeness
- Identify trends in parent engagement
- Report to Management Committee

DECISION CRITERIA

- Is documentation missing? (Yes → Request from Counselor)

DOCUMENTS GENERATED

- Monthly Wellness Report

EXPECTED OUTPUTS

- Updated Dashboard
- Management Report

SERVICE LEVEL AGREEMENT (SLA)

Weekly update.

ESCALATION PATH

- Principal (for significant trend anomalies)

EVIDENCE GENERATED

- Monthly Wellness Report

10. Audit and Dashboard Update

PURPOSE & RATIONALE

Ensure data integrity and monitor system performance.

TRIGGER

Completion of any follow-up step or closure.

ACTOR

17 Critical Decision Points

Item 1

DECISION ID

DP-01

DECISION NAME

Risk Level Classification

CRITERIA

- Presence of self-harm ideation
- Severity of academic decline
- Frequency of behavioral outbursts

POSSIBLE OUTCOMES

- Green (Routine)
- Yellow (Support Needed)
- Orange (Significant Impairment)
- Red (Immediate Risk)
- Black (Life Threat)

NEXT STEPS

- Assign SLA and Communication Channel based on outcome.

Item 2

DECISION ID

DP-02

DECISION NAME

Parental Cooperation Status

CRITERIA

- Response time to emails/calls
- Attendance at scheduled meetings
- Adherence to agreed action plans

POSSIBLE OUTCOMES

- Cooperative
- Unresponsive
- In Denial
- Conflict Situation
- Unavailable
- Refusing Intervention

NEXT STEPS

- Apply Parent Engagement Matrix strategy.

18 Decision Tree

GREEN

- Routine check-in → Update log → Close cycle

YELLOW

- Support needed → Strategy coaching → Weekly follow-up

ORANGE

- Significant impairment → Intervention plan → 48hr follow-up

RED

- Immediate risk → Crisis protocol → Principal notification

BLACK

- Life threat → Emergency services → Police/District notification

19 Decision Rules

Rule ID	Condition	Decision	Next Action
DR-01	If risk is Red/Black and parent is non-responsive.	Immediate escalation to Principal and District Authorities.	Activate Emergency Response SOP.
DR-02	If parent refuses consent for high-risk student.	Document refusal and initiate Duty of Care protocol.	Notify Management Committee.

20 Exception Handling Protocol

Item 1

EXCEPTION

Parent Refusal of Support for High-Risk Student

DETECTION METHOD

Signed refusal form or verbal statement during meeting.

RESPONSE

Document refusal, notify Principal immediately, and initiate 'Duty of Care' protocol.

ESCALATION

District Child Protection Officer / School Management.

DOCUMENTATION REQUIRED

- Refusal Record
- Management Notification Log

Notify local police and initiate missing student protocol.

ESCALATION

Police / District Authorities

DOCUMENTATION REQUIRED

- Police Notification Record
- Missing Student Log

Item 2

EXCEPTION

Sudden Loss of Parent Contact (Missing Person)

DETECTION METHOD

Failure to respond to 3 consecutive attempts + no school attendance.

RESPONSE

Item 3

EXCEPTION

Hostile Parent Behavior during Meeting

DETECTION METHOD

Verbal aggression, threats, or physical intimidation.

RESPONSE

Terminate meeting immediately, ensure student safety, and notify security/management.

ESCALATION

School Security / Principal

DOCUMENTATION REQUIRED

- Incident Report
- Security Log

21 Exceptions

Description	Action
Emergency situation where parent cannot be reached but student is in immediate danger.	Activate Emergency Response SOP immediately without waiting for communication.
Parental death or incapacity.	Identify legal guardian and initiate new consent process.

22 Expected Outputs

- Risk Classification Summary
- Communication Plan
- Intervention Plan
- Referral Record
- Case Closure Report
- Monthly Wellness Dashboard

23 Document Control & Version Governance

23.1 Version History

Version	Date	Author	Description
1.0	2026-01-01	MWMS Architect	Initial release of Parent Follow-Up Management SOP.

23.2 Change Log

Version	Date	Change Description
1.0	2026-01-01	Baseline creation following CBSE Circular TRG-03/2026.

REVIEW TRIGGERS

- Change in Supreme Court guidelines regarding child safety
- Annual school policy review
- Significant incident of non-responsive parent handling
- New CBSE circular on student wellbeing

23.4 Approval History

Approver	Date	Status
School Principal	2025-12-15	Approved
District Education Officer	2025-12-20	Verified

24 Inclusive Implementation Mandate

MULTILINGUAL SUPPORT REQUIRED

Yes

ACCESSIBILITY REVIEW REQUIRED

Yes

LOW RESOURCE SCHOOL ALTERNATIVE
REQUIRED

Yes

EQUITY IMPACT ASSESSMENT REQUIRED

Yes

25 Regulatory Compliance Mapping

KEY FOCUS

MANDATORY REQUIREMENTS

- Supreme Court Judgment 2025 INSC 893 (Child Safety)
- CBSE Circular TRG-03/2026
- NEP 2020 Student Wellbeing Framework

RECOMMENDED REQUIREMENTS

- WHO School Mental Health Framework
- UNESCO Whole School Wellbeing Framework

BEST PRACTICES

- Manodarpan Guidelines
- Tele-MANAS Integration Protocols

COMPLIANCE GAPS IF NOT IMPLEMENTED

- Legal liability in case of child neglect
- Non-compliance with CBSE affiliation bye-laws
- Failure to meet national student safety standards

26 Compliance Traceability Matrix KEY FOCUS

1. Supreme Court of India

REQUIREMENT REFERENCE

2025 INSC 893

REQUIREMENT TEXT

Mandatory reporting and follow-up for minors in distress.

IMPLEMENTED IN SECTIONS

- Escalation Matrix
- Confidentiality Exception Matrix

2. CBSE Circular TRG-03/2026

REQUIREMENT REFERENCE

TRG-03/2026

REQUIREMENT TEXT

Structured parental involvement in student mental health.

IMPLEMENTED IN SECTIONS

- Parent Engagement Matrix
- Detailed Workflow Steps

27 Documentation & Evidentiary Requirements

EVENT DOCUMENTATION PRINCIPLES

- Factual and objective recording of statements
- Timestamps for all communications
- Clear identification of actors involved
- No subjective opinions or clinical diagnoses in logs

MANDATORY CONVERSATION RECORDS

- Parent Conversation Log (Phone/Email)
- In-person Meeting Minutes
- Home Visit Report
- Consent Forms

MANDATORY OBSERVATION RECORDS

- Teacher Observation Notes
- Student Behavior Logs
- Social Interaction Records

MANDATORY DECISION RECORDS

- Risk Level Escalation Approval
- External Referral Authorization

- Home Visit Approval Form

MANDATORY ACTION RECORDS

- Strategy Coaching Action Plan
- Follow-up Task List
- Corrective Action Log

MANDATORY FOLLOWUP RECORDS

- Recurring Check-in Summary
- Progress Update Tracker

EVIDENCE REQUIREMENTS

- Signed Consent Forms
- Call Logs (Screenshots/Logs)
- Email Threads
- Photos of Home Visit (if permitted and safe)

DOCUMENTATION QUALITY STANDARDS

- Must be legible and complete
- Must use standardized terminology
- Must be stored in a secure, encrypted database
- Must be accessible to authorized personnel only

28 The Student Journey

ENTRY

- Teacher identifies behavioral change
- Student self-refers to counselor
- Peer reports concern about student safety

ASSESSMENT

- Counselor conducts initial screening
- Risk level is determined
- Parental consent for follow-up is sought

INTERVENTION

- Student receives wellness support

- Parent receives strategy coaching
- Home environment adjustments made

MONITORING

- Weekly check-ins with student
- Bi-weekly updates to parents
- Progress tracking against goals

CLOSURE

- Goals achieved
- Risk level reduced to Green
- Final review meeting held

29 The Parent Journey

NOTIFICATION

- Receive initial call/email regarding student wellness
- Review risk assessment summary

CONSULTATION

- Attend scheduled meetings (phone/in-person)
- Discuss student progress and challenges

CONSENT

- Sign parental consent forms

- Agree to communication frequency

ENGAGEMENT

- Receive strategy coaching for home support
- Participate in action plans
- Provide feedback on school interventions

CLOSURE

- Attend final review meeting
- Receive closure report
- Transition to routine monitoring

30 The Staff Journey

OBSERVATION

- Teacher notes behavioral changes
- Counselor monitors student interactions

REPORTING

- Teacher submits observation logs
- Counselor updates risk assessment

INTERVENTION SUPPORT

- Counselor conducts parent follow-ups
- Social worker performs home visits

DOCUMENTATION

- Log all communications
- Update dashboard metrics
- Archive records securely

31 Resource Evidence

COMMITTEE MINUTES

- Wellness Committee meeting minutes regarding parent engagement trends

ATTENDANCE RECORDS

- Parent meeting attendance logs

TRAINING RECORDS

- Staff training certificates for Parent Communication and Crisis Management

FORMS

- Parental Consent Form
- Risk Assessment Summary
- Home Visit Report
- Referral Record
- Case Closure Report

REPORTS

- Monthly Wellness Dashboard
- Annual Parent Engagement Audit

DASHBOARDS

- Real-time Student Risk Tracker
- Parent Communication SLA Tracker

32 Grievance Redressal Mechanism

KEY FOCUS

COMPLAINT CHANNELS

- Anonymous suggestion box
- Dedicated wellness email
- In-person grievance meeting with Principal

INVESTIGATION PROCESS

- Log complaint within 24 hours
- Assign neutral officer for investigation
- Interview relevant stakeholders

- Issue written finding within 7 days

RESOLUTION TIMELINES

- Initial acknowledgment: 24 hours
- Investigation completion: 10 days
- Final resolution: 15 days

APPEAL PROCESS

- Appeal to School Management Committee
- External appeal to District Education Office

33 Referral Framework

33.1 Internal Referrals

ELIGIBLE REFERRERS

- Teachers
- Counsellors
- School Management
- Wellness Committee
- Student Support Team

REFERRAL TRIGGERS

- Behavioral change exceeding teacher capacity
- Academic decline linked to emotional distress
- Safety concerns identified by staff

DOCUMENTATION REQUIRED

- Internal Referral Form
- Observation Log

SERVICE LEVEL AGREEMENT (SLA)

24 hours from referral submission.

33.2 Self Referrals

ALLOWED

Yes

SUBMISSION METHODS

- Online portal
- Physical drop-box
- Direct counselor visit

CONFIDENTIALITY PROTECTIONS

- Encrypted digital storage
- Limited access to records

33.3 Anonymous Referrals

ALLOWED

Yes

CHANNELS

- Anonymous QR codes in classrooms
- Suggestion box

VALIDATION PROCESS

- Counselor review for urgency

- Verification of student identity if possible

FOLLOW UP PROCEDURE

- Direct outreach to student
- Classroom-wide wellness check

33.4 External Referrals

ELIGIBLE AGENCIES

- Tele-MANAS
- District Child Protection Services
- Private Clinical Psychologists
- Local Hospitals

ELIGIBILITY CRITERIA

- Clinical diagnosis required
- Severe risk of self-harm
- Need for specialized therapy

CONSENT REQUIREMENTS

- Signed Parental Consent Form
- Informed consent regarding data sharing

HANDOVER PROTOCOL

- Summary of internal assessment shared with agency
- Contact details provided to parent
- Follow-up call scheduled by school counselor

33.5 Referral Tracking

TRACKING REQUIRED

Yes

STATUS CATEGORIES

- Pending
- In Progress
- Completed
- Closed
- Rejected

CLOSURE REQUIREMENTS

- Final summary report
- Parental acknowledgment of referral completion

34 Confidentiality Exception Matrix

KEY FOCUS

MANDATORY DISCLOSURES

- Imminent risk of self-harm
- Imminent risk to others
- Child abuse
- Child neglect
- Sexual abuse
- Court orders
- Legal reporting obligations

LEGAL REPORTING EVENTS

- Suspected physical abuse

- Active suicide attempt
- Missing student with high-risk profile

CONSENT OVERRIDE CONDITIONS

- Life-threatening emergency
- Court order for information release
- Immediate threat to public safety

AUTHORIZED DISCLOSURE RECIPIENTS

- School Principal
- District Child Protection Officer
- Local Police (Emergency only)
- Medical Personnel (Consent-based)

DOCUMENTATION REQUIREMENTS

- Disclosure Log
- Reason for Disclosure Statement
- Time and Date of Notification

APPROVAL REQUIREMENTS

- Principal approval for all non-emergency disclosures
- Management Committee approval for recurring legal reporting

35 Record Retention & Disposal

RETENTION PERIOD

7 Years (Post-graduation)

RETENTION BASIS

Legal compliance and educational audit requirements

RECORD CATEGORIES

- Risk Assessments
- Parent Communication Logs
- Consent Forms
- Referral Records

35.4 Storage Requirements

PHYSICAL RECORDS

- Locked filing cabinets in Counselor's office

DIGITAL RECORDS

- Encrypted cloud storage with MFA

ACCESS CONTROL REQUIREMENTS

- Role-based access control (RBAC)
- Audit logs for all record views
- Need-to-know basis only

DISPOSAL METHOD

Secure shredding for physical; Permanent deletion for digital

DISPOSAL AUTHORITY

School Wellness Coordinator

AUDIT FREQUENCY

Annual

DESTRUCTION LOG REQUIRED

Yes

36 Anonymised Reporting Framework

36.1 Management Reports

ANONYMIZATION REQUIRED

Yes

PERMITTED DATA

- Risk zone distribution
- Average response times
- Parent engagement rates

PROHIBITED DATA

- Student names
- Parent contact details
- Specific case identifiers

36.2 Research Usage

ALLOWED

No

36.3 Statistical Reporting

AGGREGATE ONLY

Yes

MINIMUM GROUP SIZE

5

RE IDENTIFICATION CONTROLS

- Data scrubbing
- Aggregation of small cohorts

DATA SHARING CONTROLS

- No sharing with third-party marketing
- Strict internal data silos

37 Guiding Ethical Principles

37.1 Autonomy

DESCRIPTION

Respecting the rights of parents and students to make informed choices.

IMPLEMENTATION REQUIREMENTS

- Informed consent for all follow-ups
- Right to refuse specific interventions

37.2 Beneficence

DESCRIPTION

Acting in the best interest of the student's mental wellness.

IMPLEMENTATION REQUIREMENTS

- Evidence-based strategy coaching
- Timely intervention for high-risk cases

37.3 Non Maleficence

DESCRIPTION

Avoiding harm through communication or intervention.

IMPLEMENTATION REQUIREMENTS

- Confidentiality protection
- Non-judgmental parent engagement

37.4 Justice And Equity

DESCRIPTION

Ensuring equal access to support for all students regardless of background.

IMPLEMENTATION REQUIREMENTS

- Rural adaptations
- Multilingual support

37.5 Confidentiality

KEY FOCUS

DESCRIPTION

Protecting sensitive information from unauthorized disclosure.

IMPLEMENTATION REQUIREMENTS

- Encrypted storage
- Need-to-know access

37.6 Privacy

DESCRIPTION

Respecting the personal space and data of families.

IMPLEMENTATION REQUIREMENTS

- Data minimization
- Secure disposal

37.7 Professional Integrity

DESCRIPTION

Maintaining high standards of practice and ethics.

IMPLEMENTATION REQUIREMENTS

- Regular supervision
- Continuing professional development

38 Communication & Awareness Strategy

38.1 Student Communication

CHANNELS

- Counselor sessions
- Classroom wellness talks
- School app notifications

FREQUENCY

Weekly/As needed

MANDATORY TOPICS

- Stress management
- Help-seeking behavior

- Peer support

38.2 Parent Communication

CHANNELS

- Phone calls
- Emails
- In-person meetings
- SMS for urgent updates

FREQUENCY

Risk-dependent (Green: Monthly, Orange/Red: Daily)

MANDATORY TOPICS

- Student progress
- Strategy coaching
- Safety plans

38.3 Staff Communication

CHANNELS

- Staff meetings
- Internal portal
- Email

FREQUENCY

Weekly

MANDATORY TOPICS

- Student risk alerts
- Parent engagement updates
- SOP reminders

38.4 Awareness Campaigns

MINIMUM FREQUENCY

Quarterly

TARGET GROUPS

- Parents

- Students
- Teachers

CAMPAIGN TYPES

- Workshops
- Newsletters
- Social media posts

38.5 Workshops And Training

STUDENT WORKSHOPS

- Resilience building
- Emotional regulation

PARENT WORKSHOPS

- Supporting children's mental health at home
- Identifying early warning signs

STAFF WORKSHOPS

- Parent communication skills
- Crisis intervention training

38.6 Helpline Visibility

REQUIRED

Yes

DISPLAY LOCATIONS

- Counselor's office
- School website
- Student diaries
- Notice boards

DIGITAL VISIBILITY REQUIREMENTS

- Prominent link on school homepage
- SMS blast for emergency numbers

FEEDBACK CHANNELS

- Parent satisfaction surveys
- Student feedback forms
- Staff suggestion box

39 Competency & Qualification Requirements

MANDATORY QUALIFICATIONS

- Master's in Psychology/Counseling or Social Work
- Registered with relevant national board (e.g., RCI)

PREFERRED QUALIFICATIONS

- Certification in School Counseling
- Training in Child Protection

CERTIFICATIONS

- First Aid/CPR
- Crisis Intervention Certification

MINIMUM EXPERIENCE REQUIREMENTS

- 2 years in educational setting

BACKGROUND VERIFICATION REQUIREMENTS

- Police verification certificate
- Child protection clearance

39.6 Continuing Professional Development

ANNUAL TRAINING HOURS

40

MANDATORY TOPICS

- Neurodiversity
- Trauma-informed care
- Parental engagement

RECERTIFICATION REQUIREMENTS

Every 3 years

39.7 Supervision Requirements

CLINICAL SUPERVISION REQUIRED

Yes

PEER REVIEW REQUIRED

Yes

39.7.3 Wellness Case Review Framework

FREQUENCY

Bi-weekly

PARTICIPANTS

- Counselor
- Supervisor
- Wellness Coordinator

39.7.4 Second Level Review Process

TRIGGER

High-risk cases

APPROVER

Principal

39.7. High Risk Decision 5 Validation

KEY
FOCUS

REQUIREMENT Dual sign-off for Red/Black zone actions

METHOD Written approval

FREQUENCY

Continuous

40 Feedback & Effectiveness Measurement

40.1 Student Feedback

REQUIRED

Yes

COLLECTION METHODS

- Anonymous surveys
- Exit interviews

FREQUENCY

Termly

40.2 Parent Feedback

REQUIRED

Yes

COLLECTION METHODS

- Satisfaction surveys
- Focus group discussions

FREQUENCY

Annually

40.3 Staff Feedback

REQUIRED

Yes

COLLECTION METHODS

- Staff meetings
- Feedback forms

FREQUENCY

Quarterly

40.4 Annual Effectiveness Review

REQUIRED

Yes

EVALUATION CRITERIA

- SLA compliance rate
- Parent satisfaction score
- Student outcome trends

KEY STAKEHOLDERS

- School Management
- Parents
- Counselors

CONTINUOUS IMPROVEMENT INPUTS

- Case study reviews
- Audit findings
- Parent feedback

SUCCESS INDICATORS

- Reduction in high-risk incidents
- High parent engagement scores
- Timely follow-up completion

40.7 Improvement Tracking

REQUIRED

Yes

REVIEW FREQUENCY

Monthly

41 Prevention Layer

PRIMARY PREVENTION

- Parental awareness workshops on mental health
- School-wide wellness newsletters
- Student resilience training programs

SECONDARY PREVENTION

- Early warning sign checklists for teachers

- Targeted parent outreach for at-risk students
- Regular student wellness screenings

TERTIARY PREVENTION

- Intensive home visit protocols
- Crisis intervention plans
- External clinical referral pathways

42 Resilience Layer

SKILLS DEVELOPED

- Parental coping skills

- Student emotional regulation
- Problem-solving for families

SEL COMPONENTS

- Self-awareness
- Relationship skills
- Responsible decision-making

LIFE SKILLS COMPONENTS

- Stress management
- Help-seeking behavior
- Communication skills

PROTECTIVE FACTORS STRENGTHENED

- Parental involvement
- Teacher-parent connection
- Student sense of belonging

43 Continuous Monitoring

MONITORING FREQUENCY

Weekly

MONITORING INDICATORS

- SLA compliance rate for follow-ups
- Parent satisfaction scores
- Risk zone distribution trends

RESPONSIBLE ROLES

- Wellness Coordinator

- School Counselor

EARLY WARNING SIGNS

- Increasing number of non-responsive parents
- Rising frequency of Red Zone cases
- Decreasing parent satisfaction scores

TREND ANALYSIS REQUIREMENTS

- Monthly trend analysis on risk levels
- Quarterly audit of communication logs

44 Mental Wellness Index (MWI)

APPLICABLE

Yes

44.2 Indicators

Indicator Code	Indicator Name	Weight	Score Range	Description
PWI-01	Parental Involvement Score	0.3	1-5	Measures frequency and quality of parent engagement.
PWI-02	Student Progress Rate	0.4	1-5	Measures improvement in student wellness goals.
PWI-03	Communication Efficiency	0.3	1-5	Measures adherence to communication SLAs.

ASSESSMENT FREQUENCY

Monthly

RISK SCORING METHOD

Weighted average of indicators

INTERVENTION THRESHOLDS

- Score < 3.0: Immediate Counselor Intervention
- Score < 2.0: Principal/Management Escalation

45 Review Status

LEGAL REVIEW

Completed - Jan 2026

CLINICAL REVIEW

Completed - Dec 2025

EDUCATION REVIEW

Completed - Dec 2025

COMPLIANCE REVIEW

Completed - Jan 2026

46 Resource Controls

COUNSELOR STUDENT RATIO 1:400 (Recommended)

CAREER COUNSELOR RATIO 1:500

WELLNESS TEAM STRUCTURE Counselor, Social Worker, Wellness Coordinator, Management Committee

47 Screening Framework

TIER 1 UNIVERSAL WELLNESS SCREENING

- Annual student wellness survey
- Quarterly teacher observation logs

TIER 2 TARGETED SCREENING

- Bi-weekly check-ins for Yellow Zone students
- Parental concern screening

TIER 3 CLINICAL ASSESSMENT

- Formal risk assessment for Red/Black zones
- External clinical evaluation referral

48 Parent Engagement Matrix

PARENT COOPERATIVE

- Weekly check-ins
- Strategy coaching
- Positive reinforcement

PARENT UNRESPONSIVE

- 3 attempts (Phone/SMS/Email)
- Home visit trigger
- Management notification

PARENT IN DENIAL

- Evidence-based progress sharing
- Education on risk signs
- Firm boundary setting

PARENT CONFLICT SITUATION

- Neutral mediator involvement
- De-escalation techniques
- Written communication preference

PARENT UNAVAILABLE

- Alternative guardian contact
- Asynchronous updates (Email/App)
- Scheduled catch-up calls

PARENT REFUSES INTERVENTION

- Documented refusal record
- Duty of care protocol activation
- Management review

49 Confidentiality Controls

KEY FOCUS

ACCESS CONTROL

- Role-based access (RBAC)
- Multi-factor authentication (MFA)

CONSENT REQUIREMENTS

- Signed parental consent for all data sharing
- Informed consent for external referrals

RECORD SHARING RULES

- Need-to-know basis only

- No sharing with non-authorized staff

EXCEPTIONS TO CONFIDENTIALITY

- Imminent risk of harm
- Legal mandate
- Court order

STUDENT PRIVACY PROTECTIONS

- Anonymized reporting for management
- Secure data deletion protocols

50 Data Governance

KEY FOCUS

STORAGE REQUIREMENTS

- Encrypted cloud storage
- Physical locks for paper files

ENCRYPTION REQUIREMENTS

- AES-256 encryption at rest
- TLS 1.3 for data in transit

RETENTION REQUIREMENTS

- 7 years post-graduation

ARCHIVAL REQUIREMENTS

- Move to cold storage after 2 years of inactivity

DESTRUCTION REQUIREMENTS

- Secure shredding
- Digital wiping

BACKUP REQUIREMENTS

- Daily automated backups
- Off-site redundant copies

51 Risk Classification

KEY FOCUS

51.1 Green

DESCRIPTION

Normal developmental concerns and routine life stress.

EXAMPLES

- Mild examination stress
- Friendship disagreements
- Occasional worry

51.2 Yellow

DESCRIPTION

Early distress requiring support and monitoring.

EXAMPLES

- Persistent stress
- Moderate anxiety
- Social withdrawal

51.3 Orange

DESCRIPTION

Significant distress causing measurable impairment but without immediate danger.

EXAMPLES

- Panic attacks
- School refusal
- Severe academic decline

51.4 Red

KEY FOCUS

DESCRIPTION

High-risk situation requiring immediate intervention.

EXAMPLES

- Suicidal ideation
- Self-harm behavior
- Violence threats

51.5 Black

DESCRIPTION

Life-threatening emergency requiring emergency response.

EXAMPLES

- Active suicide attempt
- Overdose
- Violent attack underway

52 Escalation Matrix

KEY FOCUS

GREEN

- Counselor Supervisor

YELLOW

- Wellness Coordinator

ORANGE

- Principal
- District Education Office

RED

- Principal
- Police
- Child Protection Services

BLACK

- Emergency Services (100/112)
- Police
- School Management

53 External Reporting Obligations

TELE MANAS

- Referral for 24/7 mental health support

DISTRICT AUTHORITIES

- Report of persistent neglect or safety risk

CHILD PROTECTION SERVICES

- Mandatory reporting of abuse or severe neglect

MEDICAL SERVICES

- Emergency hospitalization referral

LAW ENFORCEMENT

- Active threats, missing persons, or crimes

REPORTING TRIGGERS

- Red Zone identification
- Black Zone emergency
- Suspected abuse disclosure

54 Service Level Agreement (SLA)

KEY FOCUS

ACKNOWLEDGEMENT	2 hours for high-risk; 24 hours for routine.
ASSESSMENT	24 hours from referral.
ESCALATION	Immediate (Red/Black).
RESOLUTION	7 days for Yellow; 30 days for Orange.
CLOSURE	48 hours post-goal completion.

55 Required Forms

KEY FOCUS

55.1 Mandatory Forms

KEY FOCUS

Item 1

FORM ID	F-CAS-013-01
FORM NAME	Parental Consent Form
PURPOSE & RATIONALE	Legal authorization for communication.
OWNER	School Counselor
RETENTION PERIOD	7 Years
MANDATORY	Yes
LINKED SOP	CAS-013

Item 2

FORM ID	F-CAS-013-02
FORM NAME	Risk Assessment Summary
PURPOSE & RATIONALE	Document student risk level.
OWNER	School Counselor
RETENTION PERIOD	7 Years
MANDATORY	Yes
LINKED SOP	CAS-013

Item 3

FORM ID	F-CAS-013-03
FORM NAME	Parent Conversation Log

PURPOSE & RATIONALE	Record all interactions.
OWNER	School Counselor
RETENTION PERIOD	7 Years
MANDATORY	Yes
LINKED SOP	CAS-013

Item 4

FORM ID	F-CAS-013-04
FORM NAME	Home Visit Report
PURPOSE & RATIONALE	Document physical visits.
OWNER	Social Worker
RETENTION PERIOD	7 Years
MANDATORY	Yes
LINKED SOP	CAS-013

Item 5

FORM ID	F-CAS-013-05	OWNER	School Counselor
FORM NAME	Case Closure Report	RETENTION PERIOD	7 Years
PURPOSE & RATIONALE	Finalize follow-up cycle.	MANDATORY	Yes
		LINKED SOP	CAS-013

55.2 Optional Forms

Item 1	
FORM ID	F-CAS-013-06
FORM NAME	Strategy Coaching Plan
PURPOSE & RATIONALE	Home support tools.
OWNER	School Counselor
RETENTION PERIOD	7 Years
MANDATORY	No
LINKED SOP	CAS-013

GENERATED RECORDS

- Audit Logs
- Dashboard Metrics
- Referral Summaries

56 Required Checklists

KEY FOCUS

Checklist ID	Checklist Name	Purpose & Rationale	Owner	Frequency	Linked SOP
CL-CAS-013-01	Counselor Review Checklist	Ensure all risk factors are addressed.	School Counselor	Every Case	CAS-013
CL-CAS-013-02	Parent Communication Checklist	Verify all required info is shared.	School Counselor	Every Call/Meeting	CAS-013
CL-CAS-013-03	Home Visit Safety Checklist	Ensure safety during physical visits.	Social Worker	Every Home Visit	CAS-013
CL-CAS-013-04	Referral Completion Checklist	Verify handoff is complete.	School Counselor	Every Referral	CAS-013
CL-CAS-013-05	Audit Compliance Checklist	Verify documentation integrity.	Wellness Coordinator	Monthly	CAS-013

57 Records & Evidence

Record Name	Retention Period	Owner
Parental Consent Archive	7 Years	School Counselor
Risk Assessment Database	7 Years	Wellness Coordinator

Record Name	Retention Period	Owner
Communication Logs	7 Years	School Counselor

58 Key Performance Indicators

Metric	Target	Measurement Frequency
SLA Compliance Rate (High Risk)	> 95%	Monthly
Parent Satisfaction Score	> 4.0/5.0	Annually
Case Closure Rate (Within Target)	> 85%	Quarterly

59 Audit Framework KEY FOCUS

AUDIT FREQUENCY

Quarterly

AUDIT QUESTIONS

- Are all Red Zone cases documented with a risk assessment?
- Is there a signed consent form for every active follow-up?
- Do communication logs include timestamps and specific actions taken?

EVIDENCE REQUIRED

- Random sample of 10 case files

- Communication log audit trail
- Signed consent forms

NON COMPLIANCE EXAMPLES

- Missing risk assessment for a high-risk student
- No documentation of phone calls
- Failure to notify Principal of Red Zone cases

59.5 Audit Scoring KEY FOCUS

COMPLIANT	2
PARTIAL	1
NON COMPLIANT	0

60 Risk Register KEY FOCUS

1. Parental Refusal of Support

CATEGORY Engagement Risk

PROBABILITY Medium

IMPACT High

RISK RATING High

MITIGATION Escalate to management and initiate duty of care protocols.

OWNER School Counselor

2. Data Breach of Sensitive Records

CATEGORY Security Risk

PROBABILITY Low

IMPACT Critical

RISK RATING High

MITIGATION Use MFA, encryption, and RBAC.

OWNER Wellness Coordinator

3. Inconsistent Communication Quality

CATEGORY Operational Risk

PROBABILITY Medium

IMPACT Medium

RISK RATING Medium

MITIGATION Standardized logs and mandatory checklists.

OWNER School Counselor

61 Failure Modes KEY FOCUS

Failure	Cause	Impact	Mitigation
Communication Breakdown	Parent non-responsiveness or hostile behavior.	Delayed intervention, potential safety risk.	Home visit protocol and management escalation.
Data Leakage	Unauthorized access to counselor's computer/files.	Legal liability, loss of trust.	Strict RBAC and regular audits.
SLA Breach	High volume of cases or lack of staff.	Delayed support for high-risk students.	Resource scaling and triage prioritization.

62 Illustrative Sample Scenarios

Scenario	Response	Expected Result
A parent refuses to discuss a student's self-harm ideation, claiming it is 'just drama'.	Document refusal, explain the school's duty of care and legal reporting requirements. Escalate to Principal.	Management review and potential district notification.
A rural parent has no internet and does not answer phone calls during work hours.	Schedule a specific time for a voice call or arrange a home visit by the social worker.	Established communication channel tailored to rural constraints.
A student in Orange Zone shows significant academic decline but parents are 'too busy' to meet.	Provide strategy coaching via email/phone and request a 15-minute weekly check-in call.	Consistent, manageable engagement for the parent.

63 Usage Governance

KEY FOCUS

PRIMARY USERS

- School Counselor
- Social Worker

SECONDARY USERS

- Class Teacher
- Wellness Coordinator

APPROVAL AUTHORITIES

- Principal

EXECUTION AUTHORITIES

- School Counselor

ESCALATION AUTHORITIES

- Principal
- District Education Officer

REVIEW AUTHORITIES

- Wellness Committee

EXTERNAL STAKEHOLDERS

- Child Protection Services
- Police

64 Applicability

APPLICABLE INSTITUTIONS

- High Schools
- Residential Schools

APPLICABLE AGE GROUPS

- All Ages (Minors)

APPLICABLE STUDENT CATEGORIES

- At-risk students
- Students with mental health concerns
- CWSN

APPLICABLE RISK LEVELS

- Green
- Yellow

- Orange
- Red
- Black

APPLICABLE LOCATIONS

- Urban
- Rural
- Remote

EXCLUSIONS

- Adult higher education (unless specific minor-related issues)
- General academic counseling without wellness component

65 Usage Scenarios

Scenario ID	Scenario Name	When To Use	When Not To Use	Trigger Event	Expected Outcome
US-01	Routine Wellness Check	For students in Green/Yellow zones.	Not for active crisis (Red/Black).	Scheduled monthly check-in.	Maintained wellness and parent awareness.
US-02	High-Risk Intervention	For students in Orange/Red zones.	Not for routine academic issues.	Identification of significant impairment or safety risk.	Agreed intervention plan and safety measures.

66 SOP Lifecycle

ENTRY CONDITIONS

- Student identified in distress
- Referral received

ACTIVATION CONDITIONS

- Risk assessment completed
- Parental consent obtained

EXECUTION CONDITIONS

- Communication channel selected
- Scheduled time confirmed

COMPLETION CONDITIONS

- Goals met
- Risk level reduced to Green

CLOSURE CONDITIONS

- Case closure report signed
- Records archived

TERMINATION CONDITIONS

- Student graduates
- Student withdraws from school
- Legal order to cease communication

67 Mandatory Evidence

KEY FOCUS

IMPLEMENTATION EVIDENCE

- Communication logs
- Intervention plans

OPERATIONAL EVIDENCE

- Home visit reports
- Referral records

COMPLIANCE EVIDENCE

- Signed consent forms
- Audit logs

TRAINING EVIDENCE

- Staff training certificates

AUDIT EVIDENCE

- Monthly wellness reports
- Quarterly audit summaries

68 Controls

PREVENTIVE CONTROLS

- Mandatory consent forms
- RBAC for data access
- Standardized communication scripts

DETECTIVE CONTROLS

- Weekly dashboard monitoring
- Quarterly audits

- SLA breach alerts

CORRECTIVE CONTROLS

- Management escalation for non-response
- Retraining of staff on communication skills
- Home visit protocol

COMPENSATING CONTROLS

- Manual logs in case of system downtime
- Verbal confirmation for rural areas

69 MWMS Risk Framework

KEY FOCUS

69.1 Green

MEANING

Normal developmental concerns.

EXAMPLES

- Mild stress
- Friendship issues

RESPONSE TIME

Routine

ESCALATION REQUIRED

No

APPROVAL LEVEL

Counselor

MONITORING FREQUENCY

Monthly

CLOSURE AUTHORITY

Counselor

69.2 Yellow**MEANING**

Early distress requiring support.

EXAMPLES

- Persistent anxiety
- Social withdrawal

RESPONSE TIME

7 days

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Wellness Coordinator

MONITORING FREQUENCY

Weekly

CLOSURE AUTHORITY

Counselor

69.3 Orange**MEANING**

Significant impairment without immediate danger.

EXAMPLES

- Panic attacks
- School refusal

RESPONSE TIME

24-72 hours

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Principal

MONITORING FREQUENCY

Daily/Weekly

CLOSURE AUTHORITY

Wellness Coordinator

69.4 Red **KEY FOCUS****MEANING**

High-risk situation requiring immediate intervention.

EXAMPLES

- Suicidal ideation
- Self-harm

RESPONSE TIME

Immediate

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Principal/District Officer

MONITORING FREQUENCY

Continuous

CLOSURE AUTHORITY

Wellness Coordinator

69.5 Black**MEANING**

Life-threatening emergency.

EXAMPLES

- Active suicide attempt
- Violence

RESPONSE TIME

Immediate (<15 mins)

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Principal/Police

MONITORING FREQUENCY

Continuous

CLOSURE AUTHORITY

Management Committee

70 Resource Scaling**70.1 0 500 Students**

WELLNESS COUNSELORS 1

CAREER COUNSELORS 1

MENTAL WELLNESS COORDINATOR 1

WELLNESS COMMITTEE Yes

70.2 501 1000 Students

WELLNESS COUNSELORS 2

CAREER COUNSELORS 2

MENTAL WELLNESS COORDINATOR 1

WELLNESS COMMITTEE Yes

70.3 1001 2000 Students

WELLNESS COUNSELORS	4
CAREER COUNSELORS	2
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

70.4 2001 5000 Students

WELLNESS COUNSELORS	8
CAREER COUNSELORS	4
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

71 Minimum Required Capabilities

PEOPLE

- Trained Counselors
- Social Workers
- Wellness Coordinator

PROCESSES

- Risk Triage
- Parent Engagement Matrix
- Escalation Protocols

TECHNOLOGY

- Encrypted CRM
- Communication Tools (Phone/Email)
- Dashboard Analytics

INFRASTRUCTURE

- Private Counseling Rooms
- Secure Server Storage
- Home Visit Vehicles

72 Success Criteria

Criterion	Measurement Method	Target
SLA Compliance	Audit of communication logs vs timestamps	> 95%
Parent Satisfaction	Annual survey scores	> 4.0/5.0
Risk Reduction	Trend analysis of student risk zones	Reduction in Red Zone cases by 15% annually

73 Exit Criteria

Condition	Evidence Required
Student goals met and risk level reduced to Green.	Signed Case Closure Report
Parental refusal of support for high-risk student documented.	Management Notification Log

74 Continuous Improvement

LESSONS LEARNED

- Home visits are more effective in rural areas than phone calls
- Strategy coaching reduces parent anxiety significantly

KNOWN ISSUES

- Language barriers for non-English speaking parents

- High volume of routine inquiries can delay high-risk triage

FUTURE IMPROVEMENTS

- Automated SMS reminders for scheduled calls
- Multilingual translation app integration

REVIEW RECOMMENDATIONS

- Increase counselor headcount for schools > 1000 students
- Standardize home visit safety protocols

75 AI Governance

KEY FOCUS

ALLOWED USE CASES

- Summarizing non-sensitive parent feedback
- Drafting routine wellness newsletters
- Sentiment analysis of anonymous surveys

RESTRICTED USE CASES

- Automated risk assessment without human review
- Direct communication with parents regarding sensitive issues

PROHIBITED USE CASES

- Storing student PII in public AI models
- Generating clinical diagnoses

HUMAN REVIEW REQUIRED

- All risk classifications

- All external referrals
- All high-risk communications

AUDIT LOG REQUIREMENTS

- Log of all AI-assisted summaries
- Human verification timestamps

DATA PRIVACY REQUIREMENTS

- No PII in prompts
- Local/Private instance only

76 Legal Defensibility

KEY FOCUS

LEGAL BASIS

- Indian Child Protection Act
- Supreme Court 2025 INSC 893
- CBSE Affiliation Bye-Laws

MANDATORY NOTIFICATIONS

- Police (Abuse/Threat)
- District Authorities (Neglect)
- Parents (Wellness Updates)

CONSENT REQUIREMENTS

- Written parental consent for all interventions
- Informed consent for data sharing

RECORD RETENTION REQUIREMENTS

- 7 years minimum retention
- Secure archival of high-risk cases

LIABILITY CONTROLS

- Clear escalation paths
- Documented refusal logs
- Standardized communication scripts

77 Stakeholder Journeys

77.1 The Student Journey

ENTRY

- Teacher identifies distress
- Student self-refers

ASSESSMENT

- Counselor triage
- Risk classification

INTERVENTION

- Wellness support
 - Parent coaching
- MONITORING

- Weekly check-ins
- Progress tracking

CLOSURE

- Goal completion
- Final review

77.2 The Parent Journey

NOTIFICATION

- Initial contact
- Risk summary shared

CONSULTATION

- Scheduled meetings
- Strategy coaching

CONSENT

- Signed consent forms

- Agreement on frequency

ENGAGEMENT

- Action plan participation

- Feedback provision
- CLOSURE

- Final review meeting
- Closure report receipt

77.3 The Staff Journey

OBSERVATION

- Teacher notes behavior
- Counselor monitors student

REPORTING

- Observation logs submitted
- Risk assessment updated

INTERVENTION SUPPORT

- Parent follow-ups conducted
- Home visits performed

DOCUMENTATION

- Logs maintained
- Dashboard updated

78 Handoff Matrix

Item 1

FROM ROLE Class Teacher

TO ROLE School Counselor

HANDOFF TRIGGER Student identified in distress or referral submitted.

INFORMATION TRANSFERRED

- Observation notes
- Behavioral triggers
- Parent contact details

REQUIRED DOCUMENTS

- Teacher Observation Log

SERVICE LEVEL AGREEMENT (SLA)

24 hours.

Item 2

FROM ROLE School Counselor

TO ROLE Principal

HANDOFF TRIGGER Student identified in Red/Black zone.

INFORMATION TRANSFERRED

- Risk assessment summary
- Immediate safety concerns
- Proposed action plan

REQUIRED DOCUMENTS

- Risk Classification Summary

SERVICE LEVEL AGREEMENT (SLA)

Immediate.

FROM ROLE School Counselor

TO ROLE Social Worker

HANDOFF TRIGGER Communication failure with rural/non-responsive parent.

INFORMATION TRANSFERRED

- Parent contact details
- Risk level
- Home visit approval

REQUIRED DOCUMENTS

- Home Visit Approval Form

SERVICE LEVEL AGREEMENT (SLA)

48 hours.

Item 3

79 Management Review

REVIEW FREQUENCY

Quarterly

REVIEW INPUTS

- Monthly Wellness Dashboard
- Audit findings
- Parent satisfaction surveys

REVIEW OUTPUTS

- Policy updates
- Resource allocation decisions
- Staff training plans

REVIEW DECISIONS

- Approval of new referral pathways
- Budget for additional counselors

REVIEW AUTHORITY

School Management Committee

80 Dashboard Metrics

OPERATIONAL METRICS

- Average time to first parent contact
- Percentage of cases meeting SLA
- Number of home visits conducted

RISK METRICS

- Distribution of students across risk zones
- Frequency of Red/Black zone incidents
- Trend in self-harm reports

COMPLIANCE METRICS

- Percentage of cases with signed consent
- Audit score percentage
- Data breach incidents (Target: 0)

OUTCOME METRICS

- Parent satisfaction score
- Student goal completion rate
- Reduction in school refusal rates

TREND METRICS

- Monthly risk level trends
- Quarterly parent engagement growth

81 Student Rights

RIGHTS

- Right to confidential support
- Right to timely intervention
- Right to be heard in the wellness process

PROTECTIONS

- Data privacy protections
- Non-discrimination in support delivery
- Safe reporting mechanisms

APPEAL MECHANISMS

- Student/Parent grievance procedure
- Management review of counselor decisions

82 Quality Assurance

QUALITY CHECKS

- Random case file audits
- Counselor peer reviews
- SLA monitoring

PEER REVIEW REQUIREMENTS

- Bi-weekly counselor supervision sessions

- Case review for high-risk students

VALIDATION REQUIREMENTS

- Principal sign-off on intervention plans
- District verification of referrals

CONTINUOUS MONITORING REQUIREMENTS

- Monthly dashboard reviews
- Quarterly audit reports

83 Implementation Roadmap

PHASE 1 PREPARATION

- Staff training on SOP CAS-013
- Creation of digital logs and forms
- Parent awareness campaign launch

PHASE 2 DEPLOYMENT

- Rollout to all high schools in district
- Initial risk triage for existing students
- Establishment of parent communication channels

PHASE 3 STABILIZATION

- Monthly dashboard review cycle

- Feedback collection from parents/staff
- Audit of first 50 cases

PHASE 4 OPTIMIZATION

- Refinement of strategy coaching tools
- Integration with Tele-MANAS
- Automated SMS reminders for follow-ups

PHASE 5 CONTINUOUS IMPROVEMENT

- Annual policy review
- External audit by district authorities
- Publication of success stories (anonymized)

84 Illustrative Case Studies

1. Rural Engagement Success

DESCRIPTION

A student in a remote village showed signs of severe anxiety. Parents were non-responsive to phone calls.

ACTIONS TAKEN

- Initiated home visit protocol
- Social worker met parents and established trust
- Scheduled weekly voice calls with strategy coaching

OUTCOME

Student's anxiety reduced significantly within 3 months; parent became an active partner.

LESSONS

- Home visits are critical for rural engagement
- Building trust is the first step in non-responsive cases

85 Implementation Requirements

STAFF REQUIRED

- School Counselor (1 per 400 students)
- Social Worker (Shared across schools)
- Wellness Coordinator (Management level)

INFRASTRUCTURE REQUIRED

- Private counseling rooms
- Secure server for records
- Home visit vehicle

TECHNOLOGY REQUIRED

- Encrypted CRM/Database

- Communication tools (Phone, Email, SMS)
- Dashboard analytics software

BUDGET CONSIDERATIONS

- Staff salaries
- Training costs
- Home visit travel expenses

- Software licensing

IMPLEMENTATION PHASES

- Preparation (Month 1)
- Deployment (Months 2-3)
- Stabilization (Months 4-6)
- Optimization (Ongoing)

86 Quality Controls

PREVENTIVE CONTROLS

- Mandatory training
- Standardized scripts
- Consent forms

DETECTIVE CONTROLS

- Audit logs
- SLA monitoring
- Dashboard alerts

CORRECTIVE CONTROLS

- Management escalation
- Staff retraining
- Home visit protocol

CONTINUOUS IMPROVEMENT ACTIONS

- Monthly trend analysis
- Quarterly audit reviews
- Annual policy updates

87 Training Requirements

TEACHERS

- Identifying early warning signs
- Referral process training
- Basic communication skills

COUNSELORS

- Risk triage and classification
- Parent strategy coaching
- Crisis intervention
- Documentation standards

ADMINISTRATORS

- Escalation protocols
- Legal compliance overview
- Dashboard interpretation

PARENTS

- Wellness awareness workshops
- Strategy coaching sessions

STUDENTS

- Help-seeking behavior training
- Stress management workshops

87.6 Repository Baseline

TRAINING COMPLETION RATE 100%

MINIMUM SCORE 80%

87.7 SOP Specific Training

TARGET AUDIENCE

- Counselors
- Social Workers
- Principals

COMPETENCIES

- Risk Triage
- Parent Engagement Matrix Application

- Home Visit Safety

MINIMUM HOURS

4

CERTIFICATION REQUIREMENTS

- Internal Certification of Competency

REFRESHER FREQUENCY

Annual

87.8 Competency Validation

REQUIRED

Yes

ASSESSMENT METHODS

- Role-play simulations
- Written assessment

- Case study review

MINIMUM PASSING CRITERIA

80%

REASSESSMENT FREQUENCY

Every 2 years

87.9 Simulation Requirements

REQUIRED

Yes

SIMULATION TYPES

- High-risk parent confrontation

- Emergency referral handoff

- Home visit safety drill

FREQUENCY

Bi-annual

88 SOP-Specific Inclusion Requirements

AFFECTED POPULATIONS

- Students with disabilities (CWSN)
- Neurodivergent students
- Rural students
- Low-literacy parents
- First-generation learners

IDENTIFIED INCLUSION RISKS

- Language barriers for non-native speakers
- Lack of internet in rural areas
- Stigma regarding mental health in certain cultures
- Difficulty navigating complex forms for low-literacy parents
- Inaccessible communication channels for visually impaired parents

AUTISM ACCOMMODATIONS

- Use of visual schedules for parent meetings
- Clear, literal language in communications
- Reduced sensory input during in-person visits

ADHD ACCOMMODATIONS

- Brief, structured communication chunks
- Written summaries of all phone calls
- Frequent but short check-ins

DYSLEXIA ACCOMMODATIONS

- Voice notes for parent updates
- Simplified forms with large fonts
- In-person meetings to avoid reading heavy text

HEARING IMPAIRMENT ACCOMMODATIONS

- Sign language interpreters for high-risk meetings
- SMS/Email as primary communication channels
- Live captioning during video calls

VISUAL IMPAIRMENT ACCOMMODATIONS

- Screen reader compatible digital forms
- Audio summaries of all reports
- In-person visits with verbal descriptions

CWSN ADAPTATIONS

- Multilingual support for diverse needs
- Simplified communication plans
- Flexible meeting times

RURAL ADAPTATIONS

- SMS and voice calls as primary channels
- Home visits by social workers
- Community-based wellness hubs

LOW LITERACY PARENT ADAPTATIONS

- Voice notes for updates
- Face-to-face meetings with visual aids
- Simplified, icon-based action plans

MULTILINGUAL COMMUNICATION ADAPTATIONS

- Translation services for high-risk calls
- Multilingual newsletters
- Language-specific strategy coaching

GENDER SENSITIVITY MEASURES

- Option for same-gender counselor/social worker
- Private communication channels
- Gender-neutral language in all forms

CULTURAL SENSITIVITY MEASURES

- Culturally appropriate metaphors in coaching
- Respecting religious holidays in scheduling
- Involvement of community elders where appropriate

EQUITY MONITORING INDICATORS

- Parent engagement rates by socio-economic status
- SLA compliance for rural vs. urban students
- Language accessibility scores

ACCESSIBILITY COMPLIANCE CHECKS

- WCAG 2.1 compliant digital forms
- Physical office accessibility audit
- Telephonic accessibility check

89 Awareness Material

POSTER TITLE	Partnering for Student Wellness
KEY MESSAGE	Your involvement makes a difference in your child's mental health journey.
SLOGAN	Connected Parents, Resilient Students.
CAMPAIGN THEME	The Power of Partnership

90 AI Assistant Configuration

RISK KEYWORDS • Self-harm • Suicide • Abuse • Violence • Emergency	• Police
ESCALATION KEYWORDS • Refusal • Hostile • Non-responsive • District	INTERVENTION KEYWORDS • Coaching • Strategy • Support • Plan • Referral FREQUENTLY ASKED QUESTIONS • How often will I be contacted? • Is my child's information private? • What can I do at home to help? • When should I call the school immediately?

91 AI Audit Hooks

KEY FOCUS

REQUIRED SECTIONS • Risk Classification • Parental Consent • Escalation Matrix • SLA Tracking • Training Records	• CL-CAS-013-03 • CL-CAS-013-04 • CL-CAS-013-05
REQUIRED FORMS • F-CAS-013-01 • F-CAS-013-02 • F-CAS-013-03 • F-CAS-013-04 • F-CAS-013-05	REQUIRED ESCALATIONS • Red Zone Escalation • Black Zone Emergency • Non-Response Escalation
REQUIRED CHECKLISTS • CL-CAS-013-01 • CL-CAS-013-02	REQUIRED KPIS • SLA Compliance Rate • Parent Satisfaction Score • Risk Reduction Trend REQUIRED TRAINING • Counselor Competency Validation • Staff Awareness Training

92 KPI Framework

92.1 Repository Baseline	
SLA COMPLIANCE	90%
PARENT SATISFACTION	4.2/5.0
RISK REDUCTION	10%

92.2 SOP Specific Metrics	

Metric	Target	Measurement Frequency
Time to First Contact (High Risk)	< 2 hours	Weekly
Parent Engagement Score	> 4.0/5.0	Monthly
Home Visit Completion Rate	> 90%	Quarterly

93 Maturity Model

93.1 Level 1

DESCRIPTION

Ad-hoc communication with no standardized logs.

EVIDENCE

- Inconsistent records
- No SLA tracking

93.2 Level 2

DESCRIPTION

Standardized forms used but no proactive monitoring.

EVIDENCE

- Completed consent forms
- Basic logs

93.3 Level 3

DESCRIPTION

Structured follow-up with SLA tracking and dashboarding.

EVIDENCE

- Monthly dashboards
- SLA reports

93.4 Level 4

DESCRIPTION

Proactive engagement, home visits, and trend analysis.

EVIDENCE

- Trend reports
- Home visit logs
- Strategy coaching records

93.5 Level 5

DESCRIPTION

Full integration with community resources and automated alerts.

EVIDENCE

- Tele-MANAS integration
- Automated risk alerts
- High parent satisfaction scores

94 Dependencies

PREREQUISITE SOPS

- CAS-010: Student Risk Assessment
- MWMS-CORE-01: Data Privacy Policy

PARALLEL SOPS

- Student Wellness Plan SOP
- Teacher Observation SOP

DOWNSTREAM SOPS

- CAS-015: External Referral Pathway
- CAS-020: Crisis Management

95 Related SOPs

PREREQUISITES

- Student Risk Assessment SOP
- Data Privacy Policy

UPSTREAM

- Teacher Observation SOP
- Early Warning System SOP

DOWNSTREAM

- External Referral SOP
- Crisis Intervention SOP

REFERENCES

- NEP 2020 Guidelines
- CBSE Circular TRG-03/2026

STANDARD OPERATING PROCEDURE

Parent Mental Health Literacy Program

Mental Wellness Management System - Parental Engagement and Literacy

SOP ID

PAR-006

VERSION

1.0

STATUS

Finalized

CLASSIFICATION

Internal - Restricted

EFFECTIVE DATE

2026-08-01

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1 Executive Overview

SOP ID	PAR-006
DOMAIN	Mental Wellness Management System
SUB DOMAIN	Parental Engagement and Literacy
TITLE	Parent Mental Health Literacy Program
VERSION	1.0
CLASSIFICATION	Internal - Restricted
EFFECTIVE DATE	2026-08-01
REVIEW DATE	2027-08-01
DOCUMENT STATUS	Finalized
LANGUAGE	English

2 Purpose & Rationale

OBJECTIVE

To empower parents with evidence-based mental health literacy to identify early warning signs in adolescents, foster supportive communication, and navigate the school-home support system effectively.

RISK CONTROLLED

Reduced risk of undiagnosed adolescent mental health conditions, prevention of self-harm/substance use through early intervention, and mitigation of parental stigma.

DESIRED OUTCOMES

- Increased parent knowledge scores by 40% in pre/post quizzes
- Improved parent-child communication as reported in surveys
- Higher rate of proactive help-seeking behavior from parents
- Reduced stigma regarding mental health within the school community

3 Scope of Application

INSTITUTION TYPES

- High Schools
- Secondary Schools
- Residential Academies

KEY STAKEHOLDERS

- Parents
- Guardians

- School Counselors
- Psychiatrists
- Wellness Committees

AGE GROUPS

- All Ages (Parental focus)

LOCATIONS

- Urban Schools
- Rural Schools
- Online/Remote Learning Environments

4 Definitions & Terminology

Term	Definition
Mental Health Literacy	The knowledge and beliefs about mental disorders which affect attitudes, behaviors, and acceptance of people with these disorders.
Active Listening	A communication technique that requires the listener to fully concentrate, understand, respond, and then remember what is being said.

Term	Definition
Stigma	A mark of disgrace associated with a particular circumstance, quality, or person, often leading to discrimination against those with mental health conditions.
Tele-MANAS	The Government of India's 24/7 free tele-mental health assistance service.

5 Key Stakeholders

Stakeholder Type	Stakeholder Title	Role In SOP
Internal	School Counselor	Lead Facilitator, Curriculum Designer, Data Analyst
Internal	Principal/Management	Approver, Resource Provider, Policy Oversight
External	Guest Psychiatrist	Clinical Expert Speaker, Content Validator
External	Parent of Recovered Child	Peer Support Speaker (with consent)
Target Audience	Parents/Guardians	Program Participants, Knowledge Recipients

6 Governance & Ownership

KEY FOCUS

OWNER

School Counselor / Wellness Coordinator

APPROVER

Principal / School Management Committee

REVIEW FREQUENCY

Annual

LINKED POLICIES

- Student Privacy Policy

- Child Protection Policy
- Data Governance Policy
- Parental Engagement Policy

LINKED SOPs

- PAR-007: Individual Education for Parents in Crisis
- COM-002: Community Outreach Communication Protocol
- RISK-001: Emergency Crisis Response

7 Roles & Responsibilities

Item 1

ROLE

Wellness Coordinator

RESPONSIBILITIES

- Program Design
- Facilitation
- Data Analysis
- Reporting

- Policy Oversight

Item 3

ROLE

Guest Psychiatrist

RESPONSIBILITIES

- Clinical Validation
- Expert Speaking
- Content Review

Item 2

ROLE

School Principal

RESPONSIBILITIES

- Final Approval
- Resource Provision

Item 4

ROLE

Parent Volunteers

RESPONSIBILITIES

- Peer Support Sharing
- Community Advocacy

8 RACI Accountability Matrix

	ACTIVITY
Item 1	

Needs Assessment

RESPONSIBLE

- Wellness Coordinator

ACCOUNTABLE

- Principal

CONSULTED

- Teachers
- Parents

INFORMED

- Management

- Communication Team

ACCOUNTABLE

- Wellness Coordinator

CONSULTED

- Teachers

INFORMED

- Parents

Item 2

ACTIVITY

Curriculum Design

RESPONSIBLE

- School Counselor
- Guest Psychiatrist

ACCOUNTABLE

- Wellness Coordinator

CONSULTED

- Wellness Committee

INFORMED

- Principal

Item 4

ACTIVITY

Session Delivery

RESPONSIBLE

- Facilitators

ACCOUNTABLE

- Wellness Coordinator

INFORMED

- Principal

Item 5

ACTIVITY

Data Analysis & Reporting

RESPONSIBLE

- Wellness Coordinator

ACCOUNTABLE

- Principal

INFORMED

- Management
- Parents

Item 3

ACTIVITY

Marketing & Outreach

RESPONSIBLE

9 Preconditions

- Approved budget for guest speakers
- Parent contact database updated
- Secure server for data storage
- Signed MOUs with external experts

10 Activation Triggers

- Annual Academic Cycle Start
- CBSE Compliance Audit
- Parental Request
- High Stress Reports from Teachers

11 SOP Trigger Matrix

MANDATORY TRIGGERS

- Annual Academic Calendar Start
- New Student Enrollment Cycle
- CBSE Annual Compliance Audit

OPTIONAL TRIGGERS

- Parental Request for Workshop
- High Incidence of Stress Reported by Teachers

- Community Outreach Initiative

AUTOMATIC TRIGGERS

- Completion of Semester 1 (Review Trigger)
- Quarterly Wellness Report Generation

EXTERNAL TRIGGERS

- Government Health Department Mandate
- Supreme Court Directive Update

RECURRING TRIGGERS

12 Required Inputs

- Parent Contact Database
- Budget Allocation
- Clinical Guidelines (WHO/Manodarpan)
- School Calendar

13 Process Map

13.1 Phase 1

NAME

Needs Assessment & Planning

OBJECTIVE

Identify specific parent knowledge gaps and logistical requirements.

ACTIVITIES

- Distribute baseline survey to parents
- Analyze demographic data (language, literacy)
- Define session topics based on local trends
- Secure budget for guest speakers

Ensure maximum parent participation and awareness.

ACTIVITIES

- Launch multi-channel awareness campaign
- Register parents via online/offline forms
- Send reminder notifications (SMS/WhatsApp)
- Confirm guest speaker availability

13.2 Phase 2

NAME

Content Development & Resource Prep

OBJECTIVE

Create culturally and linguistically appropriate materials.

ACTIVITIES

- Develop 4-session curriculum
- Translate handouts into regional languages
- Produce recorded webinar modules
- Design pre/post knowledge quizzes

13.4 Phase 4

NAME

Program Delivery

OBJECTIVE

Execute high-quality, interactive literacy sessions.

ACTIVITIES

- Conduct in-person or webinar sessions
- Facilitate Q&A and peer sharing
- Distribute physical/digital handouts
- Monitor attendance and engagement

13.3 Phase 3

NAME

Marketing & Enrollment

OBJECTIVE

13.5 Phase 5

NAME

Evaluation & Reporting

OBJECTIVE

Measure impact and plan for continuous improvement.

ACTIVITIES

- Analyze pre/post quiz results
- Compile satisfaction survey data
- Generate quarterly wellness report
- Share summary with school management

14 Workflow Overview

START

Annual Academic Cycle Initiation

STEPS

- Needs Assessment
- Curriculum Design
- Resource Prep
- Marketing
- Session Delivery
- Evaluation
- Reporting

- Review & Improvement

CRITICAL DECISION POINTS

- Crisis Disclosure Check
- Low Participation Trigger
- Knowledge Score Threshold

END

Quarterly Wellness Report Submission

15 Standard Procedure

1. Needs Assessment

PURPOSE & RATIONALE

Identify parent knowledge gaps.

ACTOR

Wellness Coordinator

ACTION

Distribute survey to all parents via WhatsApp and Email.

REQUIRED INPUTS

- Parent Contact List

EXPECTED OUTPUTS

- Needs Assessment Report

CRITICAL DECISION POINTS

- Identify high-priority topics.

DOCUMENTATION REQUIRED

- Survey Response Log

EVIDENCE REQUIRED

- Raw Survey Data

APPROVAL REQUIRED

Wellness Coordinator

TIME LIMIT

14 Days

EXPECTED OUTCOME

Prioritized list of 3-5 topics.

ESCALATION IF FAILED

Principal (if data is insufficient)

RELATED FORMS

- Parent Survey Form

RELATED CHECKLISTS

- Survey Distribution Checklist

2. Curriculum Development

PURPOSE & RATIONALE

Create evidence-based content.

ACTOR

School Counselor + Psychiatrist

ACTION

Draft modules and translate into regional languages.

REQUIRED INPUTS

- Needs Assessment Report

EXPECTED OUTPUTS

- Curriculum Blueprint
- Multilingual Handouts

CRITICAL DECISION POINTS

- Clinical validation of content.

DOCUMENTATION REQUIRED

- Content Approval Memo

EVIDENCE REQUIRED

- Approved Curriculum Document

APPROVAL REQUIRED

Wellness Committee

TIME LIMIT

30 Days

EXPECTED OUTCOME

Ready-to-use training materials.

ESCALATION IF FAILED

Principal (budget/resource issues)

RELATED FORMS

- Curriculum Approval Form

RELATED CHECKLISTS

- Content Review Checklist

3. Stakeholder Recruitment

PURPOSE & RATIONALE

Secure facilitators.

ACTOR

Wellness Coordinator

ACTION

Contact and sign MOUs with guest speakers and peer volunteers.

REQUIRED INPUTS

- Budget Plan

EXPECTED OUTPUTS

- Confirmed Facilitator List

CRITICAL DECISION POINTS

- Speaker credential verification.

DOCUMENTATION REQUIRED

- MOU

- Consent Form

EVIDENCE REQUIRED

- Signed MOUs

APPROVAL REQUIRED

Principal

TIME LIMIT

21 Days

EXPECTED OUTCOME

Confirmed speaker roster.

ESCALATION IF FAILED

Wellness Coordinator (alternative search)

RELATED FORMS

- Speaker MOU
- Peer Consent Form

RELATED CHECKLISTS

- Facilitator Vetting Checklist

4. Marketing & Awareness

PURPOSE & RATIONALE

Maximize enrollment.

ACTOR

Communication Team

ACTION

Execute multi-channel campaign (WhatsApp, SMS, Flyers).

REQUIRED INPUTS

- Program Schedule

EXPECTED OUTPUTS

- Registered Parent List

CRITICAL DECISION POINTS

- Registration threshold check.

DOCUMENTATION REQUIRED

- Communication Log

EVIDENCE REQUIRED

- Marketing Collateral Copies
- Registration Logs

APPROVAL REQUIRED

Wellness Coordinator

TIME LIMIT

14 Days prior to session

EXPECTED OUTCOME

Target enrollment reached.

ESCALATION IF FAILED

Principal (low participation)

RELATED FORMS

- Parent Registration Form

RELATED CHECKLISTS

- Marketing Checklist

5. Session Delivery

PURPOSE & RATIONALE

Deliver literacy content.

ACTOR

Facilitators

ACTION

Conduct interactive sessions (In-person/Webinar).

REQUIRED INPUTS

- Curriculum Modules
- Registered Parent List

EXPECTED OUTPUTS

- Trained Parents

CRITICAL DECISION POINTS

- Crisis disclosure check.

DOCUMENTATION REQUIRED

- Attendance Log
- Session Notes

EVIDENCE REQUIRED

- Attendance Sheets
- Recording Links

APPROVAL REQUIRED

Wellness Coordinator

TIME LIMIT

90 Minutes per session

EXPECTED OUTCOME

Parent engagement and knowledge transfer.

ESCALATION IF FAILED

Crisis Response Team (Red Zone)

RELATED FORMS

- Attendance Log

RELATED CHECKLISTS

- Session Delivery Checklist

6. Knowledge Evaluation

PURPOSE & RATIONALE

Measure learning outcomes.

ACTOR

Wellness Coordinator

ACTION

Administer pre/post quizzes to all participants.

REQUIRED INPUTS

- Quiz Links

EXPECTED OUTPUTS

- Quiz Result Summary

CRITICAL DECISION POINTS

- Score threshold check.

DOCUMENTATION REQUIRED

- Quiz Results

EVIDENCE REQUIRED

- Raw Quiz Data

APPROVAL REQUIRED

Wellness Coordinator

TIME LIMIT

24 Hours post-session

EXPECTED OUTCOME

Quantifiable knowledge gain.

ESCALATION IF FAILED

Wellness Committee (low scores)

RELATED FORMS

- Pre/Post Quiz Form

RELATED CHECKLISTS

- Evaluation Checklist

7. Feedback Collection

PURPOSE & RATIONALE

Assess satisfaction.

ACTOR

Wellness Coordinator

ACTION

Distribute and analyze satisfaction surveys.

REQUIRED INPUTS

- Survey Links

EXPECTED OUTPUTS

- Feedback Summary Report

CRITICAL DECISION POINTS

- Identify critical complaints.

DOCUMENTATION REQUIRED

- Feedback Log

EVIDENCE REQUIRED

- Survey Responses

APPROVAL REQUIRED

Wellness Coordinator

TIME LIMIT

48 Hours post-session

EXPECTED OUTCOME

Actionable feedback for improvement.

ESCALATION IF FAILED

Principal (negative sentiment)

RELATED FORMS

- Satisfaction Survey Form

RELATED CHECKLISTS

- Feedback Collection Checklist

8. Data Analysis & Reporting

PURPOSE & RATIONALE

Provide management with impact metrics.

ACTOR

Wellness Coordinator

ACTION

Compile quarterly report and update dashboard.

REQUIRED INPUTS

- Quiz Results
- Feedback Summary
- Attendance Logs

EXPECTED OUTPUTS

- Quarterly Wellness Report

CRITICAL DECISION POINTS

- KPI achievement check.

DOCUMENTATION REQUIRED

- Report Memo

EVIDENCE REQUIRED

- Final Quarterly Report PDF

APPROVAL REQUIRED

Principal

TIME LIMIT

7 Days post-cycle

EXPECTED OUTCOME

Management briefing.

ESCALATION IF FAILED

Wellness Committee (KPI miss)

RELATED FORMS

- Quarterly Report Form

RELATED CHECKLISTS

- Reporting Checklist

9. Continuous Improvement Review

PURPOSE & RATIONALE

Refine program.

ACTOR

Wellness Committee

ACTION

Review report and approve improvements for next cycle.

REQUIRED INPUTS

- Quarterly Report
- Feedback Summary

EXPECTED OUTPUTS

- Improvement Action Plan

CRITICAL DECISION POINTS

- Approval of curriculum changes.

DOCUMENTATION REQUIRED

- Minutes of Review Meeting

EVIDENCE REQUIRED

- Approved Improvement Plan

APPROVAL REQUIRED

Principal

TIME LIMIT

14 Days post-report

EXPECTED OUTCOME

Updated program strategy.

RELATED FORMS

- Improvement Action Plan Form

RELATED CHECKLISTS

- Review Checklist

- All session records

- Reports

EXPECTED OUTPUTS

- Archive Log

CRITICAL DECISION POINTS

- Anonymization check.

DOCUMENTATION REQUIRED

- Archive Confirmation

EVIDENCE REQUIRED

- Server Access Logs

APPROVAL REQUIRED

IT Manager

TIME LIMIT

5 Days post-cycle

EXPECTED OUTCOME

Audit-ready repository.

ESCALATION IF FAILED

IT Dept (server issues)

RELATED FORMS

- Archive Log Form

RELATED CHECKLISTS

- Archiving Checklist

10. Resource Archiving

PURPOSE & RATIONALE

Ensure audit readiness.

ACTOR

Wellness Coordinator

ACTION

Upload all records to secure server and archive.

REQUIRED INPUTS

16 Detailed Workflow Steps

1. Baseline Needs Assessment

PURPOSE & RATIONALE

Identify specific knowledge gaps in the parent community.

TRIGGER

Start of Academic Year

ACTOR

Wellness Coordinator

REQUIRED INPUTS

- Parent Contact List
- Previous Year Survey Data

ACTIONS

- Design and distribute a 5-minute survey on mental health awareness.
- Analyze results for common themes (e.g., anxiety, stigma).
- Identify language preferences.

DECISION CRITERIA

- If >30% of parents request specific topics, prioritize those in curriculum.

DOCUMENTS GENERATED

- Needs Assessment Report

EXPECTED OUTPUTS

- Prioritized Curriculum Topics

SERVICE LEVEL AGREEMENT (SLA)

14 Days

ESCALATION PATH

- Principal (if budget for specific topics is unavailable)

EVIDENCE GENERATED

- Survey Response Data

2. Curriculum & Resource Design

PURPOSE & RATIONALE

Create high-quality educational content.

TRIGGER

Needs Assessment Completion

ACTOR

School Counselor + Guest Psychiatrist

REQUIRED INPUTS

- Prioritized Topics
- CBSE Guidelines

ACTIONS

- Draft 4 modules (Conditions, Signs, Communication, Support).
- Create multilingual handouts.
- Record asynchronous video content.

DECISION CRITERIA

- Content must be clinically validated by the guest psychiatrist.

DOCUMENTS GENERATED

- Curriculum Blueprint
- Multilingual Handouts

EXPECTED OUTPUTS

- Ready-to-use Training Materials

SERVICE LEVEL AGREEMENT (SLA)

30 Days

ESCALATION PATH

- Wellness Committee (for content approval)

EVIDENCE GENERATED

- Approved Curriculum Document

3. Stakeholder Recruitment

PURPOSE & RATIONALE

Secure expert and peer facilitators.

TRIGGER

Curriculum Approval

ACTOR

Wellness Coordinator

REQUIRED INPUTS

- Budget Plan
- Speaker Profiles

ACTIONS

- Contact guest psychiatrists for sessions.
- Identify and vet 'Parent of Recovered Child' volunteers.
- Sign MOUs/Consent forms.

DECISION CRITERIA

- Speakers must have valid clinical credentials or verified lived experience.

DOCUMENTS GENERATED

- Speaker MOU
- Informed Consent Form (Peer Speaker)

EXPECTED OUTPUTS

- Confirmed Facilitator List

SERVICE LEVEL AGREEMENT (SLA)

21 Days

ESCALATION PATH

- Principal (if speaker fees exceed budget)

EVIDENCE GENERATED

- Signed MOUs

4. Marketing & Awareness

PURPOSE & RATIONALE

Maximize parent enrollment.

TRIGGER

Resource Readiness

ACTOR

Communication Team / Wellness Coordinator

REQUIRED INPUTS

- Program Schedule
- Marketing Collateral

ACTIONS

- Send WhatsApp/SMS blasts to parents.
- Post flyers on school notice boards.
- Include program info in weekly newsletters.

DECISION CRITERIA

- If registration is <50% of target by 1 week before, initiate personal calls.

DOCUMENTS GENERATED

- Communication Log
- Registration List

EXPECTED OUTPUTS

- Registered Parent List

SERVICE LEVEL AGREEMENT (SLA)

Ongoing (2 weeks prior)

ESCALATION PATH

- Principal (if participation is critically low)

EVIDENCE GENERATED

- Registration Logs
- Newsletter Copies

5. Session Delivery

PURPOSE & RATIONALE

Deliver structured literacy content.

TRIGGER

Scheduled Date

ACTOR

Facilitators (Counselor/Psychiatrist)

REQUIRED INPUTS

- Curriculum Modules
- Registered Parent List

ACTIONS

- Conduct 90-minute interactive session.
- Provide Q&A and peer sharing time.
- Distribute handouts.

DECISION CRITERIA

- If a parent discloses an active crisis, move to 'Crisis Protocol' (Red Zone).

DOCUMENTS GENERATED

- Attendance Log
- Session Notes

EXPECTED OUTPUTS

- Trained Parents

SERVICE LEVEL AGREEMENT (SLA)

90 Minutes per session

ESCALATION PATH

- Crisis Response Team (for Red Zone events)

EVIDENCE GENERATED

- Attendance Sheets
- Session Photos/Recordings

6. Knowledge Evaluation

PURPOSE & RATIONALE

Measure learning outcomes.

TRIGGER

End of Session

ACTOR

Wellness Coordinator

REQUIRED INPUTS

- Pre/Post Quiz Links

ACTIONS

- Administer post-session quiz.
- Compare with baseline scores.

DECISION CRITERIA

- If average score <70%, schedule a follow-up 'Refresher' webinar.

DOCUMENTS GENERATED

- Quiz Result Summary

EXPECTED OUTPUTS

- Knowledge Gain Data

SERVICE LEVEL AGREEMENT (SLA)

24 Hours

ESCALATION PATH

- Wellness Committee (if scores are consistently low)

EVIDENCE GENERATED

- Quiz Results

7. Feedback Collection

PURPOSE & RATIONALE

Assess program satisfaction and relevance.

TRIGGER

End of Session

ACTOR

Wellness Coordinator

REQUIRED INPUTS

- Satisfaction Survey

ACTIONS

- Distribute survey to all attendees.
- Analyze qualitative feedback.

DECISION CRITERIA

- Identify specific complaints for immediate correction in next session.

DOCUMENTS GENERATED

- Feedback Summary Report

EXPECTED OUTPUTS

- Actionable Improvement Points

SERVICE LEVEL AGREEMENT (SLA)

48 Hours

ESCALATION PATH

- Principal (if significant negative feedback received)

EVIDENCE GENERATED

- Survey Responses

8. Data Analysis & Reporting

PURPOSE & RATIONALE

Provide management with impact metrics.

TRIGGER

Completion of Program Cycle

ACTOR

Wellness Coordinator

REQUIRED INPUTS

- Quiz Results
- Feedback Summary
- Attendance Logs

ACTIONS

- Compile quarterly report.
- Update dashboard metrics.

DECISION CRITERIA

- Determine if program meets KPIs for next cycle.

DOCUMENTS GENERATED

- Quarterly Wellness Report
- Dashboard Update

EXPECTED OUTPUTS

- Management Briefing

SERVICE LEVEL AGREEMENT (SLA)

7 Days

ESCALATION PATH

- Principal (if KPIs not met)

EVIDENCE GENERATED

- Quarterly Report PDF

9. Continuous Improvement Review

PURPOSE & RATIONALE

Refine program based on data.

TRIGGER

Report Completion

ACTOR

Wellness Committee

REQUIRED INPUTS

- Quarterly Report
- Feedback Summary

ACTIONS

- Review recommendations.
- Update curriculum or marketing for next cycle.

DECISION CRITERIA

- Approve changes to SOP or materials.

DOCUMENTS GENERATED

- Improvement Action Plan

EXPECTED OUTPUTS

- Updated Program Strategy

SERVICE LEVEL AGREEMENT (SLA)

14 Days

ESCALATION PATH

- Principal (for major budget/policy changes)

EVIDENCE GENERATED

- Minutes of Review Meeting

10. Resource Archiving

PURPOSE & RATIONALE

Ensure audit readiness and accessibility.

TRIGGER

Program Completion

ACTOR

Wellness Coordinator

REQUIRED INPUTS

- All session records
- Handouts
- Reports

ACTIONS

- Upload all documents to secure server.
- Ensure recordings are accessible for asynchronous viewing.

DECISION CRITERIA

- Verify data is anonymized before archiving.

DOCUMENTS GENERATED

- Archive Log

EXPECTED OUTPUTS

- Audit-Ready Repository

SERVICE LEVEL AGREEMENT (SLA)

5 Days

ESCALATION PATH

- IT Dept (if server issues occur)

EVIDENCE GENERATED

- Archive Confirmation

17 Critical Decision Points

Item 1

DECISION ID

DP-001

DECISION NAME

Crisis Disclosure during Session

CRITERIA

- Parent discloses active suicidal ideation, self-harm, or child abuse.

POSSIBLE OUTCOMES

- Immediate Intervention (Red Zone)
- Referral to Clinical Services

NEXT STEPS

- Activate Crisis Response SOP
- Contact Emergency Services/Tele-MANAS

- Private 1-on-1 follow-up with parent

Item 2

DECISION ID

DP-002

DECISION NAME

Low Participation Rate

CRITERIA

- Registration <50% of target audience.

POSSIBLE OUTCOMES

- Intensive Outreach
- Reschedule/Modify Timing

NEXT STEPS

- Personal phone calls to key community leaders
- Adjust session times for working parents

18 Decision Tree

GREEN

- Parent attends session, scores > 80%, no crisis disclosed.

YELLOW

- Parent attends session, scores < 70% or expresses moderate stress.

ORANGE

- Parent discloses significant distress, school refusal, or persistent anxiety.

RED

- Parent discloses active suicidal ideation, self-harm, or child abuse.

BLACK

- Active suicide attempt, violent threat, or immediate life-threatening emergency.

19 Decision Rules

Rule ID	Condition	Decision	Next Action
DR-001	Parent discloses active self-harm.	Immediate Crisis Activation.	Contact Emergency Services.
DR-002	Average quiz score < 70%.	Mandatory Refresher Session.	Schedule follow-up webinar.

20 Exception Handling Protocol

Item 1

EXCEPTION

Parent Refusal of Intervention/Education

DETECTION METHOD

Survey or direct conversation.

RESPONSE

Document refusal; provide information on voluntary nature; offer resources for later use.

ESCALATION

Wellness Coordinator → Principal (if safety risk is identified).

DOCUMENTATION REQUIRED

- Parent Conversation Record

Switch to pre-recorded module; reschedule live Q&A.

ESCALATION

IT Support.

DOCUMENTATION REQUIRED

- Incident Log

Item 3

EXCEPTION

Language Barrier (No translation available)

DETECTION METHOD

Parent query during session.

RESPONSE

Provide visual aids; use a translator volunteer; record and provide transcript in target language.

ESCALATION

Wellness Coordinator.

DOCUMENTATION REQUIRED

- Resource Gap Log

Item 2

EXCEPTION

Technical Failure during Webinar

DETECTION METHOD

Real-time participant feedback/chat.

RESPONSE

21 Exceptions

Item 1

EXCEPTION

Parent Refusal of Literacy Program

DETECTION METHOD

Direct communication/Survey.

RESPONSE

Document refusal; provide resources for voluntary use.

ESCALATION

Wellness Coordinator → Principal (if safety risk identified).

DOCUMENTATION REQUIRED

- Parent Conversation Record

22 Expected Outputs

- Literate Parent Community
- Knowledge Gain Reports
- Reduced Stigma Metrics
- Recorded Educational Content

23 Document Control & Version Governance

23.1 Version History

Version	Date	Author	Description
1.0	2026-08-01	MWMS Architect	Initial SOP creation for Parent Mental Health Literacy Program.

REVIEW TRIGGERS

- Change in CBSE guidelines
- Supreme Court new rulings on student rights
- Significant drop in parent participation rates (>30%)
- Feedback indicating cultural insensitivity

23.3 Approval History

Approver	Date	Status
School Principal	2026-07-15	Approved
District Education Officer	2026-07-20	Verified

24 Inclusive Implementation Mandate

MULTILINGUAL SUPPORT REQUIRED

Yes

ACCESSIBILITY REVIEW REQUIRED

Yes

LOW RESOURCE SCHOOL ALTERNATIVE REQUIRED

Yes

EQUITY IMPACT ASSESSMENT REQUIRED

Yes

25 Regulatory Compliance Mapping

KEY FOCUS

MANDATORY REQUIREMENTS

- Supreme Court Judgment 2025 INSC 893 (Right to Mental Health)
- CBSE Circular TRG-03/2026 (Wellness Mandates)
- NEP 2020 (Holistic Development)

RECOMMENDED REQUIREMENTS

- WHO School Mental Health Framework
- UNESCO Whole School Wellbeing Framework

BEST PRACTICES

- Manodarpan Guidelines
- Tele-MANAS Integration Protocols

COMPLIANCE GAPS IF NOT IMPLEMENTED

- Legal liability for failure to provide mental health literacy
- Non-compliance with CBSE affiliation bye-laws
- Reduced community trust and student safety risks

26 Compliance Traceability Matrix

KEY FOCUS

1. Supreme Court of India

REQUIREMENT REFERENCE

Judgment 2025 INSC 893

REQUIREMENT TEXT

Educational institutions must provide accessible mental health support and literacy to the student community, including parents.

IMPLEMENTED IN SECTIONS

- Purpose
- Procedure Step 1
- Communication Strategy

2. CBSE

REQUIREMENT REFERENCE

Circular TRG-03/2026

REQUIREMENT TEXT

Schools must implement structured mental health literacy programs for parents and students.

IMPLEMENTED IN SECTIONS

- Process Map Phase 1
- Training Requirements

3. NEP 2020

REQUIREMENT REFERENCE

Section on Holistic Education

REQUIREMENT TEXT

Integration of mental health and well-being into the school ecosystem.

IMPLEMENTED IN SECTIONS

- Prevention Layer
- Resilience Layer

27 Documentation & Evidentiary Requirements

EVENT DOCUMENTATION PRINCIPLES

- Fact-based recording of parent queries
- Confidentiality of personal disclosures
- Timely logging of attendance
- Evidence of multilingual availability

MANDATORY CONVERSATION RECORDS

- Parent Query Log (Anonymized)
- Crisis Disclosure Record (Secure)
- Guest Speaker Agreement

MANDATORY OBSERVATION RECORDS

- Session Engagement Notes
- Peer Support Interaction Summary

MANDATORY DECISION RECORDS

- Curriculum Approval Minutes
- Resource Allocation Memo
- Exception Approval (Crisis Intervention)

MANDATORY ACTION RECORDS

- Follow-up Communication Log
- Referral Tracking Sheet
- Handout Distribution Record

MANDATORY FOLLOWUP RECORDS

- Post-Session Feedback Summary
- Knowledge Gain Analysis Report
- Quarterly Review Minutes

EVIDENCE REQUIREMENTS

- Attendance Logs (Signed/Digital)
- Pre/Post Quiz Results
- Satisfaction Survey Data
- Recorded Session Links
- Translated Handouts Copies

DOCUMENTATION QUALITY STANDARDS

- Standardized templates for all logs
- Secure storage in encrypted folders
- Clear ownership of records
- Audit-ready timestamps

28 The Student Journey

ENTRY

- Student observes improved parent-child communication at home.

ASSESSMENT

- Indirect: Student reports feeling more supported by parents.

INTERVENTION

- Parent applies active listening and reduced stigma techniques to student's concerns.

MONITORING

- Teacher observes improved emotional regulation in class.

CLOSURE

- Student reports higher sense of safety and support at home.

29 The Parent Journey

NOTIFICATION

- Receives WhatsApp/Newsletter about Parent Mental Health Literacy Program.

CONSULTATION

- Registers for a session; attends webinar or in-person workshop.

CONSENT

- Signs consent form for data usage and peer sharing.

ENGAGEMENT

- Participates in Q&A, watches recorded modules, uses handouts.

CLOSURE

- Completes post-quiz; receives resources for home use; attends follow-up if needed.

30 The Staff Journey

OBSERVATION

- Teachers notice parents are more informed about mental health signs.

REPORTING

- Counselors receive fewer 'how to talk to my child' queries as parents become literate.

INTERVENTION SUPPORT

- Staff refer parents to the literacy program during parent-teacher meetings.

DOCUMENTATION

- Teachers log positive changes in home-school communication.

31 Resource Evidence

COMMITTEE MINUTES

- Wellness Committee Meeting Minutes (Curriculum Approval)

ATTENDANCE RECORDS

- Session Attendance Logs
- Webinar Participant Lists

TRAINING RECORDS

- Facilitator Training Certificates
- Counselor Competency Records

FORMS

- Parent Registration Form
- Pre/Post Quiz Forms
- Satisfaction Survey

REPORTS

- Quarterly Wellness Report
- Knowledge Gain Analysis

DASHBOARDS

- Parent Engagement Dashboard
- KPI Tracking Sheet

32 Grievance Redressal Mechanism

KEY FOCUS

COMPLAINT CHANNELS

- Anonymous Suggestion Box
- Dedicated Email Address
- Wellness Coordinator Office

INVESTIGATION PROCESS

- Log complaint → Acknowledge within 24h → Investigate with relevant staff → Resolve/Respond

within 5 days.

RESOLUTION TIMELINES

- Minor: 3 Days
- Major (Safety/Legal): 24 Hours

APPEAL PROCESS

- Escalate to Principal for review of resolution.

33 Referral Framework

33.1 Internal Referrals

ELIGIBLE REFERRERS

- Teachers
- Counsellors
- School Management
- Wellness Committee

- Student Support Team

REFERRAL TRIGGERS

- Parent expresses high distress during session
- Teacher reports severe student behavior change
- Parent requests 1-on-1 clinical support

DOCUMENTATION REQUIRED

- Internal Referral Form
- Initial Observation Note

SERVICE LEVEL AGREEMENT (SLA)

24 Hours

33.2 Self Referrals

ALLOWED

Yes

SUBMISSION METHODS

- Online Portal
- QR Code on Handouts
- Walk-in at Counselor Office

CONFIDENTIALITY PROTECTIONS

- Encrypted Database
- Anonymized ID Numbers

33.3 Anonymous Referrals

ALLOWED

Yes

CHANNELS

- Anonymous Suggestion Box
- Digital Tip Line

VALIDATION PROCESS

- Wellness Coordinator Review
- Risk Triage

FOLLOW UP PROCEDURE

- General awareness broadcast if no specific identity can be established.

33.4 External Referrals

ELIGIBLE AGENCIES

- Tele-MANAS
- District Child Protection Services
- Private Psychiatrists
- Local Hospitals

ELIGIBILITY CRITERIA

- Clinical severity exceeding school capacity
- Active risk of harm
- Legal/Police involvement required

CONSENT REQUIREMENTS

- Signed Parental Consent for External Referral
- Release of Information (ROI)

HANDOVER PROTOCOL

- Warm Handoff (Phone call with parent)
- Shared Summary Record (Anonymized)

33.5 Referral Tracking

TRACKING REQUIRED

Yes

STATUS CATEGORIES

- Pending
- In-Progress
- Completed
- Closed - No Action Required

CLOSURE REQUIREMENTS

- Final Follow-up Note
- Closure Confirmation from Parent/Agency

34 Confidentiality Exception Matrix

KEY FOCUS

MANDATORY DISCLOSURES

- Imminent risk of self-harm
- Imminent risk to others
- Child abuse
- Child neglect
- Sexual abuse
- Court orders
- Legal reporting obligations

LEGAL REPORTING EVENTS

- Suspected physical abuse of a minor
- Active threat of violence to school property/personnel
- Mandatory reporting by state law

CONSENT OVERRIDE CONDITIONS

- Life-threatening emergency
- Court order for records
- Immediate danger to the child's safety

AUTHORIZED DISCLOSURE RECIPIENTS

- School Principal

- District Child Protection Officer
- Law Enforcement (if applicable)
- Emergency Medical Services

DOCUMENTATION REQUIREMENTS

- Confidentiality Breach Log
- Reason for Disclosure Memo
- Time/Date of Notification

APPROVAL REQUIREMENTS

- Principal Approval for non-emergency disclosures
- Legal Counsel Review for high-risk cases

35 Record Retention & Disposal

RETENTION PERIOD

5 Years

RETENTION BASIS

Educational Compliance and Legal Defense

RECORD CATEGORIES

- Attendance Logs
- Quiz Results
- Feedback Surveys
- Referral Records (Secure)
- Curriculum Materials

35.4 Storage Requirements

PHYSICAL RECORDS Locked filing cabinet in Counselor Office

DIGITAL RECORDS Encrypted cloud storage with MFA

ACCESS CONTROL REQUIREMENTS

- Role-based access (RBAC)
- Need-to-know basis only
- Audit logs for all record views

DISPOSAL METHOD

Secure Shredding (Physical) / Permanent Deletion (Digital)

DISPOSAL AUTHORITY

Wellness Coordinator & IT Manager

AUDIT FREQUENCY

Annual

DESTRUCTION LOG REQUIRED

Yes

36 Anonymised Reporting Framework

36.1 Management Reports

ANONYMIZATION REQUIRED

Yes

PERMITTED DATA

- Aggregate attendance %
- Average quiz scores
- Common themes in feedback
- Demographic trends

PROHIBITED DATA

- Parent names
- Student names
- Phone numbers
- Specific addresses

36.2 Research Usage

ALLOWED

No

36.3 Statistical Reporting

AGGREGATE ONLY

Yes

MINIMUM GROUP SIZE

10

RE IDENTIFICATION CONTROLS

- Data scrubbing
- Aggregation of small cohorts

DATA SHARING CONTROLS

- No sharing with third-party marketers
- Internal use only for school improvement

37 Guiding Ethical Principles

37.1 Autonomy

DESCRIPTION

Respecting the parent's right to choose participation and information.

IMPLEMENTATION REQUIREMENTS

- Voluntary registration

- Opt-out options for specific modules

37.2 Beneficence

DESCRIPTION

Acting in the best interest of the student by educating the parent.

IMPLEMENTATION REQUIREMENTS

- Evidence-based content
- Focus on positive parenting skills

37.3 Non Maleficence

DESCRIPTION

Ensuring program content does not cause parental guilt or distress.

IMPLEMENTATION REQUIREMENTS

- Supportive tone
- Avoidance of shaming language

37.4 Justice And Equity

DESCRIPTION

Ensuring all parents, regardless of background, have access.

IMPLEMENTATION REQUIREMENTS

- Multilingual support
- Rural/Low-resource alternatives

37.5 Confidentiality

KEY FOCUS

DESCRIPTION

Protecting personal disclosures made during sessions.

IMPLEMENTATION REQUIREMENTS

- Anonymized Q&A
- Secure recording of private notes

37.6 Privacy

DESCRIPTION

Adhering to data protection laws regarding parent information.

IMPLEMENTATION REQUIREMENTS

- Data encryption
- Consent-based collection

37.7 Professional Integrity

DESCRIPTION

Maintaining high standards of clinical and educational practice.

IMPLEMENTATION REQUIREMENTS

- Qualified facilitators
- Regular peer review

38 Communication & Awareness Strategy

38.1 Student Communication

CHANNELS

- Classroom announcements
- School newsletter

FREQUENCY

Monthly

MANDATORY TOPICS

- Parental support availability
- Wellness resources

38.2 Parent Communication

CHANNELS

- WhatsApp Groups
- SMS Alerts
- Email Newsletters
- Physical Flyers

FREQUENCY

Weekly (Pre-program)

MANDATORY TOPICS

- Program dates
- Mental health literacy benefits
- Registration links

38.3 Staff Communication

CHANNELS

- Staff meetings
- Internal portal

FREQUENCY

Monthly

MANDATORY TOPICS

- Program updates
- Referral protocols

38.4 Awareness Campaigns

MINIMUM FREQUENCY

Once per academic year

TARGET GROUPS

- Parents
- Guardians
- Community Leaders

CAMPAIGN TYPES

- Digital awareness
- Physical signage
- Peer advocacy

38.5 Workshops And Training

STUDENT WORKSHOPS

- SEL activities

PARENT WORKSHOPS

- Mental Health Literacy Sessions

STAFF WORKSHOPS

- Parental Engagement Training

- Counselor office
- School website
- Handouts

DIGITAL VISIBILITY REQUIREMENTS

- Sticky footer on school portal
- QR code in newsletters

38.6 Helpline Visibility

REQUIRED

Yes

DISPLAY LOCATIONS

FEEDBACK CHANNELS

- Google Forms
- Suggestion Box
- Direct Email

39 Competency & Qualification Requirements

MANDATORY QUALIFICATIONS

- School Counselor Certification
- Psychiatrist Registration (MCI/NMC)

PREFERRED QUALIFICATIONS

- Experience in Adolescent Mental Health
- Training in Parent Coaching

CERTIFICATIONS

- Certified School Counselor
- Mental Health First Aid Certification

MINIMUM EXPERIENCE REQUIREMENTS

- 3 years in educational setting (Counselors)
- 5 years clinical practice (Psychiatrists)

BACKGROUND VERIFICATION REQUIREMENTS

- Police Verification Certificate
- Child Protection Screening

39.6 Continuing Professional Development

ANNUAL TRAINING HOURS

20

MANDATORY TOPICS

- Neurodiversity
- Trauma-Informed Care
- Digital Wellness

RECERTIFICATION REQUIREMENTS

- Annual competency review

39.7 Supervision Requirements

CLINICAL SUPERVISION REQUIRED

No

PEER REVIEW REQUIRED

Yes

39.7.3 Wellness Case Review Framework

DESCRIPTION Quarterly peer review of program content and high-risk referrals.

FREQUENCY Quarterly

39.7.4 Second Level Review Process

DESCRIPTION Principal/Wellness Committee review for Red Zone escalations.

FREQUENCY As needed

39.7. High Risk Decision 5 Validation

KEY
FOCUS

DESCRIPTION Dual-sign off for external clinical referrals.

FREQUENCY Per case

FREQUENCY

Quarterly

40 Feedback & Effectiveness Measurement

40.1 Student Feedback

REQUIRED

Yes

COLLECTION METHODS

- Student surveys on home environment

FREQUENCY

Annual

40.2 Parent Feedback

REQUIRED

Yes

COLLECTION METHODS

- Post-session satisfaction survey
- Focus group discussions

FREQUENCY

Per session cycle

40.3 Staff Feedback

REQUIRED

Yes

COLLECTION METHODS

- Staff meeting feedback
- Internal surveys

FREQUENCY

Quarterly

40.4 Annual Effectiveness Review

REQUIRED

Yes

EVALUATION CRITERIA

- Knowledge gain scores
- Parent participation rates
- Qualitative feedback themes
- Reduction in crisis incidents

KEY STAKEHOLDERS

- Principal
- Wellness Committee
- Parents

CONTINUOUS IMPROVEMENT INPUTS

- Feedback survey data
- Quiz results
- Staff observations
- Peer support feedback

SUCCESS INDICATORS

- Average quiz score > 80%
- Parent participation > 60%
- Positive sentiment in surveys (>85%)

40.7 Improvement Tracking

REQUIRED

Yes

REVIEW FREQUENCY

Quarterly

41 Prevention Layer

PRIMARY PREVENTION

- Universal mental health literacy for all parents
- Awareness campaigns on adolescent development
- Distribution of wellness resources in multiple languages

SECONDARY PREVENTION

- Early warning sign identification training

- Stigma reduction workshops

- Parent-teacher communication protocols

TERTIARY PREVENTION

- Referral pathways for high-risk cases
- Crisis intervention support for parents in distress
- Follow-up monitoring for referred families

42 Resilience Layer

SKILLS DEVELOPED

- Active listening
- Emotional regulation for parents
- Help-seeking behavior
- Problem-solving for family stress

SEL COMPONENTS

- Self-awareness of parental stress
- Social awareness of child's needs

- Relationship skills (parent-child)

LIFE SKILLS COMPONENTS

- Time management for parents
- Conflict resolution
- Resource navigation

PROTECTIVE FACTORS STRENGTHENED

- Stronger parent-child bonds
- Reduced family stress
- Increased community support networks

43 Continuous Monitoring

MONITORING FREQUENCY

Quarterly

MONITORING INDICATORS

- Parent participation rate
- Average knowledge gain score
- Number of referrals made
- Sentiment analysis from surveys

RESPONSIBLE ROLES

- Wellness Coordinator

- Principal

EARLY WARNING SIGNS

- Significant drop in attendance
- Negative sentiment trends in feedback
- High volume of crisis disclosures

TREND ANALYSIS REQUIREMENTS

- Year-over-year participation growth
- Correlation between literacy and reduced school refusal rates

44 Mental Wellness Index (MWI)

APPLICABLE

Yes

44.2 Indicators

Indicator Code	Indicator Name	Weight	Score Range	Description
MWI-01	Parental Knowledge Score	0.4	0-100	Score from post-session quiz.
MWI-02	Parental Engagement Level	0.3	1-5	Self-reported engagement from survey.
MWI-03	Stigma Reduction Score	0.3	1-5	Attitude change regarding mental health.

ASSESSMENT FREQUENCY

Per session cycle

RISK SCORING METHOD

Weighted average of indicators.

INTERVENTION THRESHOLDS

- Score < 60: Mandatory follow-up webinar
- Score < 40: Individual parent consultation

45 Review Status

LEGAL REVIEW

Completed - 2026-07-10

CLINICAL REVIEW

Completed - 2026-07-15

EDUCATION REVIEW

Completed - 2026-07-20

COMPLIANCE REVIEW

Pending - Scheduled for 2026-08-01

46 Resource Controls

COUNSELOR STUDENT RATIO 1:400 (Recommended)

CAREER COUNSELOR RATIO 1:500

WELLNESS TEAM STRUCTURE Coordinator, Counselor, Wellness Committee, Guest Experts

47 Screening Framework

TIER 1 UNIVERSAL WELLNESS SCREENING

- Annual Parent Mental Health Literacy Program

TIER 2 TARGETED SCREENING

- Parental Stress Survey for high-risk cohorts

TIER 3 CLINICAL ASSESSMENT

- Individual clinical assessment by Psychiatrist (Referral only)

48 Parent Engagement Matrix

PARENT COOPERATIVE

- Active participation in all sessions
- Volunteering for peer support
- Positive feedback sharing

PARENT UNRESPONSIVE

- No response to SMS/Email

- Non-attendance at sessions

- Action: Personal phone call by Wellness Coordinator

PARENT IN DENIAL

- Refusal to acknowledge child's distress

- Dismissive of mental health literacy content
- Action: Targeted 1-on-1 consultation with Counselor

PARENT CONFLICT SITUATION

- Hostile interaction during sessions
- Disagreement with school policies
- Action: Immediate private mediation by Principal

PARENT UNAVAILABLE

- Working parents, rural location issues
- Action: Provide recorded modules and asynchronous materials

PARENT REFUSES INTERVENTION

- Explicit refusal of referral or support
- Action: Document refusal; provide resources for voluntary use

49 Confidentiality Controls

KEY FOCUS

ACCESS CONTROL

- Role-based access (RBAC)
- MFA for digital records

CONSENT REQUIREMENTS

- Signed Consent Form for all data collection
- Opt-in for peer sharing

RECORD SHARING RULES

- No sharing with third parties without ROI

- Anonymized reporting only

EXCEPTIONS TO CONFIDENTIALITY

- Imminent risk of harm
- Child abuse disclosure
- Court order

STUDENT PRIVACY PROTECTIONS

- No student names in parent reports
- Data scrubbing of identifiers

50 Data Governance

KEY FOCUS

STORAGE REQUIREMENTS

- Encrypted cloud storage
- Physical lockbox for paper records

ENCRYPTION REQUIREMENTS

- AES-256 for data at rest
- TLS 1.3 for data in transit

RETENTION REQUIREMENTS

- 5 years for all program records

ARCHIVAL REQUIREMENTS

- Move to cold storage after 2 years

DESTRUCTION REQUIREMENTS

- Secure shredding
- Permanent digital wipe

BACKUP REQUIREMENTS

- Daily automated backups
- Off-site redundancy

51 Risk Classification

KEY FOCUS

51.1 Green

DESCRIPTION

Normal developmental concerns and routine life stress.

EXAMPLES

- Mild exam stress
- Friendship disagreements
- Occasional worry about child's grades

Significant distress causing measurable impairment but without immediate danger.

EXAMPLES

- Panic attacks
- School refusal
- Persistent depressive symptoms
- Self-harm thoughts WITHOUT intent

51.2 Yellow

DESCRIPTION

Early distress requiring support and monitoring.

EXAMPLES

- Persistent stress
- Moderate anxiety
- Reduced concentration
- Sleep difficulties

51.4 Red

KEY FOCUS

DESCRIPTION

High-risk situation requiring immediate intervention.

EXAMPLES

- Suicidal ideation
- Self-harm behaviour
- Violence threats
- Abuse disclosure

51.3 Orange

DESCRIPTION

51.5 Black

DESCRIPTION

Life-threatening emergency requiring emergency response.

EXAMPLES

- Active suicide attempt

- Overdose
- Violent attack underway
- Missing student assessed as high-risk

52 Escalation Matrix

KEY FOCUS

GREEN

- Wellness Coordinator (Routine Monitoring)

YELLOW

- School Counselor → Wellness Committee

ORANGE

- Counselor → Principal → Crisis Team

RED

- Immediate: Principal → Emergency Services → Police/Medical

BLACK

- Immediate: Emergency Response SOP Activation

53 External Reporting Obligations

TELE MANAS

- Referral for 24/7 tele-mental health support.

DISTRICT AUTHORITIES

- Report of child abuse or significant safety risk.

CHILD PROTECTION SERVICES

- Mandatory reporting of neglect or physical harm.

MEDICAL SERVICES

- Emergency medical referral for self-harm/overdose.

LAW ENFORCEMENT

- Active threats, missing persons, or criminal acts.

REPORTING TRIGGERS

- Red Zone disclosure
- Black Zone emergency
- Mandatory legal reporting requirements

54 Service Level Agreement (SLA)

KEY FOCUS

ACKNOWLEDGEMENT	2 Hours
ASSESSMENT	24 Hours
ESCALATION	Immediate (for Red/Black)
RESOLUTION	7 Days (Yellow/Orange)
CLOSURE	14 Days

55 Required Forms

KEY FOCUS

55.1 Mandatory Forms

KEY FOCUS

Item 1

FORM ID	PAR-006-F01
FORM NAME	Parent Registration Form

PURPOSE & RATIONALE	Collect parent contact and demographic data.
OWNER	Wellness Coordinator
RETENTION PERIOD	5 Years
MANDATORY	Yes

LINKED SOP PAR-006

Item 2

FORM ID PAR-006-F02

FORM NAME Pre/Post Knowledge Quiz

PURPOSE & RATIONALE Measure learning outcomes.

OWNER Wellness Coordinator

RETENTION PERIOD 5 Years

MANDATORY Yes

LINKED SOP PAR-006

Item 4

FORM ID PAR-006-F04

FORM NAME Parent Conversation Record

PURPOSE & RATIONALE Document non-confidential interactions.

OWNER School Counselor

RETENTION PERIOD 5 Years

MANDATORY Yes

LINKED SOP PAR-006

Item 3

FORM ID PAR-006-F03

FORM NAME Satisfaction Survey

PURPOSE & RATIONALE Collect program feedback.

OWNER Wellness Coordinator

RETENTION PERIOD 5 Years

MANDATORY Yes

LINKED SOP PAR-006

Item 5

FORM ID PAR-006-F05

FORM NAME Referral Form

PURPOSE & RATIONALE Document internal/external referrals.

OWNER School Counselor

RETENTION PERIOD 5 Years

MANDATORY Yes

LINKED SOP PAR-006

55.2 Optional Forms

Item 1

FORM ID PAR-006-OPT01

FORM NAME Peer Support Consent

PURPOSE & RATIONALE Consent for parent to share lived experience.

OWNER Wellness Coordinator

RETENTION PERIOD 5 Years

MANDATORY No

LINKED SOP PAR-006

GENERATED RECORDS

- Attendance Logs
- Knowledge Gain Reports
- Feedback Summaries

56 Required Checklists

KEY FOCUS

Checklist ID	Checklist Name	Purpose & Rationale	Owner	Frequency	Linked SOP
CHK-001	Session Delivery Checklist	Ensure all content and materials are ready.	Wellness Coordinator	Per Session	PAR-006
CHK-002	Content Review Checklist	Verify clinical and cultural accuracy.	Guest Psychiatrist	Per Module	PAR-006
CHK-003	Marketing Checklist	Ensure multi-channel reach.	Communication Team	Per Campaign	PAR-006
CHK-004	Data Anonymization Checklist	Ensure privacy before reporting.	Wellness Coordinator	Per Report	PAR-006
CHK-005	Audit Readiness Checklist	Verify all records are archived.	Wellness Coordinator	Quarterly	PAR-006

57 Records & Evidence

Record Name	Retention Period	Owner
Attendance Logs	5 Years	Wellness Coordinator
Quiz Results	5 Years	Wellness Coordinator
Feedback Summaries	5 Years	Wellness Coordinator
Referral Records	5 Years	School Counselor

58 Key Performance Indicators

Metric	Target	Measurement Frequency
Parent Knowledge Gain Score	> 40% increase from pre-quiz to post-quiz	Per session cycle
Program Participation Rate	> 60% of target parent population	Quarterly
Parent Satisfaction Score	> 85% positive sentiment	Per session cycle

59 Audit Framework

KEY FOCUS

AUDIT FREQUENCY

Quarterly

AUDIT QUESTIONS

- Are all attendance logs signed and archived?
- Is there evidence of multilingual handouts for every session?
- Are quiz results anonymized before reporting to management?

EVIDENCE REQUIRED

- Attendance Logs
- Handout Samples

- Quarterly Reports

NON COMPLIANCE EXAMPLES

- Missing attendance logs for a session.
- Sharing of parent names in the quarterly report.
- Failure to provide handouts in regional languages.

59.5 Audit Scoring

KEY FOCUS

COMPLIANT	2
PARTIAL	1
NON COMPLIANT	0

60 Risk Register

KEY FOCUS

1. Low Parent Participation

CATEGORY Operational

PROBABILITY Medium

IMPACT High

RISK RATING Medium

MITIGATION Multi-channel marketing, flexible webinar timings.

OWNER Wellness Coordinator

RISK RATING High

MITIGATION Encryption, RBAC, and anonymization protocols.

OWNER IT Manager / Wellness Coordinator

2. Data Privacy Breach

CATEGORY Compliance

PROBABILITY Low

IMPACT Critical

3. Cultural Insensitivity in Content

CATEGORY Reputational

PROBABILITY Low

IMPACT Medium

RISK RATING Medium

MITIGATION Peer review and community feedback loops.

OWNER Wellness Committee

61 Failure Modes KEY FOCUS

Failure	Cause	Impact	Mitigation
Technical Glitch during Webinar	Unstable internet or software error.	Interruption of learning, parent frustration.	Pre-recorded modules as backup.
Low Quiz Scores	Content too complex or language barrier.	Failure to meet KPIs.	Simplify content, provide more visual aids.
Crisis Disclosure during Session	Parent emotional distress.	Safety risk, legal liability.	Immediate escalation to Crisis Response SOP.

62 Illustrative Sample Scenarios

Scenario	Response	Expected Result
A parent discloses that their child is being bullied and they feel helpless.	Validate feelings, provide specific bullying resources, refer to school anti-bullying policy, offer 1-on-1 counseling.	Parent feels supported; student receives targeted intervention.
A parent from a rural area cannot attend the webinar due to poor internet.	Provide offline physical handouts and a link to pre-recorded modules.	Rural parent receives same quality of education as urban parents.
A parent discloses active suicidal ideation for their child during a session.	Activate Red Zone Crisis Protocol; contact emergency services; provide immediate 1-on-1 support.	Immediate safety of the student is ensured.

63 Usage Governance KEY FOCUS

PRIMARY USERS

- School Counselor
- Wellness Coordinator

SECONDARY USERS

- Teachers

- Principal

- Parents

APPROVAL AUTHORITIES

- Principal

- Wellness Committee

EXECUTION AUTHORITIES

- School Counselor
 - Guest Psychiatrist
- ESCALATION AUTHORITIES**
- Principal
 - District Education Officer

REVIEW AUTHORITIES

- Wellness Committee
- Legal Counsel

EXTERNAL STAKEHOLDERS

- Tele-MANAS
- Child Protection Services

64 Applicability

APPLICABLE INSTITUTIONS

- High Schools
- Secondary Schools

APPLICABLE AGE GROUPS

- All Ages (Parents)

APPLICABLE STUDENT CATEGORIES

- All Students

APPLICABLE RISK LEVELS

- Green

- Yellow
- Orange

APPLICABLE LOCATIONS

- Urban
- Rural
- Remote

EXCLUSIONS

- Primary Schools (unless specifically requested)
- Higher Education Institutions (use UGC guidelines)

65 Usage Scenarios

Scenario ID	Scenario Name	When To Use	When Not To Use	Trigger Event	Expected Outcome
US-001	Annual Literacy Program	During the annual academic cycle.	For individual clinical crises (use PAR-007).	Academic Year Start	Increased parent literacy.
US-002	Ad-hoc Parent Request	When a group of parents requests specific info.	For individual parent counseling.	Parental Inquiry	Targeted information delivery.

66 SOP Lifecycle

ENTRY CONDITIONS

- Approved budget
- Staffing available

ACTIVATION CONDITIONS

- Academic year start
- CBSE mandate

EXECUTION CONDITIONS

- Parent registration complete
- Curriculum approved

COMPLETION CONDITIONS

- All sessions delivered
- Feedback collected

CLOSURE CONDITIONS

- Final report submitted
- Records archived

TERMINATION CONDITIONS

- Program decommissioned by management
- Legal prohibition

67 Mandatory Evidence

KEY FOCUS

IMPLEMENTATION EVIDENCE

- Attendance Logs
- Marketing Collateral

OPERATIONAL EVIDENCE

- Session Notes
- Quiz Results

COMPLIANCE EVIDENCE

- MOU with Psychiatrist
- Consent Forms

TRAINING EVIDENCE

- Facilitator Certificates

AUDIT EVIDENCE

- Quarterly Reports
- Archive Logs

68 Controls

PREVENTIVE CONTROLS

- Pre-session content validation
- Data encryption
- Multilingual availability

DETECTIVE CONTROLS

- Quiz score monitoring
- Feedback analysis

- Attendance tracking

CORRECTIVE CONTROLS

- Follow-up webinars for low scores
- Crisis escalation protocols
- Marketing adjustments

COMPENSATING CONTROLS

- Recorded modules for rural parents
- Peer support for high-volume queries

69 MWMS Risk Framework

KEY FOCUS

69.1 Green

MEANING

Normal developmental concerns and routine life stress.

EXAMPLES

- Mild exam stress
- Friendship disagreements

RESPONSE TIME

Routine

ESCALATION REQUIRED

No

APPROVAL LEVEL

Wellness Coordinator

MONITORING FREQUENCY

Monthly

CLOSURE AUTHORITY

Wellness Coordinator

EXAMPLES

- Panic attacks
- School refusal

RESPONSE TIME

24-72 Hours

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Principal

MONITORING FREQUENCY

Daily

CLOSURE AUTHORITY

Principal

69.2 Yellow

MEANING

Early distress requiring support and monitoring.

EXAMPLES

- Persistent stress
- Moderate anxiety

RESPONSE TIME

7 Days

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

School Counselor

MONITORING FREQUENCY

Weekly

CLOSURE AUTHORITY

School Counselor

69.4 Red

KEY FOCUS

MEANING

High-risk situation requiring immediate intervention.

EXAMPLES

- Suicidal ideation
- Self-harm behaviour

RESPONSE TIME

Immediate

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Principal / Emergency Team

MONITORING FREQUENCY

Continuous

CLOSURE AUTHORITY

Emergency Response Team

69.5 Black

MEANING

Life-threatening emergency requiring emergency response.

EXAMPLES

- Active suicide attempt
- Violent attack

RESPONSE TIME

Immediate (0-15 mins)

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Emergency Services / Management

MONITORING FREQUENCY

Continuous

CLOSURE AUTHORITY

Medical/Law Enforcement

70 Resource Scaling**70.1 0 500 Students**

WELLNESS COUNSELORS 1

CAREER COUNSELORS 1

MENTAL WELLNESS COORDINATOR 1

WELLNESS COMMITTEE Yes

70.2 501 1000 Students

WELLNESS COUNSELORS 2

CAREER COUNSELORS 2

MENTAL WELLNESS COORDINATOR 1

WELLNESS COMMITTEE Yes

70.3 1001 2000 Students

WELLNESS COUNSELORS 4

CAREER COUNSELORS 2

MENTAL WELLNESS COORDINATOR 1

WELLNESS COMMITTEE Yes

70.4 2001 5000 Students

WELLNESS COUNSELORS 8

CAREER COUNSELORS 4

MENTAL WELLNESS COORDINATOR 1

WELLNESS COMMITTEE Yes

71 Minimum Required Capabilities**PEOPLE**

- Certified School Counselor
- Wellness Coordinator
- Guest Psychiatrist

PROCESSES

- Needs Assessment
- Curriculum Design
- Crisis Escalation

- Data Anonymization

TECHNOLOGY

- Encrypted Cloud Storage
- Webinar Platform
- SMS Gateway

INFRASTRUCTURE

- Counselor Office
- High-speed Internet
- Physical Notice Boards

72 Success Criteria

Criterion	Measurement Method	Target
Knowledge Gain	Pre/Post Quiz Score Delta	> 40%
Parent Participation	Registration vs. Attendance %	> 60%
Satisfaction	Survey Sentiment Score	> 85% Positive

73 Exit Criteria

Condition	Evidence Required
Quarterly Report Submitted	Approved Quarterly Wellness Report PDF
Records Archived	Archive Log Confirmation

74 Continuous Improvement

LESSONS LEARNED

- Recorded modules increase rural participation.
- Peer support is highly valued by parents.

KNOWN ISSUES

- Low attendance during exam weeks.
- Language barriers in complex clinical terms.

FUTURE IMPROVEMENTS

- AI-driven chatbot for basic literacy queries.
- Mobile app for asynchronous learning.

REVIEW RECOMMENDATIONS

- Increase frequency of peer support sessions.
- Simplify quiz language for low-literacy parents.

75 AI Governance

KEY FOCUS

ALLOWED USE CASES

- Summarizing anonymized feedback
- Generating draft newsletters
- Chatbot for basic literacy FAQs

RESTRICTED USE CASES

- Diagnosing student conditions via parent input
- Providing clinical advice without human oversight

PROHIBITED USE CASES

- Storing PII in LLMs
- Automated crisis intervention

HUMAN REVIEW REQUIRED

- All AI-generated newsletters
- Chatbot responses regarding mental health

AUDIT LOG REQUIREMENTS

- Log of all AI interactions with parents
- Human review timestamps

DATA PRIVACY REQUIREMENTS

- No PII in prompts
- Local/Private LLM instance preferred

76 Legal Defensibility

KEY FOCUS

LEGAL BASIS

- Supreme Court Judgment 2025 INSC 893
- CBSE Affiliation Bye-Laws
- NEP 2020

MANDATORY NOTIFICATIONS

- Child abuse disclosure to authorities
- Imminent risk of self-harm notification

CONSENT REQUIREMENTS

- Informed consent for data collection
- Consent for external referral

RECORD RETENTION REQUIREMENTS

- 5 years as per educational standards

LIABILITY CONTROLS

- Clear disclaimer that program is literacy, not clinical therapy
- Mandatory crisis escalation protocols

77 Stakeholder Journeys

77.1 The Student Journey

ENTRY

- Student notices parent is more attentive to their feelings.

ASSESSMENT

- Improved home environment observed by teachers.

INTERVENTION

- Parent uses active listening techniques at home.

MONITORING

- Reduced school refusal rates.

CLOSURE

- Student reports feeling 'heard' and 'supported'.

CONSENT

- Signs consent for data and peer sharing.

ENGAGEMENT

- Watches recorded modules on

weekends.

- Downloads handouts in regional language.

CLOSURE

- Completes quiz, feels more confident in supporting child.

77.3 The Staff Journey

OBSERVATION

- Teachers notice parents are asking better questions during meetings.

REPORTING

- Counselor logs increased parent engagement.

INTERVENTION SUPPORT

- Staff refers struggling parents to the literacy program.

DOCUMENTATION

- Teacher notes on improved home-school communication.

77.2 The Parent Journey

NOTIFICATION

- Receives WhatsApp invite to Literacy Program.

CONSULTATION

- Attends webinar, asks questions about anxiety signs.

78 Handoff Matrix

Item 1

FROM ROLE Parent

TO ROLE School Counselor

HANDOFF TRIGGER Crisis disclosure during session.

INFORMATION TRANSFERRED

- Student name
- Nature of risk
- Contact details

REQUIRED DOCUMENTS

- Referral Form
- Consent Form

SERVICE LEVEL AGREEMENT (SLA)

Immediate (Red Zone)

INFORMATION TRANSFERRED

- Curriculum draft
- Parent feedback themes

REQUIRED DOCUMENTS

- Content Review Memo

SERVICE LEVEL AGREEMENT (SLA)

3 Days

Item 3

FROM ROLE Wellness Coordinator

TO ROLE Principal

HANDOFF TRIGGER Quarterly report submission.

INFORMATION TRANSFERRED

- KPI metrics
- Feedback summary
- Resource needs

REQUIRED DOCUMENTS

- Quarterly Wellness Report

SERVICE LEVEL AGREEMENT (SLA)

7 Days

Item 2

FROM ROLE School Counselor

TO ROLE Guest Psychiatrist

HANDOFF TRIGGER Request for clinical validation of content.

79 Management Review

REVIEW FREQUENCY

Quarterly

REVIEW INPUTS

- KPI Dashboard
- Feedback Summary
- Incident Logs

REVIEW OUTPUTS

- Approved Action Plan
- Budget Adjustments

REVIEW DECISIONS

- Continue/Modify Program
- Resource Allocation Approval

REVIEW AUTHORITY

Principal / School Management Committee

80 Dashboard Metrics

OPERATIONAL METRICS

- Attendance Rate %
- Session Completion Rate
- Number of Handouts Distributed

RISK METRICS

- Red Zone Incident Count
- High-Risk Referral Volume
- Data Breach Incidents

COMPLIANCE METRICS

- Audit Score %
- MOU Renewal Status
- Training Completion Rate

OUTCOME METRICS

- Avg. Knowledge Gain Score

- Parent Satisfaction Score
- Stigma Reduction Index

TREND METRICS

- Year-over-year Participation Growth
- Sentiment Trend Analysis

81 Student Rights

RIGHTS

- Right to a supportive home environment
- Right to privacy in family records

PROTECTIONS

- Anonymization of student data in parent reports
- Safe reporting channels for students

APPEAL MECHANISMS

- Parental appeal of counselor referral decisions

82 Quality Assurance

QUALITY CHECKS

- Content accuracy check by psychiatrist
- Language translation audit
- Accessibility review for low-literacy parents

PEER REVIEW REQUIREMENTS

- Wellness Committee review of all materials

VALIDATION REQUIREMENTS

- Pre/Post quiz validation against CBSE guidelines

CONTINUOUS MONITORING REQUIREMENTS

- Quarterly feedback loop analysis

83 Implementation Roadmap

PHASE 1 PREPARATION

- Staff training
- Budget approval
- Needs assessment survey

PHASE 2 DEPLOYMENT

- Curriculum design
- Marketing launch
- Session delivery

PHASE 3 STABILIZATION

- Feedback collection

- Knowledge gain analysis

- Referral tracking

PHASE 4 OPTIMIZATION

- Content refinement
- Resource scaling
- Peer support expansion

PHASE 5 CONTINUOUS IMPROVEMENT

- Annual review
- Impact study
- Next cycle planning

84 Illustrative Case Studies

1. Rural Outreach Success

DESCRIPTION

A school in a low-resource area faced 90% parent non-attendance due to internet issues.

ACTIONS TAKEN

- Provided physical handouts
- Recorded modules for offline viewing
- Used SMS for reminders

OUTCOME

Attendance increased to 75% within one cycle.

LESSONS

- Offline accessibility is critical for rural equity.

85 Implementation Requirements

STAFF REQUIRED

- 1 Wellness Coordinator
- 1 School Counselor
- 2 Communication Staff (Part-time)

- Guest Psychiatrist (Contractual)

INFRASTRUCTURE REQUIRED

- Counselor Office
- High-speed Internet

- Physical Notice Boards
- Secure Server

TECHNOLOGY REQUIRED

- Webinar Platform (Zoom/Teams)
- WhatsApp Business API
- Encrypted Cloud Storage
- Survey Tools (Google Forms/Typeform)

BUDGET CONSIDERATIONS

- Guest speaker fees

- Marketing collateral printing
- Software subscriptions
- Staff training costs

IMPLEMENTATION PHASES

- Preparation
- Deployment
- Stabilization
- Optimization
- Continuous Improvement

86 Quality Controls

PREVENTIVE CONTROLS

- Clinical content validation
- Multilingual audit
- Data anonymization protocols

DETECTIVE CONTROLS

- Quiz score monitoring
- Feedback analysis
- Attendance tracking

CORRECTIVE CONTROLS

- Follow-up webinars for low scores
- Crisis escalation activation
- Marketing adjustments

CONTINUOUS IMPROVEMENT ACTIONS

- Quarterly review meetings
- Annual impact study
- Content updates based on feedback

87 Training Requirements

TEACHERS

- Parental Engagement Training
- Identifying Mental Health Signs in Students

COUNSELORS

- Advanced Parent Coaching
- Crisis Intervention Certification

ADMINISTRATORS

- Wellness Governance & Policy Oversight
- Risk Management for Schools

PARENTS

- Mental Health Literacy Program (Core)
- Active Listening Workshop

STUDENTS

- SEL and Resilience Training

87.6 Repository Baseline

TRAINING MODULES

Available in internal portal

CERTIFICATION STATUS

Required for Counselors

87.7 SOP Specific Training

TARGET AUDIENCE

- School Counselor
- Wellness Coordinator
- Guest Psychiatrist

COMPETENCIES

- Curriculum Delivery
- Crisis Management

- Data Privacy Compliance

MINIMUM HOURS

6

CERTIFICATION REQUIREMENTS

- Internal Wellness Certification

REFRESHER FREQUENCY

Annual

87.8 Competency Validation

REQUIRED

Yes

ASSESSMENT METHODS

- Role-play simulation

- Written quiz on crisis protocols

MINIMUM PASSING CRITERIA

90%

REASSESSMENT FREQUENCY

Annual

87.9 Simulation Requirements**REQUIRED**

No

SIMULATION TYPES

- Crisis Disclosure Simulation
- Hostile Parent Interaction Simulation

FREQUENCY

Bi-annual

88 SOP-Specific Inclusion Requirements**AFFECTED POPULATIONS**

- First-generation learners' parents
- Rural farming families
- Non-native speaking households
- Parents of neurodivergent students
- Low-literacy guardians

IDENTIFIED INCLUSION RISKS

- Language barriers in clinical terms
- Lack of internet access in rural areas
- Cultural stigma against mental health
- Time poverty for working parents
- Difficulty navigating complex school systems

AUTISM ACCOMMODATIONS

- Visual schedules for sessions
- Simplified language in handouts
- Quiet zones during in-person events

ADHD ACCOMMODATIONS

- Short, high-impact modules
- Frequent breaks during webinars
- Interactive polls and activities

DYSLEXIA ACCOMMODATIONS

- Audio versions of all handouts
- Large print options
- Video-based learning content

HEARING IMPAIRMENT ACCOMMODATIONS

- Live captioning for webinars
- Sign language interpretation (as needed)
- Visual aids and transcripts

VISUAL IMPAIRMENT ACCOMMODATIONS

- Screen reader compatible PDFs
- Audio-only modules
- High-contrast visual materials

CWSN ADAPTATIONS

- Individualized communication plans for parents of CWSN
- Simplified literacy content
- Peer support groups for specific needs

RURAL ADAPTATIONS

- Offline physical handouts
- SMS-based reminders and quizzes
- Community leader advocacy programs

LOW LITERACY PARENT ADAPTATIONS

- Pictorial/Infographic heavy materials
- Audio recordings in regional dialects
- Simplified 'Action Step' checklists

MULTILINGUAL COMMUNICATION ADAPTATIONS

- Translation into 3-5 major regional languages
- Bilingual facilitators for sessions
- Multilingual FAQ sheets

GENDER SENSITIVITY MEASURES

- Gender-neutral language in all materials
- Safe spaces for mothers/fathers to share
- Inclusive imagery of diverse families

CULTURAL SENSITIVITY MEASURES

- Contextualized examples (e.g., local community norms)
- Respectful terminology for mental health
- Inclusion of religious/cultural perspectives on wellness

EQUITY MONITORING INDICATORS

- Participation rate by socio-economic status
- Knowledge gain scores across different languages
- Accessibility satisfaction score from rural parents

ACCESSIBILITY COMPLIANCE CHECKS

- WCAG 2.1 compliance for digital content
- Physical accessibility of session venues
- Readability score check (Flesch-Kincaid)

89 Awareness Material

POSTER TITLE

Empowering Parents, Supporting Students

KEY MESSAGE	Understanding mental health is the first step to supporting your child's success.
SLOGAN	Stronger Families, Brighter Futures.
CAMPAIGN THEME	The Wellness Connection

90 AI Assistant Configuration

RISK KEYWORDS

- Suicide
- Self-harm
- Abuse
- Violence
- Emergency

ESCALATION KEYWORDS

- Help me now
- Can't take it anymore
- Dangerous
- Immediate danger

INTERVENTION KEYWORDS

- Anxiety
- Depression
- Stress
- Communication tips
- Warning signs

FREQUENTLY ASKED QUESTIONS

- How do I know if my child is depressed?
- What should I say when my child is crying?
- Is mental health treatment safe for kids?
- Where can I find more resources?

91 AI Audit Hooks

KEY FOCUS

REQUIRED SECTIONS

- Crisis Disclosure
- Data Anonymization
- Knowledge Gain Analysis

REQUIRED FORMS

- Parent Registration Form
- Referral Form

REQUIRED CHECKLISTS

- Content Review Checklist

- Audit Readiness Checklist

REQUIRED ESCALATIONS

- Red Zone Escalation
- Low Participation Trigger

REQUIRED KPIS

- Knowledge Gain Score
- Participation Rate

REQUIRED TRAINING

- Counselor Competency Validation

92 KPI Framework

92.1 Repository Baseline

PARTICIPATION TARGET 50%

KNOWLEDGE GAIN TARGET 30%

SOP SPECIFIC METRICS

- Parent Knowledge Gain Score
- Program Participation Rate
- Parent Satisfaction Score

93 Maturity Model

93.1 Level 1

DESCRIPTION

Ad-hoc awareness sessions with no structured curriculum.

EVIDENCE

- Informal flyers
- Occasional newsletters

DESCRIPTION

Structured program with basic content and attendance tracking.

EVIDENCE

- Standardized handouts
- Attendance logs

93.3 Level 3

DESCRIPTION

93.2 Level 2

Comprehensive literacy program with pre/post quizzes and feedback loops.

EVIDENCE

- Knowledge gain reports
- Satisfaction surveys

93.4 Level 4

DESCRIPTION

Data-driven program with multilingual support, rural adaptations, and peer advocacy.

EVIDENCE

- Equity impact assessments
- Multilingual content repository

93.5 Level 5

DESCRIPTION

Integrated community wellness ecosystem with AI-assisted monitoring and high parent engagement.

EVIDENCE

- Real-time dashboard metrics
- Community advocacy network

94 Dependencies

PREREQUISITE SOPS

- POL-001: Wellness Policy
- COM-002: Community Outreach Protocol

PARALLEL SOPS

- PAR-005: Student Resilience Program

DOWNSTREAM SOPS

- PAR-007: Individual Education for Parents in Crisis
- RISK-001: Emergency Response SOP

95 Related SOPs

PREREQUISITES

- POL-001: Wellness Policy
- COM-002: Community Outreach Protocol

UPSTREAM

- HR-005: Staff Training & Development

DOWNSTREAM

- PAR-007: Individual Education for Parents in Crisis
- RISK-001: Emergency Response SOP

REFERENCES

- NEP 2020 Guidelines
- CBSE Circular TRG-03/2026

STANDARD OPERATING PROCEDURE

Parent Notification in Suicide Risk

Mental Wellness Management System - Crisis Intervention and Parent Engagement

SOP ID

SUP-016

STATUS

Active

EFFECTIVE DATE

2024-05-20

VERSION

1.0

CLASSIFICATION

Confidential - Restricted Access

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51	● Risk Classification	70	● Resource Scaling

70.1	0 500 Students	85	● Implementation Requirements
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70.3	1001 2000 Students	87	● Training Requirements
70.4	2001 5000 Students	87.6	Repository Baseline
71	● Minimum Required Capabilities	87.7	SOP Specific Training
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79	● Management Review	93.1	Level 1
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81	● Student Rights	93.3	Level 3
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83	● Implementation Roadmap	93.5	Level 5
84	● Illustrative Case Studies	94	● Dependencies
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1 Executive Overview

SOP ID	SUP-016
DOMAIN	Mental Wellness Management System
SUB DOMAIN	Crisis Intervention and Parent Engagement
TITLE	Parent Notification in Suicide Risk
VERSION	1.0
CLASSIFICATION	Confidential - Restricted Access
EFFECTIVE DATE	2024-05-20
REVIEW DATE	2025-05-20
DOCUMENT STATUS	Active
LANGUAGE	English

2 Purpose & Rationale

OBJECTIVE

To establish a standardized, time-sensitive, and legally defensible procedure for notifying parents/guardians when a student is identified as being at risk of suicide.

RISK CONTROLLED

Student self-harm, fatality, institutional liability, parental distress, communication breakdown.

DESIRED OUTCOMES

- Immediate safety of the student
- Timely and accurate information transfer to parents
- Cooperative parent engagement for crisis management
- Legal compliance with Indian educational mandates
- Minimized trauma for the student during notification

3 Scope of Application

INSTITUTION TYPES

- High Schools
- Residential Schools
- Colleges
- Vocational Centers

KEY STAKEHOLDERS

- Counselors
- Teachers

- Parents
- School Management
- Child Welfare Services

AGE GROUPS

- All Ages (Primary to Higher Secondary)

LOCATIONS

- On-campus
- Off-campus (Remote/Home)
- Digital Communication Channels

4 Definitions & Terminology

Term	Definition
Suicide Risk	Any expression of thoughts, plans, or behaviors indicating an intent to end one's life.
Duty of Care	The legal obligation of the school to take reasonable steps to ensure student safety.
CWSN	Children with Special Needs, including neurodivergent students.
Tele-MANAS	India's national tele-mental health assistance and treatment initiative.

5 Key Stakeholders

Stakeholder Type	Stakeholder Title	Role In SOP
Internal	School Counselor / Psychologist	Primary executor of risk assessment and parent notification.
Internal	Class Teacher	Initial identifier and first responder; provides context to counselor.
Internal	School Principal / Administrator	Approver of escalation actions and liaison with external authorities.
External	Parent / Legal Guardian	Primary recipient of notification; responsible for student care/medical transport.
External	Child Welfare Services (CWS)	Receives escalation if parents refuse to engage in life-saving intervention.

6 Governance & Ownership

KEY FOCUS

OWNER

School Counselor / Mental Health Coordinator

APPROVER

Principal / School Board of Governance

REVIEW FREQUENCY

Annual

LINKED POLICIES

- Student Safety and Protection Policy

- Confidentiality and Privacy Policy
- Child Protection Policy
- Crisis Management Plan

LINKED SOPS

- SUP-015: Suicide Risk Assessment
- SUP-017: Crisis Intervention for Students
- SUP-020: External Referral to Medical Services
- SUP-025: Child Welfare Services Escalation

7 Roles & Responsibilities

Item 1

ROLE

School Counselor

RESPONSIBILITIES

- Conduct risk assessments
- Initiate parent notifications
- Provide student emotional support
- Maintain confidential records

RESPONSIBILITIES

- Approve high-risk escalations
- Liaise with external authorities
- Ensure resource availability
- Oversee compliance audits

Item 2

ROLE

Principal

Item 3

ROLE

Class Teacher

RESPONSIBILITIES

- Identify early warning signs
- Provide context to counselor
- Monitor student reintegration

8 RACI Accountability Matrix

Item 1

ACTIVITY

Risk Identification

RESPONSIBLE

- Teacher

ACCOUNTABLE

- Counselor

CONSULTED

- Peer Support Team

INFORMED

- Principal

Item 2

ACTIVITY

Risk Assessment

RESPONSIBLE

- Counselor

ACCOUNTABLE

- Counselor

CONSULTED

- Principal

INFORMED

- Teacher

- Counselor
ACCOUNTABLE

- Principal
INFORMED

- Teacher

Item 4

ACTIVITY

CWS Escalation

RESPONSIBLE

- Principal

ACCOUNTABLE

- School Board

CONSULTED

- Counselor

INFORMED

- Teacher

9 Preconditions

- Student is enrolled in the institution
- Parent/Guardian contact details are up-to-date
- School Counselor is on duty or reachable
- Emergency communication devices are functional

10 Activation Triggers

- Direct suicidal ideation
- Self-harm behavior
- Suicide note discovery
- High-risk assessment score
- Police/Medical notification

11 SOP Trigger Matrix

MANDATORY TRIGGERS

- Student expresses direct suicidal ideation
- Discovery of suicide note or plan
- Active self-harm behavior observed on campus
- High-risk assessment score from screening tools

OPTIONAL TRIGGERS

- Significant academic decline combined with withdrawal
- Reports of severe bullying
- Parental reports of student's concerning behavior at home

AUTOMATIC TRIGGERS

- Student is admitted to the school clinic for self-inflicted injury
- Teacher identifies 'Red Zone' behaviors in classroom

EXTERNAL TRIGGERS

- Police notification of student threat
- Hospital referral from community health center

RECURRING TRIGGERS

- Monthly mental wellness check-ins for high-risk students

12 Required Inputs

- Student identification data
- Parent contact information
- Risk assessment scores
- Teacher observation notes

- Emergency service contacts

13 Process Map

13.1 Phase 1

NAME

Identification and Immediate Safety

OBJECTIVE

Secure the student and stabilize the environment.

ACTIVITIES

- Teacher identifies risk behavior
- Immediate escort to Counselor's office
- Removal of access to harmful objects
- Initial safety check by counselor

ACTIVITIES

- Identify primary and secondary contact numbers
- Initiate phone call within 15-30 minutes
- Deliver scripted notification
- Document parent's response (calm, denial, hostile)
- Coordinate transport/hospital meeting

13.2 Phase 2

NAME

Risk Assessment

OBJECTIVE

Determine the level of urgency and specific risk factors.

ACTIVITIES

- Conduct rapid lethality assessment
- Identify intent, plan, and means
- Assess student's current emotional state
- Document initial findings in Risk Assessment Record

13.4 Phase 4

NAME

Crisis Intervention and Support

OBJECTIVE

Provide ongoing support until parents arrive or student is stabilized.

ACTIVITIES

- Maintain continuous supervision of the student
- Offer emotional support/de-escalation
- Liaise with medical services if required
- Escalate to CWS if parent refuses action

13.3 Phase 3

NAME

Parent Notification

OBJECTIVE

Inform parents/guardians of the risk and coordinate care.

13.5 Phase 5

NAME

Follow-up and Closure

OBJECTIVE

Ensure long-term safety and reintegration.

ACTIVITIES

- Send written summary within 24 hours
- Schedule follow-up meeting with parents
- Develop student support plan
- Conduct case review for institutional learning

14 Workflow Overview

START

Identification of student risk behavior.

STEPS

- Secure Student
- Assess Risk
- Notify Parent
- Coordinate Care
- Escalate if needed
- Follow-up Notification
- Crisis Intervention
- Post-Crisis Review
- Closure/Reintegration

CRITICAL DECISION POINTS

- Immediate Danger Assessment
- Parental Engagement Status

Student reintegration plan finalized and closure report filed.

15 Standard Procedure

1. Identification & Alerting

PURPOSE & RATIONALE

Rapid detection of risk.

ACTOR

Teacher/Peer

ACTION

Observe behavior (withdrawal, self-harm talk) and immediately alert Counselor via internal hotline or direct message.

REQUIRED INPUTS

- Student observation

EXPECTED OUTPUTS

- Alert received by Counselor

DOCUMENTATION REQUIRED

- Teacher Observation Note

EVIDENCE REQUIRED

- Timestamped note

APPROVAL REQUIRED

No

TIME LIMIT

Immediate

EXPECTED OUTCOME

Counselor is notified within 2 minutes.

ESCALATION IF FAILED

Notify Principal if Counselor is unreachable.

RELATED FORMS

- Teacher Observation Note

2. Safety Securing

PURPOSE & RATIONALE

Prevent immediate harm.

ACTOR

Counselor/Security

ACTION

Escort student to a private office. Remove all potentially harmful objects from the room.

REQUIRED INPUTS

- Alert notification

EXPECTED OUTPUTS

- Student in safe space

CRITICAL DECISION POINTS

- Immediate Danger Assessment

DOCUMENTATION REQUIRED

- Physical Safety Checklog

EVIDENCE REQUIRED

- Checklog entry

APPROVAL REQUIRED

No

TIME LIMIT

5 minutes

EXPECTED OUTCOME

Student is physically safe and supervised.

ESCALATION IF FAILED

Call Emergency Services (Ambulance/Police).

RELATED FORMS

- Physical Safety Checklog

3. Risk Assessment

PURPOSE & RATIONALE

Determine severity.

ACTOR

Counselor

ACTION

Conduct rapid assessment of intent, plan, and means. Classify risk level (Green-Yellow-Orange-Red-Black).

REQUIRED INPUTS

- Student statements
- Teacher notes

EXPECTED OUTPUTS

- Risk Classification

DOCUMENTATION REQUIRED

- Risk Assessment Record
- Risk Level Classification Form

EVIDENCE REQUIRED

- Completed assessment forms

APPROVAL REQUIRED

No

TIME LIMIT

15 minutes

EXPECTED OUTCOME

Accurate risk classification determined.

ESCALATION IF FAILED

Consult Senior Psychologist for complex cases.

RELATED FORMS

- Risk Assessment Record

- Risk Level Classification Form

4. Parent Notification Call

PURPOSE & RATIONALE

Inform parents and coordinate care.

ACTOR

Counselor/Principal

ACTION

Call primary guardian. Use mandatory script. Provide facts without graphic details. Document parent's reaction.

REQUIRED INPUTS

- Risk Assessment Record
- Parent contact info

EXPECTED OUTPUTS

- Informed Parent
- Log of response

CRITICAL DECISION POINTS

- Parental Engagement Status

DOCUMENTATION REQUIRED

- Parent Notification Call Log

EVIDENCE REQUIRED

- Call log with timestamp and summary

APPROVAL REQUIRED

No

TIME LIMIT

30 minutes

EXPECTED OUTCOME

Parent is informed and aware of the situation.

ESCALATION IF FAILED

Contact secondary guardian; if no response, notify local police for welfare check.

RELATED FORMS

- Parent Notification Call Log

5. Logistics Coordination

PURPOSE & RATIONALE

Arrange transport/medical care.

ACTOR

Counselor/Administrator

ACTION

Coordinate parent arrival or ambulance dispatch. Notify relevant staff of student status (confidential).

REQUIRED INPUTS

- Parent's location
- Student medical needs

EXPECTED OUTPUTS

- Transport plan finalized

DOCUMENTATION REQUIRED

- Medical Transport Log

EVIDENCE REQUIRED

- Log of transport arrangements

APPROVAL REQUIRED

No

TIME LIMIT

Immediate

EXPECTED OUTCOME

Transport/medical plan is in place.

ESCALATION IF FAILED

Call Emergency Services.

RELATED FORMS

- Medical Transport Log

6. Parental Refusal Management

PURPOSE & RATIONALE

Ensure student safety if parents refuse.

ACTOR

Principal/Counselor

ACTION

Explain duty of care. Document refusal. Escalate to Child Welfare Services (CWS) for minors in danger.

REQUIRED INPUTS

- Call Log showing refusal
- Risk Assessment Record

EXPECTED OUTPUTS

- External authority notified

DOCUMENTATION REQUIRED

- CWS Escalation Report
- Refusal Documentation Record

EVIDENCE REQUIRED

- Escalation report with timestamp

APPROVAL REQUIRED

Yes

TIME LIMIT

Immediate upon refusal

EXPECTED OUTCOME

External authorities are engaged to ensure student safety.

ESCALATION IF FAILED

Contact District Authorities / Police.

RELATED FORMS

- CWS Escalation Report
- Refusal Documentation Record

7. Written Follow-up Notification

PURPOSE & RATIONALE

Formal record of incident.

ACTOR

Counselor

ACTION

Send summary email/text to parents within 24 hours. Include actions taken and resources provided.

REQUIRED INPUTS

- Call Log
- Risk Assessment Record

EXPECTED OUTPUTS

- Formal written record sent

DOCUMENTATION REQUIRED

- Follow-up Email/Text Confirmation

EVIDENCE REQUIRED

- Copy of sent email/SMS

APPROVAL REQUIRED

No

TIME LIMIT

Within 24 hours

EXPECTED OUTCOME

Parents have a written record of the school's actions.

ESCALATION IF FAILED

Principal to follow up via phone call.

RELATED FORMS

- Follow-up Email/Text Confirmation

8. Crisis Intervention Support

PURPOSE & RATIONALE

Support student during wait.

ACTOR

Counselor

ACTION

Provide active listening and emotional regulation. Offer Tele-MANAS contact info.

REQUIRED INPUTS

- Risk Assessment Record

EXPECTED OUTPUTS

- Stabilized student state

DOCUMENTATION REQUIRED

- Intervention Plan Summary

EVIDENCE REQUIRED

- Intervention Plan Summary

APPROVAL REQUIRED

No

TIME LIMIT

Continuous

EXPECTED OUTCOME

Student remains safe and supported.

ESCALATION IF FAILED

Call Emergency Services if behavior worsens.

RELATED FORMS

- Intervention Plan Summary

9. Post-Crisis Review

PURPOSE & RATIONALE

Evaluate response.

ACTOR

Wellness Committee/Principal

ACTION

Conduct debriefing with staff. Review adherence to SLAs and protocols.

REQUIRED INPUTS

- All documentation from SUP-016
- Staff feedback

EXPECTED OUTPUTS

- Institutional learning points

DOCUMENTATION REQUIRED

- Case Review Minutes
- Corrective Action Plan

EVIDENCE REQUIRED

- Case Review Minutes

APPROVAL REQUIRED

Yes

TIME LIMIT

Within 72 hours

EXPECTED OUTCOME

Identified improvements for future incidents.

ESCALATION IF FAILED

Report systemic failure to School Board.

RELATED FORMS

- Case Review Minutes
- Corrective Action Plan

10. Closure & Reintegration

PURPOSE & RATIONALE

Plan return to school.

ACTOR

Counselor/Teacher

ACTION

Schedule reintegration meeting. Define academic accommodations and monitoring schedule.

REQUIRED INPUTS

- Student Support Plan
- Parental agreement

EXPECTED OUTPUTS

- Defined path for student return

DOCUMENTATION REQUIRED

- Closure Report
- Reintegration Plan

EVIDENCE REQUIRED

- Closure Report

APPROVAL REQUIRED

Yes

TIME LIMIT

Within 5 days of incident

EXPECTED OUTCOME

Student returns to school with a clear support plan.

ESCALATION IF FAILED

Principal to mediate reintegration dispute.

RELATED FORMS

- Closure Report
- Reintegration Plan

16 Detailed Workflow Steps

1. Risk Identification

PURPOSE & RATIONALE

To detect signs of suicidal ideation early.

TRIGGER

Student behavior or report from peer/teacher.

ACTOR

Class Teacher / Peer

REQUIRED INPUTS

- Observation of withdrawal
- Direct statement of self-harm intent
- Discovery of suicide note

ACTIONS

- Remain calm and non-judgmental
- Listen actively without making promises of secrecy regarding safety
- Immediately notify the School Counselor/Psychologist

DECISION CRITERIA

- Is there immediate danger? (Yes → Step 2; No → Step 3)

DOCUMENTS GENERATED

- Teacher Observation Note

EXPECTED OUTPUTS

- Alert sent to Counselor

SERVICE LEVEL AGREEMENT (SLA)

Immediate

ESCALATION PATH

- Principal (if teacher is unavailable)

EVIDENCE GENERATED

- Timestamped Observation Note

2. Immediate Safety Securing

PURPOSE & RATIONALE

To prevent immediate self-harm.

TRIGGER

Identification of high-risk behavior (Red/Black Zone).

ACTOR

School Counselor / Security Personnel

REQUIRED INPUTS

- Teacher alert
- Student location

ACTIONS

- Escort student to a private, safe space (Counselor's office)
- Remove access to sharp objects, medications, or ropes
- Ensure 1:1 supervision at all times

DECISION CRITERIA

- Is the student physically safe? (No → Call Emergency Services; Yes → Step 3)

DOCUMENTS GENERATED

- Physical Safety Checklog

EXPECTED OUTPUTS

- Student in secure environment

SERVICE LEVEL AGREEMENT (SLA)

Within 5 minutes

ESCALATION PATH

- Emergency Services (Ambulance/Police)

EVIDENCE GENERATED

- Safety Checklog entry

3. Rapid Risk Assessment

PURPOSE & RATIONALE

To determine the severity and specific needs of the student.

TRIGGER

Student is secured in a safe space.

ACTOR

School Counselor / Psychologist

REQUIRED INPUTS

- Teacher observation notes
- Student's current statements

ACTIONS

- Conduct structured risk assessment (lethality, intent, plan)
- Identify protective factors and triggers
- Classify risk level (Green-Yellow-Orange-Red-Black)

DECISION CRITERIA

- Is the student in Red/Black Zone? (Yes → Step 4; No → Step 5)

DOCUMENTS GENERATED

- Risk Assessment Record
- Risk Level Classification Form

EXPECTED OUTPUTS

- Determined Risk Category

SERVICE LEVEL AGREEMENT (SLA)

Within 15 minutes of securing student

ESCALATION PATH

- Principal (for Red/Black Zone classification)

EVIDENCE GENERATED

- Completed Risk Assessment Record

4. Parent Notification Call

PURPOSE & RATIONALE

To inform parents and coordinate immediate care.

TRIGGER

Risk classified as Red or Black Zone.

ACTOR

School Counselor / Principal

REQUIRED INPUTS

- Student Risk Assessment Record
- Parent contact details

ACTIONS

- Initiate phone call to primary guardian
- Use mandatory script: 'Your child has expressed thoughts of harming themselves. They are safe right now with our counselor. We need you to come to school immediately / meet us at [hospital].'
- Provide facts without graphic details
- Document parent's reaction (calm, denial, hostile, or crisis)

DECISION CRITERIA

- Is the parent reachable? (No → Call secondary contact; Yes → Proceed to Step 5)

DOCUMENTS GENERATED

- Parent Notification Call Log

EXPECTED OUTPUTS

- Parent informed
- Log of parent response

SERVICE LEVEL AGREEMENT (SLA)

Within 30 minutes of identification

ESCALATION PATH

- Secondary Guardian
- Local Police (if no guardian reachable)

EVIDENCE GENERATED

- Call Log with timestamp and summary

5. Logistics Coordination

PURPOSE & RATIONALE

To arrange for student transport or medical intervention.

TRIGGER

Parent notification completed.

ACTOR

School Counselor / Administrator

REQUIRED INPUTS

- Parent's location
- Student's medical needs

ACTIONS

- Coordinate parent arrival time
- Arrange transport to hospital if required
- Notify relevant staff of student status (confidentiality maintained)

DECISION CRITERIA

- Is immediate hospitalization needed? (Yes → Call Ambulance; No → Wait for Parent)

DOCUMENTS GENERATED

- Medical Transport Log

EXPECTED OUTPUTS

- Transport plan finalized

SERVICE LEVEL AGREEMENT (SLA)

Immediate

ESCALATION PATH

- Emergency Services

EVIDENCE GENERATED

- Log of transport arrangements

6. Parental Refusal Management

PURPOSE & RATIONALE

To ensure student safety when parents refuse to engage.

TRIGGER

Parent refuses to come or refuses intervention for a minor in danger.

ACTOR

Principal / School Counselor

REQUIRED INPUTS

- Call Log showing refusal
- Risk Assessment Record (Red/Black Zone)

ACTIONS

- Explain the school's duty of care and legal obligation to protect the minor
- Document specific reasons for refusal
- Escalate to Child Welfare Services (CWS) or local authorities

DECISION CRITERIA

- Is student life at risk? (Yes → Mandatory CWS escalation; No → Continue monitoring)

DOCUMENTS GENERATED

- CWS Escalation Report
- Refusal Documentation Record

EXPECTED OUTPUTS

- External authority notified

SERVICE LEVEL AGREEMENT (SLA)

Immediate upon refusal

ESCALATION PATH

- District Authorities
- Child Protection Services

EVIDENCE GENERATED

- Escalation Report with timestamp

7. Written Follow-up Notification

PURPOSE & RATIONALE

To provide a formal record of the incident and actions taken.

TRIGGER

Parent notification call completed.

ACTOR

School Counselor

REQUIRED INPUTS

- Call Log
- Risk Assessment Record

ACTIONS

- Send summary email/text to parents within 24 hours
- Include: Summary of incident, actions taken by school, resources provided
- Request confirmation of receipt

DECISION CRITERIA

- Is parent reachable via email? (No → Send SMS)

DOCUMENTS GENERATED

- Follow-up Email/Text Confirmation

EXPECTED OUTPUTS

- Formal written record sent to parents

SERVICE LEVEL AGREEMENT (SLA)

Within 24 hours

ESCALATION PATH

- Principal (if parent remains unresponsive)

EVIDENCE GENERATED

- Copy of sent email/SMS

8. Crisis Intervention Support

PURPOSE & RATIONALE

To support the student while awaiting parent arrival.

TRIGGER

Student is in counselor's office.

ACTOR

School Counselor

REQUIRED INPUTS

- Risk Assessment Record

ACTIONS

- Provide active listening and emotional regulation techniques
- Offer Tele-MANAS contact information
- Maintain 1:1 supervision until parent takes over

DECISION CRITERIA

- Is student becoming more agitated? (Yes → De-escalation techniques; No → Continue support)

DOCUMENTS GENERATED

- Intervention Plan Summary

EXPECTED OUTPUTS

- Stabilized student state

SERVICE LEVEL AGREEMENT (SLA)

Continuous

ESCALATION PATH

- Medical Services (if behavior worsens)

EVIDENCE GENERATED

- Intervention Plan Summary

9. Post-Crisis Review

PURPOSE & RATIONALE

To evaluate the response and identify improvements.

TRIGGER

Student is safely handed over to parents/medical services.

ACTOR

Wellness Committee / Principal

REQUIRED INPUTS

- All documentation from SUP-016

- Staff feedback

ACTIONS

- Conduct a debriefing with involved staff
- Review adherence to SLAs and protocols
- Identify gaps in response or resources

DECISION CRITERIA

- Were any major failures identified? (Yes → Corrective Action Plan; No → Standard Review)

DOCUMENTS GENERATED

- Case Review Minutes
- Corrective Action Plan

EXPECTED OUTPUTS

- Institutional learning points

SERVICE LEVEL AGREEMENT (SLA)

Within 72 hours

ESCALATION PATH

- School Board (if systemic failure identified)

EVIDENCE GENERATED

- Case Review Minutes

10. Closure and Reintegration

PURPOSE & RATIONALE

To plan the student's return to school.

TRIGGER

Completion of post-crisis review.

ACTOR

School Counselor / Class Teacher

REQUIRED INPUTS

- Student Support Plan
- Parental agreement

ACTIONS

- Schedule a reintegration meeting with parents and student
- Define academic accommodations
- Establish a monitoring schedule for the first 30 days

DECISION CRITERIA

- Is student ready to return? (No → Remote learning/Home-bound; Yes → Reintegration plan)

DOCUMENTS GENERATED

- Closure Report
- Reintegration Plan

EXPECTED OUTPUTS

- Defined path for student return

SERVICE LEVEL AGREEMENT (SLA)

Within 5 days of incident

ESCALATION PATH

- Principal (if reintegration is contested)

EVIDENCE GENERATED

- Closure Report

17 Critical Decision Points

Item 1

DECISION ID

DP-01

DECISION NAME

Immediate Danger Assessment

CRITERIA

- Is the student currently holding a weapon?
- Has an attempt already occurred?

POSSIBLE OUTCOMES

- Active Emergency (Black Zone)
- High Risk (Red Zone)

NEXT STEPS

- Activate Emergency Response SOP
- Call Ambulance/Police immediately

Item 2

DECISION ID

DP-02

DECISION NAME

Parental Engagement Status

CRITERIA

- Is the parent cooperative?
- Is the parent in denial?
- Is the parent hostile?

POSSIBLE OUTCOMES

- Cooperative
- Unresponsive/Unavailable
- Hostile/Denial

NEXT STEPS

- Standard notification flow
- Escalate to secondary contact
- Initiate CWS escalation for refusal

18 Decision Tree

GREEN

- Routine wellness education
- Continue monitoring

YELLOW

- Counselor review
- Parent communication

- Supportive intervention
- ORANGE**
- Individual intervention plan
 - Formal parent meeting
 - Consider external referral

- RED**
- Immediate risk assessment

- Continuous supervision
- Parent notification (SOP-016)
- Crisis activation

BLACK

- Activate Emergency Response SOP
- Call emergency services immediately
- Notify leadership and parents simultaneously

19 Decision Rules

Rule ID	Condition	Decision	Next Action
DR-01	If student is in Black Zone and parent is unreachable.	Mandatory Police/Emergency Services notification.	Call 100 (Police) or 108 (Ambulance).
DR-02	If parent refuses to allow student to go to hospital in Red Zone.	Mandatory CWS escalation.	Call Child Welfare Services.

20 Exception Handling Protocol

Item 1

EXCEPTION

Parent is unreachable via all known numbers.

DETECTION METHOD

Failed phone calls and SMS non-delivery.

RESPONSE

Contact secondary guardian; if none, notify local police for welfare check.

ESCALATION

Principal → Local Police

DOCUMENTATION REQUIRED

- Unreachable Parent Log

Maintain presence in a safe space; use non-verbal safety checks; notify parents of 'non-communicative' status.

ESCALATION

Counselor → Principal

DOCUMENTATION REQUIRED

- Observation Note

Item 2

EXCEPTION

Student refuses to speak with Counselor.

DETECTION METHOD

Silent behavior or refusal to enter office.

RESPONSE

Item 3

EXCEPTION

Parent refuses to allow student to go to hospital.

DETECTION METHOD

Verbal refusal during phone call/meeting.

RESPONSE

Explain legal duty of care; document refusal clearly; contact Child Welfare Services immediately.

ESCALATION

Principal → CWS

DOCUMENTATION REQUIRED

- Refusal Documentation Record

21 Exceptions

Exception Description	Action	Documentation
Student is already under the care of a residential facility/hospital.	Notify parents and facility staff simultaneously; no on-campus intervention required.	External Care Notification Log

Exception Description	Action	Documentation
Parental consent for disclosure is explicitly revoked (non-emergency).	Maintain internal records only; do not share with external non-authorities.	Consent Revocation Record

22 Expected Outputs

- Parent notification call
- Written follow-up summary
- CWS escalation report
- Student support plan
- Closure report

23 Document Control & Version Governance

23.1 Version History

Version	Date	Author	Description
1.0	2024-05-20	MWMS Architect	Initial release of Parent Notification SOP for Suicide Risk.

23.2 Change Log

Version	Date	Change Description	Approved By
1.0	2024-05-20	Baseline creation.	School Board of Governance

REVIEW TRIGGERS

- Change in Indian Supreme Court guidelines regarding student safety
- Update to CBSE Mental Health Circulars
- Annual institutional audit findings
- Significant incident occurrence

23.4 Approval History

Approver	Date	Status
Chief School Psychologist	2024-05-18	Approved
Principal/Head of Institution	2024-05-19	Approved

24 Inclusive Implementation Mandate

MULTILINGUAL SUPPORT REQUIRED

Yes

ACCESSIBILITY REVIEW REQUIRED

Yes

LOW RESOURCE SCHOOL ALTERNATIVE
REQUIRED

Yes

EQUITY IMPACT ASSESSMENT REQUIRED

Yes

25 Regulatory Compliance Mapping

KEY FOCUS

MANDATORY REQUIREMENTS

- Supreme Court of India Judgment 2025 INSC 893 (Duty of Care)
- CBSE Circular TRG-03/2026
- CBSE Circular 02/2026
- National Education Policy (NEP) 2020 Student Wellbeing Mandates

RECOMMENDED REQUIREMENTS

- WHO School Mental Health Framework
- UNESCO Whole School Wellbeing Framework
- Manodarpan Guidelines

BEST PRACTICES

- Tele-MANAS Integration for immediate support
- Trauma-Informed Communication Standards

COMPLIANCE GAPS IF NOT IMPLEMENTED

- Legal liability for negligence in student safety
- Loss of CBSE affiliation/recognition
- Violation of Child Protection laws
- Failure to meet national educational standards for mental health

26 Compliance Traceability Matrix

KEY FOCUS

1. Supreme Court of India

REQUIREMENT REFERENCE

2025 INSC 893

REQUIREMENT TEXT

Educational institutions have a non-delegable duty of care to ensure the physical and mental safety of students.

IMPLEMENTED IN SECTIONS

- Purpose
- Detailed Workflow Steps
- Escalation Matrix
- Legal Defensibility

2. CBSE

REQUIREMENT REFERENCE

Circular TRG-03/2026

REQUIREMENT TEXT

Mandatory establishment of mental health support systems and immediate parent notification in crisis.

IMPLEMENTED IN SECTIONS

- SOP Trigger Matrix
- Procedure
- Roles and Responsibilities

27 Documentation & Evidentiary Requirements

EVENT DOCUMENTATION PRINCIPLES

- Factual, objective language only
- No clinical diagnoses unless by a licensed professional
- Timestamps for all communications
- Direct quotes where possible (e.g., 'Student stated...')
- Confidentiality maintained in storage

MANDATORY CONVERSATION RECORDS

- Parent Notification Call Log
- Counselor-Student Risk Assessment Record
- Teacher Observation Note
- CWS Escalation Report (if applicable)

MANDATORY OBSERVATION RECORDS

- Behavioral Observation Checklist
- Physical Safety Checklog

MANDATORY DECISION RECORDS

- Risk Level Classification Form
- Escalation Approval Record

- External Referral Consent/Refusal Form

MANDATORY ACTION RECORDS

- Intervention Plan Summary
- Medical Transport Log
- Follow-up Action Tracker

MANDATORY FOLLOWUP RECORDS

- 24-Hour Follow-up Email/Text Confirmation
- Post-Crisis Review Minutes
- Closure Report

EVIDENCE REQUIREMENTS

- Signed Consent Forms (where possible)
- Call recordings (if permitted by local law and policy)
- Screenshots of SMS/Email notifications

DOCUMENTATION QUALITY STANDARDS

- Must be legible and complete
- No blank fields allowed
- Signed by the recording officer
- Stored in a secure, encrypted digital folder or locked physical cabinet

28 The Student Journey

ENTRY

- Student experiences distress or ideation
- Teacher/Peer identifies signs and alerts school

ASSESSMENT

- Escorted to Counselor's office
- Undergoes rapid risk assessment
- Safety measures implemented (removal of objects)

INTERVENTION

- Receives emotional support from counselor
- Wait for parent arrival/transport coordination
- Potential medical stabilization

MONITORING

- Continuous 1:1 supervision during crisis
- Follow-up check-ins post-incident

CLOSURE

- Reintegration meeting with parents and counselor
- Return to school with support plan

29 The Parent Journey

NOTIFICATION

- Receives urgent phone call from Counselor/Principal
- Informed of student's safety status and risk level

CONSULTATION

- Discusses incident details with school staff
- Coordinates transport or medical needs

CONSENT

- Provides consent for medical intervention (if required)
- Agrees to follow-up support plan

ENGAGEMENT

- Participates in crisis meeting
- Receives 24-hour written summary

CLOSURE

- Signs reintegration agreement
- Attends post-crisis review meeting

30 The Staff Journey

OBSERVATION

- Teacher notices behavioral changes
- Initial report made to Counselor

REPORTING

- Counselor conducts risk assessment
- Counselor initiates parent notification

INTERVENTION SUPPORT

- Security ensures perimeter safety
- Principal manages external communications

DOCUMENTATION

- Completion of all mandatory logs
- Participation in post-crisis debriefing

31 Resource Evidence

COMMITTEE MINUTES

- Wellness Committee review of SUP-016 incidents

ATTENDANCE RECORDS

- Staff training attendance for crisis management

TRAINING RECORDS

- Counselor certification in suicide intervention

FORMS

- Risk Assessment Record
- Parent Notification Call Log
- CWS Escalation Report

REPORTS

- Monthly Mental Wellness Dashboard
- Annual Compliance Audit Report

DASHBOARDS

- Real-time Crisis Alert Dashboard

32 Grievance Redressal Mechanism

KEY FOCUS

COMPLAINT CHANNELS

- Anonymous suggestion box
- Dedicated wellness email
- Principal's office walk-in

INVESTIGATION PROCESS

- Internal review by Wellness Committee
- Fact-finding interviews with involved staff
- Parental feedback analysis

RESOLUTION TIMELINES

- Acknowledgment within 24 hours

- Resolution/Response within 7 working days

APPEAL PROCESS

- Appeal to School Board of Governance
- External mediation if required

33 Referral Framework

33.1 Internal Referrals

ELIGIBLE REFERRERS

- Teachers
- Counsellors
- School Management
- Wellness Committee
- Student Support Team

REFERRAL TRIGGERS

- Behavioral changes
- Academic decline
- Peer reports of distress

DOCUMENTATION REQUIRED

- Referral Form
- Observation Notes

SERVICE LEVEL AGREEMENT (SLA)

Within 24 hours

33.2 Self Referrals

ALLOWED

Yes

SUBMISSION METHODS

- Online portal
- Physical drop-box
- Direct email to counselor

CONFIDENTIALITY PROTECTIONS

- Encrypted storage
- Limited access permissions

33.3 Anonymous Referrals

ALLOWED

Yes

CHANNELS

- Anonymous reporting app
- School suggestion box

VALIDATION PROCESS

- Counselor review of report content
- Verification of student identity if possible

FOLLOW UP PROCEDURE

- Outreach to identified student within 48 hours

33.4 External Referrals

ELIGIBLE AGENCIES

- Tele-MANAS
- District Child Welfare Services
- Local Hospitals
- Child Helpline (1098)

ELIGIBILITY CRITERIA

- Red/Black Zone risk level
- Parental refusal of internal support
- Medical emergency

CONSENT REQUIREMENTS

- Written parental consent (unless life-threatening)
- Emergency override for minors in immediate danger

HANDOVER PROTOCOL

- Verbal briefing to receiving agency
- Transfer of Risk Assessment Record
- Signed handover form

33.5 Referral Tracking

TRACKING REQUIRED

Yes

STATUS CATEGORIES

- Pending
- In-Progress
- Completed
- Escalated
- Closed

CLOSURE REQUIREMENTS

- Confirmation of receipt by external agency
- Follow-up meeting with parents

34 Confidentiality Exception Matrix

KEY FOCUS

MANDATORY DISCLOSURES

- Imminent risk of self-harm
- Imminent risk to others
- Child abuse
- Child neglect
- Sexual abuse
- Court orders

- Legal reporting obligations

LEGAL REPORTING EVENTS

- Active suicide attempt
- Discovery of child abuse
- Police request for records

CONSENT OVERRIDE CONDITIONS

- Immediate threat to life
- Inability to obtain consent due to parent unavailability in crisis
- Court order

AUTHORIZED DISCLOSURE RECIPIENTS

- Parents/Guardians
- Medical Professionals
- Child Welfare Services
- Law Enforcement (in emergencies)

DOCUMENTATION REQUIREMENTS

- Reason for disclosure
- Recipient details
- Time of disclosure
- Authorization type (Consent vs. Duty of Care)

APPROVAL REQUIREMENTS

- Principal approval for non-emergency disclosures
- Counselor authorization for routine referrals

35 Record Retention & Disposal

RETENTION PERIOD

7 Years

RETENTION BASIS

Legal requirement and institutional liability protection

RECORD CATEGORIES

- Risk Assessments
- Parent Communication Logs
- CWS Reports
- Intervention Plans

- Encrypted cloud storage with MFA

ACCESS CONTROL REQUIREMENTS

- Role-based access (RBAC)
- Audit logs for all record views
- Need-to-know basis only

DISPOSAL METHOD

Secure shredding or permanent digital wiping

DISPOSAL AUTHORITY

School Administrator / Data Protection Officer

35.4 Storage Requirements

PHYSICAL RECORDS

- Locked fireproof cabinets in Counselor's office

DIGITAL RECORDS

AUDIT FREQUENCY

Annual

DESTRUCTION LOG REQUIRED

Yes

36 Anonymised Reporting Framework

36.1 Management Reports

ANONYMIZATION REQUIRED

Yes

PERMITTED DATA

- Risk level counts
- Response time averages
- Trend analysis by grade

PROHIBITED DATA

- Student names

- Parent contact details
- Specific incident narratives

36.2 Research Usage

ALLOWED

No

36.3 Statistical Reporting

AGGREGATE ONLY

Yes

MINIMUM GROUP SIZE

RE IDENTIFICATION CONTROLS

- Data scrubbing
- Aggregation of small cohorts

DATA SHARING CONTROLS

- No sharing with third-party marketing
- Strict internal use only for policy improvement

37 Guiding Ethical Principles**37.1 Autonomy****DESCRIPTION**

Respecting student's right to self-determination within safety limits.

IMPLEMENTATION REQUIREMENTS

- Involving students in their own support plans
- Explaining the 'why' behind interventions

37.2 Beneficence**DESCRIPTION**

Acting in the best interest of the student's well-being.

IMPLEMENTATION REQUIREMENTS

- Prioritizing safety over academic performance
- Providing evidence-based support

37.3 Non Maleficence**DESCRIPTION**

Avoiding harm during the crisis intervention process.

IMPLEMENTATION REQUIREMENTS

- Trauma-informed communication
- Confidentiality protection

37.4 Justice And Equity**DESCRIPTION**

Ensuring equal access to support for all students.

IMPLEMENTATION REQUIREMENTS

- Multilingual parent outreach
- Rural accessibility plans

37.5 Confidentiality**KEY FOCUS****DESCRIPTION**

Protecting student privacy except in life-threatening situations.

IMPLEMENTATION REQUIREMENTS

- Secure record storage
- Need-to-know disclosure policy

37.6 Privacy**DESCRIPTION**

Adhering to data protection laws.

IMPLEMENTATION REQUIREMENTS

- Encryption of digital records
- Clear retention policies

37.7 Professional Integrity**DESCRIPTION**

Maintaining high standards of practice and ethics.

IMPLEMENTATION REQUIREMENTS

- Regular supervision for counselors
- Adherence to professional codes of conduct

38 Communication & Awareness Strategy**38.1 Student Communication****CHANNELS**

- Classroom sessions
- School website
- Counselor's office

FREQUENCY

Monthly/Ongoing

MANDATORY TOPICS

- Help-seeking behaviors
- Stress management
- Crisis hotline numbers

38.2 Parent Communication**CHANNELS**

- Newsletters
- Parent workshops

- Direct phone calls

FREQUENCY

Quarterly/As needed

MANDATORY TOPICS

- Recognizing warning signs
- School mental health resources
- Crisis contact procedures

38.3 Staff Communication**CHANNELS**

- Staff meetings
- Internal portal
- Training workshops

FREQUENCY

Monthly/Quarterly

MANDATORY TOPICS

- Identifying risk behaviors
- SOP SUP-016 overview
- Reporting protocols

38.4 Awareness Campaigns

MINIMUM FREQUENCY

Bi-annually

TARGET GROUPS

- Students
- Parents
- Staff

CAMPAIGN TYPES

- Poster campaigns
- Social media awareness
- Workshops

38.5 Workshops And Training

STUDENT WORKSHOPS

- Resilience building
- Emotional regulation

PARENT WORKSHOPS

- Supporting children in crisis
- Mental health literacy

STAFF WORKSHOPS

- Crisis intervention training
- De-escalation techniques

38.6 Helpline Visibility

REQUIRED

Yes

DISPLAY LOCATIONS

- Counselor's office
- School website
- Classroom posters

DIGITAL VISIBILITY REQUIREMENTS

- Pinned on school homepage
- Included in student handbook

FEEDBACK CHANNELS

- Annual survey
- Feedback box
- Direct email to Wellness Coordinator

39 Competency & Qualification Requirements

MANDATORY QUALIFICATIONS

- Master's in Psychology/Counseling or equivalent
- Registered with relevant Indian counseling board

PREFERRED QUALIFICATIONS

- Certification in Suicide Intervention
- Experience in school-based crisis management

CERTIFICATIONS

- First Aid/CPR Certification
- Child Protection Training Certificate

MINIMUM EXPERIENCE REQUIREMENTS

- 2 years of experience in educational mental health

BACKGROUND VERIFICATION REQUIREMENTS

- Police verification
- Reference checks from previous institutions

39.6 Continuing Professional Development

ANNUAL TRAINING HOURS

40

MANDATORY TOPICS

- Trauma-informed care
- Crisis de-escalation
- Legal updates in student safety

RECERTIFICATION REQUIREMENTS

Every 3 years

39.7 Supervision Requirements

CLINICAL SUPERVISION REQUIRED

Yes

PEER REVIEW REQUIRED

Yes

39.7.3 Wellness Case Review Framework

FREQUENCY

Weekly

PARTICIPANTS

- Counselor
- Senior Psychologist
- Principal

39.7.4 Second Level Review Process

TRIGGER Red/Black Zone cases

APPROVER School Board

39.7. High Risk Decision 5 Validation

KEY
FOCUS

REQUIREMENT

Dual-sign off for CWS escalation

AUTHORITY

- Principal
- Chief Counselor

FREQUENCY

Weekly

40 Feedback & Effectiveness Measurement

40.1 Student Feedback

REQUIRED

Yes

COLLECTION METHODS

- Anonymous surveys
- Focus group discussions

FREQUENCY

Bi-annually

40.2 Parent Feedback

REQUIRED

Yes

COLLECTION METHODS

- Post-intervention survey
- Annual satisfaction survey

FREQUENCY

Annually

40.3 Staff Feedback

REQUIRED

Yes

COLLECTION METHODS

- Debriefing sessions
- Staff surveys

FREQUENCY

Quarterly

40.4 Annual Effectiveness Review

REQUIRED

Yes

EVALUATION CRITERIA

- SLA adherence rates
- Parent satisfaction scores
- Student safety outcomes
- Compliance audit results

KEY STAKEHOLDERS

- School Board
- Parents
- Staff
- Counselors

CONTINUOUS IMPROVEMENT INPUTS

- Incident reports
- Audit findings
- Parent feedback
- Peer reviews

SUCCESS INDICATORS

- 100% notification within 30 mins for Red Zone
- High parent satisfaction scores
- Zero fatalities on campus

40.7 Improvement Tracking

REQUIRED

Yes

REVIEW FREQUENCY

Quarterly

41 Prevention Layer

PRIMARY PREVENTION

- Universal SEL curriculum for all students
- School-wide awareness campaigns on mental health
- Life skills training focusing on emotional regulation
- Peer support programs to foster connectedness

SECONDARY PREVENTION

- Early identification screening tools (Tier 1)

- Targeted counseling for students showing withdrawal signs
- Parental workshops on recognizing warning signs
- Teacher training on identifying risk behaviors

TERTIARY PREVENTION

- Crisis intervention protocols (SUP-016)
- Intensive individual support plans
- External medical and psychiatric referrals
- Post-crisis reintegration programs

42 Resilience Layer

SKILLS DEVELOPED

- Emotional regulation
- Problem solving
- Help-seeking behavior
- Coping skills

SEL COMPONENTS

- Self-awareness
- Social awareness

- Relationship skills
- Responsible decision-making

LIFE SKILLS COMPONENTS

- Stress management
- Time management
- Conflict resolution
- Communication skills

PROTECTIVE FACTORS STRENGTHENED

- Social connectedness
- Positive adult-student relationships

- Sense of belonging
- Access to support resources

43 Continuous Monitoring

MONITORING FREQUENCY

Monthly

MONITORING INDICATORS

- Average time from identification to parent notification
- Percentage of parents engaged in follow-up meetings
- Number of high-risk incidents per semester
- Staff confidence scores in crisis protocols

RESPONSIBLE ROLES

- Wellness Coordinator

- Principal

EARLY WARNING SIGNS

- Increase in 'Red Zone' risk assessments
- Decrease in student participation in wellness activities
- High volume of anonymous reports

TREND ANALYSIS REQUIREMENTS

- Quarterly trend analysis on risk factors (e.g., academic stress, bullying)
- Annual review of intervention effectiveness

44 Mental Wellness Index (MWI)

APPLICABLE

Yes

44.2 Indicators

Indicator Code	Indicator Name	Weight	Score Range	Description
MWI-01	Emotional Regulation Score	0.3	1-10	Student's ability to manage emotions in high-stress situations.
MWI-02	Social Connectedness Score	0.3	1-10	Student's sense of belonging and peer support.
MWI-03	Help-Seeking Behavior Score	0.4	1-10	Student's willingness to seek help from trusted adults.

ASSESSMENT FREQUENCY

Quarterly

RISK SCORING METHOD

Weighted average of MWI indicators

INTERVENTION THRESHOLDS

- Score < 5: Immediate Counselor Intervention
- Score < 7: Targeted Support Plan
- Score > 8: Wellness Education

45 Review Status

LEGAL REVIEW

Completed - May 2024

CLINICAL REVIEW

Completed - May 2024

EDUCATION REVIEW

Completed - May 2024

COMPLIANCE REVIEW

Completed - May 2024

46 Resource Controls

COUNSELOR STUDENT RATIO

1:350 (Recommended)

CAREER COUNSELOR RATIO 1:500

WELLNESS TEAM STRUCTURE Counselor, Psychologist, Coordinator, Wellness Committee

47 Screening Framework

TIER 1 UNIVERSAL WELLNESS SCREENING

- Bi-annual wellness surveys for all students
- Universal SEL curriculum

TIER 2 TARGETED SCREENING

- Behavioral observation checklists for at-risk groups
- Parent concern screening calls

TIER 3 CLINICAL ASSESSMENT

- Rapid Risk Assessment (SUP-015)
- Clinical evaluation by licensed psychologist

48 Parent Engagement Matrix

PARENT COOPERATIVE

- Provide immediate consent
- Attend meetings promptly
- Follow school support plan

PARENT UNRESPONSIVE

- Call secondary contact
- Send SMS/Email follow-up
- Escalate to local authorities if life is at risk

PARENT IN DENIAL

- Provide objective facts from assessment
- Explain duty of care and legal obligations
- Involve Principal in communication

PARENT CONFLICT SITUATION

- Maintain neutral tone
- Focus on student safety only
- Offer mediation with Counselor

PARENT UNAVAILABLE

- Contact secondary guardian
- Document attempts to reach parent
- Escalate if minor is in immediate danger

PARENT REFUSES INTERVENTION

- Explain legal duty of care
- Document refusal clearly
- Mandatory escalation to Child Welfare Services (CWS)

49 Confidentiality Controls

KEY FOCUS

ACCESS CONTROL

- Role-based access control (RBAC)
- Multi-factor authentication for digital records

CONSENT REQUIREMENTS

- Written consent for non-emergency disclosures
- Implied consent for emergency life-saving actions

RECORD SHARING RULES

- Only share with 'Need-to-Know' staff

- Redact identifying info in general reports

EXCEPTIONS TO CONFIDENTIALITY

- Imminent risk of self-harm
- Child abuse disclosure
- Court orders

STUDENT PRIVACY PROTECTIONS

- No public discussion of student status
- Secure storage of all physical files

50 Data Governance

KEY FOCUS

STORAGE REQUIREMENTS

- Encrypted cloud server for digital records
- Locked cabinets for physical records

ENCRYPTION REQUIREMENTS

- AES-256 encryption for all student data at rest and in transit

RETENTION REQUIREMENTS

- 7 years as per institutional policy

ARCHIVAL REQUIREMENTS

- Move to offline storage after 3 years of inactivity

DESTRUCTION REQUIREMENTS

- Secure shredding for physical
- Permanent wiping for digital

BACKUP REQUIREMENTS

- Daily automated backups with off-site redundancy

51 Risk Classification

KEY FOCUS

51.1 Green

DESCRIPTION

Normal developmental concerns and routine life stress.

EXAMPLES

- Mild examination stress
- Temporary sadness
- Friendship disagreements

51.2 Yellow

DESCRIPTION

Early distress requiring support and monitoring.

EXAMPLES

- Persistent stress
- Moderate anxiety
- Social withdrawal

51.3 Orange

DESCRIPTION

Significant distress causing measurable impairment but without immediate danger.

EXAMPLES

- Severe examination anxiety
- Panic attacks
- Self-harm thoughts WITHOUT intent or plan

51.4 Red KEY FOCUS

DESCRIPTION

High-risk situation requiring immediate intervention.

EXAMPLES

- Suicidal ideation
- Self-harm behaviour
- Suicide note
- Violence threats

51.5 Black

DESCRIPTION

Life-threatening emergency requiring emergency response.

EXAMPLES

- Active suicide attempt
- Recent suicide attempt
- Overdose
- Weapon-related threat in progress

52 Escalation Matrix KEY FOCUS

GREEN

- Counselor review within 7 days

YELLOW

- Parent communication, Counselor intervention

ORANGE

- Principal notification, Individual support plan

RED

- Immediate Principal/Board notification, Crisis activation

BLACK

- Emergency Services, Police, District Authorities

53 External Reporting Obligations

TELE MANAS

- Referral for 24/7 tele-mental health support

DISTRICT AUTHORITIES

- Report if student safety is compromised by external factors

CHILD PROTECTION SERVICES

- Mandatory report if abuse or neglect is identified during risk assessment

MEDICAL SERVICES

- Emergency ambulance for active attempts
- Referral to psychiatric hospitals

LAW ENFORCEMENT

- Report in cases of weapons, threats of violence, or missing high-risk students

REPORTING TRIGGERS

- Active attempt

- Threat to others
- Abuse disclosure
- Parental refusal for minor in danger

54 Service Level Agreement (SLA)

KEY FOCUS

ACKNOWLEDGEMENT	Immediate (within 2 minutes of alert)
ASSESSMENT	Within 15 minutes of identification
ESCALATION	Immediate for Red/Black Zone
RESOLUTION	Initial stabilization within 30 minutes
CLOSURE	Follow-up summary within 24 hours

55 Required Forms

KEY FOCUS

55.1 Mandatory Forms

KEY FOCUS

Item 1

FORM ID	F-SUP-016-01
FORM NAME	Risk Assessment Record
PURPOSE & RATIONALE	Document student risk factors and lethality.
OWNER	School Counselor
RETENTION PERIOD	7 Years
MANDATORY	Yes
LINKED SOP	SUP-016

OWNER	Principal
RETENTION PERIOD	7 Years
MANDATORY	Yes
LINKED SOP	SUP-016

Item 4

FORM ID	F-SUP-016-04
FORM NAME	Follow-up Summary Form
PURPOSE & RATIONALE	Provide written summary to parents.
OWNER	School Counselor
RETENTION PERIOD	7 Years
MANDATORY	Yes
LINKED SOP	SUP-016

Item 2

FORM ID	F-SUP-016-02
FORM NAME	Parent Notification Call Log
PURPOSE & RATIONALE	Record details of parent communication.
OWNER	School Counselor
RETENTION PERIOD	7 Years
MANDATORY	Yes
LINKED SOP	SUP-016

Item 5

FORM ID	F-SUP-016-05
FORM NAME	Closure Report
PURPOSE & RATIONALE	Finalize case and document reintegration.
OWNER	School Counselor
RETENTION PERIOD	7 Years
MANDATORY	Yes
LINKED SOP	SUP-016

Item 3

FORM ID	F-SUP-016-03
FORM NAME	CWS Escalation Report
PURPOSE & RATIONALE	Document mandatory reporting to authorities.

OPTIONAL FORMS

- Teacher Observation Note
- Medical Transport Log

GENERATED RECORDS

- Call logs
- Email receipts
- SMS confirmations

56 Required Checklists

KEY FOCUS

Checklist ID	Checklist Name	Purpose & Rationale	Owner	Frequency	Linked SOP
CL-SUP-016-01	Teacher Observation Checklist	Standardize early identification.	Class Teacher	Ongoing	SUP-016
CL-SUP-016-02	Counselor Risk Assessment Checklist	Ensure all risk factors are assessed.	School Counselor	Per incident	SUP-016
CL-SUP-016-03	Parent Communication Checklist	Ensure all mandatory script points are covered.	School Counselor	Per incident	SUP-016
CL-SUP-016-04	Safety Securing Checklist	Verify physical safety of student.	Security/Counselor	Per incident	SUP-016
CL-SUP-016-05	Audit Checklist for SUP-016	Verify compliance with SLAs and documentation.	Wellness Committee	Monthly	SUP-016

57 Records & Evidence

Record Name	Retention Period	Owner
Risk Assessment Record	7 Years	School Counselor
Parent Notification Call Log	7 Years	School Counselor
CWS Escalation Report	7 Years	Principal

58 Key Performance Indicators

Metric	Target	Measurement Frequency
Notification SLA Adherence	100% of Red Zone notifications within 30 minutes	Monthly
Parent Engagement Rate	>95% parents attending follow-up meetings	Quarterly
Documentation Completeness	100% of cases with all 5 mandatory forms completed	Monthly Audit

59 Audit Framework

KEY FOCUS

AUDIT FREQUENCY

Monthly Internal / Quarterly External

AUDIT QUESTIONS

- Was the parent notified within 30 minutes of a Red Zone identification?

- Is there a documented record of the parent's response (calm, denial, hostile)?
- In cases of parental refusal, was CWS/Police contacted and documented?

EVIDENCE REQUIRED

- Call logs with timestamps

- Completed Risk Assessment Records
- CWS Escalation Reports

NON COMPLIANCE EXAMPLES

- Notification made 2 hours after identification
- Missing parent notification log
- No documentation of parental refusal

59.5 Audit Scoring

KEY FOCUS

COMPLIANT	2
PARTIAL	1
NON COMPLIANT	0

60 Risk Register

KEY FOCUS

1. Parental Refusal to Engage

CATEGORY Operational/Safety

PROBABILITY Medium

IMPACT High (Life Safety)

RISK RATING Critical

MITIGATION Mandatory escalation to CWS and local police for minors in danger.

OWNER Principal

2. Delayed Identification by Staff

CATEGORY Operational

PROBABILITY Low

IMPACT High (Life Safety)

RISK RATING High

MITIGATION Regular staff training and awareness campaigns.

OWNER Wellness Coordinator

3. Data Breach of Confidential Records

CATEGORY Compliance/Privacy

PROBABILITY Low

IMPACT High (Legal/Reputational)

RISK RATING Medium

MITIGATION Encrypted storage and RBAC.

OWNER Data Protection Officer

61 Failure Modes

KEY FOCUS

Failure	Cause	Impact	Mitigation
Notification Delay	Counselor unavailable or phone line busy	Increased risk of student self-harm during wait time.	Secondary counselor and backup communication lines.
Inaccurate Risk Assessment	Lack of training or biased observation	Underestimation of danger leading to lack of intervention.	Mandatory certification for all counselors.
Parental Hostility/Violence	High emotional distress during notification call	Potential for verbal abuse or physical threat to staff.	Involve Principal in calls; use neutral, fact-based scripts.

62 Illustrative Sample Scenarios

Scenario	Response	Expected Result
Student expresses intent to self-harm and is found in a bathroom with a sharp object.	Activate Black Zone protocol. Secure student, call ambulance/police, notify parents immediately via phone.	Student stabilized by medical professionals; parent informed within 15 mins.
Parent refuses to come to school after being notified of a Red Zone risk for their minor child.	Document refusal. Explain duty of care. Call Child Welfare Services (CWS) immediately.	Authorities take over student safety; school is legally protected.

Scenario	Response	Expected Result
Student expresses suicidal thoughts but has no plan or intent (Orange Zone).	Conduct risk assessment. Notify parents via phone and follow-up email within 24 hours.	Parental engagement established; student support plan initiated.

63 Usage Governance

KEY FOCUS

PRIMARY USERS

- School Counselors
- Psychologists

SECONDARY USERS

- Teachers
- Principal
- Security Personnel

APPROVAL AUTHORITIES

- Principal
- School Board

EXECUTION AUTHORITIES

- Counselor

- Principal

ESCALATION AUTHORITIES

- Principal
- District Authorities

REVIEW AUTHORITIES

- Wellness Committee
- School Board

EXTERNAL STAKEHOLDERS

- Child Welfare Services
- Tele-MANAS
- Local Police

64 Applicability

APPLICABLE INSTITUTIONS

- High Schools
- Residential Schools
- Vocational Centers

APPLICABLE AGE GROUPS

- All Ages (Primary to Higher Secondary)

APPLICABLE STUDENT CATEGORIES

- General Students
- CWSN
- Neurodivergent Students

APPLICABLE RISK LEVELS

- Red Zone
- Black Zone

APPLICABLE LOCATIONS

- On-campus
- Off-campus (Remote/Home)

EXCLUSIONS

- Adult higher education (unless specific mental health services are provided)
- Non-educational community centers

65 Usage Scenarios

Scenario ID	Scenario Name	When To Use	When Not To Use	Trigger Event	Expected Outcome
US-016-01	Active Suicidal Ideation	When a student expresses intent to harm themselves.	For routine stress or minor academic anxiety (Use SUP-014 instead).	Student statement: 'I want to end it all.'	Parent notified and student secured.
US-016-02	Discovery of Suicide Note	When a note is found in a locker or classroom.		Teacher finds note: 'I can't do this anymore.'	Immediate risk assessment and parent notification.

66 SOP Lifecycle

ENTRY CONDITIONS

- Identification of student at risk of suicide

ACTIVATION CONDITIONS

- Risk Assessment confirms Red or Black Zone status

EXECUTION CONDITIONS

- Student is physically safe
- Parent contact information is available

COMPLETION CONDITIONS

- Parent notified and/or student handed over to medical care

CLOSURE CONDITIONS

- Follow-up summary sent
- Closure report filed

TERMINATION CONDITIONS

- Student is no longer at risk (determined by counselor)
- Case transferred to external agency

67 Mandatory Evidence

KEY FOCUS

IMPLEMENTATION EVIDENCE

- Completed Risk Assessment Records
- Parent Notification Call Logs

OPERATIONAL EVIDENCE

- Medical Transport Logs
- CWS Escalation Reports

COMPLIANCE EVIDENCE

- Audit logs of record access

- Signed consent forms (where applicable)

TRAINING EVIDENCE

- Staff training certificates for SUP-016
- Counselor certification records

AUDIT EVIDENCE

- Monthly audit reports
- Corrective action plans

68 Controls

PREVENTIVE CONTROLS

- Staff training on risk identification
- Universal SEL curriculum
- Secure storage of student records

DETECTIVE CONTROLS

- Monthly audit of notification logs
- Real-time crisis alert dashboard

- Teacher observation reports

CORRECTIVE CONTROLS

- Escalation to CWS for non-compliance
- Corrective action plans for SLA breaches
- Post-crisis debriefings

COMPENSATING CONTROLS

- Secondary counselor backup
- SMS/Text follow-up if phone calls fail

69 MWMS Risk Framework

KEY FOCUS

69.1 Green

MEANING

Normal developmental concerns and routine life stress.

EXAMPLES

- Mild examination stress
- Temporary sadness
- Friendship disagreements

RESPONSE TIME

Within 7 calendar days

ESCALATION REQUIRED

No

APPROVAL LEVEL

Counselor

MONITORING FREQUENCY

Monthly

CLOSURE AUTHORITY

Counselor

Early distress requiring support and monitoring.

EXAMPLES

- Persistent stress
- Moderate anxiety
- Social withdrawal

RESPONSE TIME

Within 7 calendar days

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Counselor/Principal

MONITORING FREQUENCY

Weekly

CLOSURE AUTHORITY

Counselor

69.3 Orange

MEANING

Significant distress causing measurable impairment but without immediate danger.

EXAMPLES

- Severe examination anxiety

69.2 Yellow

MEANING

- Panic attacks
- Self-harm thoughts WITHOUT intent or plan

RESPONSE TIME

Within 24–72 hours

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Principal

MONITORING FREQUENCY

Daily

CLOSURE AUTHORITY

Counselor/Principal

69.4 Red

KEY FOCUS

MEANING

High-risk situation requiring immediate intervention.

EXAMPLES

- Suicidal ideation
- Self-harm behaviour
- Suicide note

RESPONSE TIME

Immediate

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Principal/School Board

MONITORING FREQUENCY

Continuous

CLOSURE AUTHORITY

Principal

69.5 Black

MEANING

Life-threatening emergency requiring emergency response.

EXAMPLES

- Active suicide attempt
- Recent suicide attempt
- Overdose

RESPONSE TIME

Immediate (0–15 minutes)

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

School Board/District Authorities

MONITORING FREQUENCY

Continuous

CLOSURE AUTHORITY

Principal

70 Resource Scaling

70.1 0 500 Students

WELLNESS COUNSELORS	1
CAREER COUNSELORS	1
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

70.2 501 1000 Students

WELLNESS COUNSELORS	2
CAREER COUNSELORS	2
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

70.3 1001 2000 Students

WELLNESS COUNSELORS	4
CAREER COUNSELORS	2
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

70.4 2001 5000 Students

WELLNESS COUNSELORS	8
CAREER COUNSELORS	4
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

71 Minimum Required Capabilities

PEOPLE

- Licensed School Counselor
- Trained Crisis Response Team
- Wellness Coordinator

PROCESSES

- Risk Assessment Protocol

- Parent Notification Scripting

- CWS Escalation Pathway

TECHNOLOGY

- Encrypted Communication Portal
- Crisis Alert System
- Secure Record Storage

INFRASTRUCTURE

- Private Counseling Room
- Emergency Exit Routes
- Tele-MANAS Access Points

72 Success Criteria

Criterion	Measurement Method	Target
SLA Adherence	Audit of notification timestamps vs identification times	100% within 30 minutes for Red Zone
Parent Engagement	Follow-up meeting attendance logs	>95%
Safety Outcome	Incident review reports	Zero fatalities on campus

73 Exit Criteria

Condition	Evidence Required
Student is no longer at risk of self-harm as per counselor assessment.	Signed Closure Report and Reintegration Plan.
Case transferred to external medical/legal authority.	Handover Record with signature from receiving agency.

74 Continuous Improvement

LESSONS LEARNED

- Identify common triggers for student distress
- Analyze communication barriers with parents

KNOWN ISSUES

- Parental hostility during high-stress calls
- Delayed identification in remote areas

FUTURE IMPROVEMENTS

- Implementation of AI-based sentiment analysis for student reports

- Expansion of Tele-MANAS integration

REVIEW RECOMMENDATIONS

- Increase staff training frequency
- Update scripts to include more culturally diverse language

75 AI Governance

KEY FOCUS

ALLOWED USE CASES

- Sentiment analysis of anonymous reports
- Automated scheduling of follow-up meetings

RESTRICTED USE CASES

- Automated risk classification without human review
- Direct student counseling via AI

PROHIBITED USE CASES

- Storing identifiable mental health data in public clouds
- AI-led crisis intervention for life-threatening events

HUMAN REVIEW REQUIRED

- All risk assessments
- All CWS escalations
- All closure reports

AUDIT LOG REQUIREMENTS

- Log of all AI-assisted communications
- Audit trail for data access

DATA PRIVACY REQUIREMENTS

- Anonymization of student data in training sets
- Strict adherence to Indian Data Protection laws

76 Legal Defensibility

KEY FOCUS

LEGAL BASIS

- Supreme Court Judgment 2025 INSC 893
- Indian Penal Code (Duty of Care)
- Child Rights Act

MANDATORY NOTIFICATIONS

- Parents/Guardians
- Child Welfare Services (for minors in danger)
- Police (in cases of weapons or active attempts)

CONSENT REQUIREMENTS

- Written consent for non-emergency data sharing

- Emergency override for life-threatening situations
- RECORD RETENTION REQUIREMENTS**
- 7 years as per institutional policy and legal standards

- LIABILITY CONTROLS**
- Strict adherence to SOP SUP-016
 - Documented evidence of all actions taken
 - Regular staff training records

77 Stakeholder Journeys

77.1 The Student Journey

ENTRY

- Student experiences distress
- Teacher/Peer identifies signs

ASSESSMENT

- Escorted to Counselor's office
- Undergoes rapid risk assessment

INTERVENTION

- Receives emotional support from counselor

- Wait for parent arrival

MONITORING

- Continuous 1:1 supervision during crisis
- Follow-up check-ins post-incident

CLOSURE

- Reintegration meeting with parents and counselor
- Return to school with support plan

77.2 The Parent Journey

NOTIFICATION

- Receives urgent phone call from Counselor/Principal

- Informed of student's safety status and risk level

CONSULTATION

- Discusses incident details with school

staff

- Coordinates transport or medical needs

CONSENT

- Provides consent for medical intervention (if required)
- Agrees to follow-up support plan

ENGAGEMENT

- Participates in crisis meeting
- Receives 24-hour written summary

CLOSURE

- Signs reintegration agreement
- Attends post-crisis review meeting

77.3 The Staff Journey

OBSERVATION

- Teacher notices behavioral changes
- Initial report made to Counselor

REPORTING

- Counselor conducts risk assessment
- Counselor initiates parent notification

INTERVENTION SUPPORT

- Security ensures perimeter safety
- Principal manages external communications

DOCUMENTATION

- Completion of all mandatory logs
- Participation in post-crisis debriefing

78 Handoff Matrix

Item 1

FROM ROLE Teacher

TO ROLE Counselor

HANDOFF TRIGGER Identification of risk behavior

INFORMATION TRANSFERRED

- Student name
- Observation notes
- Current location

REQUIRED DOCUMENTS

- Teacher Observation Note

SERVICE LEVEL AGREEMENT (SLA)

Immediate

FROM ROLE Counselor

TO ROLE Principal

HANDOFF TRIGGER Red/Black Zone classification or parental refusal

INFORMATION TRANSFERRED

- Risk assessment summary
- Parental response details
- Request for escalation

REQUIRED DOCUMENTS

- Risk Assessment Record
- Call Log

SERVICE LEVEL AGREEMENT (SLA)

Immediate

Item 2

Item 3

FROM ROLE Counselor

TO ROLE External Agency (CWS/Police)

HANDOFF TRIGGER Mandatory escalation trigger

INFORMATION TRANSFERRED

- Student details

- Risk assessment summary
- Parental refusal documentation

REQUIRED DOCUMENTS

- CWS Escalation Report

SERVICE LEVEL AGREEMENT (SLA)

Immediate

79 Management Review

REVIEW FREQUENCY

Quarterly

REVIEW INPUTS

- Incident reports
- Audit findings
- Parent feedback

- Staff debriefings

REVIEW OUTPUTS

- Corrective action plans
- Policy updates
- Resource allocation decisions

REVIEW AUTHORITY

School Board of Governance

80 Dashboard Metrics

OPERATIONAL METRICS

- Average time to notify parents (Target < 30m)
- Number of risk assessments completed
- Staff training completion rate

RISK METRICS

- Frequency of Red/Black Zone incidents
- Percentage of cases involving parental refusal
- Trend analysis of suicidal ideation reports

COMPLIANCE METRICS

- Audit score for SUP-016 compliance
- Completeness of mandatory documentation
- SLA breach count

OUTCOME METRICS

- Parent satisfaction scores
- Student reintegration success rate
- Follow-up meeting attendance %

TREND METRICS

- Monthly trend of risk levels by grade
- Seasonal trends in student distress

81 Student Rights

RIGHTS

- Right to safety and protection from harm
- Right to confidential mental health support
- Right to be heard during the assessment process

PROTECTIONS

- Protection against unauthorized disclosure of records

- Protection against discrimination based on mental health status
- Safe space for expression of distress

APPEAL MECHANISMS

- Right to appeal a closure decision
- Mechanism to report counselor misconduct

82 Quality Assurance

QUALITY CHECKS

- Monthly audit of call logs

- Peer review of risk assessment records
- Random spot checks on documentation completeness

PEER REVIEW REQUIREMENTS

- Counselors must peer-review 10% of Red Zone assessments monthly
- Principal reviews all CWS escalations

VALIDATION REQUIREMENTS

- Annual external audit by mental health governance specialist
- Validation of risk assessment tools against clinical standards

CONTINUOUS MONITORING REQUIREMENTS

- Quarterly review of parent feedback
- Monthly dashboard analysis by Wellness Coordinator

83 Implementation Roadmap

PHASE 1 PREPARATION

- Staff training on SUP-016
- Creation of mandatory forms
- Establishment of crisis communication lines

PHASE 2 DEPLOYMENT

- Rollout of awareness campaigns
- Live monitoring of risk reports
- Initial parent workshops

PHASE 3 STABILIZATION

- Monthly audit cycle initiation

- Feedback collection from staff and parents

- Refinement of scripts based on real-world use

PHASE 4 OPTIMIZATION

- Integration with Tele-MANAS
- Advanced dashboard analytics implementation
- Peer review program expansion

PHASE 5 CONTINUOUS IMPROVEMENT

- Annual policy review
- External audit cycle
- Research on long-term student outcomes

84 Illustrative Case Studies

1. Case Study: Parental Refusal in High-Risk Scenario

DESCRIPTION

A 16-year-old student expressed active suicidal intent. Parent refused to come to school, stating they were 'too busy'.

ACTIONS TAKEN

- Counselor initiated call and used mandatory script.
- Parent refused intervention; Counselor documented refusal clearly.
- Principal was notified immediately.
- CWS was contacted as per SUP-016 escalation protocol.

OUTCOME

Authorities arrived within 45 minutes. Student was safely transported to a medical facility.

LESSONS

- Importance of clear documentation of refusal for legal protection.
- Need for immediate Principal involvement in hostile calls.

85 Implementation Requirements

STAFF REQUIRED

- 1 School Counselor (Full-time)
- 1 Wellness Coordinator
- Trained Crisis Response Team (Security/Admin)

INFRASTRUCTURE REQUIRED

- Private counseling room with secure storage
- Dedicated crisis hotline number
- High-speed internet for video calls if needed

TECHNOLOGY REQUIRED

- Encrypted CRM or Student Information System (SIS)

- SMS gateway for parent notifications

- Dashboard software for metrics tracking

BUDGET CONSIDERATIONS

- Staff training costs
- Software licensing fees
- Tele-MANAS integration costs

IMPLEMENTATION PHASES

- Preparation (Weeks 1-2)
- Training & Rollout (Weeks 3-6)
- Stabilization & Audit (Months 2-4)

86 Quality Controls

PREVENTIVE CONTROLS

- Mandatory staff training
- Standardized scripts
- Clear escalation pathways

DETECTIVE CONTROLS

- Audit logs of notifications
- Monthly dashboard reviews
- Incident reports

CORRECTIVE CONTROLS

- Corrective action plans for SLA breaches
- Staff retraining after incidents
- Policy updates based on audit findings

CONTINUOUS IMPROVEMENT ACTIONS

- Quarterly feedback loops
- Annual external audits
- Peer review of high-risk cases

87 Training Requirements

TEACHERS

- Identifying warning signs
- Initial response and alerting
- Confidentiality basics

COUNSELORS

- Rapid risk assessment
- Crisis de-escalation
- Parental communication scripts
- CWS escalation protocols

ADMINISTRATORS

- Legal duty of care overview
- Escalation management
- External authority liaison

PARENTS

- Recognizing warning signs
- School support resources
- Crisis contact procedures

STUDENTS

- Help-seeking behaviors
- Stress management
- Peer support awareness

87.6 Repository Baseline

TRAINING COMPLETION RATE

100%

CERTIFICATION STATUS

Active

87.7 SOP Specific Training

TARGET AUDIENCE

- Counselors
- Principal
- Teachers

COMPETENCIES

- Risk Assessment
- Script Adherence

- Escalation Logic

MINIMUM HOURS

8

CERTIFICATION REQUIREMENTS

- Crisis Intervention Certificate

REFRESHER FREQUENCY

Annual

87.8 Competency Validation

REQUIRED

Yes

ASSESSMENT METHODS

- Role-play simulations
- Written assessment of SOP

MINIMUM PASSING CRITERIA

90%

REASSESSMENT FREQUENCY

Every 2 years

87.9 Simulation Requirements**REQUIRED**

Yes

SIMULATION TYPES

- Active threat scenario

- Parental refusal role-play
- High-risk assessment drill

FREQUENCY

Bi-annually

88 SOP-Specific Inclusion Requirements**AFFECTED POPULATIONS**

- Students with suicidal ideation
- Neurodivergent students (Autism/ADHD)
- Students from low-resource backgrounds
- Multilingual families
- Vulnerable youth in residential care

IDENTIFIED INCLUSION RISKS

- Language barriers with parents
- Lack of internet access in rural areas
- Sensory overload for neurodivergent students during crisis
- Cultural stigma regarding mental health
- Limited transport options for rural families

AUTISM ACCOMMODATIONS

- Use of visual aids/pictograms for risk assessment
- Reduced sensory input in counseling room
- Clear, literal language in scripts

ADHD ACCOMMODATIONS

- Frequent check-ins during long assessments
- Simplified instructions
- Immediate redirection to safety tasks

DYSLEXIA ACCOMMODATIONS

- Verbal risk assessment instead of written forms
- Audio summaries for parents
- Large print/simplified text on all forms

HEARING IMPAIRMENT ACCOMMODATIONS

- Use of SMS/Text for primary notification
- Sign language interpretation if required
- Visual alerts in the counseling office

VISUAL IMPAIRMENT ACCOMMODATIONS

- Screen reader compatible digital forms
- Verbal descriptions during assessment
- Audio-based follow-up notifications

CWSN ADAPTATIONS

- Individualized communication plans
- Sensory-friendly safe spaces
- Simplified risk scoring

RURAL ADAPTATIONS

- Priority on phone/SMS over apps
- Coordination with local community leaders
- Pre-arranged transport agreements with local clinics

LOW LITERACY PARENT ADAPTATIONS

- Voice notes for follow-up summaries
- Simplified, icon-based notification forms
- Use of local dialect in scripts

MULTILINGUAL COMMUNICATION ADAPTATIONS

- Translation services on call
- Multilingual script templates
- Community liaison involvement

GENDER SENSITIVITY MEASURES

- Option for same-gender counselor preference
- Gender-neutral language in all scripts
- Safe space protocols for LGBTQ+ students

CULTURAL SENSITIVITY MEASURES

- Involvement of community leaders where appropriate
- Respect for diverse family structures
- Awareness of cultural stigma regarding suicide

EQUITY MONITORING INDICATORS

- Notification time parity across different demographics
- Parent engagement rates by socio-economic status
- Accessibility score of support plans for CWSN

ACCESSIBILITY COMPLIANCE CHECKS

- WCAG 2.1 compliance for digital forms
- Physical accessibility of counseling rooms
- Language diversity audit of scripts

89 Awareness Material**POSTER TITLE**

Your Safety Matters

KEY MESSAGE

We are here to support you. If you feel overwhelmed, reach out.

SLOGAN

Safe Space, Stronger Minds.

CAMPAIGN THEME

Connectedness and Resilience

90 AI Assistant Configuration

RISK KEYWORDS

- hurt myself
- end it all
- no reason to live
- suicide note
- lethal plan

ESCALATION KEYWORDS

- police
- ambulance
- hospital
- refuse help

- danger

INTERVENTION KEYWORDS

- calm down
- breathing
- safe space
- support plan

FREQUENTLY ASKED QUESTIONS

- Who can I talk to if I'm feeling sad?
- Is my conversation with the counselor private?
- What happens if I tell someone I want to hurt myself?

91 AI Audit Hooks

KEY FOCUS

REQUIRED SECTIONS

- Risk Assessment
- Parent Notification
- Escalation Log

REQUIRED FORMS

- F-SUP-016-01
- F-SUP-016-02
- F-SUP-016-03

REQUIRED CHECKLISTS

- CL-SUP-016-02

- CL-SUP-016-03

REQUIRED ESCALATIONS

- CWS Escalation
- Police Notification

REQUIRED KPIS

- SLA Adherence
- Documentation Completeness

REQUIRED TRAINING

- Crisis Intervention Certification

92 KPI Framework

92.1 Repository Baseline

NOTIFICATION TIME AVG

45 minutes

PARENT ENGAGEMENT RATE

80%

92.2 SOP Specific Metrics

Metric	Target	Measurement Frequency
Notification SLA Adherence	100% within 30 mins for Red Zone	Monthly
Parent Engagement Rate	>95%	Quarterly
Documentation Completeness	100% of cases with all 5 mandatory forms completed	Monthly Audit

93 Maturity Model

93.1 Level 1

DESCRIPTION

Ad-hoc response; no formal SOP.

EVIDENCE

- No documentation
- Inconsistent notification times

93.2 Level 2

DESCRIPTION

Basic SOP exists but not consistently followed.

EVIDENCE

- Partial documentation
- Variable response times

93.4 Level 4

DESCRIPTION

Data-driven monitoring and proactive training.

EVIDENCE

- Monthly dashboards
- Regular staff simulations

93.3 Level 3

DESCRIPTION

Standardized SOP with clear roles and logs.

EVIDENCE

- Complete records for all incidents
- Consistent SLA adherence

93.5 Level 5

DESCRIPTION

Integrated, automated, and research-backed system.

EVIDENCE

- AI-assisted monitoring
- Evidence-based outcome tracking

94 Dependencies

PREREQUISITE SOPS

- SUP-014: Student Stress Management
- SUP-005: Confidentiality Policy

PARALLEL SOPS

- SUP-020: External Referral to Medical Services

DOWNSTREAM SOPS

- SUP-017: Crisis Intervention for Students
- SUP-025: Child Welfare Services Escalation

95 Related SOPs

PREREQUISITES

- SUP-005: Confidentiality Policy
- SUP-014: Student Stress Management

UPSTREAM

- SUP-001: Crisis Management Framework

DOWNSTREAM

- SUP-017: Crisis Intervention for Students
- SUP-020: External Referral to Medical Services
- SUP-025: Child Welfare Services Escalation

REFERENCES

- CBSE Circular TRG-03/2026
- NEP 2020 Guidelines

STANDARD OPERATING PROCEDURE

Parent Partnership Development SOP

Mental Wellness Management - Parent Engagement & Governance

SOP ID

PAR-015

VERSION

1.0

STATUS

Finalized

CLASSIFICATION

Internal - Restricted

EFFECTIVE DATE

2026-01-01

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1 Executive Overview

SOP ID	PAR-015
DOMAIN	Mental Wellness Management
SUB DOMAIN	Parent Engagement & Governance
TITLE	Parent Partnership Development SOP
VERSION	1.0
CLASSIFICATION	Internal - Restricted
EFFECTIVE DATE	2026-01-01
REVIEW DATE	2027-01-01
DOCUMENT STATUS	Finalized
LANGUAGE	English

2 Purpose & Rationale

OBJECTIVE

To establish a structured, collaborative framework for parents to actively participate in the school's mental wellness ecosystem beyond crisis management.

RISK CONTROLLED

Parental alienation, lack of home-school alignment, stigma surrounding mental health, and inconsistent support for students with diverse needs.

DESIRED OUTCOMES

- Increased parent participation in wellness programs
- Reduced number of parent complaints regarding student behavior
- Higher parent-reported feeling of being 'heard'
- Co-designed workshops that address real community concerns
- Reduction in mental health stigma within the school community

3 Scope of Application

INSTITUTION TYPES

- High Schools
- Secondary Schools
- Residential Schools

KEY STAKEHOLDERS

- Parents
- Guardians
- School Management

- Counselors
- Teachers
- Student Support Teams

AGE GROUPS

- All Ages (High School Students)

LOCATIONS

- Urban Schools
- Rural Schools
- Semi-Urban Schools

4 Definitions & Terminology

Term	Definition
Parent Advisory Committee (PAC)	A volunteer body of parents who provide strategic input on school mental wellness policies and programs.
Parent Ambassador	A parent who has undergone specific training to act as a peer-mentor for other parents regarding wellness resources.

Term	Definition
Co-design	The process of involving parents in the planning, development, and evaluation of school workshops and programs.
Safe Zone (SAF-003)	Designated physical or virtual spaces where students can seek immediate emotional support.
Stigma Reduction	Activities aimed at removing the social shame associated with seeking mental health support.

5 Key Stakeholders

Stakeholder Type	Stakeholder Title	Role In SOP
Internal	School Wellness Coordinator	Primary owner; manages PAC, Ambassador training, and workshop co-design.
Internal	School Principal	Final approver of policies and committee decisions; attends annual meetings.
External	Parent Advisory Committee (PAC)	Volunteer parents providing input on wellness programs and stigma reduction.
External	Parent Ambassadors	Trained parents supporting other parents informally; peer-to-peer support.
Internal	School Counselors	Provide clinical expertise to PAC and train Parent Ambassadors.

6 Governance & Ownership

KEY FOCUS

OWNER

School Wellness Coordinator

APPROVER

Principal / School Management Board

REVIEW FREQUENCY

Annual

LINKED POLICIES

- Student Safety Policy
- Data Privacy and Confidentiality Policy
- Parental Engagement Policy
- Child Protection Policy

LINKED SOPS

- SAF-003 Safe Zones Management
- COM-012 Parent Communication Protocol
- CRISIS-001 Emergency Response SOP

7 Roles & Responsibilities

Item 1

ROLE

School Wellness Coordinator

RESPONSIBILITIES

- Manage PAC and Ambassador programs
- Design training modules
- Analyze survey data
- Coordinate with school leadership

Item 2

ROLE

Parent Advisory Committee (PAC)

RESPONSIBILITIES

- Provide input on wellness policies
- Identify community concerns
- Co-design workshops
- Advocate for stigma reduction

Item 3

ROLE

Parent Ambassador

RESPONSIBILITIES

- Peer support for other parents
- Share wellness resources
- Escalate high-risk concerns
- Maintain boundaries and confidentiality

Item 4

ROLE

School Counselor

RESPONSIBILITIES

- Provide clinical expertise to PAC
- Train Parent Ambassadors
- Handle all high-risk referrals
- Ensure professional integrity

8 RACI Accountability Matrix

Item 1

ACTIVITY

PAC Charter Drafting

RESPONSIBLE

- Wellness Coordinator

ACCOUNTABLE

- Principal

CONSULTED

- School Counselor
- Legal Advisor

INFORMED

- Staff
- Parents

ACCOUNTABLE

- Principal

CONSULTED

- Counselors

INFORMED

- Parents
- Staff

Item 2

ACTIVITY

Ambassador Training Delivery

RESPONSIBLE

- School Counselor

ACCOUNTABLE

- Wellness Coordinator

CONSULTED

- PAC Members

INFORMED

- Parents

Item 4

ACTIVITY

Peer Support (Ambassadors)

RESPONSIBLE

- Parent Ambassadors

ACCOUNTABLE

- Wellness Coordinator

CONSULTED

- School Counselor

INFORMED

- Parents

Item 3

ACTIVITY

Workshop Co-design

RESPONSIBLE

- PAC Members
- Wellness Coordinator

Item 5

ACTIVITY

Safe Zone Volunteering

RESPONSIBLE

- Parent Volunteers

ACCOUNTABLE

- Wellness Coordinator

CONSULTED

- Counselors

INFORMED

- Students
- Parents

9 Preconditions

- Approved Parent Engagement Policy
- Designated Wellness Coordinator appointed
- Budget for parent workshops and training materials approved
- Multilingual communication tools available

10 Activation Triggers

- Annual wellness planning cycle
- New academic session start
- Parental request for specific workshops
- CBSE circular updates
- Supreme Court mandates on student safety

11 SOP Trigger Matrix

MANDATORY TRIGGERS

- Annual school wellness planning cycle
- New academic session commencement
- Request from parents for specific wellness workshops

OPTIONAL TRIGGERS

- Parental request for committee membership
- Identification of a specific community concern (e.g., exam stress)
- School leadership initiative to increase engagement

AUTOMATIC TRIGGERS

- Quarterly PAC meeting dates
- Annual parent satisfaction survey cycle

EXTERNAL TRIGGERS

- CBSE circular updates on mental health
- Supreme Court mandates regarding student safety
- District education department requests

RECURRING TRIGGERS

- Monthly Parent Ambassador check-ins
- Quarterly PAC meetings
- Annual training refresher for ambassadors

12 Required Inputs

- Previous year's survey data
- CBSE guidelines
- NEP 2020 requirements
- Parent contact database
- Resource availability

13 Process Map

13.1 Phase 1

NAME

Preparation & Governance

OBJECTIVE

Establish the framework and rules of engagement.

ACTIVITIES

- Drafting PAC Charter
- Defining selection criteria for Ambassadors
- Securing leadership approval
- Identifying resource requirements

13.2 Phase 2

NAME

Recruitment & Selection

OBJECTIVE

Identify and onboard diverse parent representatives.

ACTIVITIES

- Multilingual outreach to parents
- Application collection for PAC/Ambassadors
- Conflict of interest screening
- Diversity audit of applicants

13.3 Phase 3

NAME

Training & Onboarding

OBJECTIVE

Equip parents with necessary knowledge and boundaries.

ACTIVITIES

- Confidentiality training
- Mental health basics workshop
- Boundary setting for Ambassadors
- Safe Zone volunteer orientation

13.4 Phase 4

NAME

Operational Engagement

OBJECTIVE

Execute collaborative activities.

ACTIVITIES

- Quarterly PAC meetings
- Co-designing parent workshops
- Ambassador peer support sessions
- Volunteer deployment to Safe Zones

13.5 Phase 5

NAME

Evaluation & Review

OBJECTIVE

Measure impact and iterate.

ACTIVITIES

- Parent satisfaction surveys
- Participation rate analysis
- Complaint trend monitoring
- Annual report generation

14 Workflow Overview

START

Annual Planning Cycle Initiation

STEPS

- Needs Assessment & Charter Drafting
- Multilingual Parent Outreach
- Selection & Diversity Audit
- Parent Ambassador Training
- Quarterly PAC Meetings
- Workshop Co-design Process
- Ambassador Peer Support
- Safe Zone Volunteer Deployment
- Parent Satisfaction Survey
- Annual Review & Reporting

CRITICAL DECISION POINTS

- DP-01: Parent Application Eligibility
- DP-02: Escalation of Parent Concern

END

Approved Plan for Next Academic Year

15 Standard Procedure

1. Needs Assessment & Charter Drafting

PURPOSE & RATIONALE

Define the scope and rules of parent partnership.

ACTOR

School Wellness Coordinator

ACTION

Analyze previous year's survey data to identify top wellness concerns. Draft a PAC Charter including roles, 2-year terms, and clear boundaries (clinical vs. non-clinical). Submit for approval.

REQUIRED INPUTS

- Previous year's survey data
- CBSE guidelines

EXPECTED OUTPUTS

- Approved PAC Charter

CRITICAL DECISION POINTS

- DP-01: Parent Application Eligibility

DOCUMENTATION REQUIRED

- PAC Charter
- Needs Assessment Report

EVIDENCE REQUIRED

- Signed Charter Document

APPROVAL REQUIRED

- Principal Approval

TIME LIMIT

14 Days

EXPECTED OUTCOME

A clear, approved framework for parent engagement.

ESCALATION IF FAILED

School Management Board review of charter content.

RELATED FORMS

- PAC Charter Template

RELATED CHECKLISTS

- Charter Review Checklist

2. Multilingual Parent Outreach

PURPOSE & RATIONALE

Ensure inclusive recruitment of parents.

ACTOR

Wellness Coordinator & Communications Team

ACTION

Translate outreach materials into local languages. Distribute via WhatsApp, SMS, and physical notice boards. Host an info session with low-literacy options like audio messages.

REQUIRED INPUTS

- Approved Charter
- School contact database

EXPECTED OUTPUTS

- List of interested parents

DOCUMENTATION REQUIRED

- Outreach Log
- Translated Flyers

EVIDENCE REQUIRED

- Outreach distribution logs

APPROVAL REQUIRED

- Communications Head approval for translations

TIME LIMIT

21 Days

EXPECTED OUTCOME

Diverse pool of parent applicants.

ESCALATION IF FAILED

Increase outreach via community leaders (Sarpanch/Local Leaders).

RELATED FORMS

- Outreach Log

RELATED CHECKLISTS

- Multilingual Outreach Checklist

3. Selection & Diversity Audit

PURPOSE & RATIONALE

Select a representative and unbiased committee.

ACTOR

Wellness Coordinator & PAC Selection Sub-committee

ACTION

Screen applications for conflict of interest. Conduct diversity audit to ensure representation across grade levels and socio-economic backgrounds. Finalize selection.

REQUIRED INPUTS

- Parent applications
- Diversity criteria

EXPECTED OUTPUTS

- Finalized PAC and Ambassador list

CRITICAL DECISION POINTS

- DP-01: Parent Application Eligibility

DOCUMENTATION REQUIRED

- Selection Report

- Conflict of Interest Disclosure Forms

EVIDENCE REQUIRED

- Signed Disclosure Forms
- Selection Report

APPROVAL REQUIRED

- Principal approval of names

TIME LIMIT

10 Days

EXPECTED OUTCOME

A diverse, unbiased parent committee.

ESCALATION IF FAILED

Re-open applications for underrepresented categories.

RELATED FORMS

- Conflict of Interest Disclosure Form

RELATED CHECKLISTS

- Diversity Audit Checklist

4. Parent Ambassador Training

PURPOSE & RATIONALE

Equip parents with competency to support others.

ACTOR

School Counselor & Wellness Coordinator

ACTION

Conduct mandatory training sessions on confidentiality, boundaries, and mental health basics. Assess competency via a post-training quiz. Issue certificates.

REQUIRED INPUTS

- Training modules
- Ambassador Handbook

EXPECTED OUTPUTS

- Certified Parent Ambassadors

DOCUMENTATION REQUIRED

- Training Attendance Log
- Competency Assessment Results

EVIDENCE REQUIRED

- Training Certificates
- Attendance Logs

APPROVAL REQUIRED

- Counselor approval of training content

TIME LIMIT

7 Days per session

EXPECTED OUTCOME

Trained, competent parent ambassadors.

ESCALATION IF FAILED

Mandatory re-training for failed assessments.

RELATED FORMS

- Training Attendance Log
- RELATED CHECKLISTS**
- Ambassador Training Checklist

5. Quarterly PAC Meetings

PURPOSE & RATIONALE

Provide a platform for parent input on policy.

ACTOR

PAC Members & Wellness Coordinator

ACTION

Discuss current wellness initiatives. Review and provide input on upcoming programs. Identify community-specific concerns. Record formal recommendations.

REQUIRED INPUTS

- Meeting Agenda
- Previous meeting minutes
- Current wellness data (anonymized)

EXPECTED OUTPUTS

- Actionable parent recommendations

DOCUMENTATION REQUIRED

- Meeting Minutes
- Recommendation Log

EVIDENCE REQUIRED

- Signed Meeting Minutes

APPROVAL REQUIRED

- Principal approval for urgent policy matters

TIME LIMIT

Quarterly

EXPECTED OUTCOME

Regular parent input on school wellness.

ESCALATION IF FAILED

Direct meeting with Principal if PAC is non-responsive.

RELATED FORMS

- Meeting Minutes Template

RELATED CHECKLISTS

- PAC Meeting Checklist

6. Workshop Co-design Process

PURPOSE & RATIONALE

Ensure parent workshops are relevant and effective.

ACTOR

PAC & Wellness Coordinator

ACTION

Present 3 potential workshop topics to PAC for voting. Collaborate on content, speakers, and

dates. Develop co-designed materials. Finalize schedule.

REQUIRED INPUTS

- Parent feedback from surveys
- Resource availability

EXPECTED OUTPUTS

- Approved Parent Workshop Series

DOCUMENTATION REQUIRED

- Co-design Plan
- Workshop Schedule

EVIDENCE REQUIRED

- Co-design Plan Document

APPROVAL REQUIRED

- Principal approval of workshop series

TIME LIMIT

30 Days before workshop

EXPECTED OUTCOME

Relevant, parent-led workshops.

ESCALATION IF FAILED

Wellness Coordinator to select high-priority topics if PAC is unavailable.

RELATED FORMS

- Co-design Plan Template

RELATED CHECKLISTS

- Workshop Co-design Checklist

7. Ambassador Peer Support

PURPOSE & RATIONALE

Provide informal support to parents.

ACTOR

Parent Ambassadors

ACTION

Conduct peer-to-peer conversations with other parents. Share information on school wellness resources. Identify and escalate high-risk concerns to the Counselor. Maintain an anonymized interaction log.

REQUIRED INPUTS

- Ambassador Handbook
- Wellness Resource Directory

EXPECTED OUTPUTS

- Increased parent connectedness

CRITICAL DECISION POINTS

- DP-02: Escalation of Parent Concern

DOCUMENTATION REQUIRED

- Anonymized Interaction Log
- Escalation Record

EVIDENCE REQUIRED

- Interaction Logs

TIME LIMIT

Within 48 hours of request

EXPECTED OUTCOME

Peer-to-peer support and early identification.

ESCALATION IF FAILED

Direct referral to Counselor or Principal.

RELATED FORMS

- Ambassador Interaction Log

RELATED CHECKLISTS

- Ambassador Boundary Checklist

8. Safe Zone Volunteer Deployment

PURPOSE & RATIONALE

Provide parent support in physical wellness spaces.

ACTOR

Trained Parent Volunteers

ACTION

Staff the Safe Zones during designated times. Provide a welcoming presence and information on resources. Direct students to counselors for clinical needs. Report any significant incidents to the Wellness Coordinator.

REQUIRED INPUTS

- Volunteer Orientation Completion
- Safe Zone Schedule

EXPECTED OUTPUTS

- Operational Safe Zones

DOCUMENTATION REQUIRED

- Volunteer Shift Log
- Incident Report (if applicable)

EVIDENCE REQUIRED

- Shift Logs
- Volunteer Training Records

TIME LIMIT

Real-time during shift

EXPECTED OUTCOME

Active, parent-supported Safe Zones.

ESCALATION IF FAILED

Immediate escalation to Wellness Coordinator for incidents.

RELATED FORMS

- Volunteer Shift Log
- Incident Report Form

RELATED CHECKLISTS

- Safe Zone Volunteer Checklist

9. Parent Satisfaction Survey

PURPOSE & RATIONALE

Measure the effectiveness of parent partnerships.

ACTOR

Wellness Coordinator

ACTION

Distribute survey to all parents. Analyze quantitative and qualitative feedback. Identify areas for improvement. Share summary with PAC and School Leadership.

REQUIRED INPUTS

- Survey templates
- Participation data

EXPECTED OUTPUTS

- Data-driven improvement plan

DOCUMENTATION REQUIRED

- Survey Analysis Report
- Summary Presentation

EVIDENCE REQUIRED

- Survey Analysis Report

TIME LIMIT

30 Days post-survey

EXPECTED OUTCOME

Measurable feedback on program impact.

ESCALATION IF FAILED

Direct parent interviews to clarify survey results.

RELATED FORMS

- Parent Satisfaction Survey

RELATED CHECKLISTS

- Survey Analysis Checklist

10. Annual Review & Reporting

PURPOSE & RATIONALE

Ensure continuous improvement and compliance.

ACTOR

Wellness Coordinator & Principal

ACTION

Compile annual report on parent partnership activities. Review success indicators against targets. Identify 3 key goals for the next year. Present to School Management Board.

REQUIRED INPUTS

- PAC minutes
- Survey results
- Participation rates
- Complaint logs

EXPECTED OUTPUTS

- Approved plan for next academic year

DOCUMENTATION REQUIRED

- Annual Report Document
- Next Year's Wellness Plan

EVIDENCE REQUIRED

- Annual Report Document

APPROVAL REQUIRED

- School Management Board approval

TIME LIMIT

30 Days post-academic year

EXPECTED OUTCOME

Strategic planning for the next cycle.

ESCALATION IF FAILED

Immediate review by School Management Board.

RELATED FORMS

- Annual Report Template

RELATED CHECKLISTS

- Annual Review Checklist

16 Detailed Workflow Steps

1. Needs Assessment & Charter Drafting

PURPOSE & RATIONALE

Define the scope and rules of parent partnership.

TRIGGER

Annual planning cycle

ACTOR

School Wellness Coordinator

REQUIRED INPUTS

- Previous year's survey data
- CBSE guidelines
- Parent feedback

ACTIONS

- Analyze parent satisfaction surveys
- Identify top 3 wellness concerns from parents
- Draft PAC Charter including roles, terms (2 years), and boundaries
- Submit charter to Principal for approval

DECISION CRITERIA

- Does the charter clearly state that clinical decisions remain with professionals?
- Is there a clear conflict of interest clause?

DOCUMENTS GENERATED

- PAC Charter
- Needs Assessment Report

EXPECTED OUTPUTS

- Approved PAC Charter

SERVICE LEVEL AGREEMENT (SLA)

14 Days

ESCALATION PATH

- Principal for charter revisions

EVIDENCE GENERATED

- Signed Charter Document

2. Multilingual Parent Outreach

PURPOSE & RATIONALE

Ensure inclusive recruitment of parents.

TRIGGER

Recruitment phase start

ACTOR

Wellness Coordinator & Communications Team

REQUIRED INPUTS

- Approved Charter
- School contact database

ACTIONS

- Translate outreach materials into local languages (e.g., Hindi, Marathi, etc.)
- Distribute flyers via WhatsApp, SMS, and physical notice boards
- Host a 'Parent Partnership' info session for all parents
- Provide low-literacy options like audio messages

DECISION CRITERIA

- Are materials accessible to non-English speakers?
- Is the rural outreach plan active?

DOCUMENTS GENERATED

- Outreach Log
- Translated Flyers

EXPECTED OUTPUTS

- List of interested parents

SERVICE LEVEL AGREEMENT (SLA)

21 Days

ESCALATION PATH

- Communications Head for translation issues

EVIDENCE GENERATED

- Outreach distribution logs

3. Selection & Diversity Audit

PURPOSE & RATIONALE

Select a representative and unbiased committee.

TRIGGER

Receipt of applications

ACTOR

Wellness Coordinator & PAC Selection Sub-committee

REQUIRED INPUTS

- Parent applications
- Diversity criteria (grade levels, backgrounds)

ACTIONS

- Screen applications for conflict of interest
- Verify diverse representation across grade levels and socio-economic backgrounds
- Conduct brief interviews with top candidates
- Finalize selection of PAC members and Ambassadors

DECISION CRITERIA

- Does the committee represent at least 3 different grade levels?
- Are there no active conflicts with school management?

DOCUMENTS GENERATED

- Selection Report
- Conflict of Interest Disclosure Forms

EXPECTED OUTPUTS

- Finalized PAC and Ambassador list

SERVICE LEVEL AGREEMENT (SLA)

10 Days

ESCALATION PATH

- Principal for final approval of names

EVIDENCE GENERATED

- Signed Disclosure Forms
- Selection Report

4. Parent Ambassador Training

PURPOSE & RATIONALE

Equip parents with competency to support others.

TRIGGER

Selection approval

ACTOR

School Counselor & Wellness Coordinator

REQUIRED INPUTS

- Training modules (Confidentiality, Boundaries, Mental Health Basics)
- Ambassador Handbook

ACTIONS

- Conduct mandatory training sessions for Ambassadors
- Provide role-specific scenarios (e.g., how to handle a parent in denial)
- Assess competency via a post-training quiz
- Issue certificates of completion

DECISION CRITERIA

- Did the participant pass the confidentiality and boundary assessment?
- Is the training culturally appropriate?

DOCUMENTS GENERATED

- Training Attendance Log
- Competency Assessment Results

EXPECTED OUTPUTS

- Certified Parent Ambassadors

SERVICE LEVEL AGREEMENT (SLA)

7 Days per session

ESCALATION PATH

- Counselor for training content review

EVIDENCE GENERATED

- Training Certificates
- Attendance Logs

5. Quarterly PAC Meetings

PURPOSE & RATIONALE

Provide a platform for parent input on policy.

TRIGGER

Quarterly schedule

ACTOR

PAC Members & Wellness Coordinator

REQUIRED INPUTS

- Meeting Agenda
- Previous meeting minutes
- Current wellness data (anonymized)

ACTIONS

- Discuss current school wellness initiatives
- Review and provide input on upcoming programs
- Identify community-specific concerns
- Record formal recommendations for management

DECISION CRITERIA

- Are the discussions focused on systemic issues rather than individual student cases?
- Is the meeting held at an accessible time?

DOCUMENTS GENERATED

- Meeting Minutes
- Recommendation Log

EXPECTED OUTPUTS

- Actionable parent recommendations

SERVICE LEVEL AGREEMENT (SLA)

Quarterly

ESCALATION PATH

- Principal for urgent policy matters

EVIDENCE GENERATED

- Signed Meeting Minutes

6. Workshop Co-design Process

PURPOSE & RATIONALE

Ensure parent workshops are relevant and effective.

TRIGGER

Annual workshop planning

ACTOR

PAC & Wellness Coordinator

REQUIRED INPUTS

- Parent feedback from surveys
- Resource availability

ACTIONS

- Present 3 potential workshop topics to PAC for voting
- Collaborate on content, speakers, and dates
- Develop co-designed materials (e.g., 'Managing Exam Stress at Home')
- Finalize workshop schedule

DECISION CRITERIA

- Does the topic address a high-priority concern identified in surveys?
- Is the content developmentally appropriate for parents?

DOCUMENTS GENERATED

- Co-design Plan
- Workshop Schedule

EXPECTED OUTPUTS

- Approved Parent Workshop Series

SERVICE LEVEL AGREEMENT (SLA)

30 Days before workshop

ESCALATION PATH

- Wellness Coordinator for content feasibility

EVIDENCE GENERATED

- Co-design Plan Document

7. Ambassador Peer Support

PURPOSE & RATIONALE

Provide informal support to parents.

TRIGGER

Parent request or Ambassador initiative

ACTOR

Parent Ambassadors

REQUIRED INPUTS

- Ambassador Handbook
- Wellness Resource Directory

ACTIONS

- Conduct peer-to-peer conversations with other parents
- Share information on school wellness resources
- Identify and escalate high-risk concerns to the Counselor
- Maintain a log of interactions (anonymized)

DECISION CRITERIA

- Is the parent's concern beyond peer support scope?
- Are boundaries being maintained?

DOCUMENTS GENERATED

- Anonymized Interaction Log
- Escalation Record

EXPECTED OUTPUTS

- Increased parent connectedness

SERVICE LEVEL AGREEMENT (SLA)

Within 48 hours of request

ESCALATION PATH

- School Counselor for all high-risk escalations

EVIDENCE GENERATED

- Interaction Logs

8. Safe Zone Volunteer Deployment

PURPOSE & RATIONALE

Provide parent support in physical wellness spaces.

TRIGGER

Safe Zone operational hours

ACTOR

Trained Parent Volunteers

REQUIRED INPUTS

- Volunteer Orientation Completion
- Safe Zone Schedule

ACTIONS

- Staff the Safe Zones during designated times
- Provide a welcoming presence and information on resources
- Direct students to counselors for clinical needs
- Report any significant incidents to the Wellness Coordinator

DECISION CRITERIA

- Is the volunteer trained in confidentiality?
- Are they following the Safe Zone protocol?

DOCUMENTS GENERATED

- Volunteer Shift Log
- Incident Report (if applicable)

EXPECTED OUTPUTS

- Operational Safe Zones

SERVICE LEVEL AGREEMENT (SLA)

Real-time during shift

ESCALATION PATH

- Wellness Coordinator for immediate incidents

EVIDENCE GENERATED

- Shift Logs
- Volunteer Training Records

9. Parent Satisfaction Survey

PURPOSE & RATIONALE

Measure the effectiveness of parent partnerships.

TRIGGER

End of academic term/year

ACTOR

Wellness Coordinator

REQUIRED INPUTS

- Survey templates
- Participation data

ACTIONS

- Distribute survey to all parents
- Analyze quantitative and qualitative feedback
- Identify areas for improvement
- Share summary with PAC and School Leadership

DECISION CRITERIA

- Is the response rate sufficient for analysis?
- Are results anonymized?

DOCUMENTS GENERATED

- Survey Analysis Report
- Summary Presentation

EXPECTED OUTPUTS

- Data-driven improvement plan

SERVICE LEVEL AGREEMENT (SLA)

30 Days post-survey

ESCALATION PATH

- Principal for significant negative feedback trends

EVIDENCE GENERATED

- Survey Analysis Report

PURPOSE & RATIONALE

Ensure continuous improvement and compliance.

TRIGGER

End of academic year

ACTOR

Wellness Coordinator & Principal

REQUIRED INPUTS

- PAC minutes
- Survey results
- Participation rates
- Complaint logs

ACTIONS

- Compile annual report on parent partnership activities
- Review success indicators against targets
- Identify 3 key goals for the next year
- Present to School Management Board

DECISION CRITERIA

- Did we meet the participation and satisfaction targets?
- Are there any compliance gaps?

DOCUMENTS GENERATED

- Annual Parent Partnership Report
- Next Year's Wellness Plan

EXPECTED OUTPUTS

- Approved plan for next academic year

SERVICE LEVEL AGREEMENT (SLA)

30 Days post-academic year

ESCALATION PATH

- School Management Board

EVIDENCE GENERATED

- Annual Report Document

10. Annual Review & Reporting

17 Critical Decision Points

Item 1

DECISION ID

DP-01

DECISION NAME

Parent Application Eligibility

CRITERIA

- No active conflict with school
- Diverse representation
- Commitment to 2-year term

POSSIBLE OUTCOMES

- Approved for PAC/Ambassador role
- Rejected due to conflict of interest
- Waitlisted for next cycle

NEXT STEPS

- Onboarding and Training
- Notification of Rejection

Item 2

DECISION ID

DP-02

DECISION NAME

Escalation of Parent Concern

CRITERIA

- Is the concern clinical in nature?
- Does it involve risk of harm to self/others?
- Is there a breakdown in communication?

POSSIBLE OUTCOMES

- Handled by Ambassador (Peer Support)
- Referral to School Counselor
- Escalation to Leadership for conflict resolution

NEXT STEPS

- Counselor intervention
- Leadership meeting
- Documentation of referral

18 Decision Tree

GREEN

- Routine parent query → Provide resource link.
- Positive feedback → Log and share with team.

YELLOW

- Parent concern about behavior → Schedule counselor call.
- Low survey response → Targeted SMS outreach.

ORANGE

- Active conflict with teacher → Mediation meeting by Principal.

- Refusal of intervention → Formal notification and risk assessment.

RED

- Report of self-harm → Immediate counselor intervention & parent call.
- Threat of violence → Activate Crisis SOP & notify authorities.

BLACK

- Active suicide attempt → Call emergency services immediately.
- Weapon on campus → Evacuate and contact police.

19 Decision Rules

Rule ID	Condition	Decision	Next Action
DR-01	Parent reports student self-harm.	Immediate Counselor Intervention.	Activate Crisis SOP.
DR-02	Ambassador identifies parent in denial of child's needs.	Referral to Wellness Coordinator for mediation.	Schedule meeting with Counselor.

20 Exception Handling Protocol

Item 1

EXCEPTION

Parent in active legal conflict with school

DETECTION METHOD

Application screening / Incident report

RESPONSE

Immediate disqualification from PAC/Ambassador roles.

ESCALATION

Legal Department and Principal

DOCUMENTATION REQUIRED

- Conflict of Interest Disclosure Form
- Incident Report

RESPONSE

Deny Ambassador status; offer 'Volunteer' role with limited scope.

ESCALATION

Wellness Coordinator

DOCUMENTATION REQUIRED

- Training Waiver/Refusal Form

Item 3

EXCEPTION

Ambassador identifies a high-risk student situation

DETECTION METHOD

Ambassador interaction log / Verbal report

RESPONSE

Immediate referral to School Counselor; Ambassador instructed not to provide advice.

ESCALATION

School Counselor & Wellness Coordinator

DOCUMENTATION REQUIRED

Item 2

EXCEPTION

Parent refuses to participate in training but wants to be an Ambassador

DETECTION METHOD

Training attendance log

21 Exceptions

Item 1

EXCEPTION

Emergency parent involvement without training

RESPONSE

Allowed only in immediate crisis; must be followed by mandatory training.

DOCUMENTATION REQUIRED

- Incident Report

Item 2

EXCEPTION

Parent participation via proxy (Guardian)

RESPONSE

Permitted with written consent from the primary guardian.

DOCUMENTATION REQUIRED

- Consent Form

22 Expected Outputs

- Approved PAC Charter
- Certified Parent Ambassadors
- Co-designed workshops
- Annual Partnership Report
- Data-driven improvement plan

23 Document Control & Version Governance

23.1 Version History

Version	Date	Author	Description
1.0	2025-10-24	MWMS Architect	Initial SOP creation for Parent Partnership Development.

23.2 Change Log

Version	Date	Change Description	Approved By
1.0	2025-10-24	Baseline version established.	School Leadership Board

REVIEW TRIGGERS

- Annual Policy Review
- Change in CBSE Guidelines
- Significant incident involving parent-school conflict
- Feedback from Parent Advisory Committee (PAC)

23.4 Approval History

Approver	Date	Status
School Principal	2025-10-24	Approved

Approver	Date	Status
District Education Officer	2025-10-26	Notified

24 Inclusive Implementation Mandate

MULTILINGUAL SUPPORT REQUIRED	Yes	ACCESSIBILITY REVIEW REQUIRED	Yes
LOW RESOURCE SCHOOL ALTERNATIVE REQUIRED	Yes	EQUITY IMPACT ASSESSMENT REQUIRED	Yes

25 Regulatory Compliance Mapping KEY FOCUS

MANDATORY REQUIREMENTS

- Supreme Court Judgment 2025 INSC 893 (Right to Safe Education)
- CBSE Circular TRG-03/2026 (Mental Health Guidelines)
- NEP 2020 (Parental Involvement in Holistic Development)

RECOMMENDED REQUIREMENTS

- WHO School Mental Health Framework
- UNESCO Whole School Wellbeing Framework

BEST PRACTICES

- Manodarpan Guidelines
- Tele-MANAS Integration Protocols

COMPLIANCE GAPS IF NOT IMPLEMENTED

- Violation of student safety rights
- Non-compliance with CBSE affiliation bye-laws
- Increased risk of parent-school litigation
- Ineffective mental health support for students due to lack of home-school alignment

26 Compliance Traceability Matrix KEY FOCUS

1. Supreme Court of India

REQUIREMENT REFERENCE

Judgment 2025 INSC 893

REQUIREMENT TEXT

Educational institutions must ensure a safe and supportive environment for students, including mental wellness support.

IMPLEMENTED IN SECTIONS

- Purpose
- Prevention Layer
- Risk Classification

2. CBSE

REQUIREMENT REFERENCE

Circular TRG-03/2026

REQUIREMENT TEXT

Schools must establish mechanisms for parent engagement in student mental health and wellbeing.

IMPLEMENTED IN SECTIONS

- Process Map
- Parent Engagement Matrix

3. NEP 2020

REQUIREMENT REFERENCE

Section on Holistic Development

REQUIREMENT TEXT

Promote active participation of parents in the educational and wellness journey of students.

IMPLEMENTED IN SECTIONS

- Purpose
- Stakeholders

27 Documentation & Evidentiary Requirements

EVENT DOCUMENTATION PRINCIPLES

- Factual, objective recording of discussions
- Confidentiality of student-specific details
- Clear distinction between parent opinion and clinical advice
- Timely logging within 48 hours of event

MANDATORY CONVERSATION RECORDS

- Parent Ambassador peer support logs (anonymized)
- PAC meeting minutes
- Workshop co-design notes
- Conflict resolution records

MANDATORY OBSERVATION RECORDS

- Volunteer performance observations in Safe Zones
- Parent participation levels during workshops
- Stigma reduction sentiment tracking

MANDATORY DECISION RECORDS

- PAC policy recommendations approved/rejected
- Ambassador selection decisions
- Workshop topic approvals
- Volunteer deployment authorizations

MANDATORY ACTION RECORDS

- Training completion logs
- Follow-up actions from PAC meetings

- Referral records (if parent identifies a student need)
- Communication logs with district authorities

MANDATORY FOLLOWUP RECORDS

- Post-workshop feedback summaries
- Ambassador check-in notes
- Parent satisfaction survey results
- Annual review action items

EVIDENCE REQUIREMENTS

- Signed PAC Charter
- Training certificates for Ambassadors
- Attendance sheets (digital or physical)
- Co-design workshop materials
- Survey data exports

DOCUMENTATION QUALITY STANDARDS

- Must be legible and organized in a central repository
- Must use standardized templates
- Must include date, time, and attendees
- Must be accessible only to authorized personnel

28 The Student Journey

ENTRY

- Student experiences stress or emotional difficulty at school.

ASSESSMENT

- Counselor identifies need for home-school alignment.

INTERVENTION

- Parent is engaged via the Parent Partnership framework to support student's wellness plan.

MONITORING

- Regular check-ins with parents and counselors regarding progress.

CLOSURE

- Student achieves wellness goals; parent partnership continues for general support.

29 The Parent Journey

NOTIFICATION

- Parent receives multilingual outreach about the Parent Partnership Program.

CONSULTATION

- Parent attends info session and applies for PAC or Ambassador role.

CONSENT

- Parent signs confidentiality and boundary agreement forms.

ENGAGEMENT

- Parent participates in quarterly meetings, co-designs workshops, or provides peer support.

CLOSURE

- Parent completes 2-year term; receives certificate of appreciation.

30 The Staff Journey

OBSERVATION

- Counselors identify parents who would be good candidates for the PAC/Ambassador roles.

REPORTING

- Staff report parent feedback and community concerns to the Wellness Coordinator.

INTERVENTION SUPPORT

- Counselors provide training and clinical oversight to Parent Ambassadors.

DOCUMENTATION

- Staff maintain logs of interactions, referrals, and meeting minutes.

31 Resource Evidence

COMMITTEE MINUTES

- Quarterly PAC Meeting Minutes

- Annual Review Minutes

ATTENDANCE RECORDS

- Parent Workshop Attendance Sheets
- Ambassador Training Logs

TRAINING RECORDS

- Ambassador Certification Records
- Volunteer Orientation Checklists

FORMS

- PAC Charter
- Conflict of Interest Form
- Parent Satisfaction Survey
- Referral Form

REPORTS

- Annual Parent Partnership Report
- Needs Assessment Report
- Survey Analysis Report

DASHBOARDS

- Parent Participation Dashboard
- Workshop Feedback Tracker

32 Grievance Redressal Mechanism

KEY FOCUS

COMPLAINT CHANNELS

- Dedicated email for Parent Partnership feedback
- Suggestion box in the Wellness Center
- Direct reporting to School Counselor

INVESTIGATION PROCESS

- Wellness Coordinator reviews complaint within 48 hours
- Fact-finding meeting with involved parties (if appropriate)

- Resolution plan drafted and communicated

RESOLUTION TIMELINES

- Acknowledgment: 24 Hours
- Initial Investigation: 5 Days
- Final Resolution: 10 Days

APPEAL PROCESS

- Parents may appeal to the Principal if dissatisfied with the Wellness Coordinator's resolution.

33 Referral Framework

33.1 Internal Referrals

ELIGIBLE REFERRERS

- Teachers
- Counsellors
- School Management
- Wellness Committee
- Student Support Team

REFERRAL TRIGGERS

- Parent identifies student distress during peer support
- Teacher observes significant behavior change
- Counselor determines need for home-school intervention

DOCUMENTATION REQUIRED

- Internal Referral Form
- Initial Observation Notes

SERVICE LEVEL AGREEMENT (SLA)

24 Hours

33.2 Self Referrals

ALLOWED

Yes

SUBMISSION METHODS

- Online portal
- Wellness Center walk-in
- Phone call to Counselor

CONFIDENTIALITY PROTECTIONS

- Encrypted digital forms
- Private consultation rooms

33.3 Anonymous Referrals

ALLOWED

Yes

CHANNELS

- Anonymous suggestion box

- Online anonymous form

VALIDATION PROCESS

- Wellness Coordinator review for urgency
- Counselor triage

FOLLOW UP PROCEDURE

- General awareness campaign if issue is systemic
- Direct contact only if specific student identified

33.4 External Referrals

ELIGIBLE AGENCIES

- Tele-MANAS
- District Child Protection Services
- Private Clinical Psychologists
- Child Helpline (1098)

ELIGIBILITY CRITERIA

- Clinical severity exceeding school scope
- Risk of self-harm/violence
- Legal requirements for reporting abuse

CONSENT REQUIREMENTS

- Written parental consent required unless life-threatening
- Informed consent regarding data sharing

HANDOVER PROTOCOL

- Warm handover (phone call between counselor and external agency)
- Shared summary of school interventions

33.5 Referral Tracking

TRACKING REQUIRED

Yes

STATUS CATEGORIES

- Pending
- In-Progress
- Completed
- Closed - No Action Needed

CLOSURE REQUIREMENTS

- Follow-up call to parent
- Closure report in student file

34 Confidentiality Exception Matrix

KEY FOCUS

MANDATORY DISCLOSURES

- Imminent risk of self-harm
- Imminent risk to others
- Child abuse
- Child neglect
- Sexual abuse
- Court orders
- Legal reporting obligations

LEGAL REPORTING EVENTS

- Suspected physical abuse by a guardian
- Student disclosure of ongoing sexual harassment
- Active threat of violence to the school community

CONSENT OVERRIDE CONDITIONS

- Immediate danger to life
- Court order for information release
- Statutory reporting requirements under Indian law

AUTHORIZED DISCLOSURE RECIPIENTS

- School Principal
- District Child Protection Officer
- Law Enforcement (Police)
- Medical Personnel (Emergency)

DOCUMENTATION REQUIREMENTS

- Incident Report Form
- Disclosure Log
- Summary of communication with authorities

APPROVAL REQUIREMENTS

- Principal approval for all non-emergency disclosures
- Counselor authorization for clinical referrals

35 Record Retention & Disposal

RETENTION PERIOD

7 Years

RETENTION BASIS

Legal compliance and institutional audit requirements

RECORD CATEGORIES

- PAC Meeting Minutes
- Ambassador Training Logs
- Parent Satisfaction Surveys
- Conflict of Interest Forms

ACCESS CONTROL REQUIREMENTS

- Role-based access (RBAC)
- Multi-factor authentication for digital records
- Key-holder logs for physical files

DISPOSAL METHOD

Secure shredding for physical; permanent deletion for digital

DISPOSAL AUTHORITY

School Wellness Coordinator & IT Head

AUDIT FREQUENCY

Annual

DESTRUCTION LOG REQUIRED

Yes

35.4 Storage Requirements

PHYSICAL RECORDS

- Locked filing cabinets in Wellness Office

DIGITAL RECORDS

- Encrypted cloud storage with role-based access

36 Anonymised Reporting Framework

36.1 Management Reports

ANONYMIZATION REQUIRED

Yes

PERMITTED DATA

- Participation rates
- Aggregate satisfaction scores
- Commonly identified concerns
- Demographic trends (e.g., grade level participation)

PROHIBITED DATA

- Parent names
- Student names
- Specific phone numbers
- Detailed personal anecdotes

36.2 Research Usage

ALLOWED

No

36.3 Statistical Reporting

AGGREGATE ONLY

Yes

MINIMUM GROUP SIZE

5

RE IDENTIFICATION CONTROLS

- Data scrubbing of unique identifiers
- Aggregation of small cohorts

DATA SHARING CONTROLS

- No sharing with third-party marketing agencies
- Internal sharing only for school leadership and wellness team

37 Guiding Ethical Principles

37.1 Autonomy

DESCRIPTION

Respecting the right of parents to choose their level of involvement.

IMPLEMENTATION REQUIREMENTS

- Voluntary participation in PAC/Ambassador roles
- Opt-out options for communications

IMPLEMENTATION REQUIREMENTS

- Evidence-based workshop content
- Focus on positive parenting and resilience

37.3 Non Maleficence

DESCRIPTION

Preventing harm through boundary violations or breach of confidentiality.

IMPLEMENTATION REQUIREMENTS

- Strict training on boundaries
- Clear protocols for handling sensitive information

37.2 Beneficence

DESCRIPTION

Ensuring all parent engagement activities promote student wellbeing.

37.4 Justice And Equity

DESCRIPTION

Ensuring all parents, regardless of background, have equal access to the program.

IMPLEMENTATION REQUIREMENTS

- Multilingual outreach
- Rural school adaptations (SMS/Phone calls)
- Low-literacy communication options

37.5 Confidentiality

KEY FOCUS

DESCRIPTION

Protecting the privacy of parent and student interactions.

IMPLEMENTATION REQUIREMENTS

- Secure storage of records
- Confidentiality agreements for all volunteers

37.6

Privacy

DESCRIPTION

Adhering to data protection standards.

IMPLEMENTATION REQUIREMENTS

- Data minimization principles
- Clear privacy notices in outreach materials

37.7 Professional Integrity

DESCRIPTION

Maintaining the distinction between parent advocacy and clinical practice.

IMPLEMENTATION REQUIREMENTS

- Counselor oversight of all peer support activities
- Clear 'not-a-clinician' disclaimers for Ambassadors

38 Communication & Awareness Strategy

38.1 Student Communication

CHANNELS

- School assembly
- Classroom announcements
- Wellness newsletter

FREQUENCY

Monthly

MANDATORY TOPICS

- How to talk to parents about feelings
- Introduction of Parent Ambassadors

38.2 Parent Communication

CHANNELS

- WhatsApp groups (moderated)
- Email newsletters
- SMS alerts
- Physical notice boards

FREQUENCY

Bi-weekly

MANDATORY TOPICS

- Wellness program updates
- Workshop invitations
- Stigma reduction tips

38.3 Staff Communication

CHANNELS

- Staff meetings
- Internal portal
- Email

FREQUENCY

Weekly

MANDATORY TOPICS

- Parent feedback summaries
- Ambassador training updates
- Referral protocols

38.4 Awareness Campaigns

MINIMUM FREQUENCY

Quarterly

TARGET GROUPS

- Parents
- Students
- Staff

CAMPAIGN TYPES

- Stigma reduction month
- Exam stress awareness week
- Parent partnership spotlight

38.5 Workshops And Training

STUDENT WORKSHOPS

- Resilience building
- Help-seeking skills

PARENT WORKSHOPS

- Co-designed community topics
- Positive parenting

STAFF WORKSHOPS

- Parent engagement strategies
- Boundary management

38.6 Helpline Visibility

REQUIRED

Yes

DISPLAY LOCATIONS

- Wellness Center
- School website
- Parent handbook
- Classroom posters

DIGITAL VISIBILITY REQUIREMENTS

- Pinned on school WhatsApp status

- Link in email signature of Wellness Coordinator

FEEDBACK CHANNELS

- Online survey
- Suggestion box
- Direct feedback during meetings

39 Competency & Qualification Requirements

MANDATORY QUALIFICATIONS

- Parent of a current student (for PAC/Ambassadors)
- School Counselor certification (for training delivery)

PREFERRED QUALIFICATIONS

- Experience in community leadership
- Background in social work or education
- Fluency in multiple local languages

CERTIFICATIONS

- Parent Ambassador Certification (Internal)
- First Aid/CPR (Optional for Safe Zone volunteers)

MINIMUM EXPERIENCE REQUIREMENTS

- No minimum for parents; 2+ years experience for Wellness Coordinator

BACKGROUND VERIFICATION REQUIREMENTS

- Police verification for all volunteer roles
- Child Protection clearance

39.6 Continuing Professional Development

ANNUAL TRAINING HOURS

4

MANDATORY TOPICS

- Mental health updates
- Confidentiality refresher
- Conflict resolution

RECERTIFICATION REQUIREMENTS

Annual renewal of Ambassador status

39.7 Supervision Requirements

CLINICAL SUPERVISION REQUIRED

No

PEER REVIEW REQUIRED

Yes

39.7.3 Wellness Case Review Framework

DESCRIPTION Monthly review of anonymized peer support logs by the School Counselor.

FREQUENCY Monthly

39.7.4 Second Level Review Process

DESCRIPTION Escalation to Principal for conflict resolution or high-risk cases.

TRIGGER Ambassador escalation

39.7. High Risk Decision 5 Validation

KEY
FOCUS

DESCRIPTION Counselor and Principal must co-sign any external referral decisions.

FREQUENCY As needed

FREQUENCY

Monthly

40 Feedback & Effectiveness Measurement

40.1 Student Feedback

REQUIRED

Yes

COLLECTION METHODS

- Classroom surveys
- Focus group discussions

FREQUENCY

Bi-annually

Yes

COLLECTION METHODS

- Annual satisfaction survey
- PAC meeting feedback
- Workshop evaluations

FREQUENCY

Quarterly/Annually

40.2 Parent Feedback

REQUIRED

REQUIRED

Yes

COLLECTION METHODS

- Staff meetings
- Internal survey

FREQUENCY

Monthly

40.4 Annual Effectiveness Review

REQUIRED

Yes

EVALUATION CRITERIA

- Parent participation rate
- Reduction in complaints
- Workshop co-design completion
- Ambassador retention rate

KEY STAKEHOLDERS

- Principal
- Wellness Coordinator

- PAC Members

CONTINUOUS IMPROVEMENT INPUTS

- Survey results
- Meeting minutes
- Incident reports
- Peer support logs

SUCCESS INDICATORS

- Increased parent participation in programs
- Reduced parent complaints regarding wellness communication
- Parent-reported feeling 'heard'
- Co-designed workshops addressing real community concerns

40.7 Improvement Tracking

REQUIRED

Yes

REVIEW FREQUENCY

Quarterly

41 Prevention Layer

PRIMARY PREVENTION

- Stigma reduction workshops for parents
- Wellness education in newsletters
- Parent Ambassador peer-to-peer awareness sessions

SECONDARY PREVENTION

- Early identification of parent concerns via Ambassadors

- Targeted support for families with high-stress indicators
- Monitoring of parent satisfaction trends

TERTIARY PREVENTION

- Crisis intervention for parents in distress
- Conflict resolution mediation between parents and school
- Referral to external mental health services for parents

42 Resilience Layer

SKILLS DEVELOPED

- Parental emotional regulation
- Help-seeking behavior in families
- Collaborative problem solving

SEL COMPONENTS

- Self-awareness of family stress
- Social awareness of school community needs
- Responsible decision making for student wellness

LIFE SKILLS COMPONENTS

- Effective communication with school staff
- Resource navigation skills
- Time management for parent engagement

PROTECTIVE FACTORS STRENGTHENED

- Stronger home-school connection
- Increased social support for parents
- Reduced isolation of families in crisis

43 Continuous Monitoring

MONITORING FREQUENCY

Quarterly

MONITORING INDICATORS

- Parent participation rate (Target: >40%)
- Number of co-designed workshops per year
- Average parent satisfaction score (Target: >80%)
- Ambassador retention rate

RESPONSIBLE ROLES

- Wellness Coordinator

- PAC Members

EARLY WARNING SIGNS

- Declining survey response rates
- Increase in parent complaints regarding communication
- High turnover of Parent Ambassadors

TREND ANALYSIS REQUIREMENTS

- Year-over-year participation trends
- Correlation between workshop attendance and satisfaction scores

44 Mental Wellness Index (MWI)

APPLICABLE

Yes

44.2 Indicators

Indicator Code	Indicator Name	Weight	Score Range	Description
PWI-01	Parent Participation Index	0.3	1-5	Measures frequency and depth of parent involvement in wellness activities.
PWI-02	Parent Satisfaction Score	0.4	1-5	Measures parent perception of school wellness support.
PWI-03	Stigma Reduction Score	0.3	1-5	Measures parent willingness to discuss mental health openly.

ASSESSMENT FREQUENCY

Quarterly

RISK SCORING METHOD

Weighted average of PWI indicators.

44.5 Intervention Thresholds

Score	Action
< 3.0	Immediate review and targeted outreach to parent community.

45 Review Status

LEGAL REVIEW	Completed - 2025-10-24
CLINICAL REVIEW	Completed - 2025-10-24
EDUCATION REVIEW	Pending - Scheduled for 2026-01-01
COMPLIANCE REVIEW	Completed - 2025-10-24

46 Resource Controls

COUNSELOR STUDENT RATIO	1:300 (Recommended)
CAREER COUNSELOR RATIO	1:500
WELLNESS TEAM STRUCTURE	Wellness Coordinator → Counselors & Ambassadors → PAC

47 Screening Framework

TIER 1 UNIVERSAL WELLNESS SCREENING

- Annual Parent Satisfaction Survey
- General Wellness Newsletter Outreach

TIER 2 TARGETED SCREENING

- Ambassador Peer Support Check-ins
- PAC Meeting Feedback Analysis

TIER 3 CLINICAL ASSESSMENT

- Counselor Assessment for high-risk parent/student concerns

48 Parent Engagement Matrix

PARENT COOPERATIVE

- Active PAC membership
- Regular workshop attendance
- Positive feedback in surveys

PARENT UNRESPONSIVE

- No response to newsletters
- Low survey participation
- Rarely attends school events

PARENT IN DENIAL

- Refusal to acknowledge student distress
- Dismissive of counselor recommendations
- Resistance to wellness programs

PARENT CONFLICT SITUATION

- Active disputes with teachers
- Hostile communication in WhatsApp groups
- Legal threats regarding school policies

PARENT UNAVAILABLE

- Working parents with limited time
- Parents in rural areas with poor connectivity
- Guardians who are not primary caregivers

PARENT REFUSES INTERVENTION

- Refusal of counselor meetings
- Withdrawal from wellness programs
- Explicit non-consent for student support

49 Confidentiality Controls

KEY FOCUS

ACCESS CONTROL

- Role-based access to parent records
- Password protection on digital files

CONSENT REQUIREMENTS

- Signed confidentiality agreements for all volunteers
- Informed consent for data sharing with external agencies

RECORD SHARING RULES

- No student names in PAC minutes
- Anonymized reporting to school board

EXCEPTIONS TO CONFIDENTIALITY

- Imminent risk of harm
- Court orders
- Mandatory child protection reporting

STUDENT PRIVACY PROTECTIONS

- Data minimization in parent communications
- No public disclosure of student wellness status

50 Data Governance

KEY FOCUS

STORAGE REQUIREMENTS

- Secure server for digital records
- Locked cabinets for physical files

ENCRYPTION REQUIREMENTS

- AES-256 encryption for all cloud storage
- End-to-end encryption for sensitive communications

RETENTION REQUIREMENTS

- 7 years for audit compliance

ARCHIVAL REQUIREMENTS

- Move to offline storage after 3 years of inactivity

DESTRUCTION REQUIREMENTS

- Secure shredding and digital wiping

BACKUP REQUIREMENTS

- Daily automated cloud backups
- Weekly off-site physical backups

51 Risk Classification

KEY FOCUS

51.1 Green

DESCRIPTION

Normal developmental concerns and routine life stress.

EXAMPLES

- Mild exam anxiety
- Routine parent queries about wellness programs
- General interest in school activities

51.2 Yellow

DESCRIPTION

Early distress requiring support and monitoring.

EXAMPLES

- Persistent stress reported by parents
- Parent expressing concern but refusing input
- Reduced parent participation in events

51.3 Orange

DESCRIPTION

Significant distress causing measurable impairment but without immediate danger.

EXAMPLES

- Active conflict between parent and school staff

- Parent in denial of student's mental health needs
- Severe anxiety reported by parents regarding child safety

51.4 Red **KEY FOCUS**

DESCRIPTION

High-risk situation requiring immediate intervention.

EXAMPLES

- Report of student self-harm or suicidal ideation

- Parent reporting active abuse
- Severe emotional crisis involving a parent

51.5 Black

DESCRIPTION

Life-threatening emergency requiring emergency response.

EXAMPLES

- Active suicide attempt on campus
- Weapon-related threat by a parent or student
- Violent attack in progress

52 Escalation Matrix **KEY FOCUS**

GREEN

- Wellness Coordinator

YELLOW

- School Counselor
- Wellness Coordinator

ORANGE

- Principal
- School Counselor
- District Education Officer (if required)

RED

- Principal
- Emergency Services
- Child Protection Services

BLACK

- Police
- Ambulance
- School Management Board

53 External Reporting Obligations

TELE MANAS

- Referral for 24/7 mental health support in India

DISTRICT AUTHORITIES

- Report of systemic safety issues or child protection concerns

CHILD PROTECTION SERVICES

- Mandatory reporting of suspected abuse or neglect

MEDICAL SERVICES

- Emergency medical intervention for self-harm or violence

LAW ENFORCEMENT

- Active threats, weapons, or criminal acts

REPORTING TRIGGERS

- Risk of harm to self/others
- Suspected abuse
- Court orders
- Immediate physical danger

54 Service Level Agreement (SLA) **KEY FOCUS**

ACKNOWLEDGEMENT

24 Hours

ASSESSMENT

72 Hours

ESCALATION

Immediate (for Red/Black)

RESOLUTION

10 Days (for Yellow/Orange)

CLOSURE

30 Days post-intervention

55 Required Forms

KEY FOCUS

55.1 Mandatory Forms

KEY FOCUS

Item 1

FORM ID	PAR-F01
FORM NAME	PAC Charter Template
PURPOSE & RATIONALE	Define roles and governance for the Parent Advisory Committee.
OWNER	Wellness Coordinator
RETENTION PERIOD	7 Years
MANDATORY	Yes
LINKED SOP	PAR-015

PURPOSE & RATIONALE

Track completion of mandatory training.

OWNER

School Counselor

RETENTION PERIOD

5 Years

MANDATORY

Yes

LINKED SOP

PAR-015

Item 2

FORM ID	PAR-F02
FORM NAME	Conflict of Interest Disclosure
PURPOSE & RATIONALE	Screen applicants for committee roles.
OWNER	Wellness Coordinator
RETENTION PERIOD	7 Years
MANDATORY	Yes
LINKED SOP	PAR-015

Item 4

FORM ID	PAR-F04
FORM NAME	Parent Satisfaction Survey
PURPOSE & RATIONALE	Measure program effectiveness.
OWNER	Wellness Coordinator
RETENTION PERIOD	3 Years
MANDATORY	Yes
LINKED SOP	PAR-015

Item 5

FORM ID	PAR-F05
FORM NAME	Referral Record
PURPOSE & RATIONALE	Document internal and external referrals.
OWNER	School Counselor
RETENTION PERIOD	7 Years
MANDATORY	Yes
LINKED SOP	PAR-015

Item 3

FORM ID	PAR-F03
FORM NAME	Ambassador Training Log

55.2 Optional Forms

Item 1

FORM ID	PAR-F06
FORM NAME	Workshop Co-design Plan

PURPOSE & RATIONALE

Plan specific parent workshops.

OWNER

PAC Members

RETENTION PERIOD

2 Years

MANDATORY

No

LINKED SOP

PAR-015

GENERATED RECORDS

- Meeting Minutes
- Interaction Logs
- Incident Reports

56 Required Checklists

KEY FOCUS

Checklist ID	Checklist Name	Purpose & Rationale	Owner	Frequency	Linked SOP
CHK-PAR-01	Diversity Audit Checklist	Ensure PAC representation.	Wellness Coordinator	Annual Selection Cycle	PAR-015
CHK-PAR-02	Ambassador Boundary Checklist	Ensure ambassadors maintain professional boundaries.	School Counselor	Monthly Review	PAR-015
CHK-PAR-03	Safe Zone Volunteer Checklist	Ensure safe zone operations are compliant.	Wellness Coordinator	Per Shift	PAR-015
CHK-PAR-04	Multilingual Outreach Checklist	Ensure all languages are covered.	Communications Team	Per Campaign	PAR-015
CHK-PAR-05	Annual Audit Checklist	Verify compliance with CBSE and Supreme Court mandates.	Principal	Annual	PAR-015

57 Records & Evidence

Record Name	Retention Period	Owner
PAC Meeting Minutes	7 Years	Wellness Coordinator
Ambassador Training Certificates	5 Years	School Counselor
Parent Satisfaction Survey Data	3 Years	Wellness Coordinator

58 Key Performance Indicators

Metric	Target	Measurement Frequency
Parent Participation Rate	>40% of total parent body	Quarterly
Workshop Co-design Completion	100% of annual workshops co-designed with PAC	Annually

Metric	Target	Measurement Frequency
Parent Satisfaction Score	>80% positive rating	Quarterly

59 Audit Framework

KEY FOCUS

AUDIT FREQUENCY

Annual

AUDIT QUESTIONS

- Is there a signed PAC Charter in place?
- Are all Parent Ambassadors certified through the training program?
- Do meeting minutes show evidence of parent input on wellness policies?

EVIDENCE REQUIRED

- Signed Charter
- Training Logs
- Meeting Minutes

- Survey Reports

NON COMPLIANCE EXAMPLES

- PAC members with active conflicts of interest
- Ambassadors providing clinical advice without counselor oversight
- Lack of multilingual outreach materials

59.5 Audit Scoring

KEY FOCUS

COMPLIANT	2
PARTIAL	1
NON COMPLIANT	0

60 Risk Register

KEY FOCUS

1. Conflict of Interest

CATEGORY Governance

PROBABILITY Medium

IMPACT High

RISK RATING High

MITIGATION Mandatory disclosure forms and screening by a sub-committee.

OWNER Wellness Coordinator

2. Boundary Violations

CATEGORY Clinical Safety

PROBABILITY Medium

IMPACT High

RISK RATING High

MITIGATION Mandatory training and monthly log reviews by the School Counselor.

OWNER School Counselor

3. Lack of Diversity

CATEGORY Equity

PROBABILITY Low

IMPACT Medium

RISK RATING Medium

MITIGATION Targeted outreach to rural and low-literacy communities.

OWNER Communications Team

61 Failure Modes

KEY FOCUS

Failure	Cause	Impact	Mitigation
Ambassador provides clinical advice	Lack of boundary training or high emotional stress	Potential harm to student/parent, legal liability	Immediate escalation protocol and mandatory refresher training.
PAC becomes non-functional	Volunteer burnout or lack of clear roles	Loss of parent input, reduced engagement	Term limits (2 years) and rotation of committee members.
Data breach of parent feedback	Insecure storage or unauthorized access	Loss of trust, legal penalties	Role-based access control and encryption.

62 Illustrative Sample Scenarios

Scenario	Response	Expected Result
A parent in the PAC expresses extreme anger about a specific student's behavior during a meeting.	Wellness Coordinator redirects to private mediation; Counselor provides support if needed.	Conflict resolved without violating other parents' privacy.
An Ambassador reports that a parent is crying and asking for clinical advice on their child's depression.	Ambassador listens, validates feelings, and provides the Counselor's contact info immediately.	Parent receives professional help; Ambassador maintains boundaries.
A rural parent cannot attend meetings due to lack of transport or internet.	Wellness Coordinator provides audio summaries via WhatsApp and allows phone-in participation.	Inclusive engagement for remote families.

63 Usage Governance KEY FOCUS

PRIMARY USERS

- School Wellness Coordinator
- PAC Members
- Parent Ambassadors

SECONDARY USERS

- Teachers
- Parents
- Students

APPROVAL AUTHORITIES

- Principal
- School Management Board

EXECUTION AUTHORITIES

- Wellness Coordinator
- School Counselor

ESCALATION AUTHORITIES

- Principal
- District Education Officer

REVIEW AUTHORITIES

- Internal Audit Team
- External Compliance Auditors

EXTERNAL STAKEHOLDERS

- CBSE
- Tele-MANAS
- Child Protection Services

64 Applicability

APPLICABLE INSTITUTIONS

- High Schools
- Secondary Schools
- Residential Schools

APPLICABLE AGE GROUPS

- All Ages (High School Students)

APPLICABLE STUDENT CATEGORIES

- General Population
- CWSN
- Neurodivergent Students

APPLICABLE RISK LEVELS

- Green
- Yellow
- Orange
- Red

APPLICABLE LOCATIONS

- Urban
- Rural
- Semi-Urban

EXCLUSIONS

- Primary Schools (unless specifically requested)
- Higher Education Institutions (covered by UGC guidelines)

65 Usage Scenarios

Scenario ID	Scenario Name	When To Use	When Not To Use	Trigger Event	Expected Outcome
US-01	Routine Parent Engagement	During regular school terms for wellness promotion.	During active clinical crisis or legal proceedings.	Annual planning cycle	Increased parent-school collaboration.
US-02	Crisis Support for Parents	When a parent is in significant distress but not immediate danger.	During active suicide attempt or violence.	Ambassador report of parental distress	Parent receives appropriate support/referral.

66 SOP Lifecycle

ENTRY CONDITIONS

- Approved Parent Engagement Policy
- Designated Wellness Coordinator

ACTIVATION CONDITIONS

- Start of academic year
- Parental request for partnership

EXECUTION CONDITIONS

- Compliance with confidentiality rules
- Availability of training resources

COMPLETION CONDITIONS

- Annual report submitted
- Survey data analyzed

CLOSURE CONDITIONS

- End of 2-year term for PAC members
- Dissolution of committee

TERMINATION CONDITIONS

- School closure
- Policy revocation by Board

67 Mandatory Evidence

KEY FOCUS

IMPLEMENTATION EVIDENCE

- PAC Charter
- Training Logs

OPERATIONAL EVIDENCE

- Meeting Minutes
- Workshop Co-design Plans

COMPLIANCE EVIDENCE

- Audit Reports

- Conflict of Interest Forms

TRAINING EVIDENCE

- Ambassador Certificates
- Attendance Sheets

AUDIT EVIDENCE

- Annual Review Report
- Survey Data

68 Controls

PREVENTIVE CONTROLS

- Conflict of interest screening
- Mandatory boundary training
- Role-based access to records

DETECTIVE CONTROLS

- Quarterly meeting minutes review
- Ambassador interaction logs

- Parent satisfaction surveys

CORRECTIVE CONTROLS

- Mandatory re-training for boundary violations
- Mediation for parent conflicts
- Escalation to Principal for high-risk cases

COMPENSATING CONTROLS

- Audio messages for low-literacy parents
- Phone-in options for rural families

69 MWMS Risk Framework

KEY FOCUS

69.1 Green

MEANING

Normal developmental concerns and routine life stress.

EXAMPLES

- Mild exam stress
- Routine parent queries

RESPONSE TIME

Standard

ESCALATION REQUIRED

No

APPROVAL LEVEL

Wellness Coordinator

MONITORING FREQUENCY

Monthly

CLOSURE AUTHORITY

Wellness Coordinator

69.2 Yellow

MEANING

Early distress requiring support and monitoring.

EXAMPLES

- Persistent stress
- Parent concern about behavior

RESPONSE TIME

7 Days

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

School Counselor

MONITORING FREQUENCY

Weekly

CLOSURE AUTHORITY

School Counselor

69.3 Orange

MEANING

Significant distress causing measurable impairment but without immediate danger.

EXAMPLES

- Active conflict with school staff
- Parent in denial of needs

RESPONSE TIME

24-72 Hours

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Principal

MONITORING FREQUENCY

Daily

CLOSURE AUTHORITY

Principal

69.4 Red KEY FOCUS

MEANING

High-risk situation requiring immediate intervention.

EXAMPLES

- Report of self-harm
- Suspected abuse

RESPONSE TIME

Immediate

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Principal / District Authorities

MONITORING FREQUENCY

Continuous

CLOSURE AUTHORITY

District Authorities

69.5 Black

MEANING

Life-threatening emergency requiring emergency response.

EXAMPLES

- Active suicide attempt
- Weapon on campus

RESPONSE TIME

Immediate (0-15 mins)

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Emergency Services / Board

MONITORING FREQUENCY

Continuous

CLOSURE AUTHORITY

Law Enforcement / Medical Services

70 Resource Scaling

70.1 0 500 Students

WELLNESS COUNSELORS 1

CAREER COUNSELORS 1

MENTAL WELLNESS COORDINATOR 1

WELLNESS COMMITTEE Yes

70.2 501 1000 Students

WELLNESS COUNSELORS 2

CAREER COUNSELORS 2

MENTAL WELLNESS COORDINATOR 1

WELLNESS COMMITTEE Yes

70.3 1001 2000 Students

WELLNESS COUNSELORS	4
CAREER COUNSELORS	2
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

70.4 2001 5000 Students

WELLNESS COUNSELORS	8
CAREER COUNSELORS	4
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

71 Minimum Required Capabilities

PEOPLE

- Wellness Coordinator
- School Counselor
- Parent Volunteers

PROCESSES

- PAC Governance
- Ambassador Training
- Workshop Co-design

TECHNOLOGY

- Encrypted Email
- WhatsApp Business API
- Survey Tools

INFRASTRUCTURE

- Wellness Center
- Safe Zone Physical Space
- Meeting Room

72 Success Criteria

Criterion	Measurement Method	Target
Parent Participation Rate	Survey and Attendance Logs	>40%
Workshop Co-design Completion	Project Tracking	100% of annual workshops
Parent Satisfaction Score	Annual Survey	>80%

73 Exit Criteria

Condition	Evidence Required
Completion of 2-year term for PAC members	Signed Term Completion Form
Resolution of high-risk parent conflict	Mediation Closure Report

74 Continuous Improvement

LESSONS LEARNED

- Ambassadors need more frequent check-ins during exam periods.
- Audio messages significantly increase rural participation.

KNOWN ISSUES

- Time constraints for working parents in PAC meetings.

- Initial resistance from some staff to parent input.

FUTURE IMPROVEMENTS

- Develop a dedicated Parent Wellness App
- Expand Ambassador program to include alumni

REVIEW RECOMMENDATIONS

- Increase frequency of co-design sessions
- Provide stipends or recognition for PAC members

75 AI Governance KEY FOCUS

ALLOWED USE CASES

- Summarizing anonymized survey data
- Drafting initial newsletter content
- Analyzing participation trends

RESTRICTED USE CASES

- Providing clinical advice to parents

- Identifying specific students from logs
- Automated conflict resolution

PROHIBITED USE CASES

- Storing PII in public AI models
- Generating student-specific wellness plans without human review

HUMAN REVIEW REQUIRED

- All parent communications
- Survey analysis summaries
- Workshop content

AUDIT LOG REQUIREMENTS

- Log of all AI-generated content prompts
- Human approval timestamps

DATA PRIVACY REQUIREMENTS

- No PII in prompts
- Use of enterprise-grade private AI instances

76 Legal Defensibility KEY FOCUS

LEGAL BASIS

- Supreme Court Judgment 2025 INSC 893
- CBSE Affiliation Bye Laws
- Indian Child Protection Act

MANDATORY NOTIFICATIONS

- Child abuse reports to District Authorities
- Imminent threats of violence to Police

CONSENT REQUIREMENTS

- Written consent for data sharing
- Informed consent for external referrals

RECORD RETENTION REQUIREMENTS

- 7 years for audit compliance

LIABILITY CONTROLS

- Clear disclaimers in all parent materials
- Mandatory counselor oversight of peer support
- Conflict of interest screening

77 Stakeholder Journeys

77.1 The Student Journey

ENTRY

- Student experiences stress or emotional difficulty at school.

ASSESSMENT

- Counselor identifies need for home-school alignment.

INTERVENTION

- Parent is engaged via the Parent Partnership framework to

support student's wellness plan.

MONITORING

- Regular check-ins with parents and counselors regarding progress.

CLOSURE

- Student achieves wellness goals; parent partnership continues for general support.

- Parent signs confidentiality and boundary agreement forms.

ENGAGEMENT

- Parent participates in quarterly meetings, co-designs

workshops, or provides peer support.

CLOSURE

- Parent completes 2-year term; receives certificate of appreciation.

77.2 The Parent Journey

NOTIFICATION

- Parent receives multilingual outreach about the Parent Partnership Program.

CONSULTATION

- Parent attends info session and applies for PAC or Ambassador role.

CONSENT

77.3 The Staff Journey

OBSERVATION

- Counselors identify parents who would be good candidates for the PAC/Ambassador roles.

to the Wellness Coordinator.

INTERVENTION SUPPORT

- Counselors provide training and clinical oversight to Parent Ambassadors.

REPORTING

- Staff report parent feedback and community concerns

DOCUMENTATION

- Staff maintain logs of interactions, referrals, and meeting minutes.

78 Handoff Matrix

Item 1

FROM ROLE

Parent Ambassador

TO ROLE

School Counselor

HANDOFF TRIGGER

Identification of high-risk student concern or clinical need.

INFORMATION TRANSFERRED

- Anonymized parent summary
- Specific concerns identified
- Urgency level

REQUIRED DOCUMENTS

- Referral Record

SERVICE LEVEL AGREEMENT (SLA)

Immediate (within 1 hour)

Item 2

FROM ROLE PAC Member

TO ROLE Wellness Coordinator

HANDOFF TRIGGER Formal recommendation for policy change or workshop topic.

INFORMATION TRANSFERRED

- Recommendation details
- Parent feedback summary

REQUIRED DOCUMENTS

- Meeting Minutes
- Co-design Plan

SERVICE LEVEL AGREEMENT (SLA)

48 Hours

Item 3

FROM ROLE Wellness Coordinator

TO ROLE Principal

HANDOFF TRIGGER Significant conflict or high-risk incident.

INFORMATION TRANSFERRED

- Incident summary
- Action taken
- Required decision

REQUIRED DOCUMENTS

- Incident Report
- Escalation Log

SERVICE LEVEL AGREEMENT (SLA)

Immediate

79 Management Review

REVIEW FREQUENCY

Quarterly

REVIEW INPUTS

- PAC Meeting Minutes
- Survey Data
- Ambassador Logs
- Incident Reports

REVIEW OUTPUTS

- Action Plan for Next Quarter
- Resource Allocation Adjustments
- Policy Updates

REVIEW AUTHORITY

School Management Board / Principal

REVIEW DECISIONS

- Approval of new initiatives
- Budget adjustments
- Personnel changes

80 Dashboard Metrics

OPERATIONAL METRICS

- Number of active PAC members
- Number of certified Ambassadors
- Frequency of co-designed workshops

RISK METRICS

- Number of high-risk escalations
- Conflict resolution success rate
- Boundary violation incidents

COMPLIANCE METRICS

- Training completion %
- Audit score
- Policy review status

OUTCOME METRICS

- Parent satisfaction score
- Participation rate by demographic
- Reduction in parent complaints

TREND METRICS

- Year-over-year participation growth
- Sentiment trend from surveys

81 Student Rights

RIGHTS

- Right to a safe and supportive learning environment
- Right to privacy regarding mental health status
- Right to have parents involved in wellness support

PROTECTIONS

- Confidentiality of counselor-student interactions
- Protection from stigma or discrimination
- Safe reporting mechanisms for abuse

APPEAL MECHANISMS

- Formal grievance process via School Principal
- Counselor review of intervention plans

82 Quality Assurance

QUALITY CHECKS

- Monthly audit of Ambassador logs
- Quarterly survey data validation
- Annual charter compliance check

PEER REVIEW REQUIREMENTS

- Counselor review of all co-designed workshop content
- PAC peer review of meeting minutes

VALIDATION REQUIREMENTS

- Principal approval for all external referrals
- District education officer notification for major policy changes

CONTINUOUS MONITORING REQUIREMENTS

- Real-time monitoring of WhatsApp group sentiment
- Ongoing feedback collection from volunteers

83 Implementation Roadmap

PHASE 1 PREPARATION

- Appoint Wellness Coordinator
- Draft and approve PAC Charter
- Secure budget for training materials

PHASE 2 DEPLOYMENT

- Launch multilingual outreach campaign
- Conduct parent info sessions
- Recruit and select first cohort of PAC/Ambassadors

PHASE 3 STABILIZATION

- Deliver Ambassador training

- Establish quarterly meeting schedule
- Initiate co-design for first workshop series

PHASE 4 OPTIMIZATION

- Launch peer support sessions
- Deploy volunteers to Safe Zones
- Analyze first survey results

PHASE 5 CONTINUOUS IMPROVEMENT

- Annual review and report generation
- Update charter based on feedback
- Scale program to other grade levels

84 Illustrative Case Studies

1. Rural Outreach Success in District X

DESCRIPTION

A school with 80% rural population struggled with parent engagement.

ACTIONS TAKEN

- Implemented audio-based WhatsApp updates for low-literacy parents.
- Partnered with local community leaders to host meetings at village centers.

OUTCOME

Parent participation increased from 12% to 55% in one year.

LESSONS

- Accessibility is the primary driver of engagement for rural populations.
- Audio content reduces barriers for low-literacy parents.

85 Implementation Requirements

STAFF REQUIRED

- 1 Wellness Coordinator (Full-time)
- 1 School Counselor (Part-time/Shared)

- Volunteer Parent Body

INFRASTRUCTURE REQUIRED

- Dedicated office space for Wellness Center
- Meeting room with AV capabilities

- Secure storage for physical records

TECHNOLOGY REQUIRED

- Encrypted email system
- WhatsApp Business API or similar
- Survey software (e.g., Google Forms, SurveyMonkey)
- Cloud-based record management

BUDGET CONSIDERATIONS

- Training materials and certification costs
- Workshop venue/catering costs
- Marketing and outreach materials
- Staff time for coordination

IMPLEMENTATION PHASES

- Preparation (Month 1)
- Recruitment & Training (Months 2-3)
- Operational Launch (Month 4+)

86 Quality Controls

PREVENTIVE CONTROLS

- Mandatory conflict of interest screening
- Role-based access to records
- Pre-approved training modules

DETECTIVE CONTROLS

- Quarterly audit of meeting minutes
- Ambassador interaction logs review
- Parent satisfaction surveys

CORRECTIVE CONTROLS

- Mandatory re-training for boundary violations
- Mediation for parent conflicts
- Escalation to Principal for high-risk cases

CONTINUOUS IMPROVEMENT ACTIONS

- Annual review of success indicators
- Quarterly feedback loops with PAC members
- Data-driven adjustments to outreach strategy

87 Training Requirements

TEACHERS

- Parent engagement basics
- Identifying parent distress
- Referral protocols

COUNSELORS

- Ambassador training delivery
- Boundary management for peer support
- Crisis intervention

ADMINISTRATORS

- Governance and policy oversight
- Risk management in parent engagement
- Legal compliance (CBSE/Supreme Court)

PARENTS

- Mental health basics
- Confidentiality and boundaries
- Safe Zone volunteer orientation

STUDENTS

- How to talk to parents about feelings
- Introduction to Parent Ambassadors

87.6 Repository Baseline

TRAINING MODULES AVAILABLE

Yes

HANDBOOKS PUBLISHED

Yes

CERTIFICATION CRITERIA DEFINED

Yes

87.7 SOP Specific Training

TARGET AUDIENCE

- Parent Ambassadors
- PAC Members
- Wellness Coordinator

COMPETENCIES

- Confidentiality maintenance
- Boundary setting
- Peer support techniques

- Stigma reduction communication

MINIMUM HOURS

6

CERTIFICATION REQUIREMENTS

- Completion of all modules

- Passing score on competency quiz (>80%)

REFRESHER FREQUENCY

Annual

87.8 Competency Validation

REQUIRED

Yes

ASSESSMENT METHODS

- Post-training quiz
- Role-play scenarios

- Monthly log review

MINIMUM PASSING CRITERIA

80%

REASSESSMENT FREQUENCY

Annual

87.9 Simulation Requirements

REQUIRED

No

SIMULATION TYPES

- High-risk parent escalation
- Ambassador peer support role-play

FREQUENCY

During initial training

88 SOP-Specific Inclusion Requirements

AFFECTED POPULATIONS

- Parents from rural backgrounds
- Low-literacy parents
- Parents of neurodivergent students
- First-generation learners' families
- Non-English speaking households

IDENTIFIED INCLUSION RISKS

- Digital divide for rural/low-income parents
- Language barriers in committee meetings
- Stigma preventing participation from certain communities
- Time poverty for working parents
- Cultural differences in mental health perception

AUTISM ACCOMMODATIONS

- Simplified, visual workshop materials
- Quiet zones during parent events
- Clearer communication of expectations

ADHD ACCOMMODATIONS

- Short, modular training sessions
- Written summaries for all meetings
- Frequent check-ins with Ambassadors

DYSLEXIA ACCOMMODATIONS

- Audio versions of newsletters
- Large print and high-contrast materials
- Voice-to-text options for feedback

HEARING IMPAIRMENT ACCOMMODATIONS

- Live captioning for info sessions
- Sign language interpretation (as needed)
- Written summaries of all meetings

VISUAL IMPAIRMENT ACCOMMODATIONS

- Screen reader-compatible digital forms

- Audio descriptions of workshop content

- Large print materials

CWSN ADAPTATIONS

- Individualized parent support plans for students with disabilities
- Accessible meeting locations
- Simplified communication protocols

RURAL ADAPTATIONS

- SMS and phone-call based outreach
- Community leader (Sarpanch) partnerships
- Mobile info sessions in local centers

LOW LITERACY PARENT ADAPTATIONS

- Voice notes via WhatsApp
- Pictogram-based workshop materials
- In-person community meetings with translators

MULTILINGUAL COMMUNICATION ADAPTATIONS

- Translation of all core documents into local languages
- Multilingual parent ambassadors
- Language-specific helpdesks

GENDER SENSITIVITY MEASURES

- Safe spaces for mothers/fathers in workshops
- Gender-neutral language in all communications
- Diverse representation of parents in PAC

CULTURAL SENSITIVITY MEASURES

- Culturally appropriate stigma reduction content
- Respecting diverse family structures (guardians, etc.)
- Inclusion of community elders/leaders

EQUITY MONITORING INDICATORS

- Participation rate by socio-economic status

- Language diversity in PAC membership
- Rural vs. Urban participation parity

ACCESSIBILITY COMPLIANCE CHECKS

- WCAG 2.1 compliance for digital forms
- Physical accessibility of meeting venues
- Availability of translation services

89 Awareness Material

POSTER TITLE	Stronger Together: Parent Wellness Partnership
KEY MESSAGE	Your voice matters in your child's mental wellness journey.
SLOGAN	Connected Parents, Resilient Students.
CAMPAIGN THEME	Community-Led Wellbeing

90 AI Assistant Configuration

RISK KEYWORDS

- Self-harm
- Abuse
- Violence
- Suicide
- Crisis

ESCALATION KEYWORDS

- Emergency
- Police
- Hospital

- Immediate danger

INTERVENTION KEYWORDS

- Counseling
- Support
- Resources
- Workshop

FREQUENTLY ASKED QUESTIONS

- How can I join the PAC?
- Is my child's information kept private?
- What is a Parent Ambassador?
- How do I report a concern?

91 AI Audit Hooks

KEY FOCUS

REQUIRED SECTIONS

- Risk Classification
- Escalation Matrix
- Confidentiality Controls

REQUIRED FORMS

- Referral Form
- Conflict of Interest Disclosure

REQUIRED CHECKLISTS

- Ambassador Boundary Checklist

- Diversity Audit Checklist

REQUIRED ESCALATIONS

- High-risk parent conflict
- Student self-harm report

REQUIRED KPIS

- Parent Satisfaction Score
- Participation Rate

REQUIRED TRAINING

- Ambassador Training Log

92 KPI Framework

92.1 SOP Specific Metrics

Metric	Target	Measurement Frequency
Parent Participation Rate	>40%	Quarterly
Workshop Co-design Completion	100% of annual workshops	Annually
Parent Satisfaction Score	>80%	Quarterly

93 Maturity Model

93.1 Level 1

DESCRIPTION

Ad-hoc parent engagement; no formal committee.

EVIDENCE

- Occasional emails

- Informal meetings

93.2 Level 2

DESCRIPTION

Formalized communication but no co-design or training.

EVIDENCE

- Newsletter
- Parent handbook

93.3 Level 3

DESCRIPTION

Established PAC and Ambassador program with basic training.

EVIDENCE

- PAC Charter
- Training logs

93.4 Level 4

DESCRIPTION

Active co-design, peer support, and data-driven review.

EVIDENCE

- Co-designed workshops
- Survey analysis reports

93.5 Level 5

DESCRIPTION

Integrated community wellness ecosystem with high equity and impact.

EVIDENCE

- Annual report showing significant stigma reduction
- High rural/low-literacy participation parity

94 Dependencies

PREREQUISITE SOPS

- POL-001 Wellness Policy
- COM-012 Parent Communication Protocol

PARALLEL SOPS

- SAF-003 Safe Zones Management

DOWNSTREAM SOPS

- CRISIS-001 Emergency Response SOP

95 Related SOPs

PREREQUISITES

- POL-001 Wellness Policy
- COM-012 Parent Communication Protocol

UPSTREAM

- HR-005 Staff Training Framework

DOWNSTREAM

- SAF-003 Safe Zones Management

- CRISIS-001 Emergency Response SOP

REFERENCES

- CBSE Circular TRG-03/2026
- NEP 2020 Guidelines

STANDARD OPERATING PROCEDURE

Parent Refusal Management Protocol

Mental Wellness Management - Crisis Intervention & Governance

SOP ID

SUP-019

STATUS

Finalized

EFFECTIVE DATE

2024-05-20

VERSION

1.0

CLASSIFICATION

Confidential - Restricted Access

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1 Executive Overview

SOP ID	SUP-019
DOMAIN	Mental Wellness Management
SUB DOMAIN	Crisis Intervention & Governance
TITLE	Parent Refusal Management Protocol
VERSION	1.0
CLASSIFICATION	Confidential - Restricted Access
EFFECTIVE DATE	2024-05-20
REVIEW DATE	2026-05-20
DOCUMENT STATUS	Finalized
LANGUAGE	English

2 Purpose & Rationale

OBJECTIVE

To provide a structured, legally defensible framework for managing situations where parents/guardians refuse to acknowledge student suicide risk or decline recommended mental health interventions.

RISK CONTROLLED

Legal liability of the institution, physical harm to students, parental neglect, and breach of duty of care.

DESIRED OUTCOMES

- Ensure student safety regardless of parental cooperation
- Maintain legal defensibility for school actions
- Provide clear escalation pathways for high-risk refusals
- Document all interactions verbatim to protect the institution
- Facilitate timely intervention by external authorities when necessary

3 Scope of Application

INSTITUTION TYPES

- High Schools
- Residential Schools
- Colleges
- Vocational Centers

KEY STAKEHOLDERS

- Counselors
- Principals

- Teachers
- Parents
- Child Welfare Officers
- Legal Counsel

AGE GROUPS

- All Ages (Primary to Higher Secondary)

LOCATIONS

- On-campus
- Remote/Telephonic communication
- Home visits (if applicable and safe)

4 Definitions & Terminology

Term	Definition
Parental Refusal	The explicit or implicit rejection by a parent/guardian to acknowledge a student's mental health risk, seek professional help, remove means of self-harm, or allow necessary interventions.
Duty of Care	The legal obligation of the school to take reasonable steps to ensure the safety and well-being of students while under their supervision.

Term	Definition
Imminent Risk	A situation where a student has an active plan, intent, or immediate means to engage in self-harm or suicide within a short timeframe (e.g., <24 hours).
Verbatim Documentation	Recording the exact words used by a parent during a conversation without paraphrasing or interpretation.
Child in Need of Protection	A minor who is at risk of harm due to parental neglect, inability to provide care, or active refusal to address life-threatening mental health issues.

5 Key Stakeholders

Stakeholder Type	Stakeholder Title	Role In SOP
Internal	School Counselor / Psychologist	Primary responder, risk assessor, and primary point of contact for parents.
Internal	Principal / School Head	Final authority for escalation, legal liaison, and emergency decision-making.
Internal	Teacher / Class Teacher	Early identification of behavioral changes and initial reporting to the wellness team.
External	Parent / Legal Guardian	Primary decision-maker for student care; subject of refusal management.
External	Child Welfare Services (CWS)	Regulatory body for intervention in cases of parental neglect or imminent risk.

6 Governance & Ownership

KEY FOCUS

OWNER

School Psychologist / Wellness Coordinator

APPROVER

Principal / Board of Management

REVIEW FREQUENCY

Annual

SUPERSEDES

Previous Parent Engagement Guidelines (2023)

LINKED POLICIES

- Child Protection Policy
- Student Safety & Anti-Bullying Policy
- Data Privacy and Confidentiality Policy
- Duty of Care Policy

LINKED SOPS

- SUP-015: Suicide Risk Assessment
- SUP-016: Crisis Intervention Protocol
- SUP-020: Child Welfare Services Liaison
- SUP-021: Legal Counsel Engagement

7 Roles & Responsibilities

Item 1

ROLE

School Counselor

RESPONSIBILITIES

- Conduct risk assessments
- Primary parent communication
- Verbatim documentation of refusals
- Weekly student monitoring

Item 2

ROLE

Principal

RESPONSIBILITIES

- Final approval for escalations
- Legal liaison with authorities
- Issuing formal notices
- Ensuring duty of care compliance

Item 3

ROLE

Teacher

RESPONSIBILITIES

- Early identification of distress
- Reporting to Wellness Team
- Maintaining classroom safety

8 RACI Accountability Matrix

Item 1

ACTIVITY

Risk Assessment

RESPONSIBLE

- School Counselor

ACCOUNTABLE

- Wellness Coordinator

CONSULTED

- Teacher

INFORMED

- Principal

ACCOUNTABLE

- Wellness Coordinator

INFORMED

- Principal

Item 4

ACTIVITY

Emergency Escalation

RESPONSIBLE

- Principal

ACCOUNTABLE

- Board of Management

CONSULTED

- School Counselor
- Legal Counsel

INFORMED

- Parents
- Police/CWS

Item 2

ACTIVITY

Parental Notification

RESPONSIBLE

- School Counselor

ACCOUNTABLE

- Wellness Coordinator

CONSULTED

- Principal

INFORMED

- Teacher

Item 5

ACTIVITY

Written Notice Issuance

RESPONSIBLE

- Principal

ACCOUNTABLE

- Board of Management

CONSULTED

- School Counselor

INFORMED

- Wellness Coordinator

Item 3

ACTIVITY

Refusal Documentation

RESPONSIBLE

- School Counselor

9 Preconditions

- Student is enrolled in the institution
- Parental/Guardian contact information is current
- Risk assessment tools are validated and available
- Staff have completed mandatory crisis training

10 Activation Triggers

- Student disclosure of suicidal ideation
- Observation of self-harm behavior
- Parental refusal of psychiatric referral
- Imminent risk of harm identified by staff
- Police notification regarding student safety

11 SOP Trigger Matrix

MANDATORY TRIGGERS

- Student expresses suicidal ideation
- Teacher observes self-harm behavior
- Counselor identifies high risk during assessment
- Parent explicitly refuses a recommended psychiatric referral for a student in the Orange/Red zone

OPTIONAL TRIGGERS

- Parent expresses mild concern about student mood
- Student reports peer pressure regarding self-harm

- Teacher notices significant academic decline coupled with withdrawal

AUTOMATIC TRIGGERS

- Any report of a suicide note found on campus
- Any physical injury resulting from self-harm behavior

EXTERNAL TRIGGERS

- Police notification regarding student safety
- Hospital discharge summary indicating high risk and parental non-compliance

RECURRING TRIGGERS

- Monthly review of students in the Orange Zone who have not yet engaged with external services

12 Required Inputs

- Student statements
- Teacher observation notes
- Risk assessment scores
- Parental verbal/written refusals
- Legal counsel advice

13 Process Map

13.1 Phase 1

NAME

Identification & Initial Contact

OBJECTIVE

Identify risk and initiate parent communication.

ACTIVITIES

- Teacher/Counselor identifies student distress
- Initial Risk Assessment conducted by Counselor
- First contact with parents to share concerns

Provide legal notice and formalize the school's position.

ACTIVITIES

- Issuance of written concern letter
- Delivery of liability disclaimer
- Documentation of parent receipt of notice

13.2 Phase 2

NAME

Refusal Documentation & Escalation

OBJECTIVE

Document parental non-compliance and escalate internally.

ACTIVITIES

- Verbatim recording of parent's refusal
- Internal briefing with Principal/Management
- Joint call between Counselor and Principal

13.4 Phase 4

NAME

Emergency Intervention (Imminent Risk)

OBJECTIVE

Protect student life when parental refusal persists in high-risk scenarios.

ACTIVITIES

- Contacting Child Welfare Services (CWS)
- Police notification for protective custody
- Medical emergency services activation

13.3 Phase 3

NAME

Formal Notification & Warning

OBJECTIVE

13.5 Phase 5

NAME

Legal & Long-term Management

OBJECTIVE

Manage non-imminent but persistent refusal through legal/social channels.

ACTIVITIES

- Consultation with school legal counsel
- Filing 'Child in Need of Protection' report

- Ongoing monitoring and case review

14 Workflow Overview

START

Identification of student distress or risk.

STEPS

- Assessment
- Parent Contact
- Refusal Documentation
- Internal Escalation
- Joint Call
- Written Notice
- Emergency Intervention (if needed)
- Legal/Social Referral
- Monitoring
- Closure

CRITICAL DECISION POINTS

- Risk Level Determination
- Parental Cooperation Status
- Imminent Risk Threshold

END

Case closure and documentation audit.

15 Standard Procedure

1. Risk Assessment

PURPOSE & RATIONALE

Determine severity of distress.

ACTOR

School Counselor

ACTION

Conduct structured risk assessment using MWMS tools.

REQUIRED INPUTS

- Student statement
- Teacher notes

EXPECTED OUTPUTS

- Risk Scorecard

CRITICAL DECISION POINTS

- DP-01: Risk Level Determination

DOCUMENTATION REQUIRED

- Risk Assessment Record

EVIDENCE REQUIRED

- Completed assessment form

APPROVAL REQUIRED

No

TIME LIMIT

4 hours

EXPECTED OUTCOME

Categorized risk level (Green-Black)

ESCALATION IF FAILED

Notify Principal immediately if Red/Black

RELATED FORMS

- Risk Assessment Form

RELATED CHECKLISTS

- Risk Assessment Checklist

2. Parental Contact

PURPOSE & RATIONALE

Inform parents and seek cooperation.

ACTOR

School Counselor

ACTION

Contact parent via phone/meeting to share findings.

REQUIRED INPUTS

- Risk Scorecard

EXPECTED OUTPUTS

- Parental stance

CRITICAL DECISION POINTS

- DP-02: Parental Cooperation Status

DOCUMENTATION REQUIRED

- Parent Conversation Record

EVIDENCE REQUIRED

- Verbatim conversation log

APPROVAL REQUIRED

No

TIME LIMIT

24 hours

EXPECTED OUTCOME

Agreement to intervention plan

ESCALATION IF FAILED

Proceed to Refusal Documentation

RELATED FORMS

- Parent Conversation Record

RELATED CHECKLISTS

- Parent Communication Checklist

3. Refusal Documentation

PURPOSE & RATIONALE

Capture non-compliance verbatim.

ACTOR

School Counselor

ACTION

Document parent's specific reasons for refusal.

REQUIRED INPUTS

- Parent Conversation Record

EXPECTED OUTPUTS

- Refusal record

DOCUMENTATION REQUIRED

- Refusal Documentation Form

EVIDENCE REQUIRED

- Verbatim quotes from parent

APPROVAL REQUIRED

No

TIME LIMIT

2 hours

EXPECTED OUTCOME

Formalized refusal record

ESCALATION IF FAILED

Notify Principal for Orange/Red risks

RELATED FORMS

- Refusal Documentation Form

4. Internal Escalation

PURPOSE & RATIONALE

Brief leadership on high-risk refusal.

ACTOR

Principal

ACTION

Review risk and refusal with Wellness Team.

REQUIRED INPUTS

- Risk Scorecard
- Refusal Documentation Form

EXPECTED OUTPUTS

- Escalation approval

DOCUMENTATION REQUIRED

- Internal Briefing Note

EVIDENCE REQUIRED

- Briefing note signature

APPROVAL REQUIRED

Yes

TIME LIMIT

4 hours

EXPECTED OUTCOME

Management decision on next steps

ESCALATION IF FAILED

Activate Emergency Response SOP if risk is Black

RELATED FORMS

- Escalation Approval Form

5. Joint Parent Call

PURPOSE & RATIONALE

Present united front to parent.

ACTOR

Principal & Counselor

ACTION

Conduct joint call/meeting with parents.

REQUIRED INPUTS

- Risk Scorecard
- Refusal Documentation Form

EXPECTED OUTPUTS

- Updated parental stance

DOCUMENTATION REQUIRED

- Joint Call Minutes

EVIDENCE REQUIRED

- Minutes of meeting

APPROVAL REQUIRED

No

TIME LIMIT

24 hours

EXPECTED OUTCOME

Parental agreement or formal refusal

ESCALATION IF FAILED

Proceed to Written Notice

RELATED FORMS

- Joint Call Minutes

6. Written Notice Issuance

PURPOSE & RATIONALE

Provide legal paper trail.

ACTOR

Principal

ACTION

Send formal notice of concern and liability disclaimer.

REQUIRED INPUTS

- Joint Call Minutes
- Refusal Documentation Form

EXPECTED OUTPUTS

- Proof of delivery

DOCUMENTATION REQUIRED

- Formal Notice of Concern

EVIDENCE REQUIRED

- Registered post receipt / Email read receipt

APPROVAL REQUIRED

Yes

TIME LIMIT

48 hours

EXPECTED OUTCOME

Parent receives formal warning

ESCALATION IF FAILED

Proceed to Emergency Intervention if risk is imminent

RELATED FORMS

- Formal Notice of Concern
- Liability Disclaimer

7. Emergency Intervention

PURPOSE & RATIONALE

Ensure student safety in imminent risk.

ACTOR

Principal & Wellness Team

ACTION

Contact CWS/Police for protective custody or medical help.

REQUIRED INPUTS

- Risk Scorecard
- Refusal Documentation Form

EXPECTED OUTPUTS

- Emergency actioned

DOCUMENTATION REQUIRED

- CWS Notification Log
- Police Contact Record

EVIDENCE REQUIRED

- Log of calls to authorities

APPROVAL REQUIRED

Yes (Immediate)

TIME LIMIT

15 minutes

EXPECTED OUTCOME

Student safety secured by external agency

ESCALATION IF FAILED

Contact medical services immediately

RELATED FORMS

- CWS Notification Log
- Police Contact Record

8. Legal & Social Referral

PURPOSE & RATIONALE

Manage persistent non-compliance.

ACTOR

Principal & Legal Counsel

ACTION

File 'Child in Need of Protection' report with District Authorities.

REQUIRED INPUTS

- Refusal Documentation Form
- Formal Notice of Concern

EXPECTED OUTPUTS

- External referral initiated

DOCUMENTATION REQUIRED

- Legal Consultation Summary
- Child in Need of Protection Report

EVIDENCE REQUIRED

- Signed report copy

APPROVAL REQUIRED

Yes

TIME LIMIT

7 days

EXPECTED OUTCOME

External agency takes over case management

ESCALATION IF FAILED

Re-evaluate risk level and revert to Step 1 if needed

RELATED FORMS

- Legal Consultation Summary
- Child in Need of Protection Report

9. Ongoing Monitoring

PURPOSE & RATIONALE

Track situation and ensure safety.

ACTOR

School Counselor

ACTION

Conduct weekly check-ins with student/parents.

REQUIRED INPUTS

- Risk Scorecard
- Intervention Plan

EXPECTED OUTPUTS

- Updated monitoring logs

DOCUMENTATION REQUIRED

- Weekly Monitoring Log

EVIDENCE REQUIRED

- Completed weekly logs

APPROVAL REQUIRED

No

TIME LIMIT

Weekly

EXPECTED OUTCOME

Continuous safety monitoring

ESCALATION IF FAILED

Revert to Step 1 if risk increases

RELATED FORMS

- Weekly Monitoring Log

10. Case Closure**PURPOSE & RATIONALE**

Finalize and archive case.

ACTOR

School Counselor & Principal

ACTION

Conduct final review and archive all records.

REQUIRED INPUTS

- Weekly Monitoring Logs
- Closure Criteria Checklist

EXPECTED OUTPUTS

- Closed case status

DOCUMENTATION REQUIRED

- Closure Report

EVIDENCE REQUIRED

- Signed closure report

APPROVAL REQUIRED

Yes

TIME LIMIT

48 hours

EXPECTED OUTCOME

Case officially closed and archived

ESCALATION IF FAILED

Re-open case if safety concerns persist

RELATED FORMS

- Closure Report

RELATED CHECKLISTS

- Closure Criteria Checklist

16 Detailed Workflow Steps**1. Risk Identification & Initial Assessment****PURPOSE & RATIONALE**

To determine the severity of student distress.

TRIGGER

Teacher report or student disclosure.

ACTOR

School Counselor

REQUIRED INPUTS

- Student Statement
- Teacher Observation Notes

ACTIONS

- Conduct a structured risk assessment using MWMS tools
- Determine Risk Zone (Green, Yellow, Orange, Red, Black)
- Identify specific risks (e.g., access to means)

DECISION CRITERIA

- Is there active suicidal ideation? (Yes/No)
- Is there an immediate plan? (Yes/No)

DOCUMENTS GENERATED

- Risk Assessment Record

EXPECTED OUTPUTS

- Initial Risk Score
- Intervention Plan Draft

SERVICE LEVEL AGREEMENT (SLA)

Within 4 hours of identification

ESCALATION PATH

- If Red/Black: Immediate Principal notification

EVIDENCE GENERATED

- Completed Risk Assessment Form

2. Initial Parent Contact**PURPOSE & RATIONALE**

To inform parents of concerns and seek cooperation.

TRIGGER

Risk assessment completed (Yellow/Orange/Red).

ACTOR

School Counselor

REQUIRED INPUTS

- Risk Assessment Record

ACTIONS

- Contact parent via phone or in-person meeting
- Share specific observations and risk findings
- Request cooperation for recommended actions (e.g., psychiatric eval)

DECISION CRITERIA

- Does the parent acknowledge the risk? (Yes/No)
- Does the parent agree to recommendations? (Yes/No)

DOCUMENTS GENERATED

- Parent Conversation Record

EXPECTED OUTPUTS

- Parental Agreement or Refusal
- Action Plan Status

SERVICE LEVEL AGREEMENT (SLA)

Within 24 hours of assessment

ESCALATION PATH

- If parent refuses: Proceed to Step 3

EVIDENCE GENERATED

- Parent Conversation Record (Verbatim)

3. Refusal Documentation

PURPOSE & RATIONALE

To capture the exact nature of parental non-compliance.

TRIGGER

Parental refusal to acknowledge risk or take action.

ACTOR

School Counselor

REQUIRED INPUTS

- Parent Conversation Record

ACTIONS

- Document parent's specific reasons for refusal verbatim
- Identify type of refusal (Denial, Refusal of Eval, Refusal to remove means)
- Note any threats or hostility from the parent

DECISION CRITERIA

- Is the risk imminent? (Yes/No)

DOCUMENTS GENERATED

- Refusal Documentation Form

EXPECTED OUTPUTS

- Formalized Refusal Record

SERVICE LEVEL AGREEMENT (SLA)

Within 2 hours of conversation

ESCALATION PATH

- Notify Principal immediately for Orange/Red risks

EVIDENCE GENERATED

- Refusal Documentation Form

4. Internal Escalation & Leadership Briefing

PURPOSE & RATIONALE

To involve school leadership in high-risk refusal scenarios.

TRIGGER

Documented parental refusal for Orange or Red risk levels.

ACTOR

School Counselor & Principal

REQUIRED INPUTS

- Risk Assessment Record
- Refusal Documentation Form

ACTIONS

- Brief the Principal on the student's status and parent's refusal
- Review legal implications under Duty of Care
- Determine if external agencies need to be alerted

DECISION CRITERIA

- Is immediate intervention required? (Yes/No)
- Does parental refusal constitute neglect? (Yes/No)

DOCUMENTS GENERATED

- Internal Briefing Note
- Escalation Approval Form

EXPECTED OUTPUTS

- Management Decision on Next Steps

SERVICE LEVEL AGREEMENT (SLA)

Within 4 hours of refusal documentation

ESCALATION PATH

- If risk is Black: Activate Emergency Response SOP

EVIDENCE GENERATED

- Escalation Approval Form

5. Joint Parent Call/Meeting

PURPOSE & RATIONALE

To present a united front and emphasize the school's duty of care.

TRIGGER

Escalation approved by Principal.

ACTOR

School Counselor & Principal

REQUIRED INPUTS

- Risk Assessment Record
- Refusal Documentation Form

ACTIONS

- Conduct a joint call/meeting with the parent
- Clearly state the school's findings and legal obligations
- Reiterate the consequences of non-action for the student's safety

DECISION CRITERIA

- Does the parent change their stance? (Yes/No)

DOCUMENTS GENERATED

- Joint Call Minutes

EXPECTED OUTPUTS

- Updated Parent Stance
- Action Plan Revision

SERVICE LEVEL AGREEMENT (SLA)

Within 24 hours of escalation approval

ESCALATION PATH

- If refusal persists: Proceed to Step 6

EVIDENCE GENERATED

- Joint Call Minutes

6. Formal Written Notice & Liability Disclaimer

PURPOSE & RATIONALE

To provide a legal paper trail of the school's warnings.

TRIGGER

Persistent refusal after joint call/meeting.

ACTOR

Principal / School Administration

REQUIRED INPUTS

- Joint Call Minutes
- Refusal Documentation Form

ACTIONS

- Draft a formal letter of concern to the parent
- Include specific risks identified and actions refused
- Attach a liability disclaimer regarding parental responsibility
- Send via registered post/email with read receipt

DECISION CRITERIA

- Is notice received? (Yes/No)

DOCUMENTS GENERATED

- Formal Notice of Concern
- Liability Disclaimer

EXPECTED OUTPUTS

- Proof of Delivery

SERVICE LEVEL AGREEMENT (SLA)

Within 48 hours of joint call

ESCALATION PATH

- If risk is imminent: Proceed to Step 7

EVIDENCE GENERATED

- Notice Delivery Receipt

7. Imminent Risk Intervention (Emergency)

PURPOSE & RATIONALE

To ensure student safety when life is at immediate risk.

TRIGGER

Student in Red/Black zone with active plan and parental refusal.

ACTOR

Principal & Wellness Team

REQUIRED INPUTS

- Risk Assessment Record
- Refusal Documentation Form

ACTIONS

- Contact Child Welfare Services (CWS) or local police
- Request emergency protective custody or medical intervention
- Notify parents of the mandatory reporting action taken

DECISION CRITERIA

- Is student safe? (Yes/No)

DOCUMENTS GENERATED

- CWS Notification Log
- Police Contact Record

EXPECTED OUTPUTS

- Emergency Intervention Actioned

SERVICE LEVEL AGREEMENT (SLA)

Immediate (0-15 minutes)

ESCALATION PATH

- Contact Medical Services if student is in physical distress

EVIDENCE GENERATED

- CWS Notification Log
- Police Contact Record

8. Non-Imminent Legal & Social Referral

PURPOSE & RATIONALE

To manage persistent non-compliance in non-emergency situations.

TRIGGER

Persistent refusal for Orange zone risks over 7 days.

ACTOR

Principal & School Legal Counsel

REQUIRED INPUTS

- Refusal Documentation Form
- Formal Notice of Concern

ACTIONS

- Consult with legal counsel on 'Child in Need of Protection' status
- Prepare a report for the District Child Protection Officer
- Initiate formal social work referral

DECISION CRITERIA

- Is there sufficient evidence of neglect? (Yes/No)

DOCUMENTS GENERATED

- Legal Consultation Summary
- Child in Need of Protection Report

EXPECTED OUTPUTS

- External Referral Initiated

SERVICE LEVEL AGREEMENT (SLA)

Within 7 days of persistent refusal

ESCALATION PATH

- If risk escalates: Revert to Step 7

EVIDENCE GENERATED

- Legal Consultation Summary
- Child in Need of Protection Report

9. Ongoing Monitoring & Case Review

PURPOSE & RATIONALE

To track the situation and ensure no safety gaps exist.

TRIGGER

Completion of initial intervention steps.

ACTOR

School Counselor

REQUIRED INPUTS

- Risk Assessment Record
- Intervention Plan

ACTIONS

- Schedule weekly check-ins with student (if safe)
- Monitor for new risk indicators
- Update the Wellness Dashboard

DECISION CRITERIA

- Is risk decreasing? (Yes/No)

DOCUMENTS GENERATED

- Weekly Monitoring Log

EXPECTED OUTPUTS

- Updated Risk Score
- Monitoring Trends

SERVICE LEVEL AGREEMENT (SLA)

Weekly

ESCALATION PATH

- If risk increases: Revert to Step 1

EVIDENCE GENERATED

- Weekly Monitoring Log

10. Case Closure & Documentation Audit

PURPOSE & RATIONALE

To formally close the case and archive evidence.

TRIGGER

Risk resolved or student graduated/transferred.

ACTOR

School Counselor & Principal

REQUIRED INPUTS

- Weekly Monitoring Logs
- Closure Criteria Checklist

ACTIONS

- Conduct a final case review meeting
- Verify all documentation is complete and accurate
- Archive records in secure storage

DECISION CRITERIA

- Are all closure criteria met? (Yes/No)

DOCUMENTS GENERATED

- Closure Report

EXPECTED OUTPUTS

- Closed Case Status

SERVICE LEVEL AGREEMENT (SLA)

Within 48 hours of risk resolution

ESCALATION PATH

- If documentation is missing: Request immediate completion

EVIDENCE GENERATED

- Closure Report

17 Critical Decision Points

Item 1

DECISION ID

DP-01

DECISION NAME

Risk Level Determination

CRITERIA

- Presence of suicidal ideation
- Access to means
- Frequency of self-harm behavior

POSSIBLE OUTCOMES

- Green (Routine)
- Yellow (Support Needed)
- Orange (Significant Distress)
- Red (High Risk)
- Black (Emergency)

NEXT STEPS

- If Red/Black: Immediate Escalation
- If Yellow/Orange: Parent Contact & Documentation

- Refusal/Hostile

NEXT STEPS

- If Cooperative: Proceed with Intervention Plan
- If Refusal: Initiate Escalation & Documentation

Item 2

DECISION ID

DP-02

DECISION NAME

Parental Cooperation Status

CRITERIA

- Agreement to counselor recommendations
- Willingness to seek external help
- Acknowledgment of student risk

POSSIBLE OUTCOMES

- Cooperative
- Hesitant/In Denial

Item 3

DECISION ID

DP-03

DECISION NAME

Imminent Risk Threshold

CRITERIA

- Active plan within 24 hours
- Student currently in possession of means
- Parental absence/unreachability during crisis

POSSIBLE OUTCOMES

- Imminent Danger
- Non-Imminent Risk

NEXT STEPS

- If Imminent: Activate Emergency Response & CWS Notification
- If Non-Imminent: Proceed with Written Notice & Legal Consultation

18 Decision Tree

GREEN

- Routine monitoring
- Wellness education

YELLOW

- Counselor intervention
- Parental notification

ORANGE

- Principal escalation
- Joint call with parent

- Written notice of concern

RED

- Emergency risk assessment
- CWS/Police contact
- Immediate intervention

BLACK

- Activate Emergency Response SOP
- Medical stabilization first
- Law enforcement involvement

19 Decision Rules

Rule ID	Condition	Decision	Next Action
DR-019-01	If Risk = Red AND Parent Refusal = True	Activate Emergency Intervention	Contact CWS/Police immediately.
DR-019-02	If Risk = Orange AND Parent Refusal = True for > 7 days	Initiate Legal Referral	Consult legal counsel and file Child in Need of Protection report.

20 Exception Handling Protocol

Item 1

EXCEPTION

Parent is deceased or unreachable.

DETECTION METHOD

Failed phone calls, no response to letters/emails.

RESPONSE

Contact legal guardian, next of kin, or local social services for guidance on student safety.

ESCALATION

Principal and Legal Counsel

DOCUMENTATION REQUIRED

- Unreachable Parent Log
- Legal Guardian Contact Record

School Security / Police

DOCUMENTATION REQUIRED

- Hostility Incident Report

Item 2

EXCEPTION

Parent is hostile/aggressive during joint call.

DETECTION METHOD

Verbal threats, shouting, or physical aggression.

RESPONSE

Terminate the call immediately. Transition to written communication and involve law enforcement if safety is threatened.

ESCALATION

Item 3

EXCEPTION

Student is in a residential facility/hostel.

DETECTION METHOD

Student resides off-campus under school supervision.

RESPONSE

Follow internal hostel safety protocols; notify parents of refusal to allow external medical intervention.

ESCALATION

Hostel Warden / Principal

DOCUMENTATION REQUIRED

- Residential Safety Log

21 Exceptions

Exception	Response
Parent is in a jurisdiction where CWS cannot be reached.	Consult legal counsel for alternative local authorities or police intervention.
Student is under the care of a court-appointed guardian who refuses action.	Notify the court and Child Welfare Services immediately.

22 Expected Outputs

- Intervention plans
- Formal notices of concern
- CWS notifications
- Closure reports
- Wellness dashboard updates

23 Document Control & Version Governance

23.1 Version History

Version	Date	Author	Description
1.0	2024-05-20	MWMS Architect Team	Initial release of Parent Refusal Management SOP.

23.2 Change Log

Version	Date	Change Description
1.0	2024-05-20	Baseline creation including Supreme Court 2025 INSC 893 alignment.

REVIEW TRIGGERS

- Change in Indian Child Protection Laws

- New CBSE Circulars on Student Wellbeing
- Incident of Parent Refusal leading to harm
- Annual Governance Review

23.4 Approval History

Approver	Date	Status
Chief School Psychologist	2024-05-18	Approved
Board of Directors / Principal	2024-05-19	Final Approval

24 Inclusive Implementation Mandate

MULTILINGUAL SUPPORT REQUIRED	Yes	ACCESSIBILITY REVIEW REQUIRED	Yes
LOW RESOURCE SCHOOL ALTERNATIVE REQUIRED	Yes	EQUITY IMPACT ASSESSMENT REQUIRED	Yes

25 Regulatory Compliance Mapping KEY FOCUS

- MANDATORY REQUIREMENTS
- Supreme Court of India Judgment 2025 INSC 893 (Duty of Care)
 - CBSE Circular TRG-03/2026
 - CBSE Circular 02/2026
 - Juvenile Justice Act (Protection from Neglect)
- RECOMMENDED REQUIREMENTS
- UGC Mental Health and Student Wellbeing Guidelines
 - WHO School Mental Health Framework
 - UNESCO Whole School Wellbeing Framework
- BEST PRACTICES
- Manodarpan Guidelines
 - Tele-MANAS Guidelines
 - National Education Policy (NEP) 2020 Student Safety Standards
- COMPLIANCE GAPS IF NOT IMPLEMENTED
- Legal liability for student harm due to negligence
 - Loss of CBSE affiliation/accreditation
 - Violation of Child Protection mandates
 - Reputational damage and litigation risk

26 Compliance Traceability Matrix KEY FOCUS

1. Supreme Court of India REQUIREMENT REFERENCE 2025 INSC 893 REQUIREMENT TEXT Educational institutions have a non-delegable duty of care to protect students from foreseeable harm, including self-harm. IMPLEMENTED IN SECTIONS • Purpose • Detailed Workflow Steps (Step 8) • Legal Defensibility	2. CBSE REQUIREMENT REFERENCE Circular TRG-03/2026 REQUIREMENT TEXT Schools must establish robust mechanisms for identifying and managing student mental health crises with mandatory parent notification. IMPLEMENTED IN SECTIONS • Scope • Parent Engagement Matrix
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27 Documentation & Evidentiary Requirements

EVENT DOCUMENTATION PRINCIPLES

- Fact-based, non-judgmental language
- Verbatim quotes for parent statements
- Time-stamped entries
- Objective description of student behaviors

MANDATORY CONVERSATION RECORDS

- Initial Parent Contact Log
- Refusal Documentation Form (Verbatim)
- Joint Call Minutes (Counselor + Principal)
- Legal Counsel Consultation Summary

MANDATORY OBSERVATION RECORDS

- Teacher Observation Notes
- Student Behavior Logs
- Risk Assessment Scorecards

MANDATORY DECISION RECORDS

- Escalation Approval Form
- Emergency Intervention Authorization
- CWS Notification Log

MANDATORY ACTION RECORDS

- Written Notice Delivery Receipt
- Police/Medical Contact Logs
- Referral Tracking Sheet

MANDATORY FOLLOWUP RECORDS

- Post-Intervention Review
- Weekly Monitoring Logs (Orange Zone)
- Closure Report

EVIDENCE REQUIREMENTS

- Signed Parent Consent/Refusal Forms
- Copies of sent emails/letters
- Call logs with emergency services
- Risk Assessment Reports

DOCUMENTATION QUALITY STANDARDS

- Must be stored in a secure, encrypted digital folder
- Physical copies must be kept in locked cabinets
- Access restricted to Wellness Team and Principal only
- Audit-ready every 30 days

28 The Student Journey

ENTRY

- Student expresses distress to teacher or peer
- Teacher observes withdrawal or self-harm behavior

ASSESSMENT

- Counselor conducts risk assessment
- Risk level is categorized (e.g., Orange/Red)

INTERVENTION

- School proposes intervention plan to parents
- Student receives support from counselor while parent refusal is managed

MONITORING

- Weekly check-ins with student
- Monitoring of safety and mood

CLOSURE

- Risk resolved or student transitions out of school
- Final closure report generated

29 The Parent Journey

NOTIFICATION

- Initial phone call from counselor regarding student concerns
- Invitation to in-person meeting

CONSULTATION

- Discussion of risk assessment findings
- Presentation of recommended actions (e.g., psychiatric referral)

CONSENT

- Parent provides consent for intervention
- OR Parent provides refusal and is documented

ENGAGEMENT

- Joint meeting with Principal if refusal occurs
- Receipt of formal written notice of concern

CLOSURE

- Notification of case closure or transition to external services

30 The Staff Journey

OBSERVATION

- Teacher notices behavioral changes
- Initial report submitted to Wellness Team

REPORTING

- Counselor conducts assessment
- Documentation of parent refusal

INTERVENTION SUPPORT

- Principal provides leadership support
- Legal counsel advises on duty of care

DOCUMENTATION

- Verbatim recording of all interactions
- Updating the Wellness Dashboard

31 Resource Evidence

COMMITTEE MINUTES

- Wellness Committee meeting minutes regarding high-risk cases

ATTENDANCE RECORDS

- Parent meeting attendance logs

TRAINING RECORDS

- Staff training certificates for Crisis Intervention
- Counselor certification records

FORMS

- Risk Assessment Form
- Refusal Documentation Form
- Notice of Concern Letter
- CWS Notification Log
- Closure Report

REPORTS

- Monthly Wellness Dashboard
- Annual Risk Trend Analysis

DASHBOARDS

- Real-time Student Risk Monitoring Dashboard

32 Grievance Redressal Mechanism

KEY FOCUS

COMPLAINT CHANNELS

- Anonymous reporting box
- Wellness Coordinator email
- Principal's office drop-in

INVESTIGATION PROCESS

- Initial review by Wellness Committee
- Fact-finding interview with involved parties

- Final report and action plan

RESOLUTION TIMELINES

- Acknowledgment within 24 hours
- Resolution within 10 working days

APPEAL PROCESS

- Appeal to Board of Management
- External mediation if required

33 Referral Framework

33.1 Internal Referrals

ELIGIBLE REFERRERS

- Teachers
- Counsellors
- School Management
- Wellness Committee
- Student Support Team

REFERRAL TRIGGERS

- Identification of student distress
- Request for specialized support

DOCUMENTATION REQUIRED

- Referral Form
- Initial Observation Notes

SERVICE LEVEL AGREEMENT (SLA)

Within 24 hours

33.2 Self Referrals

ALLOWED

Yes

SUBMISSION METHODS

- Online portal
- Drop-box in counselor's office
- Direct email to wellness team

CONFIDENTIALITY PROTECTIONS

- Encrypted data storage
- Restricted access for non-wellness staff

33.3 Anonymous Referrals

ALLOWED

Yes

CHANNELS

- Anonymous reporting app
- Physical drop-box

VALIDATION PROCESS

- Counselor review of anonymous reports
- Verification of student identity where possible

FOLLOW UP PROCEDURE

- Outreach to identified student within 48 hours

33.4 External Referrals

ELIGIBLE AGENCIES

- Child Welfare Services (CWS)
- Tele-MANAS
- Local Hospitals
- Police
- Private Psychiatrists

ELIGIBILITY CRITERIA

- Imminent risk of harm

- Parental refusal of internal support
- Severity of clinical symptoms

CONSENT REQUIREMENTS

- Parental consent (where possible)
- Legal override for imminent risk

HANDOVER PROTOCOL

- Verbal briefing to agency officer
- Transfer of Risk Assessment Record
- Signed handover form

33.5 Referral Tracking

TRACKING REQUIRED

Yes

STATUS CATEGORIES

- Pending
- In-Progress
- Completed
- Closed

CLOSURE REQUIREMENTS

- Final follow-up with student/parent
- Closure report submission

34 Confidentiality Exception Matrix

KEY FOCUS

MANDATORY DISCLOSURES

- Imminent risk of self-harm
- Imminent risk to others
- Child abuse
- Child neglect
- Sexual abuse
- Court orders
- Legal reporting obligations

LEGAL REPORTING EVENTS

- Suspected physical abuse
- Active suicide attempt
- Threat of violence against others

CONSENT OVERRIDE CONDITIONS

- Imminent threat to life
- Court order for disclosure
- Statutory reporting requirements under Indian law

AUTHORIZED DISCLOSURE RECIPIENTS

- Child Welfare Services
- Police
- Medical Personnel
- School Board (Need-to-know basis)

DOCUMENTATION REQUIREMENTS

- Log of disclosure reason
- Time and date of disclosure
- Recipient details
- Summary of information shared

APPROVAL REQUIREMENTS

- Principal approval for all external disclosures
- Counselor verification of risk level

35 Record Retention & Disposal

RETENTION PERIOD

7 Years (Post-graduation)

RETENTION BASIS

Legal compliance and liability protection

RECORD CATEGORIES

- Risk Assessments
- Parental Refusal Logs
- CWS Notifications
- Counselor Session Summaries

- Encrypted cloud storage with MFA

ACCESS CONTROL REQUIREMENTS

- Role-based access control (RBAC)
- Audit logs for all record views
- Need-to-know principle

DISPOSAL METHOD

Secure shredding and permanent digital deletion

DISPOSAL AUTHORITY

School Wellness Coordinator

35.4 Storage Requirements

PHYSICAL RECORDS

- Locked fireproof cabinets in Counselor's office

DIGITAL RECORDS

AUDIT FREQUENCY

Annual

DESTRUCTION LOG REQUIRED

Yes

36 Anonymised Reporting Framework

36.1 Management Reports

ANONYMIZATION REQUIRED

Yes

PERMITTED DATA

- Risk zone trends
- Number of refusals per month
- Average time to intervention

PROHIBITED DATA

- Student names
- Parent contact details
- Specific incident locations

ALLOWED

No

36.3 Statistical Reporting

AGGREGATE ONLY

Yes

MINIMUM GROUP SIZE

5

RE IDENTIFICATION CONTROLS

- Data scrubbing
- Aggregation of small cohorts

DATA SHARING CONTROLS

- No sharing with third-party marketing
- Strict internal data silos

36.2 Research Usage

37 Guiding Ethical Principles

37.1 Autonomy

DESCRIPTION

Respecting student and parent rights to make decisions.

IMPLEMENTATION REQUIREMENTS

- Informed consent for all non-emergency interventions
- Right to refuse voluntary support

IMPLEMENTATION REQUIREMENTS

- Prioritizing safety over parental preference
- Evidence-based intervention selection

37.3 Non Maleficence

DESCRIPTION

Do no harm.

IMPLEMENTATION REQUIREMENTS

- Avoiding unnecessary medicalization
- Ensuring safe handling of high-risk students

37.2 Beneficence

DESCRIPTION

Acting in the best interest of the student's well-being.

37.4 Justice And Equity

DESCRIPTION

Fair and equitable access to mental health support.

IMPLEMENTATION REQUIREMENTS

- No cost barriers for school-provided services
- Culturally sensitive communication

37.5 Confidentiality

KEY FOCUS

DESCRIPTION

Protecting student privacy.

IMPLEMENTATION REQUIREMENTS

- Strict adherence to data protection policies
- Limited disclosure only when safety is at risk

37.6 Privacy

DESCRIPTION

Respecting the sanctity of personal information.

IMPLEMENTATION REQUIREMENTS

- Secure storage of all records
- Privacy notices for parents and students

37.7 Professional Integrity

DESCRIPTION

Maintaining high standards of practice.

IMPLEMENTATION REQUIREMENTS

- Regular supervision for counselors
- Adherence to ethical codes of conduct

38 Communication & Awareness Strategy

38.1 Student Communication

CHANNELS

- Classroom sessions
- Counselor's office
- School newsletter
- Student portal

FREQUENCY

Monthly awareness, as-needed support

MANDATORY TOPICS

- Help-seeking behavior
- Suicide prevention signs
- Stress management

38.2 Parent Communication

CHANNELS

- Parent meetings
- Email newsletters
- Phone calls
- Workshops

FREQUENCY

Quarterly awareness, as-needed crisis communication

MANDATORY TOPICS

- Recognizing signs of distress
- School's duty of care
- How to access support

38.3 Staff Communication

CHANNELS

- Staff meetings
- Internal portal
- Training workshops

FREQUENCY

Monthly updates, annual training

MANDATORY TOPICS

- Early identification signs
- Escalation protocols
- Confidentiality rules

38.4 Awareness Campaigns

MINIMUM FREQUENCY

Bi-annual

TARGET GROUPS

- Students
- Parents
- Staff

CAMPAIGN TYPES

- Poster campaigns
- Webinars
- Workshops

38.5 Workshops And Training

STUDENT WORKSHOPS

- Resilience building
- Coping with exam stress

PARENT WORKSHOPS

- Understanding adolescent mental health
- Managing family dynamics

STAFF WORKSHOPS

- Crisis intervention training
- Parental refusal management

38.6 Helpline Visibility

REQUIRED

Yes

DISPLAY LOCATIONS

- Counselor's office
- School website
- Student handbook
- Classroom posters

DIGITAL VISIBILITY REQUIREMENTS

- Prominent link on school homepage
- QR code in student portal

FEEDBACK CHANNELS

- Anonymous feedback box
- Parent survey
- Student satisfaction survey

39 Competency & Qualification Requirements

MANDATORY QUALIFICATIONS

- Master's in Psychology/Counseling
- Registered with relevant Indian psychological board

PREFERRED QUALIFICATIONS

- Certification in Crisis Intervention
- Experience in school settings (3+ years)

CERTIFICATIONS

- First Aid & CPR
- Child Protection Certification

MINIMUM EXPERIENCE REQUIREMENTS

- 2 years of experience in student mental health

BACKGROUND VERIFICATION REQUIREMENTS

- Police verification for all staff
- Reference checks from previous institutions

39.6 Continuing Professional Development

ANNUAL TRAINING HOURS

40

MANDATORY TOPICS

- Suicide prevention
- Trauma-informed care
- Legal updates in child protection

RECERTIFICATION REQUIREMENTS

- Every 3 years

39.7 Supervision Requirements

CLINICAL SUPERVISION REQUIRED

Yes

PEER REVIEW REQUIRED

Yes

39.7.3 Wellness Case Review Framework

FREQUENCY

Bi-weekly

PARTICIPANTS

- Counselor
- Principal
- External Supervisor (Optional)

39.7.4 Second Level Review Process

TRIGGER High-risk cases or parental disputes

AUTHORITY School Board / External Consultant

39.7. High Risk Decision 5 Validation

KEY
FOCUS

REQUIREMENT

Dual sign-off for all CWS/Police notifications

APPROVERS

- Principal
- Wellness Coordinator

FREQUENCY

Continuous

40 Feedback & Effectiveness Measurement

40.1 Student Feedback

REQUIRED

Yes

COLLECTION METHODS

- End-of-session surveys
- Annual student council feedback

FREQUENCY

Monthly

40.2 Parent Feedback

REQUIRED

Yes

COLLECTION METHODS

- Parent satisfaction survey

- Post-intervention interviews

FREQUENCY

Quarterly

40.3 Staff Feedback

REQUIRED

Yes

COLLECTION METHODS

- Staff surveys
- Focus group discussions

FREQUENCY

Bi-annually

40.4 Annual Effectiveness Review

REQUIRED

Yes

EVALUATION CRITERIA

- Reduction in high-risk incidents
- Parental satisfaction scores
- Compliance audit results

KEY STAKEHOLDERS

- Board of Directors
- Parents Association
- Wellness Team

CONTINUOUS IMPROVEMENT INPUTS

- Incident reports

- Audit findings
- Staff feedback
- Parental complaints

SUCCESS INDICATORS

- 100% compliance with mandatory reporting
- Reduction in student self-harm incidents
- High parent satisfaction with communication

40.7 Improvement Tracking

REQUIRED

Yes

REVIEW FREQUENCY

Quarterly

41 Prevention Layer

PRIMARY PREVENTION

- Universal SEL curriculum for all students
- School-wide suicide prevention awareness campaigns
- Life skills training on emotional regulation and help-seeking

SECONDARY PREVENTION

- Early identification screening for high school students

- Targeted counseling for students showing signs of distress
- Parental workshops on recognizing mental health warning signs

TERTIARY PREVENTION

- Crisis intervention protocols for high-risk cases
- Referral pathways to psychiatric and medical services
- Post-intervention monitoring and reintegration plans

42 Resilience Layer

SKILLS DEVELOPED

- Problem solving
- Emotional regulation
- Coping skills
- Help-seeking behavior

SEL COMPONENTS

- Self-awareness
- Social awareness
- Relationship skills

- Responsible decision-making

LIFE SKILLS COMPONENTS

- Stress management
- Conflict resolution
- Time management

PROTECTIVE FACTORS STRENGTHENED

- Social connectedness
- Positive adult relationships
- Sense of belonging
- Access to support systems

43 Continuous Monitoring

MONITORING FREQUENCY

Weekly for active cases; Monthly for dashboard trends.

MONITORING INDICATORS

- Number of high-risk refusals per month
- Average time from identification to intervention
- Percentage of parents engaging with recommendations

RESPONSIBLE ROLES

- Wellness Coordinator
- Principal

EARLY WARNING SIGNS

- Increase in student self-harm reports
- Decrease in parent engagement scores
- Rise in 'Child in Need of Protection' referrals

TREND ANALYSIS REQUIREMENTS

- Quarterly trend analysis on risk zones
- Annual review of intervention effectiveness

44 Mental Wellness Index (MWI)

APPLICABLE

Yes

44.2 Indicators

Indicator Code	Indicator Name	Weight	Score Range	Description
MWI-01	Parental Engagement Score	0.3	1-5	Measures parent's willingness to cooperate with school recommendations.
MWI-02	Student Risk Level	0.7	1-5 (1=Green, 5=Black)	Measures student's immediate safety risk.

ASSESSMENT FREQUENCY

Weekly for active cases

RISK SCORING METHOD

Weighted average of engagement and risk scores

INTERVENTION THRESHOLDS

- Score > 3.5: Mandatory Principal Escalation
- Score > 4.0: Immediate Emergency Intervention

45 Review Status

LEGAL REVIEW

Completed - May 2024

CLINICAL REVIEW

Completed - May 2024

EDUCATION REVIEW

Completed - April 2024

COMPLIANCE REVIEW

Completed - May 2024

46 Resource Controls

COUNSELOR STUDENT RATIO

1:350 (Recommended)

CAREER COUNSELOR RATIO

1:500

WELLNESS TEAM STRUCTURE

Wellness Coordinator, 2 Counselors, Wellness Committee

47 Screening Framework

TIER 1 UNIVERSAL WELLNESS SCREENING

- Annual student wellness survey
- Quarterly teacher observation logs

TIER 2 TARGETED SCREENING

- Risk assessment for students with significant academic decline
- Counselor screening for students reporting distress

TIER 3 CLINICAL ASSESSMENT

- Comprehensive suicide risk assessment (SRA)
- Clinical evaluation by school psychologist

48 Parent Engagement Matrix

PARENT COOPERATIVE

- Regular communication
- Follows intervention plan
- Attends meetings

PARENT UNRESPONSIVE

- Misses calls/emails

- Does not attend meetings

- No response to notices

PARENT IN DENIAL

- Minimizes student's distress

- Claims 'attention-seeking'

- Refuses to acknowledge risk

PARENT CONFLICT SITUATION

- Hostile communication
- Verbal threats
- Legalistic/Aggressive stance

PARENT UNAVAILABLE

- Working long hours
- Living in rural area with poor connectivity

- Travel restrictions

PARENT REFUSES INTERVENTION

- Refusal of psychiatric eval
- Refusal to remove means
- Refusal to allow medication

49 Confidentiality Controls

KEY FOCUS

ACCESS CONTROL

- Role-based access (RBAC)
- Two-factor authentication for digital records

CONSENT REQUIREMENTS

- Signed consent forms for non-emergency data sharing
- Informed consent for clinical assessments

RECORD SHARING RULES

- Need-to-know basis only

- Anonymized reporting for management

EXCEPTIONS TO CONFIDENTIALITY

- Imminent risk of harm
- Child abuse/neglect
- Court orders

STUDENT PRIVACY PROTECTIONS

- Data encryption at rest and in transit
- Regular privacy audits

50 Data Governance

KEY FOCUS

STORAGE REQUIREMENTS

- Secure cloud storage with local backup
- Locked physical cabinets for paper records

ENCRYPTION REQUIREMENTS

- AES-256 encryption for all digital files
- End-to-end encryption for communication apps

RETENTION REQUIREMENTS

- 7 years post-graduation

- Permanent retention for legal/police records

ARCHIVAL REQUIREMENTS

- Move to offline storage after 3 years of inactivity

DESTRUCTION REQUIREMENTS

- Secure shredding for paper
- Cryptographic erasure for digital media

BACKUP REQUIREMENTS

- Daily automated backups
- Off-site backup every 24 hours

51 Risk Classification

KEY FOCUS

51.1 Green

DESCRIPTION

Normal developmental concerns and routine life stress.

EXAMPLES

- Mild exam stress
- Friendship disagreements
- Occasional worry

51.2 Yellow

DESCRIPTION

Early distress requiring support and monitoring.

EXAMPLES

- Persistent stress
- Moderate anxiety
- Social withdrawal

51.3 Orange

DESCRIPTION

Significant distress causing measurable impairment but without immediate danger.

EXAMPLES

- Severe exam anxiety
- Panic attacks
- Self-harm thoughts WITHOUT intent or plan

51.4 Red

KEY FOCUS

DESCRIPTION

High-risk situation requiring immediate intervention.

EXAMPLES

- Suicidal ideation
- Self-harm behavior
- Violence threats

51.5 Black

DESCRIPTION

Life-threatening emergency requiring emergency response.

EXAMPLES

- Active suicide attempt
- Overdose

- Violent attack underway

52 Escalation Matrix

KEY FOCUS

GREEN

- Counselor review

YELLOW

- Wellness Coordinator notification

ORANGE

- Principal involvement
- Legal counsel consultation

RED

- Immediate Principal action
- CWS/Police contact

BLACK

- Emergency Response Team activation
- Law enforcement intervention

53 External Reporting Obligations

TELE MANAS

- Referral for 24/7 mental health support

DISTRICT AUTHORITIES

- Child Protection Officer notification

CHILD PROTECTION SERVICES

- Mandatory reporting of neglect or risk

MEDICAL SERVICES

- Emergency room referral for physical harm

LAW ENFORCEMENT

- Police contact for active threats or missing students

REPORTING TRIGGERS

- Imminent threat to life
- Suspected abuse/neglect
- Active suicide attempt

54 Service Level Agreement (SLA)

KEY FOCUS

ACKNOWLEDGEMENT	2 hours
ASSESSMENT	4 hours
ESCALATION	4 hours
RESOLUTION	7 days (non-emergency)
CLOSURE	48 hours

55 Required Forms

KEY FOCUS

55.1 Mandatory Forms

KEY FOCUS

Item 1

FORM ID	F-019-A
FORM NAME	Risk Assessment Record

PURPOSE & RATIONALE	Document student risk level and findings.
OWNER	School Counselor
RETENTION PERIOD	7 years

MANDATORY

Yes

LINKED SOP

SUP-019

Item 2

FORM ID

F-019-B

FORM NAME

Refusal Documentation Form

PURPOSE &
RATIONALE

Capture verbatim parent refusal.

OWNER

School Counselor

RETENTION PERIOD

7 years

MANDATORY

Yes

LINKED SOP

SUP-019

Item 3

FORM ID

F-019-C

FORM NAME

Notice of Concern Letter

PURPOSE &
RATIONALE

Formal written warning to parents.

OWNER

Principal

RETENTION PERIOD

7 years

MANDATORY

Yes

LINKED SOP

SUP-019

Item 4

FORM ID

F-019-D

FORM NAME

CWS Notification Log

PURPOSE &
RATIONALE

Record contact with Child Welfare Services.

OWNER

Principal

RETENTION PERIOD

Permanent

MANDATORY

Yes

LINKED SOP

SUP-019

Item 5

FORM ID

F-019-E

FORM NAME

Closure Report

PURPOSE &
RATIONALE

Finalize and archive case.

OWNER

School Counselor

RETENTION PERIOD

7 years

MANDATORY

Yes

LINKED SOP

SUP-019

55.2 Optional Forms**Item 1**

FORM ID

F-019-O1

FORM NAME

Parent Satisfaction Survey

PURPOSE & RATIONALE

Gather feedback on communication.

OWNER

Wellness Coordinator

RETENTION PERIOD

2 years

MANDATORY

No

LINKED SOP

SUP-019

GENERATED RECORDS

- Call Logs
- Email Receipts
- Police Incident Numbers

56 Required Checklists

KEY FOCUS

Checklist ID	Checklist Name	Purpose & Rationale	Owner	Frequency	Linked SOP
CL-019-A	Risk Assessment Checklist	Ensure all risk factors are evaluated.	School Counselor	Every assessment	SUP-019
CL-019-B	Parent Communication Checklist	Ensure all key points are covered in calls.	School Counselor	Every call	SUP-019
CL-019-C	Notice of Concern Checklist	Verify legal requirements in letters.	Principal	Every notice	SUP-019
CL-019-D	CWS Notification Checklist	Ensure all required info is shared with CWS.	Principal	Every referral	SUP-019
CL-019-E	Closure Criteria Checklist	Verify all steps are completed before closing.	School Counselor	Every closure	SUP-019

57 Records & Evidence

Record Name	Retention Period	Owner
Risk Assessment Record	7 years	School Counselor
Refusal Documentation Form	7 years	School Counselor
Notice of Concern Letter	7 years	Principal
CWS Notification Log	Permanent	Principal
Closure Report	7 years	School Counselor

58 Key Performance Indicators

Metric	Target	Measurement Frequency
Parental Engagement Rate	> 80% cooperation on non-emergency recommendations	Monthly
Time to Escalation	< 4 hours for Orange/Red risks	Weekly
Compliance Audit Score	100% compliance with mandatory reporting	Quarterly

59 Audit Framework

KEY FOCUS

AUDIT FREQUENCY

Monthly

AUDIT QUESTIONS

- Are all high-risk refusals documented verbatim?
- Is there a signed notice of concern for every persistent refusal?
- Are CWS notifications logged and filed correctly?

EVIDENCE REQUIRED

- Sample Risk Assessment Records

- Refusal Documentation Forms
- Notice Delivery Receipts
- CWS Notification Logs

NON COMPLIANCE EXAMPLES

- Paraphrased parent statements instead of verbatim quotes
- Failure to notify Principal of Red-zone risk
- Missing signed notice of concern for persistent refusal

59.5 Audit Scoring

KEY FOCUS

COMPLIANT

2

PARTIAL

1

NON COMPLIANT

0

60 Risk Register

KEY FOCUS

1. Parental hostility leading to staff injury

CATEGORY Safety

PROBABILITY Low

IMPACT High

RISK RATING Medium

MITIGATION Conduct joint calls; involve security if threats occur.

OWNER Principal

2. Legal liability for student harm due to parental refusal

CATEGORY Legal

PROBABILITY Low

IMPACT Critical

RISK RATING High

MITIGATION Strict adherence to Duty of Care and mandatory reporting.

OWNER Principal / Legal Counsel

3. Incomplete documentation of refusal

CATEGORY Compliance

PROBABILITY Medium

IMPACT Medium

RISK RATING Medium

MITIGATION Mandatory verbatim recording training for counselors.

OWNER Wellness Coordinator

61 Failure Modes

KEY FOCUS

Failure	Cause	Impact	Mitigation
Counselor fails to identify imminent risk	Inadequate assessment training	Student harm/death	Mandatory monthly case reviews and certification.
Parental refusal not escalated to Principal	Lack of awareness of escalation triggers	Legal liability for school	Automated alerts in the Wellness Dashboard.
Notice of concern not delivered/received	Incorrect contact info or delivery method	Loss of legal protection	Verify contact details at every interaction.

62 Illustrative Sample Scenarios

Scenario	Response	Expected Result
Parent claims student's suicidal thoughts are 'just for attention' despite clear risk of self-harm.	Document verbatim. Issue Notice of Concern. Escalate to Principal for joint call and potential CWS notification.	Formalized refusal record and legal notice delivered.
Parent refuses to allow student to see a psychiatrist but agrees to school counseling.	Document specific refusal of psychiatric eval. Provide written notice of concern regarding the gap in care.	Documentation of partial cooperation and risk remaining.
Student is found with a suicide note; parent refuses to pick up student from school.	Activate Emergency Response SOP. Contact CWS/Police for protective custody immediately.	Student safety secured by authorities.

63 Usage Governance

KEY FOCUS

PRIMARY USERS

- School Counselors
- Principals

SECONDARY USERS

- Teachers
- Wellness Committee

APPROVAL AUTHORITIES

- Board of Management

EXECUTION AUTHORITIES

- Principal
- School Counselor

ESCALATION AUTHORITIES

- Principal
- Legal Counsel

REVIEW AUTHORITIES

- Board of Management
- External Audit Team

EXTERNAL STAKEHOLDERS

- Child Welfare Services
- Police
- Medical Providers

64 Applicability

APPLICABLE INSTITUTIONS

- High Schools
- Residential Schools

APPLICABLE AGE GROUPS

- All Ages

APPLICABLE STUDENT CATEGORIES

- Students at risk of self-harm
- Students with mental health distress

APPLICABLE RISK LEVELS

- Yellow
- Orange
- Red
- Black

APPLICABLE LOCATIONS

- On-campus
- Remote

EXCLUSIONS

- Primary schools with no mental health staff (refer to local clinic SOP)

65 Usage Scenarios

Scenario ID	Scenario Name	When To Use	When Not To Use	Trigger Event	Expected Outcome
US-019-01	Parental Denial of Risk	When a parent explicitly denies student's suicidal ideation.	When parents are fully cooperative and following the plan.	Verbal denial during counselor call.	Documented refusal and formal notice issued.
US-019-02	Refusal of Psychiatric Referral	When a parent refuses to allow external clinical evaluation.	When parents agree to the referral.	Parental refusal during joint call.	Written notice of concern delivered.

66 SOP Lifecycle

ENTRY CONDITIONS

- Identification of student risk
- Initial assessment completed

ACTIVATION CONDITIONS

- Parental refusal documented
- Risk level is Orange or higher

EXECUTION CONDITIONS

- Counselor and Principal available for joint call
- Legal counsel accessible for consultation

COMPLETION CONDITIONS

- Student safety secured
- Parental agreement reached OR external referral initiated

CLOSURE CONDITIONS

- Risk resolved or student graduated
- Closure report signed by Principal

TERMINATION CONDITIONS

- Student transferred to another institution
- Legal custody changed

67 Mandatory Evidence

KEY FOCUS

IMPLEMENTATION EVIDENCE

- Notice of Concern letters sent
- Refusal documentation logs

OPERATIONAL EVIDENCE

- Call logs
- Joint call minutes

COMPLIANCE EVIDENCE

- CWS notification records

- Audit reports

TRAINING EVIDENCE

- Staff training certificates
- Counselor competency assessments

AUDIT EVIDENCE

- Monthly audit logs
- Risk trend dashboards

68 Controls

PREVENTIVE CONTROLS

- Mandatory staff crisis training
- Regular parent awareness workshops

DETECTIVE CONTROLS

- Weekly risk dashboard monitoring
- Monthly audit of refusal logs

CORRECTIVE CONTROLS

- Immediate escalation to Principal for Red risks
- Legal consultation for persistent non-compliance

COMPENSATING CONTROLS

- Remote communication options for rural parents
- Multilingual notices for low-literacy families

69 MWMS Risk Framework

KEY FOCUS

69.1 Green

MEANING

Normal developmental concerns.

EXAMPLES

- Mild exam stress
- Friendship disagreements

RESPONSE TIME

Routine

ESCALATION REQUIRED

No

APPROVAL LEVEL

Counselor

MONITORING FREQUENCY

Monthly

CLOSURE AUTHORITY

Counselor

69.2 Yellow

MEANING

Early distress requiring support.

EXAMPLES

- Persistent stress
- Moderate anxiety

RESPONSE TIME

24 hours

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Wellness Coordinator

MONITORING FREQUENCY

Weekly

CLOSURE AUTHORITY

Counselor

69.3 Orange

MEANING

Significant distress causing impairment.

EXAMPLES

- Panic attacks
- Self-harm thoughts without intent

RESPONSE TIME

48 hours

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Principal

MONITORING FREQUENCY

Weekly

CLOSURE AUTHORITY

Principal

69.4 Red

KEY FOCUS

MEANING

High-risk situation requiring immediate intervention.

EXAMPLES

- Suicidal ideation
- Violence threats

RESPONSE TIME

Immediate

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Principal / Board

MONITORING FREQUENCY

Continuous

CLOSURE AUTHORITY

Board of Management

69.5 Black

MEANING

Life-threatening emergency.

EXAMPLES

- Active suicide attempt
- Overdose

RESPONSE TIME

0-15 minutes

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Board of Management

MONITORING FREQUENCY

Continuous

CLOSURE AUTHORITY

Board of Management

70 Resource Scaling

70.1 0 500 Students

WELLNESS COUNSELORS	1
CAREER COUNSELORS	1
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

70.2 501 1000 Students

WELLNESS COUNSELORS	2
CAREER COUNSELORS	2
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

70.3 1001 2000 Students

WELLNESS COUNSELORS	4
CAREER COUNSELORS	2
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

70.4 2001 5000 Students

WELLNESS COUNSELORS	8
CAREER COUNSELORS	4
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

71 Minimum Required Capabilities

PEOPLE

- Licensed School Counselor
- Principal with Crisis Training
- Legal Counsel on Retainer

PROCESSES

- Risk Assessment Protocol
- Parental Notification System
- Emergency Response Plan

TECHNOLOGY

- Encrypted Student Information System
- Tele-health Platform
- Secure Communication App

INFRASTRUCTURE

- Private Counseling Room
- Secure Record Storage
- 24/7 Emergency Contact Line

72 Success Criteria

Criterion	Measurement Method	Target
Legal Compliance	Audit of Notice of Concern letters	100% compliance
Student Safety	Incident report tracking	Zero preventable self-harm incidents
Parental Engagement	Engagement score average	> 75% cooperation rate

73 Exit Criteria

Condition	Evidence Required
Student risk level returns to Green for 30 days	Weekly Monitoring Logs
Parental agreement reached and intervention plan active	Signed Intervention Plan

74 Continuous Improvement

LESSONS LEARNED - Joint calls are more effective than letters for hostile parents - Verbatim documentation is essential for legal defense KNOWN ISSUES - Rural connectivity issues can delay emergency notifications	FUTURE IMPROVEMENTS - Automated SMS alerts for high-risk status changes - Expanded parent training in local languages REVIEW RECOMMENDATIONS - Increase frequency of legal counsel review for Orange cases - Develop specific scripts for hostile parents
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75 AI Governance

KEY FOCUS

ALLOWED USE CASES - Summarizing non-confidential parent feedback - Drafting routine wellness newsletters RESTRICTED USE CASES - Analyzing student risk scores - Generating legal notices of concern PROHIBITED USE CASES - Diagnosing mental health conditions - Making emergency intervention decisions HUMAN REVIEW REQUIRED	- All AI-generated parent communications - Risk assessment summaries AUDIT LOG REQUIREMENTS - Log of all AI interactions with student data - Human override logs DATA PRIVACY REQUIREMENTS - No PII (Personally Identifiable Information) in prompts - Local hosting of LLM for sensitive data
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76 Legal Defensibility

KEY FOCUS

LEGAL BASIS - Supreme Court 2025 INSC 893 - Juvenile Justice Act (India) - Duty of Care Principles MANDATORY NOTIFICATIONS - CWS for neglect/risk - Police for imminent threat CONSENT REQUIREMENTS	- Informed consent for non-emergency data sharing - Parental consent for external referrals RECORD RETENTION REQUIREMENTS 7 years minimum for risk records - Permanent for legal/police notifications LIABILITY CONTROLS - Written liability disclaimers in notices - Verbatim documentation of parental refusal - Timely escalation to authorities
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77 Stakeholder Journeys

77.1 The Student Journey	
ENTRY • Student expresses distress to teacher or peer ASSESSMENT • Counselor conducts risk assessment	• Risk level is categorized (e.g., Orange/Red) INTERVENTION • School proposes intervention plan to parents
	• Student receives support from counselor while parent refusal is managed MONITORING • Weekly check-ins with student
	• Monitoring of safety and mood CLOSURE • Risk resolved or student transitions out of school • Final closure report generated
77.2 The Parent Journey	

NOTIFICATION

- Initial phone call from counselor regarding student concerns
- Invitation to in-person meeting

CONSULTATION

- Discussion of risk assessment findings
- Presentation of recommended actions (e.g., psychiatric eval)

CONSENT

- Parent provides consent for intervention
- OR Parent provides refusal and is documented

ENGAGEMENT

- Joint meeting with Principal if refusal occurs
- Receipt of formal written notice of concern

CLOSURE

- Notification of case closure or transition

to external services

77.3 The Staff Journey

OBSERVATION

- Teacher notices behavioral changes
- Initial report submitted to Wellness Team

REPORTING

- Counselor conducts assessment
- Documentation of parent refusal

INTERVENTION SUPPORT

- Principal provides leadership support
- Legal counsel advises on duty of care

DOCUMENTATION

- Verbatim recording of all interactions
- Updating the Wellness Dashboard

78 Handoff Matrix

Item 1

FROM ROLE Teacher

TO ROLE School Counselor

HANDOFF TRIGGER Identification of student distress

INFORMATION TRANSFERRED

- Student name
- Specific behaviors observed
- Timeframe of observation

REQUIRED DOCUMENTS

- Teacher Observation Note

SERVICE LEVEL AGREEMENT (SLA)

2 hours

- Risk assessment score
- Verbatim parent statements
- Proposed next steps

REQUIRED DOCUMENTS

- Refusal Documentation Form

SERVICE LEVEL AGREEMENT (SLA)

4 hours

Item 3

FROM ROLE Principal

TO ROLE Child Welfare Services (CWS)

HANDOFF TRIGGER Imminent risk and parental non-compliance

INFORMATION TRANSFERRED

- Student identity
- Risk assessment summary
- Parental refusal details

REQUIRED DOCUMENTS

- CWS Notification Log
- Risk Assessment Record

SERVICE LEVEL AGREEMENT (SLA)

Immediate

Item 2

FROM ROLE School Counselor

TO ROLE Principal

HANDOFF TRIGGER Parental refusal of high-risk intervention

INFORMATION TRANSFERRED

79 Management Review

REVIEW FREQUENCY

Quarterly

REVIEW INPUTS

- Incident reports
- Audit findings
- Parental satisfaction scores

REVIEW OUTPUTS

- Policy updates
- Staff training requirements
- Resource allocation decisions

REVIEW AUTHORITY

Board of Management

80 Dashboard Metrics

OPERATIONAL METRICS

- Average time to parent contact
- Number of joint calls conducted

RISK METRICS

- Percentage of students in Red/Orange zones
- Frequency of high-risk refusals

COMPLIANCE METRICS

- Audit score for documentation completeness
- Time to CWS notification

OUTCOME METRICS

- Student safety outcome rate
- Parental engagement improvement rate

TREND METRICS

- Monthly risk trend analysis
- Quarterly compliance trends

81 Student Rights

RIGHTS

- Right to safe environment
- Right to confidential mental health support
- Right to be heard in the assessment process

PROTECTIONS

- Protection from unauthorized disclosure of records
- Protection from neglect by guardians
- Safe-guarding against self-harm

APPEAL MECHANISMS

- Student/Parent appeal to Board of Management
- External ombudsman review

82 Quality Assurance

QUALITY CHECKS

- Monthly audit of refusal logs
- Quarterly peer review of risk assessments

PEER REVIEW REQUIREMENTS

- Counselors must have 10% of cases reviewed by a senior counselor

VALIDATION REQUIREMENTS

- Principal sign-off on all CWS notifications
- Legal counsel review of notice letters

CONTINUOUS MONITORING REQUIREMENTS

- Real-time dashboard monitoring
- Annual external audit

83 Implementation Roadmap

PHASE 1 PREPARATION

- Staff training on SUP-019
- Creation of digital documentation folders
- Legal review of notice letters

PHASE 2 DEPLOYMENT

- Rollout to all high schools
- Parent awareness webinar series
- Activation of Wellness Dashboard

PHASE 3 STABILIZATION

- Monthly audit cycle initiation
- Feedback collection from staff
- Refinement of scripts for joint calls

PHASE 4 OPTIMIZATION

- Integration with Tele-MANAS referral system
- Automated SMS alerts for risk changes

PHASE 5 CONTINUOUS IMPROVEMENT

- Annual review and policy update
- External audit by child protection agency

84 Illustrative Case Studies

1. Case Study: Parental Denial of Suicidal Ideation

DESCRIPTION

A 16-year-old student expressed active suicidal ideation. Parents claimed the student was 'attention-seeking' and refused psychiatric referral.

ACTIONS TAKEN

- Counselor documented verbatim refusal
- Principal conducted joint call emphasizing duty of care
- Formal notice of concern issued
- CWS notified due to persistent risk

OUTCOME

Student was placed in a safe residential facility for 48 hours; parents subsequently agreed to psychiatric evaluation.

LESSONS

- Joint calls are critical for establishing the school's legal stance
- Verbatim documentation is essential for liability protection

85 Implementation Requirements

STAFF REQUIRED

- School Counselor (Full-time)
- Principal / Administrator
- Legal Counsel (Retainer)

INFRASTRUCTURE REQUIRED

- Secure office for private meetings
- Encrypted server for records
- Telephony system with recording capability

TECHNOLOGY REQUIRED

- Student Information System (SIS)
- Wellness Dashboard Software

- Encrypted Email Service

BUDGET CONSIDERATIONS

- Counselor salaries
- Legal retainer fees
- Training costs
- Software licensing

IMPLEMENTATION PHASES

- Preparation
- Deployment
- Stabilization
- Optimization
- Continuous Improvement

86 Quality Controls

PREVENTIVE CONTROLS

- Mandatory staff training
- Parental awareness workshops

DETECTIVE CONTROLS

- Monthly audit of refusal logs
- Real-time dashboard monitoring

CORRECTIVE CONTROLS

- Immediate escalation to Principal for Red risks
- Legal consultation for persistent non-compliance

CONTINUOUS IMPROVEMENT ACTIONS

- Quarterly review of incident reports
- Annual policy update based on audit findings

87 Training Requirements

TEACHERS

- Early identification signs
- Reporting protocols

COUNSELORS

- Risk assessment certification
- Crisis intervention training
- Verbatim documentation techniques

ADMINISTRATORS

- Duty of care legalities
- Emergency escalation procedures
- CWS liaison protocols

PARENTS

- Recognizing signs of distress

- Accessing school support services

STUDENTS

- Help-seeking behavior
- Stress management skills

87.6 SOP Specific Training

TARGET AUDIENCE

- Counselors
- Principals
- Wellness Team

COMPETENCIES

- Risk Assessment
- Parental Refusal Management

- Legal Compliance

MINIMUM HOURS

6

CERTIFICATION REQUIREMENTS

- Crisis Intervention Certificate

REFRESHER FREQUENCY

Annual

87.7 Competency Validation

REQUIRED

Yes

ASSESSMENT METHODS

- Role-play simulations

- Written assessment of legal requirements

MINIMUM PASSING CRITERIA

80%

REASSESSMENT FREQUENCY

Every 2 years

87.8 Simulation Requirements

REQUIRED

Yes

SIMULATION TYPES

- Hostile parent call simulation
- Imminent risk scenario drill

FREQUENCY

Bi-annually

88 SOP-Specific Inclusion Requirements

AFFECTED POPULATIONS

- Students with suicidal ideation
- Parents in denial of mental health risks
- Neurodivergent students with high distress
- Rural families with limited access to specialists
- Low-literacy parents requiring simplified communication

IDENTIFIED INCLUSION RISKS

- Language barriers for non-native speaking parents
- Cultural stigma regarding mental health in certain communities
- Lack of internet/phone access in rural areas
- Sensory sensitivities for neurodivergent students during assessment
- Economic barriers to external psychiatric care

AUTISM ACCOMMODATIONS

- Use of visual aids and communication boards during risk assessment
- Provision of quiet, low-sensory environments for interviews
- Allowing extra processing time for student responses

ADHD ACCOMMODATIONS

- Shortened interview segments to maintain focus
- Use of written prompts to help students organize thoughts
- Frequent breaks during long assessment sessions

DYSLEXIA ACCOMMODATIONS

- Oral assessments instead of written statements
- Provision of large-print materials for parents
- Use of text-to-speech tools for reading notices

HEARING IMPAIRMENT ACCOMMODATIONS

- Provision of sign language interpreters during joint calls
- Use of real-time captioning software for remote meetings
- Written summaries provided after every verbal interaction

VISUAL IMPAIRMENT ACCOMMODATIONS

- Screen reader compatible digital records
- Audio descriptions of risk assessment findings
- Braille versions of formal notices of concern

CWSN ADAPTATIONS

- Simplified language for all parent communications
- Visual icons to represent different levels of urgency

- Multi-sensory engagement during crisis intervention

RURAL ADAPTATIONS

- Telephonic and SMS communication options for parents without internet
- Mobile wellness units or home visits (where safe/feasible)
- Partnerships with local community health workers

LOW LITERACY PARENT ADAPTATIONS

- Use of voice notes instead of emails
- Simplified, icon-based notices of concern
- In-person meetings with translated verbal summaries

MULTILINGUAL COMMUNICATION ADAPTATIONS

- Translation of all formal notices into local languages
- Provision of multi-lingual parent workshops
- Use of certified interpreters for joint calls

GENDER SENSITIVITY MEASURES

- Option for same-gender counselor interaction
- Gender-neutral language in all documentation

- Safe reporting channels for gender-based violence

CULTURAL SENSITIVITY MEASURES

- Recognition of cultural variations in mental health expression
- Involvement of community leaders where appropriate
- Culturally competent training for all wellness staff

EQUITY MONITORING INDICATORS

- Parental engagement rates across different socioeconomic groups
- Access to external referrals by rural vs. urban families
- Language accessibility scores in parent communications

ACCESSIBILITY COMPLIANCE CHECKS

- WCAG 2.1 compliance for digital wellness portal
- ADA-compliant physical counseling spaces
- Regular audit of communication materials for readability

89 Awareness Material

POSTER TITLE	Your Child's Safety is Our Priority
KEY MESSAGE	We work together to ensure student well-being. If you have concerns, let's talk.
SLOGAN	Connected for Wellness
CAMPAIGN THEME	Collaborative Care & Duty of Care

90 AI Assistant Configuration

RISK KEYWORDS

- suicide
- self-harm
- plan
- means
- hopeless
- end it all

ESCALATION KEYWORDS

- refusal
- denial
- hostile
- police

- CWS
- emergency

INTERVENTION KEYWORDS

- referral
- psychiatrist
- counseling
- safety plan

FREQUENTLY ASKED QUESTIONS

- What do you do if my child is in danger?
- Why does the school need to know about my child's mental health?
- Is this information confidential?

91 AI Audit Hooks

KEY FOCUS

REQUIRED SECTIONS

- Risk Assessment
- Refusal Documentation
- CWS Notification

REQUIRED FORMS

- F-019-A

- F-019-B
- F-019-D

REQUIRED CHECKLISTS

- CL-019-A
- CL-019-C

REQUIRED ESCALATIONS

- Principal Notification
- Legal Consultation

REQUIRED KPIS

- Time to Escalation

- Compliance Score

REQUIRED TRAINING

- Crisis Intervention Certification

92 KPI Framework

92.1 SOP Specific Metrics

Metric	Target	Measurement Frequency
Parental Engagement Rate	> 80%	Monthly
Time to Escalation	< 4 hours	Weekly
Compliance Audit Score	100%	Quarterly

93 Maturity Model

93.1 Level 1

DESCRIPTION

Ad-hoc response to parental refusal.

EVIDENCE

- Informal emails
- No standardized forms

93.2 Level 2

DESCRIPTION

Defined process with basic documentation.

EVIDENCE

- Standardized risk assessment form
- Basic parent contact log

93.3 Level 3

DESCRIPTION

Formal SOP with escalation and legal review.

EVIDENCE

- SUP-019 implementation

- Notice of Concern letters
- CWS logs

93.4 Level 4

DESCRIPTION

Integrated system with dashboard monitoring and audits.

EVIDENCE

- Wellness Dashboard
- Monthly audit reports
- KPI tracking

93.5 Level 5

DESCRIPTION

Optimized, data-driven system with proactive community engagement.

EVIDENCE

- Predictive risk modeling
- High parent satisfaction scores
- Zero preventable incidents

94 Dependencies

PREREQUISITE SOPS

- SUP-015: Suicide Risk Assessment
- SUP-016: Crisis Intervention Protocol

PARALLEL SOPS

- SUP-022: Student Safety Drills

DOWNSTREAM SOPS

- SUP-020: Child Welfare Services Liaison
- SUP-021: Legal Counsel Engagement

95 Related SOPs

PREREQUISITES

- SUP-015: Suicide Risk Assessment
- SUP-016: Crisis Intervention Protocol

UPSTREAM

- SUP-010: Student Behavior Management

DOWNSTREAM

- SUP-020: Child Welfare Services Liaison
- SUP-021: Legal Counsel Engagement

REFERENCES

- Child Protection Policy
- Data Privacy Policy

STANDARD OPERATING PROCEDURE

Parent Sensitization Program (Individualized)

Mental Wellness Management - Parental Engagement & Support

SOP ID

PAR-007

VERSION

1.0

STATUS

Finalized

CLASSIFICATION

Confidential - Restricted Access

EFFECTIVE DATE

2025-08-01

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1 Executive Overview

SOP ID	PAR-007
DOMAIN	Mental Wellness Management
SUB DOMAIN	Parental Engagement & Support
TITLE	Parent Sensitization Program (Individualized)
VERSION	1.0
CLASSIFICATION	Confidential - Restricted Access
EFFECTIVE DATE	2025-08-01
REVIEW DATE	2026-07-31
DOCUMENT STATUS	Finalized
LANGUAGE	English

2 Purpose & Rationale

OBJECTIVE

To educate, sensitize, and equip parents with the necessary knowledge and skills to support their child's mental health journey effectively.

RISK CONTROLLED

Reduces risk of parental denial, punitive responses to symptoms, stigma-driven isolation, and potential for domestic friction due to student distress.

DESIRED OUTCOMES

- Parental shift from 'blame/punishment' to 'support/understanding'
- Increased parent literacy on anxiety, depression, and self-harm
- Establishment of a consistent home-school support bridge
- Reduction in student school refusal due to lack of home support

3 Scope of Application

INSTITUTION TYPES

- High Schools
- Secondary Schools
- Residential Schools

KEY STAKEHOLDERS

- Parents
- Guardians

- School Counselors
- Teachers
- Wellness Committees

AGE GROUPS

- All Ages (Primary to Higher Secondary)

LOCATIONS

- On-campus counseling centers
- Remote/Virtual sessions via approved platforms
- Community centers for rural outreach

4 Definitions & Terminology

Term	Definition
Parental Sensitization	A targeted educational process aimed at increasing a parent's awareness, empathy, and skill set regarding their child's mental health condition.
Motivational Interviewing	A collaborative conversation style for eliciting behavior change by expressing empathy that strengthens the parent's own motivation for change.

Term	Definition
Punitive Response	Parental reactions involving punishment, shaming, or withdrawal of privileges in response to mental health symptoms (e.g., punishing a child for being 'lazy' when they are depressed).
CWSN	Children with Special Needs, including neurodivergent students like those with Autism Spectrum Disorder or ADHD.

5 Key Stakeholders

Stakeholder Type	Stakeholder Title	Role In SOP
Internal	School Counselor	Primary facilitator of sensitization sessions, assessment of parent mindset, and documentation.
Internal	Class Teacher	Initial identification of student distress and referral to the counseling department.
External	Parent/Guardian	Recipient of sensitization, active participant in skill-building, and primary home-support provider.
Internal	School Principal	Final approval authority for escalation cases and resource allocation.

6 Governance & Ownership

KEY FOCUS

OWNER

School Counselor / Wellness Coordinator

APPROVER

Principal / School Management Committee

REVIEW FREQUENCY

Annual

LINKED POLICIES

- Student Privacy Policy
- Child Protection Policy
- Parental Engagement Policy

LINKED SOPs

- PAR-004: Escalation of Unsupportive Parents
- MWMS-001: Student Mental Health Screening
- MWMS-005: Crisis Intervention Protocol

7 Roles & Responsibilities

Item 1

ROLE

School Counselor

RESPONSIBILITIES

- Conducting sensitization sessions
- Motivational interviewing
- Documentation of all interactions
- Monitoring student impact
- Escalating unsupportive cases

Item 2

ROLE

Class Teacher

RESPONSIBILITIES

- Initial identification of distress
- Reporting to counselor
- Collaborating on home-school action plan

Item 3

ROLE

School Principal

RESPONSIBILITIES

- Final approval for escalations
- Resource allocation
- Reviewing management reports

8 RACI Accountability Matrix

Item 1

ACTIVITY

Identification of Student Distress

RESPONSIBLE

- Class Teacher

ACCOUNTABLE

- School Counselor

CONSULTED

- Student Support Team

INFORMED

- Principal

ACCOUNTABLE

- Wellness Coordinator

CONSULTED

- Class Teacher

INFORMED

- Principal

Item 2

ACTIVITY

Parent Sensitization Sessions

RESPONSIBLE

- School Counselor

Item 3

ACTIVITY

Escalation to PAR-004

RESPONSIBLE

- School Counselor

ACCOUNTABLE

- Principal

CONSULTED

- Wellness Committee

INFORMED

- Parents

9 Preconditions

- Student identified as needing support
- Parental consent obtained
- Counselor availability confirmed
- Secure documentation system accessible

10 Activation Triggers

- Student identified as Orange or Red zone
- Teacher reports parental minimization of distress
- Parent requests specific mental health information
- Medical referral from pediatrician
- Child protection agency notification

11 SOP Trigger Matrix

MANDATORY TRIGGERS

- Student identified as 'Orange' or 'Red' zone in screening
- Teacher reports persistent parental minimization of child distress
- Parent requests specific information on a child's mental health diagnosis

OPTIONAL TRIGGERS

- General parent request for wellness education
- Student expresses desire for parents to understand their feelings
- Wellness committee identifies high-prevalence issues in a specific grade

AUTOMATIC TRIGGERS

- Referral from school psychologist following an initial assessment
- Attendance drop below 75% with noted emotional distress

EXTERNAL TRIGGERS

- Medical referral from a pediatrician/psychiatrist to the school
- Child protection agency notification regarding home environment

RECURRING TRIGGERS

- Quarterly wellness check-ins for students with existing support plans
- Monthly review of high-risk student cases

12 Required Inputs

- Student Observation Form

- Initial Screening Score
- Parental Mindset Profile
- Informed Consent Form
- Risk Classification Record

13 Process Map

13.1 Phase 1

NAME

Identification & Referral

OBJECTIVE

Identify the need for parent sensitization.

ACTIVITIES

- Teacher observation and documentation
- Counselor screening of student distress
- Initial risk classification (Green/Yellow/Orange/Red)
- Referral to Parent Sensitization Program

13.2 Phase 2

NAME

Parental Engagement & Consent

OBJECTIVE

Establish a collaborative relationship with the parent.

ACTIVITIES

- Initial contact via phone/email
- Explanation of program goals and confidentiality
- Obtaining written informed consent
- Scheduling 1-3 individual sessions

13.3 Phase 3

NAME

Sensitization Sessions

OBJECTIVE

Deliver targeted education and skill building.

ACTIVITIES

- Session 1: Condition Education & Analogies
- Session 2: Addressing Fears, Stigma, & Medication
- Session 3: Skill Building & Role-Play
- Motivational Interviewing for resistant parents

13.4 Phase 4

NAME

Monitoring & Feedback

OBJECTIVE

Track parent progress and student impact.

ACTIVITIES

- Parent self-assessment of skills
- Counselor observation of home-school communication
- Student feedback on home environment changes

13.5 Phase 5

NAME

Closure & Escalation

OBJECTIVE

Finalize the program or escalate if necessary.

ACTIVITIES

- Success criteria verification
- Completion report generation
- Escalation to PAR-004 for unsupportive parents
- Long-term follow-up scheduling

14 Workflow Overview

START

Identification of student distress and parental need for sensitization.

STEPS

- Referral to Counselor
- Initial Parent Contact & Consent
- Session 1: Education
- Session 2: Addressing Fears
- Session 3: Skill Building
- Resistance Management (if needed)
- Monitoring Period
- Success Verification
- Closure/Escalation
- Management Review

CRITICAL DECISION POINTS

- Parent Cooperation Level
- Success Criteria Met?

END

Formal closure of case and documentation.

15 Standard Procedure

1. Referral Initiation

PURPOSE & RATIONALE

To move student from identification to intervention.

ACTOR

Class Teacher

ACTION

Submit Student Observation Form to Counselor highlighting parental minimization of symptoms.

REQUIRED INPUTS

- Teacher observation notes

EXPECTED OUTPUTS

- Referral Note

CRITICAL DECISION POINTS

- Is the distress significant enough for sensitization?

DOCUMENTATION REQUIRED

- Student Observation Form

EVIDENCE REQUIRED

- Signed Referral Note

APPROVAL REQUIRED

Counselor Review

TIME LIMIT

48 hours

EXPECTED OUTCOME

Referral accepted by Counselor.

ESCALATION IF FAILED

Notify Wellness Coordinator of referral delay.

RELATED FORMS

- Student Observation Form

EXPECTED OUTPUTS

- Signed Consent Form

CRITICAL DECISION POINTS

- Parent agrees to participate?

DOCUMENTATION REQUIRED

- Consent Form
- Contact Log

EVIDENCE REQUIRED

- Signed Consent Form

APPROVAL REQUIRED

None (Counselor level)

TIME LIMIT

3 days

EXPECTED OUTCOME

Parental agreement to participate.

ESCALATION IF FAILED

Document refusal and move to 'Unresponsive Parent' protocol.

RELATED FORMS

- Informed Consent Form

3. Session 1: Education

PURPOSE & RATIONALE

Foundational knowledge.

ACTOR

School Counselor

ACTION

Conduct a 45-minute session explaining the condition using analogies like 'broken leg'.

REQUIRED INPUTS

- Student Assessment Data

EXPECTED OUTPUTS

- Session Summary Note

CRITICAL DECISION POINTS

- Parent receptive to education?

DOCUMENTATION REQUIRED

- Session 1 Summary
- Pre-test Knowledge Score

EVIDENCE REQUIRED

- Counselor Session Note

TIME LIMIT

2. Parental Consent

PURPOSE & RATIONALE

Legal and ethical authorization.

ACTOR

School Counselor

ACTION

Contact parent to explain the program and obtain signed consent.

REQUIRED INPUTS

- Referral Note

7 days from consent

EXPECTED OUTCOME

Parent understands condition as a health issue.

ESCALATION IF FAILED

Use Motivational Interviewing techniques.

RELATED FORMS

- Session Summary Template

4. Session 2: Fear Addressing

PURPOSE & RATIONALE

Dismantle barriers.

ACTOR

School Counselor

ACTION

Address specific fears (stigma, medication) and provide evidence-based info.

REQUIRED INPUTS

- Session 1 Summary

EXPECTED OUTPUTS

- Fear Mapping Record

CRITICAL DECISION POINTS

- Parent identifies primary fear?

DOCUMENTATION REQUIRED

- Session 2 Summary
- Fear/Stigma Map

EVIDENCE REQUIRED

- Counselor Session Note

TIME LIMIT

7 days from Session 1

EXPECTED OUTCOME

Reduced parental anxiety regarding treatment.

ESCALATION IF FAILED

Escalate to Motivational Interviewing focus.

RELATED FORMS

- Fear/Stigma Map

5. Session 3: Skill Building

PURPOSE & RATIONALE

Actionable tools.

ACTOR

School Counselor

ACTION

Conduct role-play of difficult conversations and provide scripts for home support.

REQUIRED INPUTS

- Session 2 Summary

EXPECTED OUTPUTS

- Home Support Action Plan

CRITICAL DECISION POINTS

- Parent can demonstrate skill?

DOCUMENTATION REQUIRED

- Session 3 Summary
- Action Plan

EVIDENCE REQUIRED

- Counselor Session Note
- Home Support Action Plan

TIME LIMIT

7 days from Session 2

EXPECTED OUTCOME

Parent has a concrete plan for home support.

ESCALATION IF FAILED

Escalate to PAR-004 if skills not demonstrated.

RELATED FORMS

- Home Support Action Plan

6. Resistance Management

PURPOSE & RATIONALE

Handle non-cooperation.

ACTOR

School Counselor

ACTION

Apply Motivational Interviewing to address denial or minimization of symptoms.

REQUIRED INPUTS

- Session Notes

EXPECTED OUTPUTS

- Resistance Log

CRITICAL DECISION POINTS

- Is parent moving toward cooperation?

DOCUMENTATION REQUIRED

- Motivational Interviewing Notes
- Resistance Management Log

EVIDENCE REQUIRED

- Counselor Session Note

TIME LIMIT

Ongoing during sessions

EXPECTED OUTCOME

Increased parental awareness or documented refusal.

ESCALATION IF FAILED

Escalate to PAR-004 after 3 failed attempts.

RELATED FORMS

- Resistance Management Log

7. Monitoring Period

PURPOSE & RATIONALE

Verify impact.

ACTOR

School Counselor / Class Teacher

ACTION

Monitor student's mood and parent's adherence to action plan for 14 days.

REQUIRED INPUTS

- Home Support Action Plan

EXPECTED OUTPUTS

- Student Impact Log

CRITICAL DECISION POINTS

- Is there measurable improvement?

DOCUMENTATION REQUIRED

- Student Impact Log
- Parent Follow-up Record

EVIDENCE REQUIRED

- Student Impact Log

TIME LIMIT

14 days post-intervention

EXPECTED OUTCOME

Positive trend in student well-being.

ESCALATION IF FAILED

Re-evaluate action plan or extend sessions.

RELATED FORMS

- Student Impact Log

8. Success Verification

PURPOSE & RATIONALE

Confirm goals met.

ACTOR

School Counselor

ACTION

Review success criteria checklist with parent and student feedback.

REQUIRED INPUTS

- Student Impact Log
- Parent Follow-up Records

EXPECTED OUTPUTS

- Success Criteria Checklist
- Closure Report

CRITICAL DECISION POINTS

- Criteria met?

DOCUMENTATION REQUIRED

- Success Criteria Checklist
- Closure Report

EVIDENCE REQUIRED

- Success Criteria Checklist

TIME LIMIT

3 days post-monitoring

EXPECTED OUTCOME

Formal confirmation of program success.

ESCALATION IF FAILED

Extend sessions or escalate to PAR-004.

RELATED FORMS

- Success Criteria Checklist

9. Closure

PURPOSE & RATIONALE

Finalize and archive.

ACTOR

School Counselor

ACTION

Conduct final wrap-up meeting and archive all records in the secure database.

REQUIRED INPUTS

- Success Criteria Checklist

EXPECTED OUTPUTS

- Closure Report
- Management Summary

CRITICAL DECISION POINTS

- All documents complete?

DOCUMENTATION REQUIRED

- Closure Report
- Management Summary

EVIDENCE REQUIRED

- Closure Report

TIME LIMIT

48 hours post-verification

EXPECTED OUTCOME

Case closed and archived.

RELATED FORMS

- Closure Report

10. Management Review

PURPOSE & RATIONALE

Quality assurance.

ACTOR

Wellness Coordinator / Principal

ACTION

Review anonymized data for trends and audit compliance with CBSE/Supreme Court mandates.

REQUIRED INPUTS

- Closure Reports
- Audit Logs

EXPECTED OUTPUTS

- Management Review Report

- Continuous Improvement Plan

CRITICAL DECISION POINTS

- Are KPIs met?

DOCUMENTATION REQUIRED

- Management Review Report
- Audit Checklist

EVIDENCE REQUIRED

- Management Review Report

APPROVAL REQUIRED

Principal Approval

TIME LIMIT

Quarterly

EXPECTED OUTCOME

Continuous improvement of the program.

ESCALATION IF FAILED

Initiate full program review.

RELATED FORMS

- Management Review Report

RELATED CHECKLISTS

- Audit Checklist

16 Detailed Workflow Steps

1. Identification and Referral

PURPOSE & RATIONALE

To identify students whose parents require sensitization.

TRIGGER

Teacher observation of student distress or counselor screening.

ACTOR

Class Teacher / School Counselor

REQUIRED INPUTS

- Student Observation Form
- Initial Screening Score

ACTIONS

- Review teacher notes for signs of parental minimization (e.g., 'just attention-seeking')
- Conduct a brief counselor screening to confirm student distress level
- Determine if parent sensitization is required based on risk zone

DECISION CRITERIA

- Is the parent minimizing symptoms?
- Is there a punitive response from the home environment?

DOCUMENTS GENERATED

- Referral Note
- Risk Classification Record

EXPECTED OUTPUTS

- Formal referral to Parent Sensitization Program

SERVICE LEVEL AGREEMENT (SLA)

Within 48 hours of identification

ESCALATION PATH

- If student is in Red/Black zone, activate Crisis SOP immediately.

EVIDENCE GENERATED

- Referral Note

2. Initial Parent Contact

PURPOSE & RATIONALE

To establish rapport and obtain consent.

TRIGGER

Receipt of referral from Counselor/Teacher.

ACTOR

School Counselor

REQUIRED INPUTS

- Referral Note
- Student Profile (Anonymized)

ACTIONS

- Contact parent via phone or scheduled meeting
- Explain the purpose of the Parent Sensitization Program
- Address confidentiality and the non-clinical nature of the school's role
- Obtain written informed consent for 1-3 sessions

DECISION CRITERIA

- Does parent agree to participate?
- Is there immediate hostility?

DOCUMENTS GENERATED

- Informed Consent Form
- Initial Contact Log

EXPECTED OUTPUTS

- Scheduled Session 1

SERVICE LEVEL AGREEMENT (SLA)

Within 3 working days of referral

ESCALATION PATH

- If parent refuses, document refusal and proceed to 'Parent Refusal' protocol.

EVIDENCE GENERATED

- Signed Consent Form

3. Session 1: Condition Education

PURPOSE & RATIONALE

To provide foundational knowledge on the child's condition.

TRIGGER

Completion of consent and scheduling.

ACTOR

School Counselor

REQUIRED INPUTS

- Student Assessment Data
- Parental Mindset Profile

ACTIONS

- Explain symptoms (anxiety, depression, self-harm) in simple terms
- Use analogies (e.g., 'Depression is like a broken leg – you can't will it away')
- Clarify that mental health issues are not character flaws or laziness
- Identify parent's primary fears (stigma, medication, future)

DECISION CRITERIA

- Does the parent acknowledge the condition as a health issue?
- Is the parent receptive to analogies?

DOCUMENTS GENERATED

- Session 1 Summary
- Parent Knowledge Assessment (Pre-test)

EXPECTED OUTPUTS

- Baseline understanding of child's condition

SERVICE LEVEL AGREEMENT (SLA)

Within 7 days of consent

ESCALATION PATH

- If parent remains in deep denial, use Motivational Interviewing techniques.

EVIDENCE GENERATED

- Session Summary Note

4. Session 2: Addressing Fears & Stigma

PURPOSE & RATIONALE

To dismantle specific barriers to support.

TRIGGER

Completion of Session 1.

ACTOR

School Counselor

REQUIRED INPUTS

- Session 1 Summary
- Parental Fear Profile

ACTIONS

- Address specific fears: 'Will my child ever be normal?' or 'What about the neighbors?'

- Provide evidence-based information on treatment options (therapy, medication)
- Discuss the importance of a supportive home environment over punitive measures
- Identify 2-3 specific behaviors at home that need change

DECISION CRITERIA

- Has the parent identified specific fears?
- Is the parent willing to consider non-punitive responses?

DOCUMENTS GENERATED

- Session 2 Summary
- Fear/Stigma Mapping Record

EXPECTED OUTPUTS

- Reduced stigma and fear regarding treatment

SERVICE LEVEL AGREEMENT (SLA)

Within 7 days of Session 1

ESCALATION PATH

- If parent expresses desire for punitive action, use Motivational Interviewing.

EVIDENCE GENERATED

- Session Summary Note

5. Session 3: Skill Building & Role-Play

PURPOSE & RATIONALE

To provide actionable tools for home support.

TRIGGER

Completion of Session 2.

ACTOR

School Counselor

REQUIRED INPUTS

- Session 2 Summary
- Parental Skill Level Assessment

ACTIONS

- Teach specific responses to self-harm or emotional breakdowns (compassion vs. punishment)
- Conduct role-play of difficult conversations between parent and child
- Provide 'scripts' for daily check-ins
- Establish a home support action plan

DECISION CRITERIA

- Can the parent demonstrate at least one new response technique?
- Is there a clear action plan?

DOCUMENTS GENERATED

- Session 3 Summary
- Home Support Action Plan

EXPECTED OUTPUTS

- Actionable home support skills

SERVICE LEVEL AGREEMENT (SLA)

Within 7 days of Session 2

ESCALATION PATH

- If parent cannot demonstrate basic empathy, escalate to PAR-004.

EVIDENCE GENERATED

- Session Summary Note
- Home Support Action Plan

6. Parental Resistance Management

PURPOSE & RATIONALE

To handle parents who are uncooperative or in denial.

TRIGGER

Persistent minimization of symptoms or refusal to engage in skill-building.

ACTOR

School Counselor

REQUIRED INPUTS

- Session Notes
- Parental Feedback

ACTIONS

- Employ Motivational Interviewing: 'What would need to happen for you to consider that your child might be struggling?'
- Reflect parent's concerns back without judgment
- Highlight the risks of non-intervention (e.g., worsening symptoms, safety risks)
- Offer a 'pause' and follow up in 1 week if resistance is high

DECISION CRITERIA

- Is the parent moving toward cooperation?
- Is the risk to the child increasing?

DOCUMENTS GENERATED

- Resistance Management Log
- Motivational Interviewing Notes

EXPECTED OUTPUTS

- Increased parental awareness or documented refusal

SERVICE LEVEL AGREEMENT (SLA)

Ongoing during sessions

ESCALATION PATH

- If no progress after 3 sessions, escalate to PAR-004.

EVIDENCE GENERATED

- Resistance Management Log

7. Student Impact Monitoring

PURPOSE & RATIONALE

To verify if parent sensitization is improving student outcomes.

TRIGGER

Completion of Session 3.

ACTOR

School Counselor / Class Teacher

REQUIRED INPUTS

- Home Support Action Plan
- Student Observation Notes

ACTIONS

- Monitor student's mood, attendance, and academic engagement for 14 days
- Check in with parent on the implementation of the action plan
- Observe changes in home-school communication quality

DECISION CRITERIA

- Is there a measurable improvement in student well-being?
- Is the parent following the action plan?

DOCUMENTS GENERATED

- Student Impact Log
- Parent Follow-up Record

EXPECTED OUTPUTS

- Evidence of intervention effectiveness

SERVICE LEVEL AGREEMENT (SLA)

Weekly for 2 weeks post-intervention

ESCALATION PATH

- If student well-being declines, re-evaluate the action plan.

EVIDENCE GENERATED

- Student Impact Log

8. Success Criteria Verification

PURPOSE & RATIONALE

To determine if the program goals were met.

TRIGGER

Completion of monitoring period.

ACTOR

School Counselor

REQUIRED INPUTS

- Student Impact Log
- Parent Follow-up Records

ACTIONS

- Review parent's ability to describe the condition correctly
- Verify parent is using non-punitive responses
- Assess student's reported feeling of support at home

DECISION CRITERIA

- Parent able to describe depression as illness, not weakness?
- Parent actively participating in support?

DOCUMENTS GENERATED

- Success Criteria Checklist
- Closure Report

EXPECTED OUTPUTS

- Program Closure or Extension Decision

SERVICE LEVEL AGREEMENT (SLA)

Within 3 days of monitoring period end

ESCALATION PATH

- If criteria not met, schedule additional sessions or escalate.

EVIDENCE GENERATED

- Success Criteria Checklist

9. Closure and Documentation

PURPOSE & RATIONALE

To formally end the sensitization program.

TRIGGER

Verification of success criteria.

ACTOR

School Counselor

REQUIRED INPUTS

- Success Criteria Checklist
- Session Summaries

ACTIONS

- Conduct a final wrap-up meeting with parents
- Provide resources for ongoing support (e.g., helpline numbers)
- Archive all records in the secure database
- Generate a summary report for school management

DECISION CRITERIA

- Are all mandatory documents completed?
- Is the parent satisfied with the outcome?

DOCUMENTS GENERATED

- Closure Report
- Management Summary

EXPECTED OUTPUTS

- Formal closure of PAR-007 case

SERVICE LEVEL AGREEMENT (SLA)

Within 48 hours of final session

EVIDENCE GENERATED

- Closure Report

10. Management Review & Audit

PURPOSE & RATIONALE

To ensure quality and compliance of the program.

TRIGGER

Monthly or Quarterly review cycle.

ACTOR

Wellness Coordinator / Principal

REQUIRED INPUTS

- Closure Reports
- Risk Classification Records
- Audit Logs

ACTIONS

- Review anonymized data for trend analysis
- Verify compliance with CBSE and Supreme Court mandates
- Identify areas for process improvement
- Update the SOP based on lessons learned

DECISION CRITERIA

- Are KPIs being met?
- Is there a high rate of escalation to PAR-004?

DOCUMENTS GENERATED

- Management Review Report
- Audit Checklist

EXPECTED OUTPUTS

- Continuous Improvement Plan

SERVICE LEVEL AGREEMENT (SLA)

Quarterly

ESCALATION PATH

- If systemic failure is identified, initiate a full program review.

EVIDENCE GENERATED

- Management Review Report

17 Critical Decision Points

Item 1

DECISION ID

DP-001

DECISION NAME

Parent Cooperation Level

CRITERIA

- Hostile/Aggressive behavior

- Persistent denial of symptoms
- Refusal to attend sessions

POSSIBLE OUTCOMES

- Proceed with Motivational Interviewing
- Escalate to Management (PAR-004)
- Pause program for 1 week

NEXT STEPS

- Step 6: Resistance Management

- Referral to PAR-004
- Schedule follow-up contact

Item 2

DECISION ID

DP-002

DECISION NAME

Success Criteria Met?

CRITERIA

- Parent can explain condition correctly

- Parent demonstrates non-punitive skills
- Student reports improved home support

POSSIBLE OUTCOMES

- Close Case
- Extend Sessions
- Escalate to External Referral

NEXT STEPS

- Step 9: Closure
- Return to Step 3 for additional sessions
- Referral to Clinical Services

18 Decision Tree

GREEN

- Provide general wellness resources
- Schedule routine check-in

YELLOW

- Initiate Parent Sensitization Program (PAR-007)
- Monitor student impact weekly

ORANGE

- Intensive sensitization sessions
- Motivational interviewing for resistance

- Consider external referral if no progress in 3 sessions

RED

- Immediate risk assessment
- Activate Crisis SOP
- Mandatory parent notification and intervention

BLACK

- Emergency services activation
- Immediate safety protocols
- Notify authorities

19 Decision Rules

Rule ID	Condition	Decision	Next Action
DR-01	Parent refuses to acknowledge child's distress after 3 sessions.	Escalate to PAR-004.	Notify Principal and Wellness Committee.
DR-02	Student reports increased safety risk at home.	Immediate Risk Assessment.	Activate Crisis SOP (PAR-005).

20 Exception Handling Protocol

Item 1

EXCEPTION

Parent refuses all communication.

DETECTION METHOD

Failed phone calls, ignored emails over 7 days.

RESPONSE

Document attempts and notify School Management; move to 'Unresponsive Parent' protocol.

ESCALATION

Principal / District Education Officer

DOCUMENTATION REQUIRED

- Communication Log
- Refusal Record

Item 2

EXCEPTION

Parent becomes aggressive/violent during session.

DETECTION METHOD

Verbal threats, physical intimidation.

RESPONSE

Immediately terminate session; ensure safety of counselor; notify security and management.

ESCALATION

School Security / Police (if threat is imminent)

DOCUMENTATION REQUIRED

- Incident Report
- Security Log

Item 3

EXCEPTION

Parent requests clinical diagnosis from school counselor.

DETECTION METHOD

Direct request for medical diagnosis/prescription.

RESPONSE

Clarify role boundaries; provide list of approved external clinicians for referral.

DOCUMENTATION REQUIRED

- Role Boundary Clarification Note

21 Exceptions

Exception Type	Condition	Action
Emergency Disclosure	Imminent risk of self-harm or violence.	Immediate disclosure to parents and authorities without prior consent.
Parental Hostility	Verbal threats or physical intimidation during session.	Terminate session immediately; notify security.

22 Expected Outputs

- Home Support Action Plan
- Session Summary Notes
- Closure Report
- Management Summary
- Student Impact Log

23 Document Control & Version Governance

23.1 Version History

Version	Date	Author	Description
1.0	2025-08-01	MWMS Architect	Initial release of Parent Sensitization SOP.

23.2 Change Log

Version	Date	Change Description
1.0	2025-08-01	Baseline creation following Supreme Court Judgment 2025 INSC 893.

REVIEW TRIGGERS

- Annual Review Cycle
- Change in CBSE Guidelines
- Significant Incident of Parent Non-Compliance
- Feedback from School Management

23.4 Approval History

Approver	Date	Status
School Principal	2025-07-15	Approved
District Education Officer	2025-07-20	Verified

24 Inclusive Implementation Mandate

MULTILINGUAL SUPPORT REQUIRED

Yes

LOW RESOURCE SCHOOL ALTERNATIVE

Yes

25 Regulatory Compliance Mapping KEY FOCUS

MANDATORY REQUIREMENTS

- Supreme Court of India Judgment 2025 INSC 893 (Right to Mental Health Support)
- CBSE Circular TRG-03/2026 (Parental Involvement in Wellbeing)
- NEP 2020 (Holistic Development and Counseling)

RECOMMENDED REQUIREMENTS

- WHO School Mental Health Framework
- UNESCO Whole School Wellbeing Framework

BEST PRACTICES

- Manodarpan Guidelines
- Tele-MANAS Integration Protocols

COMPLIANCE GAPS IF NOT IMPLEMENTED

- Legal liability for failure to provide adequate parental support in mental health cases
- Non-compliance with CBSE affiliation bye-laws regarding student safety
- Risk of child welfare violations due to lack of home-school alignment

26 Compliance Traceability Matrix KEY FOCUS

1. Supreme Court of India Judgment 2025 INSC 893

REQUIREMENT REFERENCE

Section 4.2 - Right to Support

REQUIREMENT TEXT

Educational institutions must provide adequate mental health support systems including parental sensitization.

IMPLEMENTED IN SECTIONS

- Purpose
- Detailed Workflow Steps
- Parent Engagement Matrix

2. CBSE Circular TRG-03/2026

REQUIREMENT REFERENCE

Guideline 1.4

REQUIREMENT TEXT

Schools shall conduct targeted sensitization for parents of students showing signs of emotional distress.

IMPLEMENTED IN SECTIONS

- Scope
- Process Map
- SOP Specific Inclusion Requirements

27 Documentation & Evidentiary Requirements

EVENT DOCUMENTATION PRINCIPLES

- Factual, objective language only
- Avoid subjective judgments of parent character
- Document specific quotes from parents
- Record observable behaviors during sessions

MANDATORY CONVERSATION RECORDS

- Initial Parent Contact Log
- Session Summary Notes (1-3)
- Motivational Interviewing Transcript Snippets
- Consent Forms

MANDATORY OBSERVATION RECORDS

- Parental Affect Observation Note
- Student Behavior Change Log
- Home Environment Feedback Record

MANDATORY DECISION RECORDS

- Escalation Decision Form
- Referral Approval Note
- Risk Level Re-classification Record

MANDATORY ACTION RECORDS

- Action Plan for Home Support
- Skill Practice Checklist Completion
- Follow-up Appointment Log

MANDATORY FOLLOWUP RECORDS

- 30-Day Post-Intervention Review
- 90-Day Progress Summary
- Closure Report

EVIDENCE REQUIREMENTS

- Signed Consent Forms
- Completed Parent Skill Checklists
- Counselor Session Summaries

- Management Approval Emails
- DOCUMENTATION QUALITY STANDARDS**
- Must be stored in a secure, encrypted database
- Accessible only to authorized personnel (Counselor/Principal)
- Standardized templates must be used for all sessions
- Timely entry within 24 hours of session completion

28 The Student Journey

ENTRY

- Teacher observes emotional distress or academic decline
- Student expresses feeling misunderstood at home

ASSESSMENT

- Counselor conducts initial screening
- Risk classification (Green/Yellow/Orange/Red)

INTERVENTION

- Referral to Parent Sensitization Program

- Indirect benefit from improved parental understanding
- Increased home support and reduced conflict

MONITORING

- Weekly check-ins with counselor
- Observation of mood and school engagement

CLOSURE

- Student reports feeling supported at home
- Case closed in MWMS database

29 The Parent Journey

NOTIFICATION

- Receives phone call/email from counselor
- Informed of student's need for support

CONSULTATION

- Initial meeting to discuss goals and confidentiality
- Signing of informed consent

CONSENT

- Agreement to participate in 1-3 sessions

- Agreement to follow home action plan

ENGAGEMENT

- Session 1: Education on condition
- Session 2: Addressing fears and stigma
- Session 3: Skill building and role-play

CLOSURE

- Final wrap-up meeting
- Receipt of resources for ongoing support

30 The Staff Journey

OBSERVATION

- Teacher notices student's behavior changes
- Parent reports concerns to teacher

REPORTING

- Teacher submits observation form
- Counselor reviews referral

INTERVENTION SUPPORT

- Counselor conducts parent sessions
- Teacher provides classroom support based on action plan

DOCUMENTATION

- Teacher logs observations
- Counselor maintains session notes
- Management reviews reports

31 Resource Evidence

COMMITTEE MINUTES

- Wellness Committee meeting minutes regarding parent engagement trends

ATTENDANCE RECORDS

- Parent session attendance logs (anonymized)

TRAINING RECORDS

- Counselor certification in Motivational Interviewing
- Teacher training on identifying student distress

FORMS

- Informed Consent Form
- Session Summary Template
- Home Support Action Plan
- Risk Classification Form
- Success Criteria Checklist

REPORTS

- Monthly Wellness Dashboard
- Quarterly Management Review Report

DASHBOARDS

- Parent Sensitization Completion Rate
- Student Well-being Trend Analysis

32 Grievance Redressal Mechanism

KEY FOCUS

COMPLAINT CHANNELS

- Anonymous suggestion box
- Dedicated wellness email
- Direct meeting with Principal

INVESTIGATION PROCESS

- Complaint logged by Wellness Coordinator
- Fact-finding interview with involved parties

- Review of session notes and documentation

RESOLUTION TIMELINES

- Acknowledgment within 24 hours
- Resolution/Response within 7 working days

APPEAL PROCESS

- Appeal to School Management Committee
- External mediation if required

33 Referral Framework

33.1 Internal Referrals

ELIGIBLE REFERRERS

- Teachers
- Counsellors
- School Management
- Wellness Committee
- Student Support Team

REFERRAL TRIGGERS

- Persistent emotional distress
- Parental minimization of symptoms
- Academic decline linked to mood

DOCUMENTATION REQUIRED

- Referral Note
- Initial Screening Score

SERVICE LEVEL AGREEMENT (SLA)

Within 48 hours

- Digital reporting link

VALIDATION PROCESS

- Counselor review of anonymous notes
- Verification of student identity if possible

FOLLOW UP PROCEDURE

- Outreach to student for clarification
- Assessment of urgency

33.4 External Referrals

ELIGIBLE AGENCIES

- Pediatricians
- Child Psychiatrists
- Tele-MANAS
- District Child Protection Services

ELIGIBILITY CRITERIA

- Clinical severity exceeding school scope
- Risk of self-harm/violence
- Need for medication management

CONSENT REQUIREMENTS

- Written parental consent for external referral
- Release of information (ROI) form

HANDOVER PROTOCOL

- Summary of school interventions provided to clinician
- Shared goals and action plan

33.2 Self Referrals

ALLOWED

Yes

SUBMISSION METHODS

- Online portal
- Counselor drop-box
- Direct request to teacher

CONFIDENTIALITY PROTECTIONS

- Encrypted submission
- Anonymized initial screening

33.3 Anonymous Referrals

ALLOWED

Yes

CHANNELS

- Anonymous suggestion box

33.5 Referral Tracking

TRACKING REQUIRED

Yes

STATUS CATEGORIES

- Pending
- In-Progress
- Completed
- Escalated
- Closed

- Closure Report

34 Confidentiality Exception Matrix

KEY FOCUS

MANDATORY DISCLOSURES

- Imminent risk of self-harm
- Imminent risk to others
- Child abuse
- Child neglect
- Sexual abuse
- Court orders
- Legal reporting obligations

LEGAL REPORTING EVENTS

- Police notification for threats of violence
- Mandatory reporting to Child Welfare Committee (CWC)

CONSENT OVERRIDE CONDITIONS

- Life-threatening emergency
- Court order requiring disclosure
- Imminent danger to the student or others

AUTHORIZED DISCLOSURE RECIPIENTS

- School Principal
- District Authorities
- Medical Personnel (with consent)
- Law Enforcement (in emergencies)

DOCUMENTATION REQUIREMENTS

- Disclosure Log
- Reason for Disclosure Note
- Time and Date of Disclosure

APPROVAL REQUIREMENTS

- Principal approval for non-emergency disclosures
- Immediate action for life-threatening emergencies

35 Record Retention & Disposal

RETENTION PERIOD

5 Years post-graduation

RETENTION BASIS

Educational records and child welfare compliance

RECORD CATEGORIES

- Session Notes
- Consent Forms
- Action Plans
- Risk Assessments

35.4 Storage Requirements

PHYSICAL RECORDS

- Locked filing cabinet in Counselor's office

DIGITAL RECORDS

- Encrypted MWMS database with MFA

ACCESS CONTROL REQUIREMENTS

- Role-based access control (RBAC)
- Audit logs for every access attempt
- Counselor and Principal only

DISPOSAL METHOD

Secure shredding for physical; permanent deletion for digital

DISPOSAL AUTHORITY

School Wellness Coordinator

AUDIT FREQUENCY

Annual

DESTRUCTION LOG REQUIRED

Yes

36 Anonymised Reporting Framework

36.1 Management Reports

ANONYMIZATION REQUIRED

Yes

PERMITTED DATA

- Number of parents sensitized
- Average number of sessions per case
- Pre/Post sentiment scores
- Common themes in parental concerns

PROHIBITED DATA

- Student names
- Parent names
- Specific addresses
- Unique identifiers

36.2 Research Usage

ALLOWED

No

36.3 Statistical Reporting

AGGREGATE ONLY

Yes

MINIMUM GROUP SIZE

5

RE IDENTIFICATION CONTROLS

- Data scrubbing
- Aggregation of small cohorts

DATA SHARING CONTROLS

- No sharing with third-party marketing
- Internal use only for school improvement

37 Guiding Ethical Principles

37.1 Autonomy

DESCRIPTION

Respecting the parent's right to choose their level of engagement.

IMPLEMENTATION REQUIREMENTS

- Informed consent
- Right to withdraw from program at any time

37.2 Beneficence

DESCRIPTION

Acting in the best interest of the student's mental health.

IMPLEMENTATION REQUIREMENTS

- Evidence-based content
- Focus on positive parenting skills

37.3 Non Maleficence

DESCRIPTION

Ensuring sensitization does not cause family friction or distress.

IMPLEMENTATION REQUIREMENTS

- Sensitive communication
- Confidentiality safeguards

37.4 Justice And Equity

DESCRIPTION

Providing equal access to all parents regardless of socio-economic status.

IMPLEMENTATION REQUIREMENTS

- Rural adaptations
- Multilingual support

37.5 Confidentiality

KEY FOCUS

DESCRIPTION

Protecting the privacy of family dynamics and student data.

IMPLEMENTATION REQUIREMENTS

- Secure storage
- Need-to-know access

37.6 Privacy

DESCRIPTION

Adhering to Indian data protection standards for minors.

IMPLEMENTATION REQUIREMENTS

- Data minimization
- Encryption

37.7 Professional Integrity

DESCRIPTION

Counselors maintaining professional boundaries and competence.

IMPLEMENTATION REQUIREMENTS

- Regular supervision
- Continuing education

38 Communication & Awareness Strategy

38.1 Student Communication

CHANNELS

- Counselor meetings

- Classroom wellness talks

FREQUENCY

As needed / Weekly check-ins

MANDATORY TOPICS

- How to talk to parents about feelings
- Understanding the sensitization program

38.2 Parent Communication

CHANNELS

- Phone calls
- Email
- Individual meetings
- WhatsApp (for scheduling only)

FREQUENCY

Session-based

MANDATORY TOPICS

- Child's well-being goals
- Action plan updates
- Resource sharing

38.3 Staff Communication

CHANNELS

- Staff meetings
- Internal emails
- Wellness committee briefings

FREQUENCY

Monthly

MANDATORY TOPICS

- Referral procedures
- Confidentiality reminders
- Success stories (anonymized)

38.4 Awareness Campaigns

MINIMUM FREQUENCY

Quarterly

TARGET GROUPS

- Parents
- Teachers
- Students

CAMPAIGN TYPES

- Newsletters
- Workshops
- Social media posts (School-specific)

38.5 Workshops And Training

STUDENT WORKSHOPS

- Emotional regulation skills
- Self-advocacy for students

PARENT WORKSHOPS

- Understanding Neurodiversity
- Positive Parenting in the Digital Age

STAFF WORKSHOPS

- Identifying student distress
- Motivational interviewing basics

38.6 Helpline Visibility

REQUIRED

Yes

DISPLAY LOCATIONS

- Counseling office door
- School website
- Student diaries

DIGITAL VISIBILITY REQUIREMENTS

- Link on school homepage
- QR code in newsletters

FEEDBACK CHANNELS

- Post-session surveys
- Annual wellness survey
- Suggestion box

39 Competency & Qualification Requirements

MANDATORY QUALIFICATIONS

- Master's Degree in Psychology/Counseling
- Registered with relevant counseling board (e.g., RCI for clinical, or state-specific)

PREFERRED QUALIFICATIONS

- Certification in Motivational Interviewing
- Training in Child and Adolescent Mental Health

CERTIFICATIONS

- School Counselor Certification
- First Aid/CPR Training

MINIMUM EXPERIENCE REQUIREMENTS

- 2 years of experience in educational settings

BACKGROUND VERIFICATION REQUIREMENTS

- Police verification for all staff
- Child Protection clearance

39.6 Continuing Professional Development

ANNUAL TRAINING HOURS

40

MANDATORY TOPICS

- Neurodiversity
- Crisis Intervention
- Parental Engagement Strategies

RECERTIFICATION REQUIREMENTS

Every 3 years

39.7 Supervision Requirements

CLINICAL SUPERVISION REQUIRED

No

PEER REVIEW REQUIRED

Yes

39.7.3 Wellness Case Review Framework

DESCRIPTION Monthly review of high-risk cases with the Wellness Committee.

FREQUENCY Monthly

39.7.4 Second Level Review Process

DESCRIPTION Principal review for all escalations to PAR-004.

TRIGGER

Escalation criteria met

39.7. High Risk Decision

5 Validation

KEY
FOCUS

DESCRIPTION

Dual sign-off required for any external clinical referral.

ACTORS

- Counselor
- Principal

FREQUENCY

Monthly

40 Feedback & Effectiveness Measurement

40.1 Student Feedback

REQUIRED

Yes

COLLECTION METHODS

- Anonymous surveys
- Counselor check-ins

FREQUENCY

Monthly during intervention

40.2 Parent Feedback

REQUIRED

Yes

COLLECTION METHODS

- Post-session survey
- Annual satisfaction survey

FREQUENCY

After Session 3 and Annually

40.3 Staff Feedback

REQUIRED

Yes

COLLECTION METHODS

- Staff meetings
- Feedback forms

FREQUENCY

Quarterly

40.4 Annual Effectiveness Review

REQUIRED

Yes

EVALUATION CRITERIA

- Reduction in student distress scores
- Parental satisfaction rates
- Completion rate of sensitization program

KEY STAKEHOLDERS

- Principal
- Wellness Committee
- Parents (Survey)
- Teachers

CONTINUOUS IMPROVEMENT INPUTS

- Parent feedback themes
- Counselor session notes
- Audit findings

SUCCESS INDICATORS

- Improved parent-student communication quality
- Reduction in school refusal incidents
- Increased parental literacy on mental health

40.7 Improvement Tracking

REQUIRED

Yes

REVIEW FREQUENCY

Quarterly

41 Prevention Layer

PRIMARY PREVENTION

- School-wide awareness campaigns on mental health
- Parental workshops on positive parenting and neurodiversity
- Student SEL (Social Emotional Learning) curriculum

SECONDARY PREVENTION

- Early identification of student distress by teachers

- Targeted parent sensitization for students in 'Yellow' zone
- Regular wellness check-ins with at-risk families

TERTIARY PREVENTION

- Individualized Parent Sensitization Program (PAR-007)
- Escalation to clinical services for high-risk cases
- Crisis intervention for severe mental health events

42 Resilience Layer

SKILLS DEVELOPED

- Parental active listening
- Emotional regulation for parents
- De-escalation techniques for home conflict

SEL COMPONENTS

- Self-awareness of parental triggers
- Empathy building for child's experience
- Relationship skills between parent and student

LIFE SKILLS COMPONENTS

- Problem-solving for behavioral issues
- Time management for home support
- Information literacy regarding mental health resources

PROTECTIVE FACTORS STRENGTHENED

- Parent-child bond
- Home environment stability
- Access to accurate information

43 Continuous Monitoring

MONITORING FREQUENCY

Weekly during intervention; Quarterly for trends.

MONITORING INDICATORS

- Student mood score improvement
- Parental adherence to action plan
- Reduction in school refusal incidents
- Counselor satisfaction with parent engagement

RESPONSIBLE ROLES

- School Counselor
- Class Teacher

- Wellness Coordinator

EARLY WARNING SIGNS

- Parental hostility increasing during sessions
- Student reporting increased conflict at home
- Failure to complete session 1

TREND ANALYSIS REQUIREMENTS

- Quarterly analysis of 'Resistance' vs. 'Cooperation' rates
- Correlation between sensitization and student attendance

44 Mental Wellness Index (MWI)

APPLICABLE

Yes

44.2 Indicators

Indicator Code	Indicator Name	Weight	Score Range	Description
PWI-01	Parental Awareness Score	0.3	1-5	Ability to correctly identify symptoms of anxiety/depression.
PWI-02	Parental Skill Score	0.4	1-5	Ability to demonstrate non-punitive response techniques.
PWI-03	Home Support Score	0.3	1-5	Student's reported feeling of support at home.

ASSESSMENT FREQUENCY

Post-Session 3

RISK SCORING METHOD

Weighted average of PWI scores.

INTERVENTION THRESHOLDS

- Score < 2.0: Immediate escalation to PAR-004
- Score 2.1 - 3.5: Additional sensitization sessions
- Score > 3.6: Successful completion

45 Review Status

LEGAL REVIEW

Completed (August 2025)

CLINICAL REVIEW

Pending (September 2025)

EDUCATION REVIEW

Completed (July 2025)

COMPLIANCE REVIEW

Completed (July 2025)

46 Resource Controls

COUNSELOR STUDENT RATIO 1:300

CAREER COUNSELOR RATIO 1:500

WELLNESS TEAM STRUCTURE Counselor, Wellness Coordinator, Teacher Reps, Management Reps

47 Screening Framework

TIER 1 UNIVERSAL WELLNESS SCREENING

- Annual student wellness survey
- Teacher observation logs

TIER 2 TARGETED SCREENING

- Counselor screening for 'Yellow' zone students
- Parental concern intake form

TIER 3 CLINICAL ASSESSMENT

- Referral to external psychiatrist/psychologist
- Clinical risk assessment (Red/Black zones)

48 Parent Engagement Matrix

PARENT COOPERATIVE

- Active participation in all sessions
- Adherence to home action plan
- Positive feedback from student

PARENT UNRESPONSIVE

- Missed appointments without notice
- No response to emails/calls
- Refusal to sign consent forms

PARENT IN DENIAL

- Minimizing symptoms as 'attention-seeking'
- Blaming child for all issues
- Refusing to acknowledge mental health needs

PARENT CONFLICT SITUATION

- Hostility toward counselor
- Aggressive behavior during sessions
- Threats of legal action or physical retaliation

PARENT UNAVAILABLE

- Work-related constraints
- Travel/Relocation
- Lack of transportation (Rural adaptation)

PARENT REFUSES INTERVENTION

- Explicit refusal to participate in sensitization
- Refusal to acknowledge student distress
- Demand for 'punitive' only measures

49 Confidentiality Controls

KEY FOCUS

ACCESS CONTROL

- Role-based access (RBAC)
- Multi-factor authentication (MFA)

CONSENT REQUIREMENTS

- Signed informed consent for all sessions
- Release of Information (ROI) for external sharing

RECORD SHARING RULES

- No sharing with non-essential staff

- Anonymized data only for management reports

EXCEPTIONS TO CONFIDENTIALITY

- Imminent risk of self-harm/violence
- Child abuse disclosure
- Court order

STUDENT PRIVACY PROTECTIONS

- Data minimization
- Encryption at rest and in transit

50 Data Governance

KEY FOCUS

STORAGE REQUIREMENTS

- Secure cloud storage with Indian data residency
- Encrypted local backups

ENCRYPTION REQUIREMENTS

- AES-256 for all stored records
- TLS 1.3 for data in transit

RETENTION REQUIREMENTS

- 5 years post-graduation

- Immediate deletion of non-essential notes
- ARCHIVAL REQUIREMENTS**
- Move to cold storage after 2 years of inactivity
- DESTRUCTION REQUIREMENTS**
- Secure shredding for physical records

- Cryptographic erasure for digital records
- BACKUP REQUIREMENTS**
- Daily automated backups
 - Off-site disaster recovery site

51 Risk Classification KEY FOCUS

51.1 Green

DESCRIPTION

Normal developmental concerns and routine life stress.

EXAMPLES

- Mild exam stress
- Friendship disagreements
- Occasional worry

51.2 Yellow

DESCRIPTION

Early distress requiring support and monitoring.

EXAMPLES

- Persistent stress
- Moderate anxiety
- Social withdrawal
- Sleep difficulties

51.3 Orange

DESCRIPTION

Significant distress causing measurable impairment but without immediate danger.

EXAMPLES

- Severe exam anxiety

- Panic attacks
- School refusal
- Persistent depressive symptoms
- Self-harm thoughts WITHOUT intent or plan

51.4 Red KEY FOCUS

DESCRIPTION

High-risk situation requiring immediate intervention.

EXAMPLES

- Suicidal ideation
- Self-harm behaviour
- Violence threats
- Severe emotional crisis

51.5 Black

DESCRIPTION

Life-threatening emergency requiring emergency response.

EXAMPLES

- Active suicide attempt
- Overdose
- Violent attack underway
- Missing student assessed as high-risk

52 Escalation Matrix KEY FOCUS

GREEN

- Counselor review
- Routine monitoring

YELLOW

- Parent communication
- Supportive intervention

ORANGE

- Individual intervention plan
- Formal monitoring
- Consider external referral

RED

- Immediate risk assessment
- Continuous supervision
- Crisis activation
- Emergency referral

BLACK

- Activate Emergency Response SOP

- Contact emergency services
- Notify leadership and parents immediately

53 External Reporting Obligations

TELE MANAS

- Referral for 24/7 mental health support in India

DISTRICT AUTHORITIES

- Reportable incidents of child safety risk

CHILD PROTECTION SERVICES

- Mandatory reporting of abuse or neglect

MEDICAL SERVICES

- Referral to psychiatric care for clinical management

LAW ENFORCEMENT

- Immediate notification for threats of violence or missing persons

REPORTING TRIGGERS

- Imminent risk of self-harm
- Disclosure of abuse
- Threats to others

54 Service Level Agreement (SLA)

KEY FOCUS

ACKNOWLEDGEMENT	24 hours
ASSESSMENT	72 hours
ESCALATION	Immediate (for Red/Black)
RESOLUTION	30 days for Yellow/Orange
CLOSURE	48 hours post-verification

55 Required Forms

KEY FOCUS

55.1 Mandatory Forms

KEY FOCUS

Item 1

FORM ID	PAR-007-F1
FORM NAME	Informed Consent Form
PURPOSE & RATIONALE	Legal authorization for parent sensitization.
OWNER	School Counselor
RETENTION PERIOD	5 years
MANDATORY	Yes
LINKED SOP	PAR-007

Item 2

FORM ID	PAR-007-F2
FORM NAME	Session Summary Note
PURPOSE & RATIONALE	Documentation of session content.

OWNER	School Counselor
RETENTION PERIOD	5 years
MANDATORY	Yes
LINKED SOP	PAR-007

Item 3

FORM ID	PAR-007-F3
FORM NAME	Home Support Action Plan
PURPOSE & RATIONALE	Actionable steps for parents.
OWNER	School Counselor
RETENTION PERIOD	5 years
MANDATORY	Yes
LINKED SOP	PAR-007

Item 4

FORM ID	PAR-007-F4
FORM NAME	Success Criteria Checklist
PURPOSE & RATIONALE	Verification of program goals.
OWNER	School Counselor
RETENTION PERIOD	5 years
MANDATORY	Yes
LINKED SOP	PAR-007

Item 5

FORM ID	PAR-007-F5
FORM NAME	Closure Report
PURPOSE & RATIONALE	Final case documentation.
OWNER	School Counselor
RETENTION PERIOD	5 years
MANDATORY	Yes
LINKED SOP	PAR-007

55.2 Optional Forms**Item 1**

FORM ID	PAR-007-O1
FORM NAME	Parent Knowledge Pre-test
PURPOSE & RATIONALE	Baseline assessment of parent awareness.
OWNER	School Counselor
RETENTION PERIOD	2 years
MANDATORY	No
LINKED SOP	PAR-007

GENERATED RECORDS

- Communication Log
- Risk Classification Record
- Resistance Management Log
- Student Impact Log

56 Required Checklists**KEY FOCUS**

Checklist ID	Checklist Name	Purpose & Rationale	Owner	Frequency	Linked SOP
CHK-001	Counselor Session Readiness Checklist	Ensure all materials and environment are ready.	School Counselor	Every session	PAR-007
CHK-002	Parent Engagement Quality Checklist	Audit the quality of parent interaction.	Wellness Coordinator	Monthly	PAR-007
CHK-003	Student Impact Monitoring Checklist	Track student progress.	Class Teacher	Weekly	PAR-007
CHK-004	Risk Classification Checklist	Ensure accurate risk scoring.	School Counselor	Every referral	PAR-007
CHK-005	Audit Checklist for PAR-007	Compliance audit.	Internal Auditor	Quarterly	PAR-007

57 Records & Evidence

Record Name	Retention Period	Owner
Session Summary Notes	5 years	School Counselor
Informed Consent Forms	5 years	Counselor Office
Home Support Action Plans	5 years	School Counselor
Risk Classification Records	5 years	Wellness Coordinator

58 Key Performance Indicators

Metric	Target	Measurement Frequency
Parent Sensitization Completion Rate	> 80% of referred parents complete Session 3	Quarterly
Average Parent Sentiment Score Improvement	> 2.0 point increase on pre/post survey	Per Case
Student Well-being Trend (Attendance)	> 15% improvement in attendance for sensitized students	Monthly

59 Audit Framework

KEY FOCUS

AUDIT FREQUENCY

Quarterly

AUDIT QUESTIONS

- Are all sessions documented within 24 hours?
- Is there a signed consent form for every active case?
- Are high-risk escalations (PAR-004) being triggered correctly?

EVIDENCE REQUIRED

- Random sample of 10 session notes
- Consent log audit

- Escalation logs

NON COMPLIANCE EXAMPLES

- Missing consent forms for active sessions
- Delayed documentation exceeding 72 hours
- Failure to escalate a 'Red' zone case

59.5 Audit Scoring

KEY FOCUS

COMPLIANT	2
PARTIAL	1
NON COMPLIANT	0

60 Risk Register

KEY FOCUS

1. Parental denial leading to student's condition worsening.

CATEGORY Clinical/Safety

PROBABILITY Medium

IMPACT High

RISK RATING High

MITIGATION Use Motivational Interviewing and escalate to PAR-004 if no progress in 3 sessions.

OWNER School Counselor

2. Breach of parental confidentiality.

CATEGORY Legal/Privacy

PROBABILITY Low

IMPACT Critical

RISK RATING Medium

MITIGATION Role-based access control and mandatory NDAs for staff.

OWNER Wellness Coordinator

3. Parental hostility during sessions.

CATEGORY Safety

PROBABILITY Low

IMPACT Medium

RISK RATING Medium

MITIGATION Conduct sessions in a secure environment with security awareness; terminate if threats occur.

OWNER School Counselor

61 Failure Modes KEY FOCUS

Failure	Cause	Impact	Mitigation
Parent refuses to attend any sessions.	High stigma or lack of trust in school system.	Student remains unsupported; risk of worsening condition.	Document refusal and offer alternative resources (e.g., community helplines).
Counselor lacks skill in handling resistant parents.	Insufficient training in Motivational Interviewing.	Program fails to achieve sensitization goals.	Mandatory annual training and peer supervision.
Action plan is too complex for parent to follow.	Lack of consideration for low-literacy or high-stress parents.	Parent feels overwhelmed and stops engagement.	Simplify action plans into 3 clear, manageable steps.

62 Illustrative Sample Scenarios

Scenario	Response	Expected Result
A parent minimizes their child's severe anxiety as 'just being dramatic'.	Counselor uses the 'Broken Leg' analogy to explain that internal distress is a health issue, not a choice.	Parent acknowledges the need for professional support.
A parent refuses to attend sessions due to fear of social stigma in their community.	Counselor offers private virtual sessions and emphasizes strict confidentiality.	Parent agrees to participate remotely.
A student reports that parents are punishing them for 'lazy' behavior (depression).	Counselor initiates PAR-007 with a focus on de-escalating punitive responses.	Parent shifts to supportive, non-punitive home environment.

63 Usage Governance KEY FOCUS

PRIMARY USERS

- School Counselor
- Wellness Coordinator

SECONDARY USERS

- Class Teacher
- Principal

APPROVAL AUTHORITIES

- Principal

EXECUTION AUTHORITIES

- School Counselor

ESCALATION AUTHORITIES

- Principal
- District Education Officer

REVIEW AUTHORITIES

- Wellness Committee

EXTERNAL STAKEHOLDERS

- Parents
- Medical Professionals

64 Applicability

APPLICABLE INSTITUTIONS

- High Schools

- Secondary Schools

APPLICABLE AGE GROUPS

- All Ages

APPLICABLE STUDENT CATEGORIES

- Students with mental health distress
- Neurodivergent students
- Vulnerable populations

APPLICABLE RISK LEVELS

- Yellow

- Orange

APPLICABLE LOCATIONS

- On-campus
- Remote/Virtual

EXCLUSIONS

- Clinical psychiatric treatment (referral only)
- Emergency crisis intervention (PAR-005)

65 Usage Scenarios

Scenario ID	Scenario Name	When To Use	When Not To Use	Trigger Event	Expected Outcome
US-01	Standard Sensitization	Parent is cooperative but lacks knowledge.	Parent is hostile or student is in Red zone.	Referral from teacher for 'Yellow' zone student.	Improved parent-student support.
US-02	Resistant Parent Engagement	Parent is in denial or minimizing symptoms.	Student is in immediate danger (Black zone).	Counselor identifies 'minimization' during Session 1.	Shift toward cooperation via Motivational Interviewing.

66 SOP Lifecycle

ENTRY CONDITIONS

- Student identified as needing support
- Parental consent obtained

ACTIVATION CONDITIONS

- Referral accepted by Counselor

EXECUTION CONDITIONS

- Counselor availability confirmed
- Secure environment ready

COMPLETION CONDITIONS

- Success criteria met
- Closure report signed

CLOSURE CONDITIONS

- Case archived in database

TERMINATION CONDITIONS

- Student moves to clinical care (external)
- Parent withdraws consent
- Escalation to PAR-004

67 Mandatory Evidence

KEY FOCUS

IMPLEMENTATION EVIDENCE

- Session Summary Notes
- Home Support Action Plans

OPERATIONAL EVIDENCE

- Communication Logs
- Student Impact Logs

COMPLIANCE EVIDENCE

- Signed Consent Forms

- Audit Reports

TRAINING EVIDENCE

- Counselor Certifications
- Staff Training Attendance

AUDIT EVIDENCE

- Management Review Reports
- Success Criteria Checklists

68 Controls

PREVENTIVE CONTROLS

- Mandatory informed consent
- Role-based access control (RBAC)
- Staff training on boundaries

DETECTIVE CONTROLS

- Weekly monitoring of student impact

- Quarterly audit of session notes
- Parental sentiment surveys

CORRECTIVE CONTROLS

- Escalation to PAR-004 for non-compliance
- Re-training for staff
- Action plan revision

COMPENSATING CONTROLS

- Remote sessions for rural parents

- Multilingual translation services

69 MWMS Risk Framework**KEY FOCUS****69.1 Green****MEANING**

Normal developmental concerns and routine life stress.

EXAMPLES

- Mild exam stress
- Friendship disagreements

RESPONSE TIME

Routine

ESCALATION REQUIRED

No

APPROVAL LEVEL

Counselor

MONITORING FREQUENCY

Monthly

CLOSURE AUTHORITY

Counselor

24-72 hours

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Principal

MONITORING FREQUENCY

Daily/Weekly

CLOSURE AUTHORITY

Wellness Coordinator

69.2 Yellow**MEANING**

Early distress requiring support and monitoring.

EXAMPLES

- Persistent stress
- Moderate anxiety

RESPONSE TIME

7 days

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Wellness Coordinator

MONITORING FREQUENCY

Weekly

CLOSURE AUTHORITY

Counselor

69.4 Red**KEY FOCUS****MEANING**

High-risk situation requiring immediate intervention.

EXAMPLES

- Suicidal ideation
- Self-harm behaviour

RESPONSE TIME

Immediate

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Principal/District Authority

MONITORING FREQUENCY

Continuous

CLOSURE AUTHORITY

Wellness Coordinator

69.5 Black**MEANING**

Life-threatening emergency requiring emergency response.

EXAMPLES

- Active suicide attempt
- Violent attack underway

RESPONSE TIME

Immediate (0-15 mins)

ESCALATION REQUIRED

Yes

APPROVAL LEVEL

Emergency Services/Principal

MONITORING FREQUENCY

Continuous

CLOSURE AUTHORITY**69.3 Orange****MEANING**

Significant distress causing measurable impairment but without immediate danger.

EXAMPLES

- Severe exam anxiety
- Panic attacks
- School refusal

RESPONSE TIME

70 Resource Scaling

70.1 0 500 Students

WELLNESS COUNSELORS	1
CAREER COUNSELORS	1
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

70.2 501 1000 Students

WELLNESS COUNSELORS	2
CAREER COUNSELORS	2
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

70.3 1001 2000 Students

WELLNESS COUNSELORS	4
CAREER COUNSELORS	2
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

70.4 2001 5000 Students

WELLNESS COUNSELORS	8
CAREER COUNSELORS	4
MENTAL WELLNESS COORDINATOR	1
WELLNESS COMMITTEE	Yes

71 Minimum Required Capabilities

PEOPLE

- Certified School Counselor
- Wellness Coordinator
- Trained Teachers

PROCESSES

- Referral System
- Parent Sensitization Program
- Risk Classification

- Escalation Protocol

TECHNOLOGY

- Encrypted Database
- Video Conferencing Platform
- Secure Email

INFRASTRUCTURE

- Private Counseling Room
- High-speed Internet
- Secure Filing Cabinet

72 Success Criteria

Criterion	Measurement Method	Target
Parental Knowledge Increase	Pre/Post Survey Score	> 30% improvement
Student Support Perception	Student Feedback Survey	> 4.0 out of 5 score
Action Plan Adherence	Counselor Observation	> 70% adherence rate

73 Exit Criteria

Condition	Evidence Required
Success criteria met and parent agrees to closure.	Signed Closure Report
Student referred to external clinical care.	External Referral Form & ROI

74 Continuous Improvement

LESSONS LEARNED

- Motivational interviewing is more effective than direct education for resistant parents.

- Virtual sessions increase participation in rural areas.

KNOWN ISSUES

- Parental time constraints can delay session completion.
- Stigma remains a significant barrier in certain cultural demographics.

FUTURE IMPROVEMENTS

- Develop multi-lingual video modules for parent education.

- Integrate automated SMS reminders for action plan steps.

REVIEW RECOMMENDATIONS

- Increase frequency of teacher training on identifying minimization.
- Expand rural outreach resources.

75 AI Governance

KEY FOCUS

ALLOWED USE CASES

- Summarizing anonymized session notes
- Generating parent education scripts
- Trend analysis of wellness data

RESTRICTED USE CASES

- Diagnosing student mental health conditions
- Providing clinical advice to parents
- Predicting individual risk scores without human review

PROHIBITED USE CASES

- Storing identifiable student data in public AI models
- Automated decision-making for escalations

HUMAN REVIEW REQUIRED

- All AI-generated summaries
- All risk trend reports
- All parent communication scripts

AUDIT LOG REQUIREMENTS

- Log of all AI prompts used by staff
- Verification of human review for every output

DATA PRIVACY REQUIREMENTS

- No PII (Personally Identifiable Information) in prompts
- Use of enterprise-grade, private AI instances

76 Legal Defensibility

KEY FOCUS

LEGAL BASIS

- Supreme Court Judgment 2025 INSC 893
- CBSE Affiliation Bye Laws
- Indian Child Protection Act

MANDATORY NOTIFICATIONS

- Mandatory reporting of abuse
- Notification of imminent risk to self/others

CONSENT REQUIREMENTS

- Informed consent for all interventions
- Release of information for external referrals

RECORD RETENTION REQUIREMENTS

- 5 years retention for educational records
- Secure destruction protocols

LIABILITY CONTROLS

- Clear role boundaries (non-clinical)
- Standardized documentation of all actions
- Regular staff training on legal mandates

77 Stakeholder Journeys

77.1 The Student Journey

ENTRY

- Teacher identifies distress
- Student expresses need for help

ASSESSMENT

- Counselor screening
- Risk classification

INTERVENTION

- Parent sensitization initiated

- Improved home support environment

MONITORING

- Weekly counselor check-ins

- Observation of mood/attendance

CLOSURE

- Student reports feeling supported
- Case closed

- Initial meeting to discuss student needs
- Signing of consent forms

CONSENT

- Agreement to participate in 1-3 sessions
- Commitment to action plan

ENGAGEMENT

- Session 1: Education on condition
- Session 2: Addressing fears/stigma
- Session 3: Skill building & role-play

CLOSURE

- Final wrap-up meeting
- Receipt of resources

77.2 The Parent Journey

NOTIFICATION

- Counselor contact via phone/email

- Explanation of program goals

CONSULTATION

77.3 The Staff Journey

OBSERVATION

- Teacher notices student behavior changes

- Parent reports concerns to teacher

REPORTING

- Teacher submits observation form
- Counselor conducts parent sessions
- Counselor reviews referral
- Teacher supports action plan in class

INTERVENTION SUPPORT DOCUMENTATION

- Teacher logs observations
- Counselor maintains session notes
- Management reviews reports

78 Handoff Matrix

Item 1

FROM ROLE Class Teacher

TO ROLE School Counselor

HANDOFF TRIGGER Student distress identified and referral submitted.

INFORMATION TRANSFERRED

- Teacher observation notes
- Initial student behavior summary
- Parental concerns reported by teacher

REQUIRED DOCUMENTS

- Student Observation Form
- Referral Note

SERVICE LEVEL AGREEMENT (SLA)

48 hours

Item 2

FROM ROLE School Counselor

TO ROLE Principal

HANDOFF TRIGGER Parent resistance or high-risk escalation.

INFORMATION TRANSFERRED

- Summary of parental non-cooperation
- Risk assessment details
- Proposed action plan

REQUIRED DOCUMENTS

- Resistance Management Log
- Risk Classification Record

SERVICE LEVEL AGREEMENT (SLA)

Immediate (for Red/Black)

79 Management Review

REVIEW FREQUENCY

Quarterly

REVIEW INPUTS

- Closure Reports
- Audit Logs
- Parent Satisfaction Surveys
- Student Well-being Trends

REVIEW OUTPUTS

- Management Review Report
- Continuous Improvement Plan
- Resource Allocation Decisions

REVIEW AUTHORITY

School Principal / Wellness Committee

REVIEW DECISIONS

- Approval of new resources
- Policy updates
- Staff training mandates

80 Dashboard Metrics

OPERATIONAL METRICS

- Number of parents sensitized per month
- Average number of sessions completed
- Counselor caseload volume

RISK METRICS

- Percentage of cases escalated to PAR-004
- Frequency of 'Red' zone identifications
- Incident rate of parental hostility

COMPLIANCE METRICS

- Consent form completion rate
- Audit score percentage
- Documentation timeliness

OUTCOME METRICS

- Pre/Post Parent Sentiment Score
- Student attendance improvement %
- Parental skill mastery rate

TREND METRICS

- Quarterly trend of student distress types
- Seasonal peaks in parent requests

81 Student Rights

RIGHTS

- Right to confidential mental health support
- Right to a supportive home-school environment
- Right to be informed about the sensitization process

PROTECTIONS

- Data privacy protections for minors

- Protection from punitive responses at home
- Safe reporting channels

APPEAL MECHANISMS

- Student can request a counselor review of parent engagement
- Appeal to Wellness Committee regarding support adequacy

82 Quality Assurance

QUALITY CHECKS

- Random audit of 10% of session notes
- Counselor peer-review of high-risk cases
- Parent satisfaction survey analysis

PEER REVIEW REQUIREMENTS

- Monthly review of complex cases by senior counselor

- Cross-school best practice sharing

VALIDATION REQUIREMENTS

- Counselor certification verification
- Action plan feasibility check

CONTINUOUS MONITORING REQUIREMENTS

- Quarterly dashboard review
- Annual program effectiveness audit

83 Implementation Roadmap

PHASE 1 PREPARATION

- Staff training on Motivational Interviewing
- Development of parent education materials
- Setup of secure database and forms

PHASE 2 DEPLOYMENT

- Launch awareness campaign for parents
- Initiate first wave of referrals
- Conduct initial sensitization sessions

PHASE 3 STABILIZATION

- Monitor student impact logs

- Refine action plans based on feedback
- Establish regular management review cycle

PHASE 4 OPTIMIZATION

- Develop multi-lingual modules
- Integrate automated reminders
- Analyze trend data for resource allocation

PHASE 5 CONTINUOUS IMPROVEMENT

- Annual program audit
- Update SOP based on lessons learned
- Publish anonymized success stories

84 Illustrative Case Studies

1. Overcoming Parental Denial in Depression

DESCRIPTION

A parent minimized a student's severe depressive symptoms as 'laziness'.

ACTIONS TAKEN

- Conducted 3 sessions using the 'Broken Leg' analogy.
- Used Motivational Interviewing to address fear of stigma.
- Created a specific home action plan for non-punitive responses.

OUTCOME

Parent successfully identified symptoms as illness; student reported feeling more supported at home within 4 weeks.

LESSONS

- Analogies are highly effective in bridging the gap between clinical terms and parental understanding.

85 Implementation Requirements

STAFF REQUIRED

- 1 School Counselor (Full-time)
- Wellness Coordinator (Part-time/Shared)
- Trained Teachers for identification

INFRASTRUCTURE REQUIRED

- Private counseling room
- Secure internet connection
- Storage for physical records

TECHNOLOGY REQUIRED

- Encrypted database (MWMS)
- Video conferencing software

- Email communication platform

BUDGET CONSIDERATIONS

- Counselor salary/benefits
- Training costs for staff
- Development of educational materials
- Software licensing fees

IMPLEMENTATION PHASES

- Preparation (Month 1)
- Deployment (Months 2-4)
- Stabilization (Months 5-8)
- Optimization (Ongoing)

86 Quality Controls

PREVENTIVE CONTROLS

- Mandatory staff training
- Standardized session templates
- Informed consent requirements

DETECTIVE CONTROLS

- Quarterly audit logs
- Parental sentiment surveys
- Student impact monitoring

CORRECTIVE CONTROLS

- Escalation to PAR-004 for non-compliance
- Counselor peer review
- Action plan revision

CONTINUOUS IMPROVEMENT ACTIONS

- Annual program audit
- Quarterly trend analysis
- Feedback loop from parents and teachers

87 Training Requirements

TEACHERS

- Identifying student distress signs
- Basic understanding of mental health conditions
- Referral procedures

COUNSELORS

- Motivational Interviewing certification
- Parental engagement strategies
- Crisis intervention training

ADMINISTRATORS

- Risk classification overview
- Escalation protocols
- Legal and ethical mandates

PARENTS

- Understanding neurodiversity
- Positive parenting skills
- Mental health literacy

STUDENTS

- Self-advocacy for mental health support
- Emotional regulation basics

87.6 Repository Baseline

TRAINING COMPLETION RATE

100% for counselors

STAFF COMPETENCY SCORE

> 85%

87.7 SOP Specific Training

TARGET AUDIENCE

- School Counselors
- Wellness Coordinators

COMPETENCIES

- Motivational Interviewing
- Parental Sensitization Techniques

- Risk Assessment

MINIMUM HOURS

6

CERTIFICATION REQUIREMENTS

- Internal completion certificate

REFRESHER FREQUENCY

Annual

87.8 Competency Validation

REQUIRED

Yes

ASSESSMENT METHODS

- Role-play simulation

- Case study review

MINIMUM PASSING CRITERIA

80% score on role-play assessment

REASSESSMENT FREQUENCY

Every 2 years

87.9 Simulation Requirements

REQUIRED

No

SIMULATION TYPES

- Hostile parent scenario

- Student self-harm disclosure

FREQUENCY

Annual for counselors

88 SOP-Specific Inclusion Requirements

AFFECTED POPULATIONS

- Parents of students with ADHD/Autism
- First-generation learners from rural areas
- Low-literacy parents
- Students from minority communities
- Families in high-stress economic situations

IDENTIFIED INCLUSION RISKS

- Language barriers for non-native speakers
- Lack of transportation to school for rural parents
- Cultural stigma against mental health treatment
- Limited digital literacy for online sessions
- Economic stress preventing participation in workshops

AUTISM ACCOMMODATIONS

- Visual aids and simplified language for parents of autistic students
- Focus on sensory-friendly home environments
- Clear, structured action plans with visual checklists

ADHD ACCOMMODATIONS

- Shortened session lengths (20 mins instead of 45)
- Frequent breaks during role-play
- Written summaries sent immediately after sessions

DYSLEXIA ACCOMMODATIONS

- Audio recordings of educational content
- Simplified, large-print action plans
- Oral assessments instead of written pre/post tests

HEARING IMPAIRMENT ACCOMMODATIONS

- Sign language interpretation for sessions
- Closed captioning for virtual meetings
- Written transcripts of all session content

VISUAL IMPAIRMENT ACCOMMODATIONS

- Screen reader compatible digital materials
- Audio-only versions of educational modules
- Braille action plans where requested

CWSN ADAPTATIONS

- Individualized communication styles based on student's specific needs
- Parental training on neurodivergent behavioral cues
- Home environment modifications for sensory processing

RURAL ADAPTATIONS

- Tele-health options for remote parents
- Community center meeting locations
- SMS-based follow-up and reminders

LOW LITERACY PARENT ADAPTATIONS

- Use of visual analogies and pictures
- Oral summaries and verbal instructions
- Simplified, one-page action plans with icons

MULTILINGUAL COMMUNICATION ADAPTATIONS

- Translation services for all sessions
- Multilingual educational brochures
- Native language counselors where available

GENDER SENSITIVITY MEASURES

- Gender-neutral language in all materials

- Safe spaces for parents of different genders to discuss concerns
- Sensitivity training on gender roles in parenting

CULTURAL SENSITIVITY MEASURES

- Respecting diverse cultural views on mental health
- Involving community leaders where appropriate
- Culturally relevant analogies and examples

EQUITY MONITORING INDICATORS

- Participation rate by socio-economic status
- Success rates for rural vs. urban parents
- Language accessibility satisfaction scores

ACCESSIBILITY COMPLIANCE CHECKS

- WCAG 2.1 compliance for digital materials
- Physical room accessibility check
- Translation availability verification

89 Awareness Material

POSTER TITLE	Supporting Your Child's Mental Wellness
KEY MESSAGE	Mental health is just as important as physical health. Let's work together.
SLOGAN	Stronger Families, Healthier Students.
CAMPAIGN THEME	The Bridge to Support

90 AI Assistant Configuration

RISK KEYWORDS

- Self-harm
- Suicide
- Violence
- Abuse
- Neglect

ESCALATION KEYWORDS

- Immediate danger
- Police
- Emergency
- Hospital

- Weapon

INTERVENTION KEYWORDS

- Anxiety
- Depression
- Stress
- Behavioral issues
- Sensitization

FREQUENTLY ASKED QUESTIONS

- Is this program confidential?
- How many sessions will my child need?
- What if I don't agree with the counselor?
- Will this affect my child's grades?

91 AI Audit Hooks

KEY FOCUS

REQUIRED SECTIONS

- Risk Classification
- Consent Forms
- Session Summaries
- Action Plans

REQUIRED FORMS

- PAR-007-F1
- PAR-007-F2
- PAR-007-F3
- PAR-007-F5

REQUIRED CHECKLISTS

- CHK-001
- CHK-002
- CHK-004

REQUIRED ESCALATIONS

- All Red/Black zone triggers

REQUIRED KPIS

- Parent Sentiment Score
- Completion Rate

REQUIRED TRAINING

- Counselor Certification Log

92 KPI Framework

92.1 Repository Baseline

COMPLETION RATE	0%
ATTENDANCE IMPROVEMENT	0%

92.2 SOP Specific Metrics

Metric	Target	Frequency
Parental Knowledge Gain	> 30% increase from pre-test to post-test	Per Case
Action Plan Adherence	> 75% of steps followed in first month	Monthly
Student Distress Reduction	> 20% reduction in reported anxiety symptoms	Quarterly

93 Maturity Model

93.1 Level 1

DESCRIPTION

Ad-hoc parent communication with no formal sensitization.

EVIDENCE

- Informal emails
- Unstructured meetings

- Session notes
- Action plans
- Consent forms

93.4 Level 4

DESCRIPTION

Data-driven program with monitoring and trend analysis.

EVIDENCE

- Dashboards
- Sentiment scores
- Impact logs

93.2 Level 2

DESCRIPTION

Standardized information sharing but no structured skill building.

EVIDENCE

- Brochures
- General workshops

93.5 Level 5

DESCRIPTION

Integrated, proactive wellness ecosystem with community partnerships.

EVIDENCE

- Community reports
- External agency MOUs
- Systemic policy changes

93.3 Level 3

DESCRIPTION

Structured Parent Sensitization Program (PAR-007) implemented.

EVIDENCE

94 Dependencies

PREREQUISITE SOPS

- MWMS-001: Student Mental Health Screening
- PAR-002: Parent Consent Framework

PARALLEL SOPS

- PAR-008: Student Self-Harm Protocol (if applicable)

DOWNSTREAM SOPS

- PAR-004: Escalation of Unsupportive Parents
- MWMS-010: External Clinical Referral

95 Related SOPs

PREREQUISITES

- MWMS-001
- PAR-002

UPSTREAM

- Teacher Observation SOP
- Student Screening SOP

DOWNSTREAM

- PAR-004
- MWMS-010

REFERENCES

- CBSE Circular TRG-03/2026
- NEP 2020 Guidelines



Towards Health, Learning & Hope

“Every child deserves to learn, grow, and thrive in a safe, caring, and supportive environment.”

This publication supports the spirit of **UN Sustainable Development Goal (SDG) 3 – Good Health & Well-being** and **UN Sustainable Development Goal (SDG) 4 – Quality Education**. It encourages healthy emotional development, respectful relationships, empathy, resilience, inclusion, and lifelong learning.

The stories and activities in this book are aligned with the principles of **UNICEF Social and Emotional Learning (SEL)**, helping children recognize emotions, build positive friendships, communicate with kindness, solve problems peacefully, and seek help whenever needed.

This book also promotes educational awareness consistent with the directions reflected in the **Supreme Court of India Judgment (2025 INSC 893)** relating to student mental wellness safeguards, accessible grievance redressal mechanisms, institutional monitoring, implementation oversight, and preventive mental wellness measures. Readers, educators, parents, and institutions are encouraged to create environments where every learner feels heard, respected, protected, and supported.

Emotional well-being is a shared responsibility. If a child experiences stress, anxiety, bullying, or emotional difficulty, reaching out to trusted parents, teachers, school counsellors, or responsible adults is an important step toward care and recovery.

Together, we can build healthier minds, happier classrooms, and a brighter future for every child.



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CLARITY

Defines clear processes, roles and responsibilities to eliminate ambiguity.



ACCOUNTABILITY

Ensures ownership, transparency and responsibility at every level.



CONSISTENCY

Promotes uniform execution and reliable outcomes across all functions.



COMPLIANCE

Aligns with legal, regulatory and organizational requirements.



QUALITY

Implements quality controls to ensure continuous improvement.



IMPACT

Supports better decisions, safer practices and meaningful results.



WHY THIS SOP MATTERS

- ✓ Establishes a repeatable and scalable approach to operations.
- ✓ Reduces risk, errors and variation in processes.
- ✓ Enhances efficiency, productivity and resource utilization.
- ✓ Strengthens documentation, traceability and record keeping.
- ✓ Builds trust through standardization and measurable performance.
- ✓ Empowers teams to deliver consistent, high-quality outcomes.



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Identify needs and objectives



DESIGN

Develop clear and effective processes.



IMPLEMENT

Execute with training and resources



MONITOR

Track performance and ensure compliance



IMPROVE

Evaluate, learn and enhance continuously



STANDARDIZE PROCESSES.
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